911 Nurse Navigation

Job Training & Reference Materials

Last Updated: November 1, 2026

Policies & Guidelines

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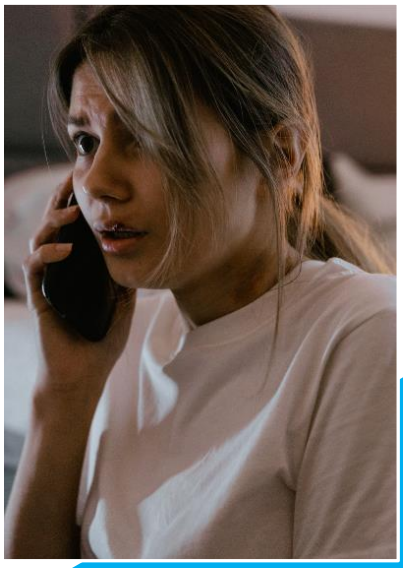

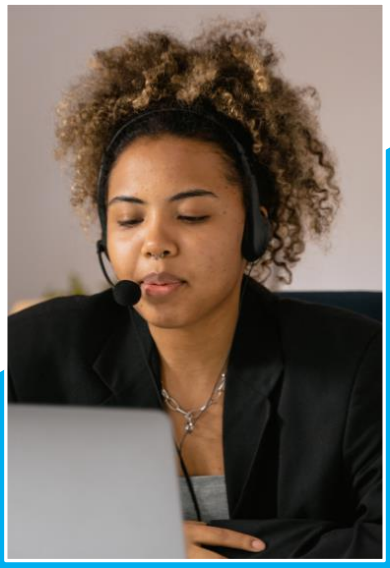

Global Medical Response

To request updates to this document, please send an email to: nntraining@gmr.net

Policies & Guidelines

In This Section

The ICC People Policies contained in this section serve as a guide to our culture, operational practices, and general workplace expectations. You will also find 911 Nurse Navigation-specific policies and procedures outlined here. For a complete list of all GMR policies, including the Code of Conduct, please visit the GMR People Services GPS Portal.

Reference Topics

- New Hire Attestation Form
- Company Recognized Holidays
- Attendance Policy
- Breaks & Shift Relief
- Paid Time Off
- Shift Swaps & Giveaways
- Job Preparedness & Shift Restrictions
- Distractions
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New Hire Attestation Form

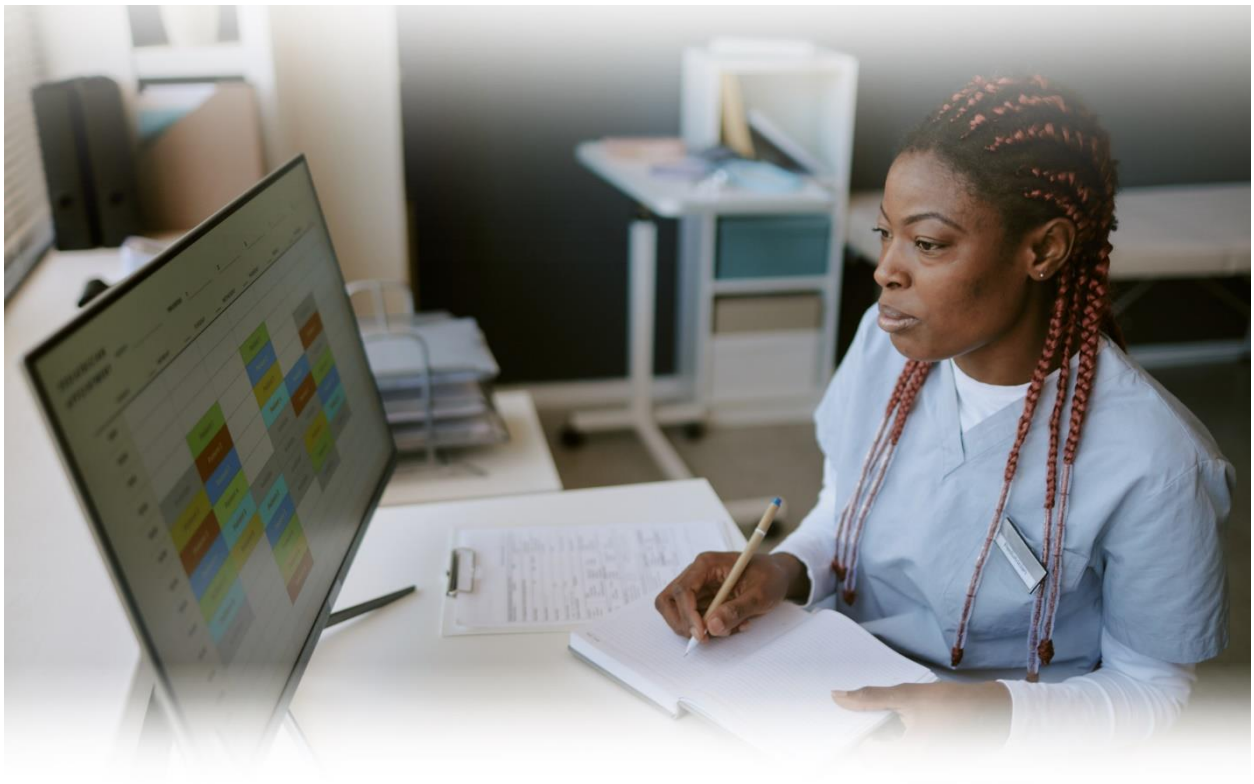
As part of your onboarding journey at Global Medical Resource (GMR), all hired employees MUST attest to their review and understanding of the policies in this section. Complete your attestation by submitting the following form:

100-ICC People Policies Attestation_NN

Compliance Consideration

In addition to the policies herein, you are required to complete mandatory compliance training on a yearly basis, for more information, please reach out to your trainer or supervisor.

View your pending learning assignments and transcripts on CornerStone.



Company Recognized Holidays

New Years Day

Memorial Day

Independence Day
(July 3rd if on a Friday)

Labor Day

Thanksgiving

Day After Thanksgiving

Christmas Eve

Christmas Day

Attendance Policy

Policy Number: 100-01	**Subject:** Attendance
Revision Date: January 8th, 2026	**Effective Date:** January 8th, 2026

I. PURPOSE

To establish expectations that employees report to work for all scheduled hours, including regular shifts, assigned overtime, shift swaps, giveaways, as well as mandatory meetings and classes.

II. POLICY

Expectations

- Employees are expected to arrive early enough to be performing assigned duties at the start of their shift and remain until its scheduled end time without extending allotted break periods.
- Employees may clock in no more than 10 minutes before their shift begins.
- Employees are required to be physically present in the ICC when clocking in unless they are attending a course, meeting or other business-related obligation located offsite.

III. PROCEDURE

If an employee is unable to meet the above expectations, the subsequent guidelines should be followed.

Notification Procedure

1. Employees who will be absent or anticipate being late are expected to call the Communications Center and speak with the on-duty leader on a recorded line.
    - Employees are expected to call as soon as they know they will not be reporting for duty at the start of their shift.
2. In the event an employee needs an extended break, to leave early or requires any other shift modification, they must speak with the on-duty leader in person.

- If an on-duty leader is not present in the ICC, the employee must attempt to contact the on-duty leader and receive acknowledgement of their schedule modification. Should the employee be unable to reach their on-duty leader, the matter should be escalated to another departmental ICC on-duty leader.

Attendance Record Documentation

- Attendance records are kept using a rolling twelve (12) month period, calculating attendance occurrences over the previous 365 days. Occurrences are archived and deducted from the employee's record 365 days after they occur.
- The highest level of corrective action taken during the merit period (January 1st – December 31st) will be included in an employee's annual review, regardless of the total number of occurrences on record at the conclusion of the calendar year.

Appeal Process

1. Employees who wish to contest an occurrence must do so via email to the Communications Manager within thirty (30) calendar days of the date of the occurrence in question.
2. Employees will receive a response to their appeal via email within ten (10) business days. Appeals that are not submitted as specified above, or those denied by the Manager, will become a permanent part of the employee's attendance record.

PTO Application

- PTO will be applied to all absences from an employee's regularly scheduled shift, including those documented as FMLA.
- PTO will be applied to any partial shift absence (late arrival or early departure) in one-hour increments, except in the case of Voluntary Time Off (VTO).
- PTO will not be applied to absences on overtime shifts (scheduled in addition to an employee's regular schedule).

Types of Absences

- Approved Absences - Absences arranged and authorized in advance. They may include:
    - Company observed holidays (for specified employees).
    - Pre-approved PTO, shift swaps, shift giveaways and scheduled shift modifications documented in the Company-approved scheduling system.
    - Family Medical Leave (if stated at the time of notification with an approved FMLA case).
        - Additionally, employees are required to notify GMR's third-party leave management company, in accordance with their established guidelines, of any shift, full or partial, that is missed for which FMLA has been claimed.
        - Time that is not submitted to or has been denied by GMR's third-party leave management company will result in the occurrence being issued according to this policy.
    - Approved Disability Leave.
    - Absences related to an on-the-job injury, wherein the injury has been reported to leadership, and a claim has been filed with and approved by GMR's third-party risk management company.
    - Bereavement per GMR policy with proper documentation.
    - Military Leave with proper documentation.
    - Jury Duty with proper documentation.
    - Work-related court appearances with proper documentation.
    - Any additional absence permitted under applicable law.
- Unapproved Absences - Absences not classified as "approved absences" in this policy.
- Multiple Day Absences - A subsequent unapproved absence on a consecutive calendar day will be recorded as one-half of an occurrence.
    - Unapproved absences spanning three (3) or more consecutive calendar days will result in one (1) occurrence being recorded for each shift missed after the initial two (2) absences.

Absence Exceptions

- Return to work with a provider note: Should an employee return to work with an official signed document bearing the signature of a licensed medical provider, absences of greater than one (1) shift may be considered one (1) occurrence. Documentation must be provided to leadership upon the employee's return to work.
- Return to work following hospitalization: Absences during a hospital admission will be recorded as zero occurrences if valid paperwork inclusive of date of admission, discharge and a return-to-work date are provided to leadership upon the employee's return to work.

Documentation

- Be advised that any documentation provided to leadership for the purpose of this policy is subject to verification, and any instance of falsification of documents may result in corrective action, up to and including termination.

Definitions

- Tardy: Reporting for duty one to ten (1-10) minutes after the scheduled start time of an employee's shift. Employees will be issued one-quarter of an occurrence.
- Late: Reporting for duty more than 10 minutes after the scheduled start time but not exceeding 25% of the employee's shift duration. Employees will be issued one-half of an occurrence.
- Early Departure: Leaving assigned duty prior to the scheduled end time of an employee's shift.
    - Employees who work less than 75% of their scheduled shift and leave early will be issued one occurrence.
    - Employees who worked more than 75% of their scheduled shift and leave early will be issued one-half of an occurrence.
    - Notification of early departure is to be made directly to the on-duty leader, and the employee must confirm receipt of the notification. If proper notification procedures are not followed, this may be considered job abandonment and may result in corrective action, up to and including termination.
- Voluntary Time Off (VTO): Unpaid time off during periods of low demand, as determined by an on-duty leader.

- No Call / No Show: Not reporting to work within two hours of a scheduled shift without proper notification to an on-duty leader, provided extraordinary circumstances do not apply.

Multiple Occurrences (same shift)

Should an employee incur a tardy or late arrival in combination with an early departure during the same scheduled shift, each instance will be documented individually; however, no more than one occurrence will be issued.

Mandatory Meetings and Classes

Required meetings in addition to mandatory training or classes are considered part of an employee's work schedule. Failure to attend these events will be recorded as an occurrence.

Occurrences of Significant Impact

Certain types of absences may have significant adverse effects on staffing plans. Such instances are particularly detrimental to operational efficiency and will result in an expedited corrective action process. The initial instance of any of the following occurrences will result in a final warning. A subsequent instance (any combination) of the occurrences listed below within a rolling 365-day period will be grounds for termination.

- Calling out on a company-observed holiday.
- Calling out for a scheduled shift for which PTO had been requested and denied.
- Calling out unable to report to work due to inclement weather after accommodations have been offered by the Company.
- No Call / No Show

Progressive Discipline

Excessive occurrence accumulation will lead to progressive discipline. "Excessive" is defined by the following criteria:

- 3 – Attendance Notification
- 4 – Verbal Warning
- 5 – Written Warning
- 7 – Final Warning
- 9 – Termination

Redemptions

Employees may earn the opportunity to have occurrences removed from their attendance record as follows:

- An employee who accepts a last-minute shift coverage request from a departmental leader may have up to one full occurrence removed from their attendance record to a maximum of one per calendar quarter.
    - Coverage of one full shift removes one occurrence.
    - Coverage of a half shift removes one-half of an occurrence.
- Shift redemptions will be forfeited if:
    - The employee has previously been offered and declined an on-call or overtime opportunity for the date in question. (Failure to respond to an overtime offer will be considered declination.)
    - The employees does not work all of their scheduled shifts within that pay period.

Employees may not have a negative attendance occurrence balance.

Breaks & Shift Relief

Policy Number: 100-02	**Subject:** Breaks and Shift Relief
Revision Date: January 8th, 2026	**Effective Date:** January 8th, 2026

I. PURPOSE

To establish standards for breaks as well as proper shift relief for all GMR ICC employees.

II. POLICY

Breaks

This basic outline establishes typical break periods for employees based on their total shift time. Due to variable demands in our workplace, these breaks are provided on paid company time to allow for necessary flexibility in managing fluctuating workloads.

Employees may be allotted breaks as outlined in the following chart. The specific duration and scheduling of breaks will be determined by operational leadership.

Total Shift Hours	Total Break Time
6 Hours	20 Minutes
8 Hours	40 Minutes
10+ Hours	60 Minutes

While we make every effort to provide breaks as outlined in this policy, they are not guaranteed and are contingent upon staffing, workload and operational needs of the department. Additional break expectations are as follows:

- Employees are expected to remain logged into their workstations and all software, as well as to utilize the designated telephone status codes appropriately during break periods.
- Employees must remain on-site and be reachable by telephone during break periods, able to return to the ICC immediately if needed.
- In the rare case that there is a need to leave the premises during a break, direct acknowledgment and approval from the on-duty leader must be obtained. Employees should clock out before leaving and clock in upon return.

- Employees are required to obtain direct acknowledgment and approval from the on-duty leader before leaving the room, except during regularly scheduled breaks.
- Trainees and trainers are required to take their breaks together, unless otherwise instructed to stagger them by the on-duty leader.

Shift Relief

To ensure continuous operational oversight, employees should remain logged in and available at their workstations until properly relieved by the on-coming employee taking over their assignment.

- Employees must receive the approval of the on-duty leader to end their shift if their scheduled relief is not present at the conclusion of their shift.
- Air employees without proper relief may be held over for up to six (6) hours.

Paid Time Off (PTO)

Policy Number: 100-03	**Subject:** Paid Time Off (PTO)
Revision Date: January 8th, 2026	**Effective Date:** January 8th, 2026

I. PURPOSE

This policy outlines procedures for requesting and approving Paid Time Off (PTO). It does not govern absences protected by federal, state, or local law, including but not limited to FMLA, military leave, jury duty, disability-related leave, or pregnancy-related leave.

II. POLICY

PTO Requests

- Should be submitted via the company-approved scheduling system and departmental process for approval.
- Will be reviewed on a first requested-first considered basis.
- Will be limited by departmental guidelines for staffing.
- May be submitted for consideration up to 90 days prior to the intended date(s).
- Must be submitted at least 30 days prior to the intended date(s).
    - Requests made less than 30 days in advance will be denied unless the requestor arranges shift coverage as outlined in the Shift Swaps and Giveaways Policy.
- Will be reviewed by a member of leadership and a response will be provided within ten (10) business days of submission when submitted per policy.
    - Responses will be designated as approved, denied or pending.
- Will be denied if the requestor will not have sufficient PTO hours to cover the intended date(s).
- For which the requestor's PTO balance is insufficient at the time of submission will remain in a pending status until the proper number of hours have accrued to cover the intended date(s).
- That have been previously approved are subject to the approval being withdrawn if the requestor's PTO balance becomes insufficient for the intended date(s). The employee will then remain responsible for their originally scheduled shift(s).
    - Employees may not take unpaid time off.

- For Company-observed holidays will be reviewed with consideration for past holiday usage.

Leadership will review all employee requests for PTO and make every effort to accommodate eligible requests. However, requests may be denied due to operational requirements, staffing levels, or other business considerations.

It is the responsibility of the employee to confirm approval for all requested PTO date(s). Should an employee be absent without approval, corrective action per attendance standards may apply.

Shift Swaps & Giveaways

Policy Number: 100-04	**Subject:** Shift Swaps & Giveaways
Revision Date: January 8th, 2026	**Effective Date:** January 8th, 2026

I. PURPOSE

This policy outlines procedures for employees to swap or give away shifts. It does not govern absences protected by federal, state, or local law, including but not limited to FMLA, military leave, jury duty, disability-related leave, or pregnancy-related leave.

II. POLICY

Shift Swaps

Employees are eligible to swap shifts if:

- They have been fully cleared from training.
- Both parties have equivalent title and capabilities.
- The shifts or partial shifts involved are equal in duration.
- Both shifts included in the swap must occur within the same pay period.
- Both employees acknowledge the shift swap via the department-approved process.
- The request is approved by leadership before the first shift involved begins.

Shift Giveaways

If an employee's PTO request is denied, the employee may arrange shift coverage and process such as a shift giveaway if:

- The accepting employee has an equivalent title and capabilities.
- When giving away an overtime assignment, the receiving employee must meet the shift's minimum requirements, not the title and capabilities of the employee giving the shift away.
- They have sufficient PTO available to cover the shift they intend to give away if it is a regularly scheduled shift.
    - If the shift giveaway applies to an overtime assignment that the requesting employee initially accepted, the use of PTO is not required.
- The request is approved by leadership before the shift begins.

Once approved, the covering employee assumes responsibility for the shift under attendance standards.

Job Preparedness & Shift Restrictions

Policy Number: 100-05	**Subject:** Job Preparedness and Shift Restrictions
Revision Date: November 19th, 2025	**Effective Date:** January 1st, 2026

I. PURPOSE

To establish standards for employees to work efficiently, provide excellent service and ensure high-quality performance.

II. POLICY

Employees are required to arrive well-rested and capable of fulfilling the needs of the public and our patients. Maintaining alertness fosters a safe workplace and upholds high standards of service and patient care. Fatigue can negatively impact individuals and may endanger both patients and colleagues. The use of drugs or alcohol, as well as sleeping during work hours, is strictly prohibited. Employees who are found to be in potential violation of these standards may be sent home without pay and could be subject to corrective action up to and including termination.

In some situations, an employee may be required to remain at work beyond their scheduled shift. The maximum permitted shift length is eighteen (18) hours.

Distractions

Policy Number: 100-06	**Subject:** Distractions
Revision Date: N/A	**Effective Date:** January 1st, 2026

I. PURPOSE

To establish standards to mitigate distractions in the workplace.

II. POLICY

Personal Electronic Devices

Personal electronic devices are allowed on the communications center floor provided:

- They do not inhibit the ability to perform work duties.
- They remain in silent mode.

The following items/activities are strictly prohibited:

- Smart glasses.
- Earbuds or any listening device that isn't a company-issued headset.
- Media Capture, Recording, & Streaming devices.
- Making or receiving phone calls or voice messages with a personal electronic device.
- The use of cell phones in any capacity by trainees or trainers while they are training.

If an employee chooses to take their break at their workstation and must utilize their cell phone to make or receive a call they are required to do so outside of the ICC.

Use of Company Equipment

Televisions in the ICC are the property of GMR. Employees should not request special viewing of programming that does not involve local, state or national news events and/or weather updates.

Use of Company equipment, including but not limited to computers, phones, email and messaging, for personal purposes is prohibited. There is no reasonable expectation of privacy for the use of Company equipment. The Company may access and monitor all employee electronic usage, such as computer applications, internet, voicemail, email and messages.

The Company will not exercise its rights as described for the purpose of investigating whether employees have engaged in activity protected by the National Labor Relations Act.

Streaming

Employees are prohibited from streaming or viewing non-work-related videos or music on Company or personal electronic devices within the ICC.

Socialization

Casual conversation is encouraged but must not occur across the room or distract from assigned duties. Employees should be mindful of noise levels and always avoid unprofessional conversation or language. Employees should refrain from visiting others' workstations or departments, even during breaks.

Gossip and rumors has a way of spreading negativity and creating unnecessary tension. To maintain a harmonious and productive environment, it's essential to focus on clear, direct communication and to address any concerns or questions openly. By fostering a culture of respect and transparency, we can ensure that everyone feels valued and understood, paving the way for a more collaborative and supportive workplace.

Appearance

Policy Number: 100-07	**Subject:** Appearance
Revision Date: January 8th, 2026	**Effective Date:** January 8th, 2026

I. PURPOSE

To establish guidelines for dress code and overall appearance expectations for employees.

II. POLICY

A casual professional dress code is permitted, although leadership may require more professional attire when certain business needs arise. Employees are expected to maintain a well-groomed and professional appearance while at work or attending business-related meetings, courses, conferences and other events.

Company-Issued Uniform Components

- Photo ID Badge
- This access card will be issued to all employees and must be displayed at all times.
- See policy 100-08 for additional details on this item.
- Shirts
- GMR polos will be issued to each employee at no cost.
- Outerwear
- One (1) GMR jacket will be issued to each employee at no cost.

Employee Responsibility Uniform Components

- Headwear
    - Head scarves or other hair accessories should be solid white, tan/khaki, gray, dark blue or black.
    - Hats must be baseball style/brimmed and bear a GMR-affiliated logo or be solid white, tan/khaki, gray, dark blue or black and free of other logos. Hats must be worn with the brim facing forward.
    - Sunglasses may not be worn.

- Undershirts
  - May be short or long sleeved
  - If worn, any visible part of the shirt should be solid white, tan / khaki, gray, dark blue or black.
- Outerwear
  - Employees may also choose to wear GMR branded jackets purchased from the GMR Brand Store provided that they are gray, dark blue, or black.
- Pants / Skirts
  - Employees can wear pants/skirts of their own choosing within the guidelines below.
  - Pants / Skirts should be solid white, tan/khaki, gray, dark blue or black.
  - While sitting, the skirt hem should remain at knee length.
  - Denim is allowed, but jeans must be in good condition, regardless of current trends.
  - No holes, faded, frayed or distressed denim.
  - No ill-fitting or "sagging" jeans.
  - No overly embellished designs.
  - No shorts, athletic / leisure wear, leggings or overalls.
- Footwear
  - Must have a non-slip sole and be clean in appearance and in good condition.
  - House slippers are not permitted.

General Appearance

The ICC is a business environment with frequent visitors. All attire must be neat, clean, free of wrinkles, and in presentable condition.

On weekdays (Monday – Friday, non-company holidays) employees must adhere to the previously outlined dress code.

On weekends (Saturday and Sunday) and company holidays, a GMR branded t-shirt or hoodie may be worn. The other dress code expectations should still be adhered to.

Personal Appearance and Hygiene

- Hair
  - Must always be kept clean and neat.
  - Must be styled in a way that does not interfere with the field of vision or assigned duties.
  - Must not present an unprofessional image. Including, but not limited to:
    - Mohawks and other unusual styles.
    - Neon, glittering or multiple artificial hues visible.
- Facial hair
  - Must be kept trimmed and tidy.
- Fingernails
  - Must be kept neat and clean.
  - Length must not inhibit performance of job duties.
- Jewelry
  - Must portray a professional image.
  - Must not compromise safety.
  - Must not inhibit performance of job duties.
- Tattoos
  - Must be covered if they are perceived as offensive or vulgar.
- Scents
  - All employees should take steps to avoid strong scents in the workplace. Including, but not limited to:
    - Bad breath
    - Body odor
    - Tobacco products
    - Heavy colognes/perfumes/lotions/essential oils
    - Communications staff must comply with field grooming and safety policies if they are to attend any ride-along sessions.

Reasonable Accommodations for Dress and Grooming

The Company is committed to complying with all applicable federal, state, and local laws regarding reasonable accommodations. Employees who require modifications to the dress code due to a sincerely held religious belief, observance, or practice, or because of a disability or medical condition, may request an accommodation.

Examples of accommodations include, but are not limited to:

- Allowing religious attire or grooming practices (e.g., hijabs, turbans, yarmulkes, modest clothing, facial hair).
- Adjusting uniform requirements for medical reasons (e.g., alternative fabrics or styles to prevent irritation or accommodate medical devices).

Employees seeking an accommodation should notify their leadership and Human Resources as soon as possible. The Company will engage in an interactive process to determine a reasonable solution that does not impose an undue hardship on business operations or compromise workplace safety.

Uniform Replacement

Leadership may approve uniform replacements for wear and tear or size changes, with items exchanged on a one-to-one basis. Employees will be responsible for the cost of replacing uniform items due to loss, misuse, abuse or lack of care.

Return of Uniforms

All company-issued clothing remains the property of GMR. Upon the conclusion of employment in the ICC, employees are required to return all company-issued clothing and materials. Failure to do so will result in the cost of these items being deducted from the final paycheck.

Additional Information

If an employee's appearance or grooming does not align with Company standards, they may be required to leave work without pay to address the issue before resuming their duties.

Company-Issued ID Badge

Policy Number: 100-08	**Subject:** Company-Issued ID Badge
Revision Date: November 11th, 2025	**Effective Date:** January 1st, 2026

I. PURPOSE

To establish procedures outlining the use and possession of company-issued ID badges.

II. POLICY

Badge Possession and Use

- Employees must always display their ID badges inside of GMR facilities.
- Employees must not lend their badge to another individual for any reason.
- If an employee arrives for duty without their badge, they are required to inform the on-duty leader upon arrival to request a replacement or temporary badge.
- Employees are required to swipe their ID badges on any door equipped with a badge reader before entry to ensure that access is properly documented; tailgating is not permitted.
- All visitors, including GMR employees not based at HQ, must register at the front desk. Contact an on-duty leader for assistance.
- Employees must not grant anyone access to GMR property without the approval of leadership.

Missing Badge

Missing ID badges present a security concern and must be reported to the on-duty leader immediately.

- The initial replacement will be provided at no cost to the employee.
- Subsequent replacements may result in a $15.00 payroll deduction.

Return of Badge

ID badges are to be turned in to the on-duty leader upon separation of employment.

Lockers & Personal Belongings

Policy Number: 100-09	**Subject:** Lockers and Personal Belongings
Revision Date: November 11th, 2025	**Effective Date:** January 1st, 2026

I. PURPOSE

To establish standards for the property of ICC employees left in lockers or in the common areas of the ICC.

II. POLICY

Lockers

- Are the property of GMR.
- Will be assigned by departmental leadership.
- May have to be shared by employees due to limited availability.
- May only be used by the designated staff member(s).
- May not have anything affixed or attached to either the interior or the exterior.
- May be unlocked or reprogrammed by leadership without employee consent if locker procedures are in question or in the event of an emergency.

Items prohibited from being stored in lockers include, but are not limited to:

- Alcohol
- Illicit Drugs
- Dangerous chemicals
- Flammable or explosive materials
- Perishable items, such as food and beverages
- Unsealed containers holding liquids of any kind
- Weapons of any kind

Improper use of a locker may result in the suspension or termination of locker privileges.

Employees have no reasonable expectation of privacy for items placed in lockers. Leadership may inspect items, and any contents within, at any time based on reasonable suspicion, with or without prior notice.

Personal Belongings

- Must be stored in an employee's assigned locker when off duty.
- May be discarded without notice if left outside of a locker.

GMR is not liable for any personal belongings, including those in lockers. The company will not reimburse employees for discarded items.

Employees are expected to cooperate in any search or inspection of their person or property requested for reasonable suspicion as described in the Company handbook.

Condition & Upkeep of Facilities

Policy Number: 100-10	**Subject:** Condition and Upkeep of Facilities
Revision Date: N/A	**Effective Date:** January 1st, 2026

I. PURPOSE

To establish standards and expectations of workplace cleanliness and facility upkeep.

II. POLICY

Food and Beverage

Employees are prohibited from storing food or beverages in locations other than in the breakroom or in personal lunch bags or coolers. The GMR HQ refrigerator policy should be followed, including attention to the cleaning schedule.

Food and beverages may be consumed at workstations if:

- Work duties are not impacted.
- No residue is left behind.
- Equipment and furniture are not damaged.

Employees are responsible for keeping their work areas clean, including addressing spills, crumbs and overflowing trash. For cleaning supplies or assistance, employees should see an on-duty leader.

Maintenance

Employees should report any malfunctioning, missing, or broken equipment, furniture, etc. to an on-duty leader.

Regular Cleaning

Shift supervisors or their designee will inspect and clean departmental workstations regularly. Personal belongings are not permitted to be stored at any workstation, including in drawers or cabinets. Personal items left outside lockers may be discarded without notice. GMR is not liable for any personal belongings, including those in lockers. The Company will not reimburse employees for damaged, discarded, lost or missing items.

Tobacco & Smoke-Free Workplace

Policy Number: 100-11	**Subject:** Tobacco and Smoke-Free Workplace
Revision Date: November 19th, 2025	**Effective Date:** January 1st, 2026

I. PURPOSE

To establish a policy regarding tobacco or synthetic product use when reporting for and while on duty in the ICC.

II. POLICY

To ensure a safe, healthy workplace, all tobacco products, including, but not limited to those as defined below, are prohibited in company workspaces, common areas and company vehicles. This policy applies to all employees, clients, contractors and visitors.

- The term “smoking” refers to the burning of all forms of tobacco included in cigarettes, cigars, and pipes.
- The term “smokeless tobacco” refers to non-burning forms of tobacco and synthetics, including chewing tobacco and electronic cigarettes/vapes.

Users should do their best to be free of the scent of tobacco or synthetics on their person when reporting for duty and at the conclusion of each break should they partake during their break.

Smoking and the use of smokeless tobacco products is solely permitted in designated outdoor areas during scheduled break times.

All tobacco products and associated paraphernalia must be stored out of sight while in Company facilities or vehicles.

Break schedules will not be modified to accommodate smoking or tobacco use.

Phoenix: Tobacco & Electronic Cigarettes

Policy Number: N/A	**Subject:** Phoenix: Tobacco and Electronic Cigarettes
Revision Date: N/A	**Effective Date:** N/A

I. PURPOSE

The Communication Center policy is to provide a safe and pleasant environment for our customers and employees. With this in mind, the use of tobacco and electronic smoking products are restricted unless otherwise governed by applicable law.

II. POLICY

1. Any type of smoking and the use of smokeless tobacco is only allowed in designated smoking areas during designated break times.
2. Designated smoking area for the Communications Center is the West side of the building at the curb and beyond.
3. No designated area is allowed inside of the Communications Center
4. Within the designated smoking area smokers are to use the disposal method provided for cigarettes. The area is to be kept clean of trash and any smoking material.
5. The sale of tobacco and electronic smoking products on company premises by vending or other means is also prohibited.
6. No smoking within 100 feet of the building.

Any employee who violates this policy may be subject to corrective action, up to and including termination.

III. TOBACCO USE IMPLICATIONS

1. The use of tobacco products will impact the rates you pay for the company provided medical plans.
2. For those interested in quitting, referral programs are available. Please contact the benefits Service Center at 1-888-301-2123 for details, Designated smoking area for the Communications Center is the West side of the building at the curb and beyond.

Inclement Weather

Policy Number: 100-12	**Subject:** Inclement Weather
Revision Date: November 19th, 2025	**Effective Date:** January 1st, 2026

I. PURPOSE

To establish a procedure to maintain adequate staffing at all times, regardless of weather, and to outline expectations when lodging accommodations are made for inclement conditions.

II. POLICY

GMR requires all essential personnel to report for their scheduled shift regardless of weather conditions.

III. PROCEDURE

GMR's mission statement is to provide care to the world at a moment's notice. Although severe weather or disasters may make commuting to work challenging, employees are required to report for duty unless instructed otherwise.

During periods of inclement weather, employees are advised to allocate sufficient time to arrive at work for the start of their scheduled shift. Employees on duty may be expected to remain until their replacement has arrived.

Employees are responsible for planning for inclement weather. Tardiness or absences will not be excused due to weather events and failure to report for duty when accommodations have been offered will count as an Occurrence of Significant Impact per the Attendance Policy.

The Company will provide reasonable lodging accommodation when deemed necessary. Employees must agree to all stipulated terms prior to accepting the offer of company-sponsored accommodation. Those stipulations are:

- Employees must act professionally as company representatives during GMR-sponsored stays.
- Booking confirmations are sent to the employee's GMR email address.
- A government-issued ID is required for check-in.

- GMR covers accommodation and taxes. Employees must present a credit or debit card at check-in for an incidental hold. Hold amounts and refund timing vary by hotel policy.
- Employees are responsible for any expenses beyond the room rate and taxes, including but not limited to:
    - Additional fees charged if an employee elects to bring their pet(s).
    - Charges for missing items or damaged contents of the room.
    - Room service or incidental fees charged to the room.
- Due to limited hotel availability, it may not be possible to place all staff in the same hotel.
- Transportation between the hotel and office will be the responsibility of each employee.
- Employees are expected to arrive equipped with all necessary items required to stay throughout the entirety of the weather event. These provisions may include, but are not limited to:
    - Food
    - Beverages
    - Toiletries
    - Extra work clothes
    - Outerwear, including appropriate footwear for the expected weather
- Snacks, sandwich-making material, and various microwaveable food options may be available while on duty.
    - Off-duty meals are not provided.
    - Employees should bring their own food to accommodate any personal dietary needs or preferences.

Lactation Accommodation

Policy Number: 100-13	**Subject:** Lactation Accommodation
Revision Date: N/A	**Effective Date:** January 1st, 2026

I. PURPOSE

To establish departmental expectations and procedures for lactation accommodations in alignment with the GMR Employee Handbook.

II. POLICY

This policy ensures compliance with applicable laws and supports employees returning to work after childbirth who require lactation accommodations.

Employees needing lactation accommodations should notify their Leader upon returning to work following childbirth. Upon notification, departmental leadership will meet with the employee to review the request and coordinate appropriate arrangements that balance operational needs and legal requirements.

III. PROCEDURE

Break Time

Employees may take reasonable break time to express milk. When possible, these breaks should coincide with existing break periods. Additional unpaid break time may be arranged and must be accurately recorded using the Company's timekeeping system.

Private Lactation Space

The Company will provide access to a private, non-bathroom space that is shielded from view and free from intrusion. This space will be equipped for milk expression, including seating, electrical outlets, and surfaces as needed.

Milk Storage

Employees may use personal cooler packs or store expressed milk in a designated refrigerator/freezer. All containers must be clearly labeled with the employee's name and date. Employees are responsible for removing containers at the end of each shift. GMR is not liable for items left in shared spaces.

Legal Compliance

In jurisdictions with specific lactation accommodation laws, the Company will comply with all applicable requirements, including providing greater or different benefits than those outlined here.

Contracts, Territories, & Licensing

Please see the Contract Quick Links section for a full list of the territories we serve.

911 Contracts

- Fire Departments handle Emergency Medical Services
- NN is typically contracted through County Fire and Rescue
  - Sometimes as stand-alone solution
  - Sometimes in conjunction with AMR ground transport services
    - AMR offers 911 call center, emergency dispatch, and emergent/non-emergent transport solutions

Licensure

Compact Licenses	Non-Compact License
Compact licensure covers *most* of our bigger contracts	States not covered • CA, DC, NY, OR, MI

Please provide the following by the end of your first week of employment with GMR:

- Marriage and/or divorce certificates
- How to order official school transcripts (high school diploma needed for LVNs)
- Name and/or gender change documents
- List of schools attended
- We will do fingerprint hard cards in house (must bring a photo ID)

About GMR

In This Section

Discover the purpose and drive that makes GMR an incredible place to work. Each call we manage, and each concern we resolve, helps our neighbors engage with an improved system of health care; one where they are valued, heard, invited to engage with the resources of their communities. Thank you for being a member of our team!

Reference Topics

- Welcome 911 Nurse Navigation
- Introduction
- Our Vision & Mission
- Core Behaviors
- Respect in the Workplace
- Meet Our Team
- Company Jargon
- 911 Nurse Navigation (NN) Deep Dive

Welcome to 911 Nurse Navigation

Dear Team Member

Welcome to 911 Nurse Navigation at Global Medical Response (GMR)!

I am excited to welcome you to our team and thank you for choosing to join us. As a Nurse Navigator, you play a critical role in helping patients access the right level of care at the right time. Every interaction you have has the potential to provide reassurance, guidance, and support to individuals and families during moments when they need it most.

At 911 Nurse Navigation, our mission is centered on delivering exceptional clinical guidance while promoting patient safety, improving healthcare access, and supporting appropriate care decisions. We are proud of the work we do and the positive impact we make every day across the communities we serve.

As you begin your journey with us, I encourage you to embrace the opportunities to learn, ask questions, and collaborate with your colleagues. We believe that success comes from continuous learning, teamwork, accountability, and a shared commitment to excellence. Your unique experiences, skills, and perspectives are valued and will contribute to the ongoing success of our program.

Our leadership team is committed to creating an environment where team members feel supported, respected, and empowered to grow professionally. Throughout your onboarding and beyond, you will have access to leaders, educators, and peers who are invested in your success.

We recognize that starting a new role can be both exciting and challenging. Please know that you are not expected to know everything on day one. Give yourself grace as you learn new systems, processes, and workflows. We are here to support you every step of the way.

Thank you for bringing your clinical expertise, compassion, and dedication to our organization. We are honored to have you as part of the 911 Nurse Navigation family and look forward to the contributions you will make.

Welcome aboard. We are excited to have you with us.

Warm regards,

Purity Nyaga
Director, Clinical Operations
911 Nurse Navigation

Introduction

Global Medical Response delivers comprehensive emergency medical services to **over five million patients annually**. Our extensive network of ground and air clinical teams ensures the right level of care at the right time and place, regardless of the situation. With a focus on patient care and emergency response, GMR stands out as a leader in the emergency medical services industry.

Most Experienced Program Offering More Complete Care to 911 Callers

An estimated 30% of calls to 911 don't need an immediate EMS response. These callers need help, but often don't know where else to go, or they aren't able to access the care they need.

In the past, the only method communities had to help callers with non-life-threatening needs was to dispatch EMS and send the patient to the Emergency Department (ED). This diverted EMS resources from urgent, life-threatening issues and led to higher costs for patients who didn't require a visit to the ED.

In **2018**, Global Medical Response (GMR) pioneered a solution with its introduction of 911 Nurse Navigation in order to provide care to these callers, often without the use of an ambulance. In communities across the country, GMR's Nurse Navigators use evidence-based clinical protocols to screen a patient's current condition and provide an appropriate resource to meet the caller's unique healthcare needs.

Resolutions include; dispatching a ride-share to an urgent care accepting patients, an appointment at a Federally Qualified Health Center, or virtual care with a physician on the spot.

Learn more on our website at:

🔗 https://globalmedicalresponse.com/services/healthcare-innovation/nurse-navigatio

Better for Patients and Communities

*In **2024**, more than **50,000 911 callers** from across the country were screened by GMR's 911 Nurse Navigation program and redirected to more clinically appropriate and cost-effective care options.*

Every **10 minutes** someone called 911 seeking help and GMR's team of 911 Nurse Navigators connected them to the best healthcare resource

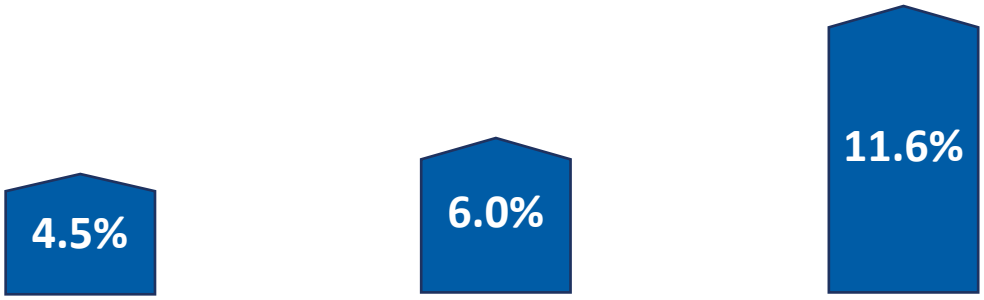
$21 million estimated in savings for patients, first response agencies, payors and health systems

81,000-unit hours of capacity back into the EMS system by avoiding unnecessary responses, ambulance transports and ED visits

Better for EMS Providers

Our first responders in 911 Nurse Navigation markets are also more satisfied with their work. GMR's data indicates decreased burnout and improved satisfaction when providers are not overly burdened with low acuity calls.

In GMR's 911 Nurse Navigation markets:



4.5% increase in field providers who stated they feel like their job is important

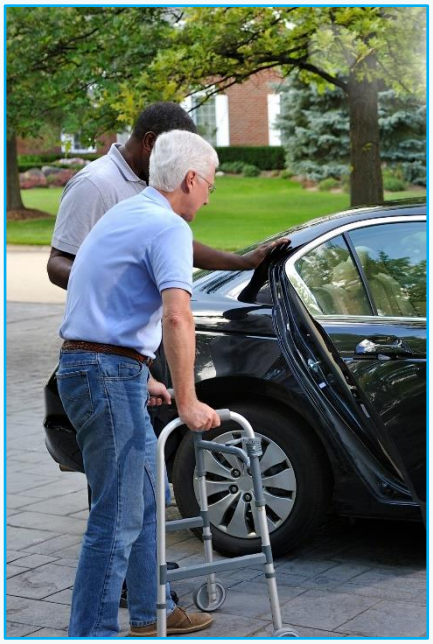
6.0% increase in field providers who stated they feel like their training and skills are well utilized

11.6% increase in field providers who stated they have high job satisfaction

Our Vision & Mission

Our Vision

We will provide a healthier tomorrow for millions of people, one patient at a time, by providing access to the right care and serving as a trusted EMS healthcare partner in every community.



Our Mission

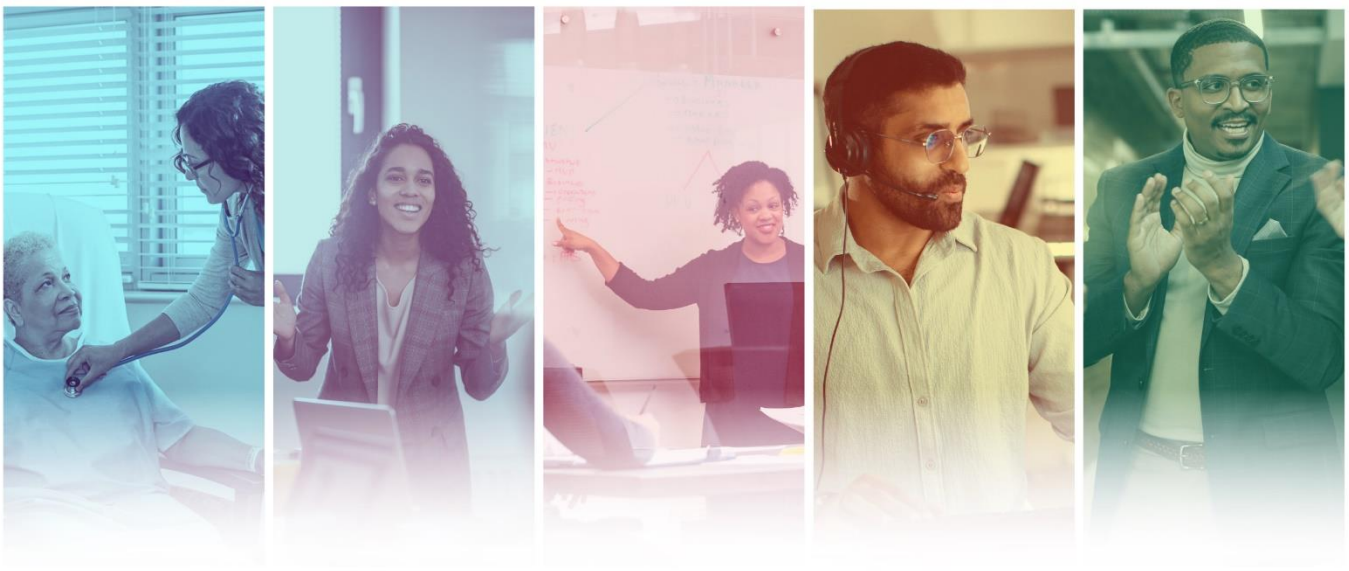
Providing care to the world at a moment's notice

Core Behaviors

GMR Core Behaviors Outline

KEEP CARE AT THE CENTER	RAISE YOUR HAND	SEEK TO UNDERSTAND	FIND A WAY TOGETHER	BE ACCOUNTABLE
We prioritize the care of our patients and our teams when we plan or act.	We ensure that people can feely and safely "raise their hands" about issues, concerns, and opportunities – and it is everyone's duty to do so.	We treat each other respectfully. We gather information, listen and seek to understand before acting	We work together to solve problems and find solutions that support our mission.	We set clear expectations, systems, and processes and trust those closest to the issue to make good decisions and take accountability.

For more information, visit the GMR Home Page on Sharepoint.



Respect in the Workplace

The importance of Respect in the Workplace

- Provides a feeling of security at work
- Shows appreciation for patients, coworkers, and leaders
- Maintains efficiency
- Fosters a happy work environment

Cambridge dictionary defines respect as: *"to treat something or someone with kindness and care".*

How to Foster Respect

Avoid gossip	**Be mindful of everyone's feelings**	**Show appreciation to each other**
Greet everyone with kindness	**Value diversity and other's opinions**	

The Main Floor

Rules

1. Use teams when collaborating with coworkers, whenever possible
2. When you do need to talk on the floor, please speak as quiet as possible
3. No horseplay or other noisy activities will be tolerated

Why?

- Helps maintain a professional work environment
- Assists dispatchers and nurse navigating with hearing patient needs, while on the phone
- Assists with keeping patient's calm when there is minimal background noise

Teams Decorum

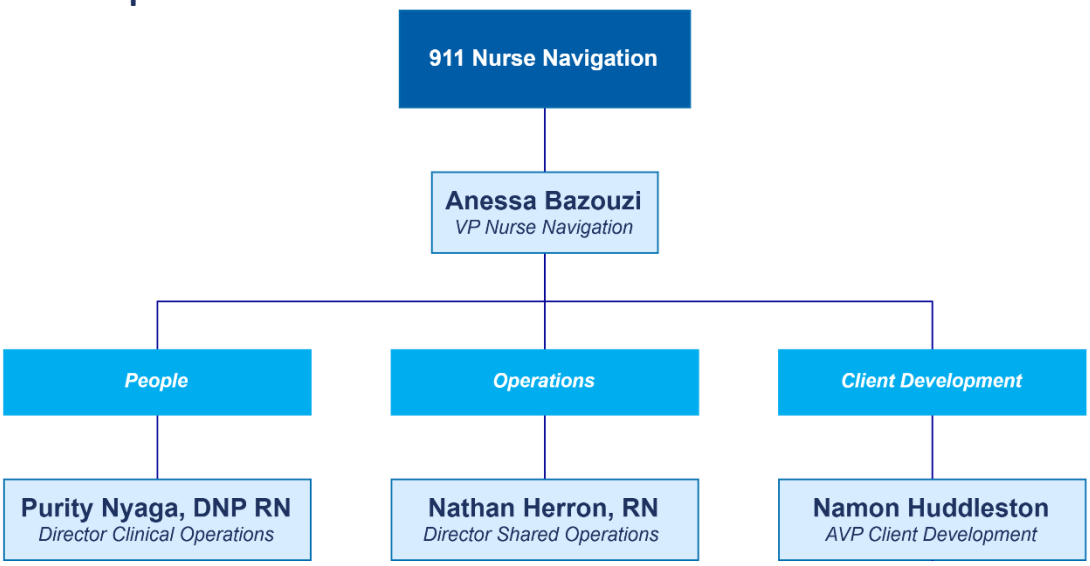
1. 911 Nurse Navigation promotes a collaborative work environment when trying to aid our patients or troubleshoot issues we may have run into.
2. This will be a no judgment zone!
3. Feel free to see if others may have handled the situation differently or let us know how you are feeling about a particular situation.
4. This forum is also useful to discuss obstacles you may have experienced. Let's troubleshoot and work together.

Meet Our Team

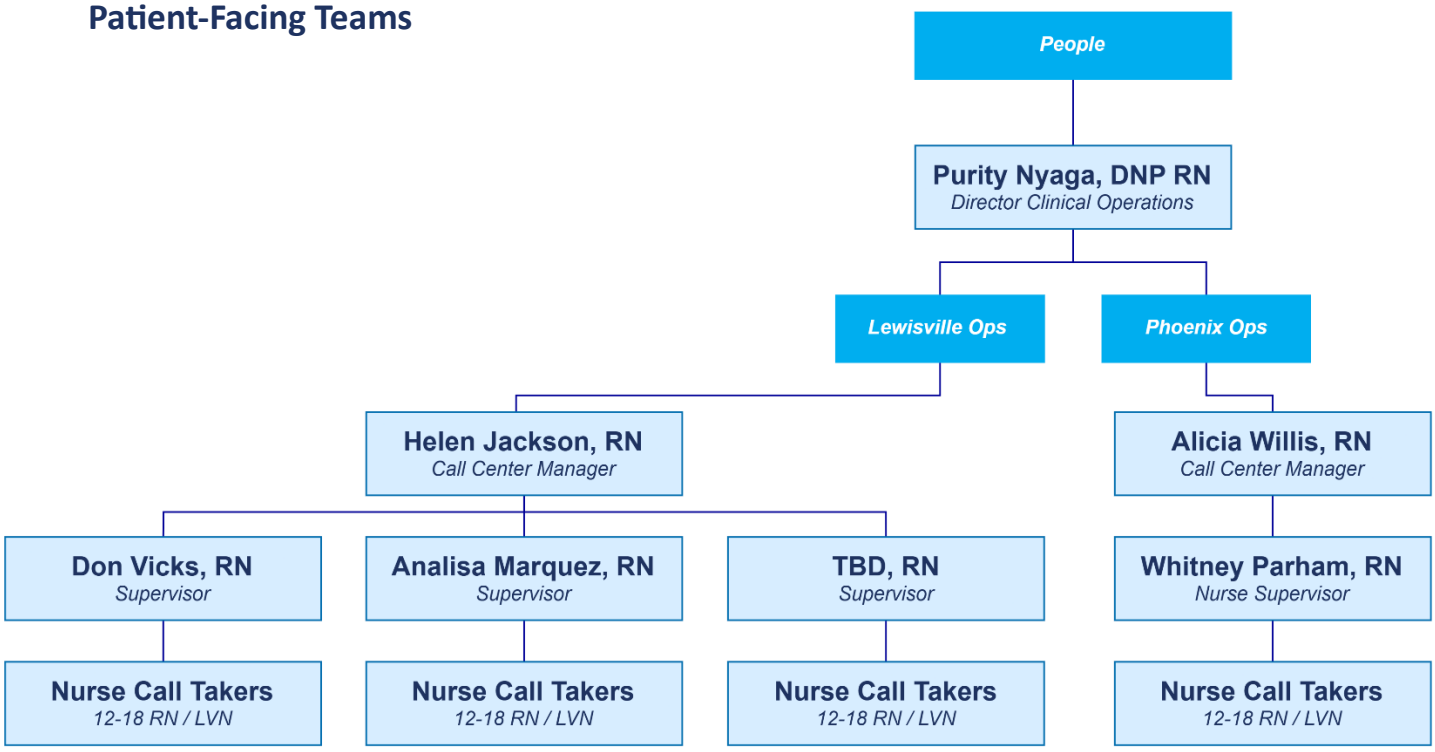
Organizational Chart

Full Organizational Chart: 🔗 911 Nurse Navigation Organizational Chart

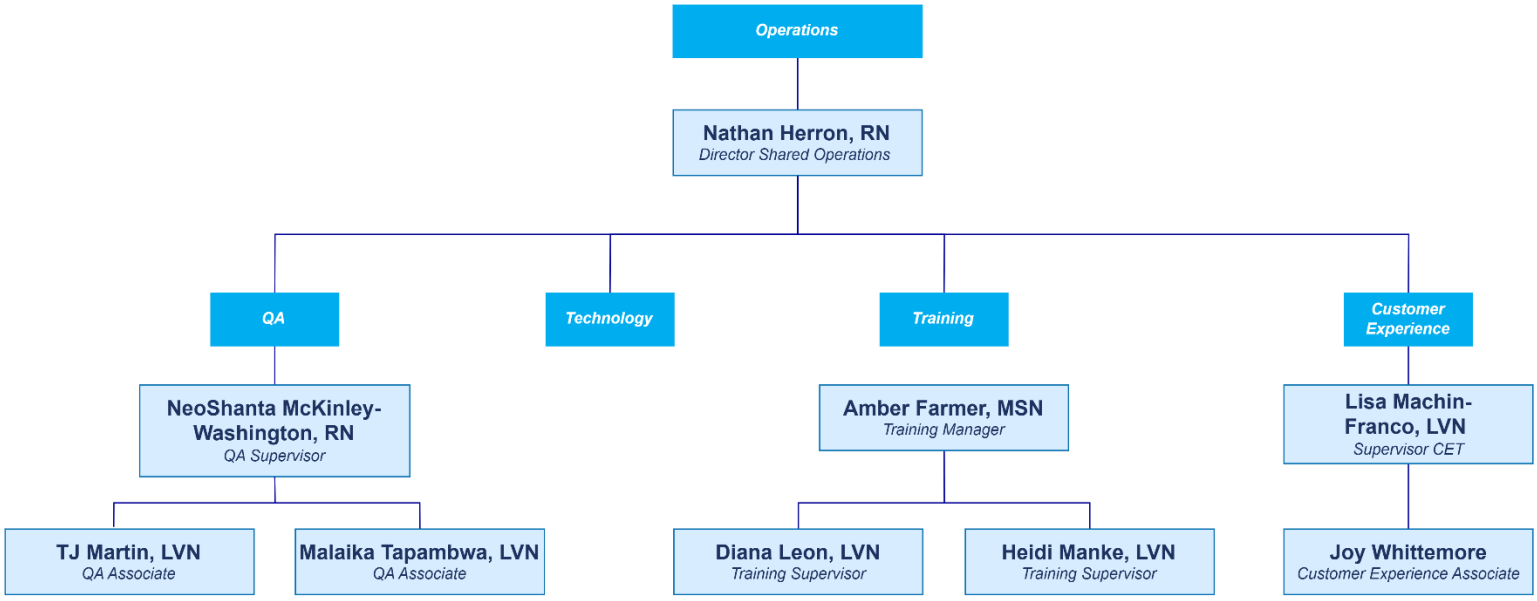
Our Leadership Team



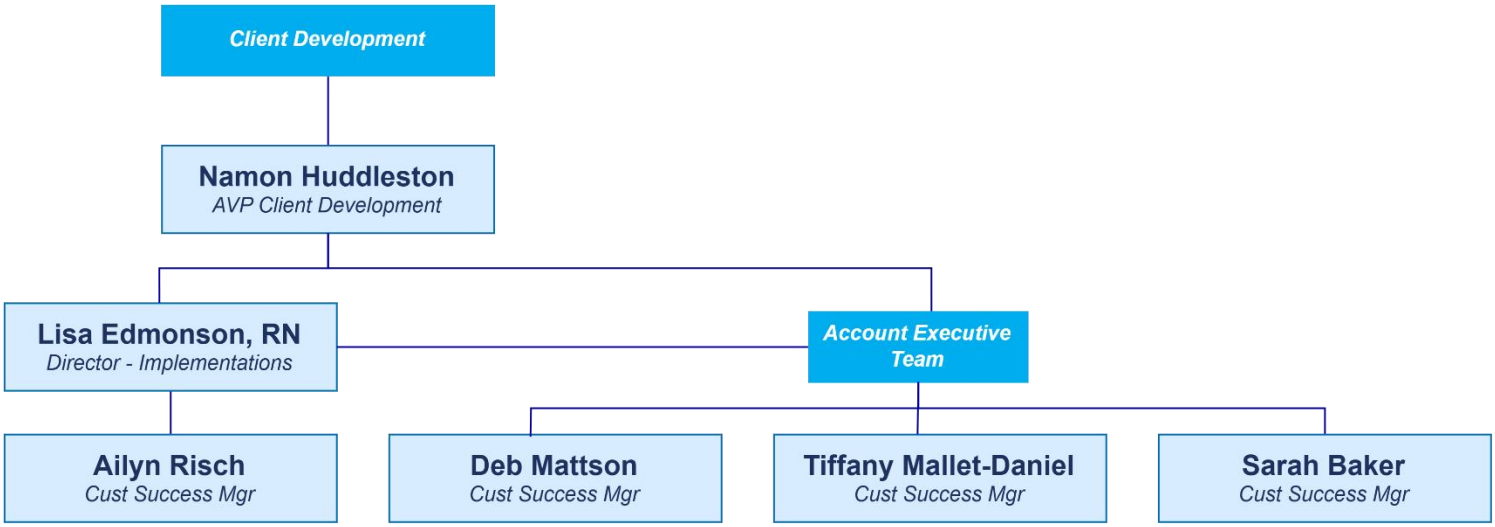
Patient-Facing Teams



Operations



Customer Success



ACW

After Call Work

This is the designated time after disconnecting with the caller / dispatcher to complete documentation related to the call.

- A five-minute timer will start immediately after the call has ended. When the timer ends, the nurse will be automatically placed back On-Queue to receive another call.
- If the nurse completes documentation before the timer ends, the nurse will click the blue done button to go back on-queue.

Additional details about ACW can be reviewed in our Genesys software tutorial.

ALS

Advanced Life Support

This response is for immediate life threats. Onboard an ambulance with lights and sirens are paramedics and EMTS who are trained to intervene and have access to life-saving meds and equipment. The main reasons we call are for loss of consciousness, hemorrhage, stroke, chest pain (MI), and respiratory distress.

AMR

American Medical Response

Our ground transportation company which responds to emergencies and conducts interfacility transfers.

BLS

Basic Life Support

This response type means an ambulance has been dispatched with limited ability for intervention (no IVs, no meds, no intubation). Crew typically consists of two EMTs who provide basic CPR and first aid.

CAD

Computer Aided Dispatch

Dispatchers use this to send calls over to 911 Nurse Navigation (NN) and to Fire / EMS via computer. We have 5 contracts that are CAD-to-CAD:

- DCFEMS
- Rochester
- Onondaga
- Dekalb
- Heartland

When calls come through the CAD, click it on the right side of the screen to grab it and click being evaluated to claim it as yours.

For more information, please review our CAD-to-CAD software tutorial.

CRISP

Chesapeake Regional Information System

CRISP is a regional health information exchange (HIE) serving Maryland and the District of Columbia. We use CRISP to find (DC only) patient's insurance ID numbers and home clinics. You must log in once monthly to maintain access.

Please review our CRISP DC software tutorial for more information.

CSM

Customer Success Manager

This team is client (contract) facing. They ensure clear communication and collaboration between the client and 911 NN. There are four who are assigned to certain regions of the United States and conduct quarter meetings called JOCs with each contract. This team is managed by Namon Huddleston, AVP Client Success.

DC FEMS

District of Columbia Fire and Emergency Services

Our first NN contract in 2018. Located in the US Capital.

- They have a distinct phone greeting:
  - "Right Care, right now this is nurse [*name*]. How can I help you?"

EACW

Extended After Call Work

This Genesys status allows nurses to complete call-related tasks:

- This is a manual status change and should only be selected if the standard ACW timer is running out. Nurses should not immediately enter this status when the call is disconnected.

EMD

Emergency Medical Dispatch

Refers to a set of protocols used by emergency dispatchers to handle medical emergencies. It involves asking specific questions to assess the patient's condition, prioritizing the call based on severity and determining if the call meets criteria for 911 Nurse Navigation.

EMS

Emergency Medical Services

This is an ambulance response. EMS is further broken down into ALS and BLS response types.

FQHC

Federally Qualified Health Center

They provide comprehensive healthcare services to underserved communities regardless of ability to pay. They receive federal funding and offer a sliding fee pay scale based on income and family size.

GMR

Global Medical Response

The company you work for!

As a provider of care to the world at a moment's notice, we are a leader in the emergency medical services industry.

ICC

Integrated Communications Center

The call center (at headquarters in Lewisville) containing air and ground communications and 911 Nurse Navigation.

NMTARA

Needs Matched Time Appropriate Resource Allocation

This is how we determine the time to care. An NMTARA level (level 0-6) is reached by selecting the appropriate Corti pathway that corresponds with the patient's most urgent complaint.

- **NMTARA 0**: Emergent care needed (immediately)
- **NMTARA 1**: Care needed within 1 hour
- **NMTARA 2**: Care needed within 2-4 hours
- **NMTARA 3**: Care needed within 12 hours
- **NMTARA 4**: Care needed within 24 hours
- **NMTARA 5**: Care needed within 1 week

NN

911 Nurse Navigation

Also called Nurse Nav. This is us!

NNRide-ER

911 Nurse Navigation Ride ER

This disposition is the rideshare to the emergency room. We primarily use Lyft but also use Uber. This disposition is not an option in DC (use DC Taxi or Medical Transportation Management (MTM))

NTL

Nurse Triage Line

This is the terminology used for 911 Nurse Navigation in the DC market.

- We use this distinct phone greeting for these communications:
  - "Right Care, right now this is nurse [*name*]. How can I help you?"

NURSYS

Nursys is the only national database for verification of nurse licensure, discipline and practice privileges for RNs and LPN / VNs licensed in participating boards of nursing, including all states in the Nurse Licensure Compact (NLC).

Nursys.com provides online verification for endorsement to a nurse requesting to practice in another state and anyone who wants to verify a nurse license.

OUC

Office of Unified Communications

This is the PSAP in DC.

PSAP

Public Safety Answering Point

This is the 911 call center. 911 calls are dispatched from here directly to Fire / EMS or 911 Nurse Navigation. Each contract has at least one PSAP. They handle over 200 million 911 calls annually.

REF

Referral

This disposition refers patients to a brick-and-mortar clinic like an urgent care or behavioral health resource like Crisis Connection.

RONA

Ring Offered No Answer

This is a missed call. The nurse was available in Genesys but did not answer the phone. The nurse will need to place themselves back On-Queue to exit the Not Responding status.

RSC

Return to Self-Care

This disposition is selected when patients opt to follow the home-care instructions provided by the nurse or when patients decline all options including an EMS response.

- Patients may also opt to go to the ER by private vehicle as well which would be an RSC

SBARF

Situation, Background, Assessment, Recommendation, Follow Through

This is standardized documentation used by nursing that borrows from the traditional SBAR format.

For more information, please review our SBARF documentation segment.

TL;DR

Too Long; Didn't Read

Used to present a summary for those who possess an innate or learned aversion to reading.

TNC

Triage Not Complete

No NMTARA level is reached due to the following reasons:

- Caller hangs up and no answer on callback x2
- Patient refuses triage process
- EMS arrives on scene
- Exclusion list criteria met.

This replaces the former internal designation NMTARA 6.

UKG

Ultimate Kronos Group

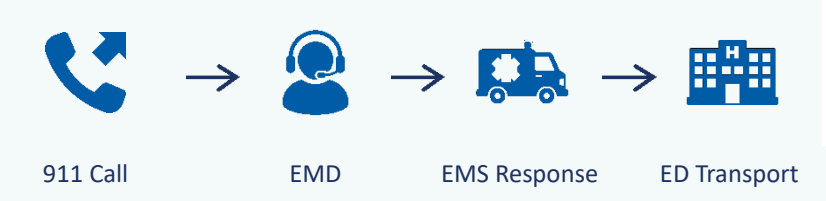
This chiclet is how you clock in and out, fix missed punches, check the schedule, and enter PTO requests. You can also clock in and out using the time clock on the wall near the printer.

VC

Virtual Care

This tele-health option is great for patients who need further evaluation without labs / imaging. RelyMD (the company we use) works like Uber for providers. This disposition allows patients to be treated in place. They can even get (certain) prescriptions called in to their pharmacy.

911 Traditional Model: You Call, We Haul!



Timeframe: Emergency

Resource: Ambulance

This traditional model cannot keep up with modern changes, concerns, or capacity needs.

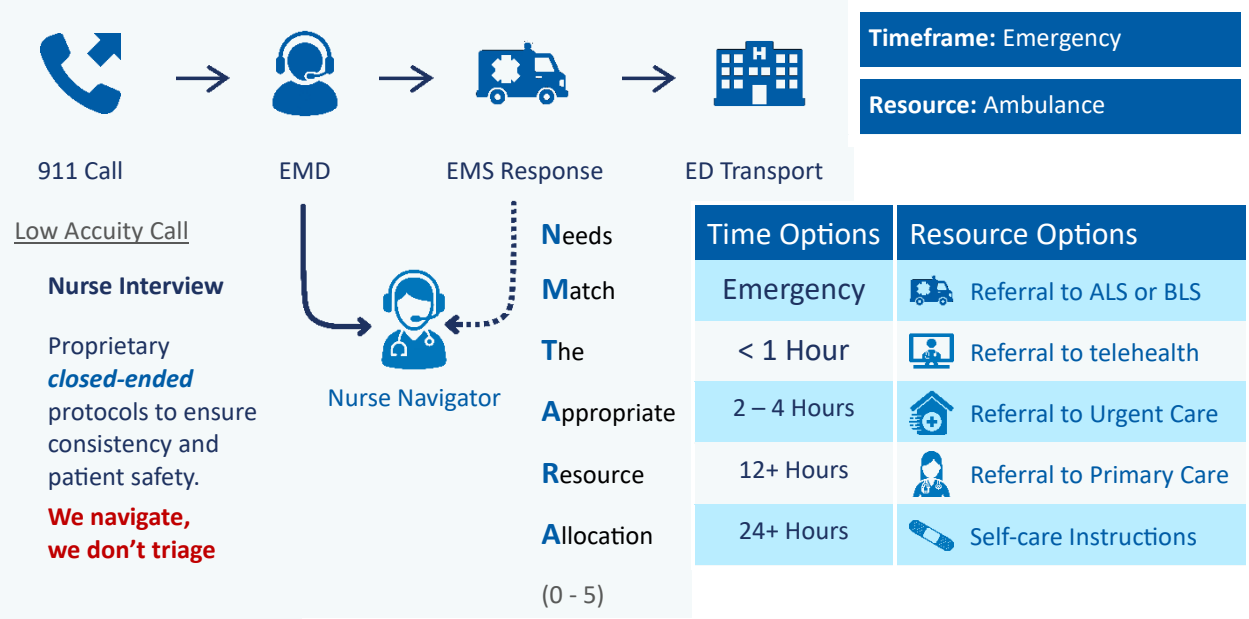
Consider the modern challenges of maintaining this model. Think about challenges you have faced in your career caused by resource limitations.

- Caller in need contacts 911
- The answering 911 Emergency Call Dispatcher asks Emergency Medical Dispatch questions
- Fire department sends paramedic / EMTs to the scene
- Pt treated in place or transported to the hospital for further eval
  - The individual in need may decline care
- All medical 911 calls treated as emergency w/ ambulance sent to patient
  - Not all ambulances responses are the same!
    - ALS
    - BLS

Current Challenges the Traditional Model Faces

- Shrinking workforce: COVID prompted drastic exodus from EMS positions
- Skyrocketing expenses: wages, vehicle maintenance, gas, technology
- Stagnant revenues: Reimbursements have not kept pace with rising costs
- Ever increasing demand from:
- Growing public
- Uninsured and under-insured patients
- Growing medical illiteracy

Our Modern Model



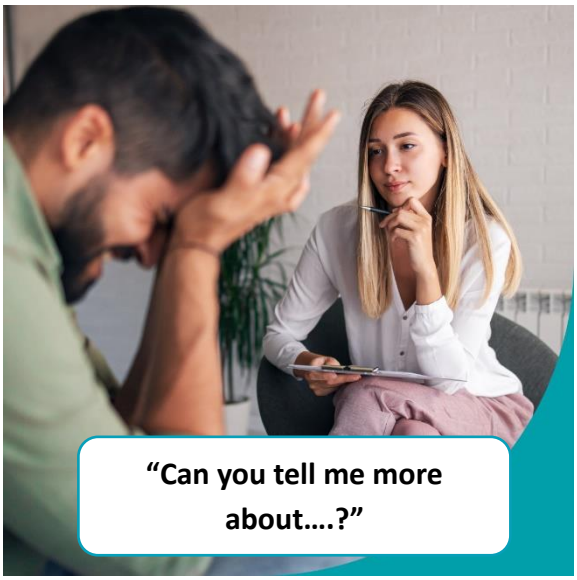
- 911 NN started as a little project within "Integrated Health", a GMR department linked to several patient care projects
  - Example: Medically at Home, Envision Physician Services, etc.
- COVID-19 saw NN pursue all kinds of services:
  - Medical screenings
  - Worker's Comp claims
  - After-hours triage for MD offices
  - Other similar services
- Most of these ancillary services ended by 2023 coupled by rapid growth in the 911 space
- In 2025, GMR executive leadership refocused GMR from transport to healthcare and positioned 911 NN as the "scaffolding on which EMS 2.0 can be built."

The Ideal 911 Nurse Navigator

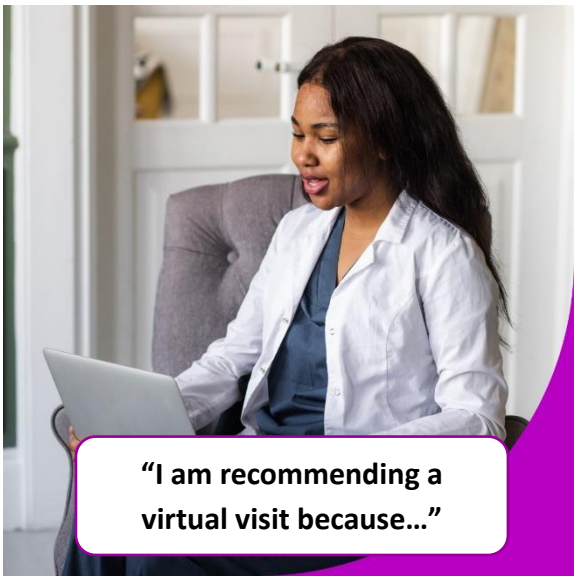
Shows Empathy

Advocates

Actively Listens



Educates



Investigates

Follows Through

We match the patient's need to care resources. It's not just a GMR core value, it is core to the nursing profession!

Low Acuity Callers

We give low acuity callers treatment options without engaging EMS

Most low acuity callers:

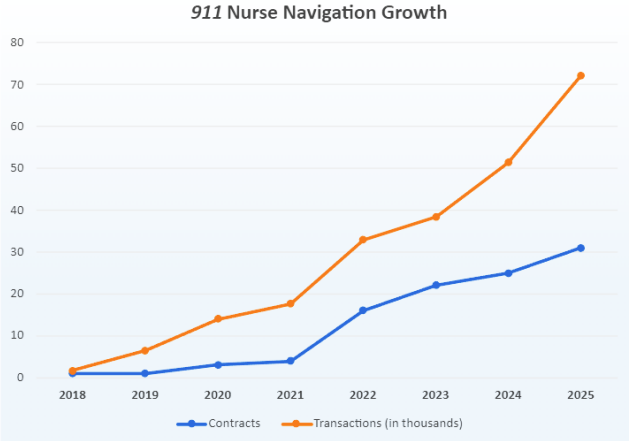
We know:

- 7-8% of 911 EMS calls need time-sensitive, life-saving intervention.
- Conservative estimates show 30-40% of low-acuity, non-emergent calls can be resolved outside an ED: Urgent Care, Virtual Care, BMH services, social services.
- Less conservative estimates have this at 70-90%.

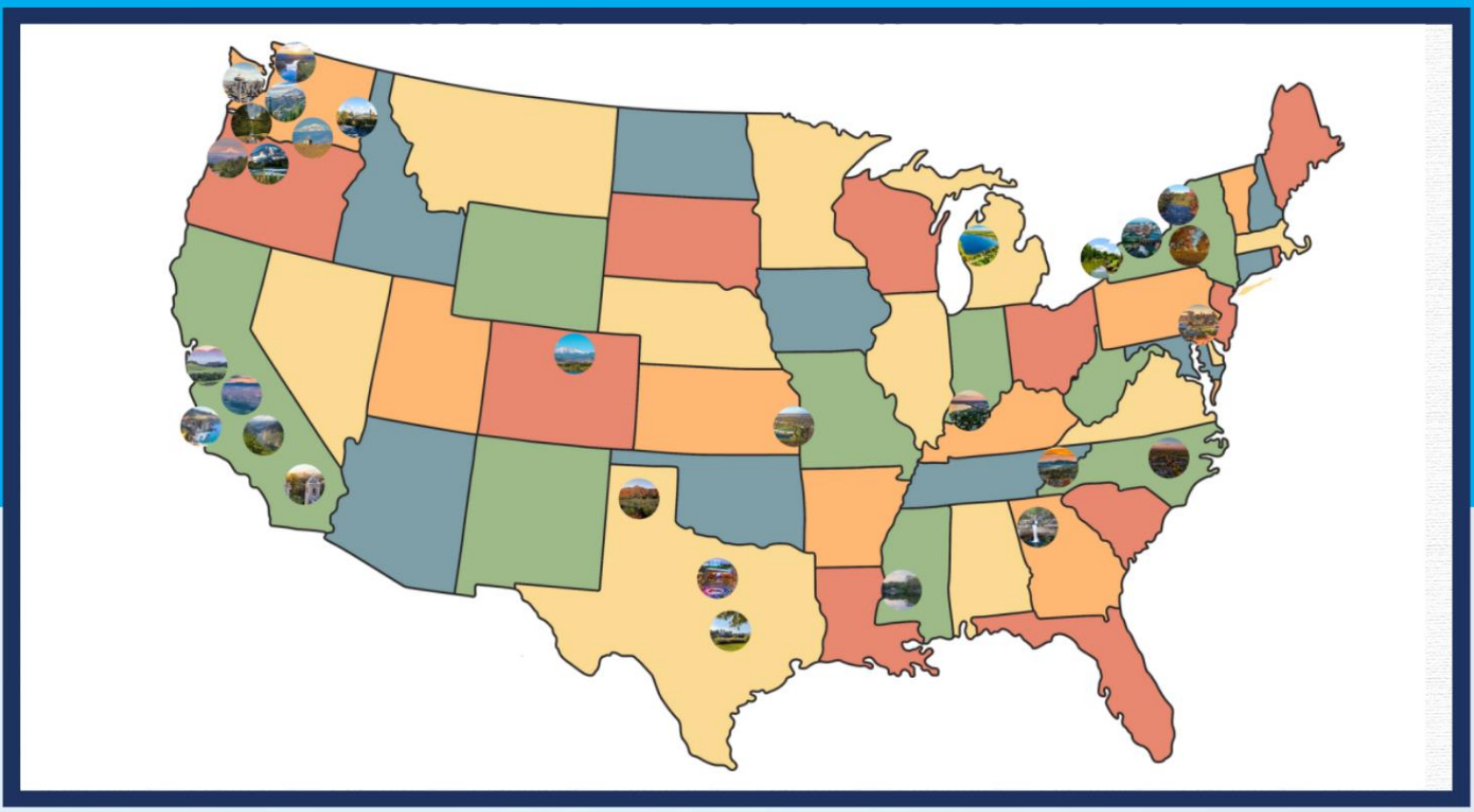
When we fulfill our purpose, secondary benefits include:

- More ambulances available for emergency calls
- Less ER crowding
- Increased patient satisfaction
- Increased EMS provider satisfaction

911 Nurse Navigation Growth



- 5 contracts in implementation
- 10 in negotiation
- 10+ being pursued
- Current contracts looking to increase volume:
  - Expand determinants
  - Start field services
  - Implement CAD-CAD



Contracts

In This Section

Please allow us this opportunity to introduce our contracts for what they are; communities that we help each and every day! Learn about the locations we serve, how they communicate with 911 Nurse Navigation, and the resources they offer to their residents. We have also included any contract-specific nuances or required call scripting.

Reference Topics

Contract Links by Time Zone

Comms Center Supervisor Contact List

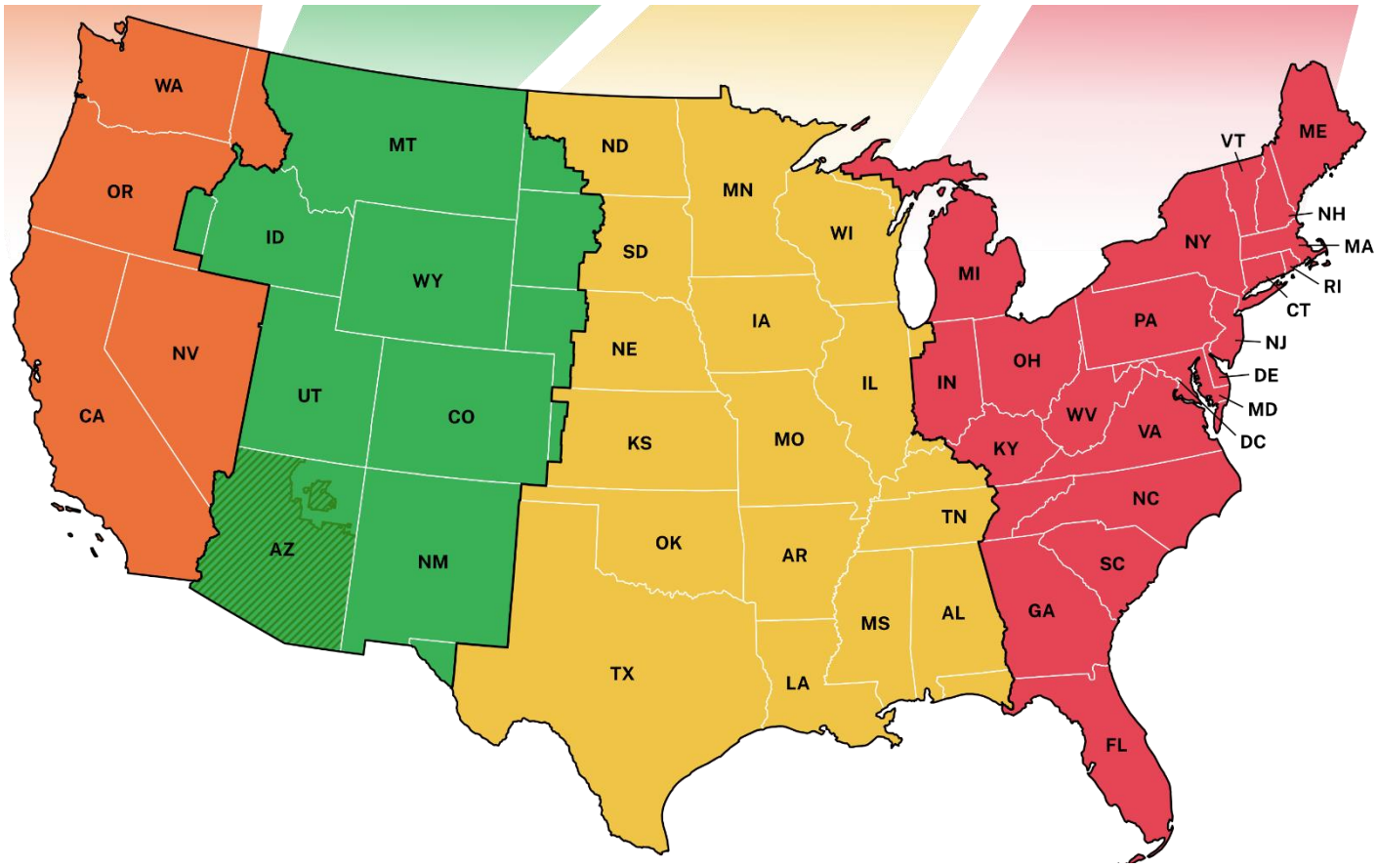
Contracts Quick Overview Map

Contracts by Year Timeline

2018	911 DC, Fire & EMS		
2019			
2020	911 GA, DeKalb County		
2021			
2022	911 NY, Rochester	911 NC, Wake County	911 NY, Onondaga
2023			
2024			
2025	911 MA, Baltimore	911 MI, Kent County	911 NY, Erie County
2026	911 GA, Barrow County		

Contract Links by Time Zone

Time zone reference images are provided to the public by the National Institution of Standards & Technology at: https://nist.time.gov/



Pacific Time (PT)	Mountain Time (MT)	Central Time (CT)	Eastern Time (ET)
			DC-FEMS
			GA - Barrow County
			GA – DeKalb County
			MA – Baltimore
			MI – Kent County
			NC – Wake County
			NY – Erie County
			NY – Onondaga
			NY - Rochester

911 DC, Fire & EMS

Founded: April 2018

DC is Different!

Contact With

DC Fire and EMS (FEMS)

PSAP

DC OUC

Federal District

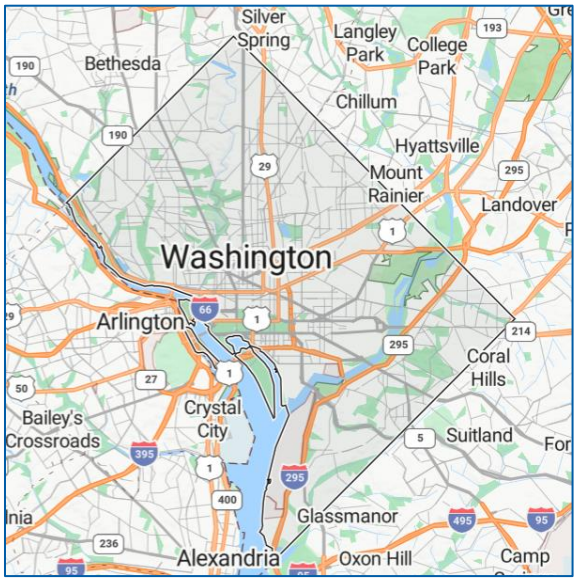
Washington, D.C **Eastern Time**

Unique Scripting

"Right Care, Right Now. This is [NAME]. I'm a nurse on a recorded line. How may I assist you?"

Communications

ALS	DC FEMS
BLS	AMR Capital Region
CAD-to-CAD	*Yes*
Field Referrals	*Yes (Field Fire)*
NN RideER	*No (Never)*



Test Your Knowledge

RIGHTCARE RIGHTNOW

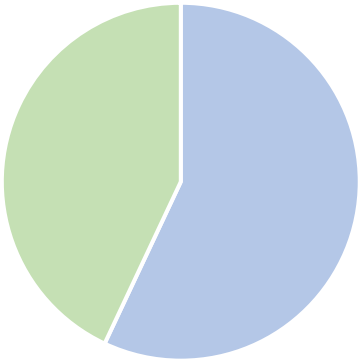
DC uses "Nurse Triage Line" or "NTL" to describe Nurse Navigation

"The goals of the Right Care, Right Now program are to improve patients' health outcomes and to preserve the DC Fire and EMS Department's (FEMS) resources for those patients with life threatening injuries and illnesses. This should also free up beds in crowded hospital emergency departments, which will benefit all critically ill emergency patients. The District has one of the highest per capita EMS call volumes in the nation. Our high non-emergency call volume strains the Department's resources for emergencies."

Call Volume Data

2023

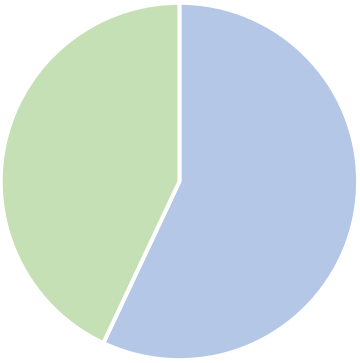
Calls triaged: 7,154



Call Taker	57%
Field	43%
Navigated	67%

Program-to-Date

Calls Triggered: 66,000+



Call Taker	70%
Field	30%
Navigated	59%

Services & Resources

CRISP DC

CRISP DC

CRISP is a database that allows providers and organizations to more efficiently coordinate care.

911 Nurse Navigation uses CRISP to verify the patient's insurance and "home" clinic.

Insurance type should be asked in all contracts, but in DC it is especially important because we will arrange rides to clinics based on insurance type.

Click Here to review our full CRISP DC tutorial in the System Applications section of this reference guide.

Transportation

MTM

Use if the patient has:

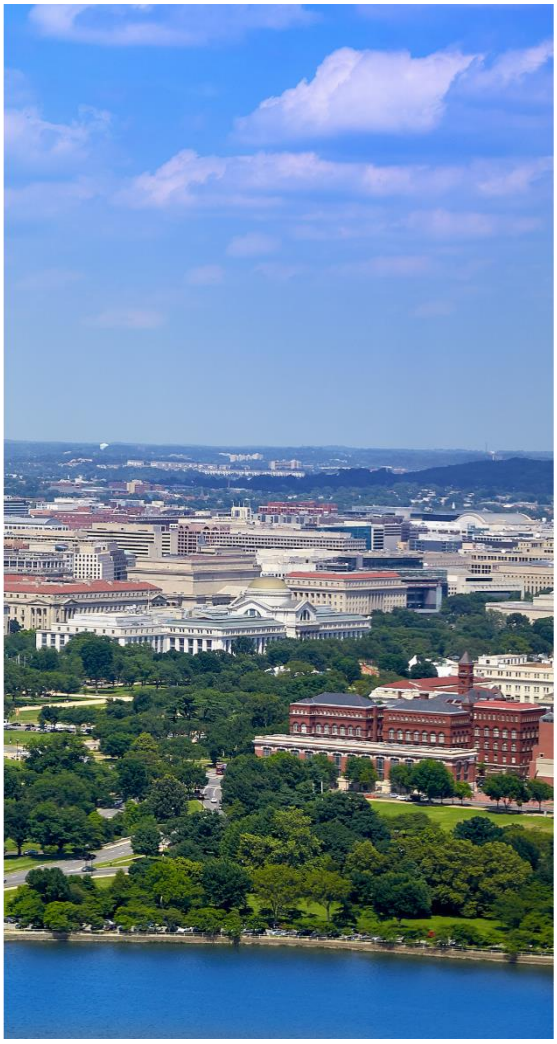
- Medicaid
- Managed Care Organization (MCO)
- AmeriHealth
- Straight Medicaid
- Amerigroup (FFS)

DC Taxi

Use if the patient has Non-Medicaid (commercial insurance) or is Uninsured.

DC Taxi platform is the “catch all” if A2C, MTM or Saferide aren’t working.

Click here to review our full DC Taxi tutorial in the System Applications section of this reference guide.



Kaiser Saferide

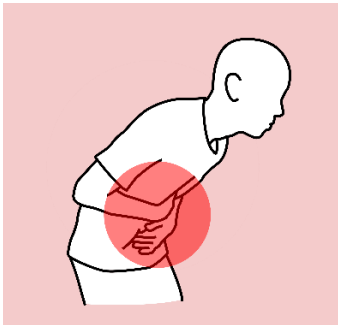
Use if the patient has Kaise

Lyft

Lyft Concierge can only be used if DC Taxi has been tried and is failing.

Click here to review our Lyft & Uber guide in the Workflows section of this reference guide.

Special Plan of Care Conditions



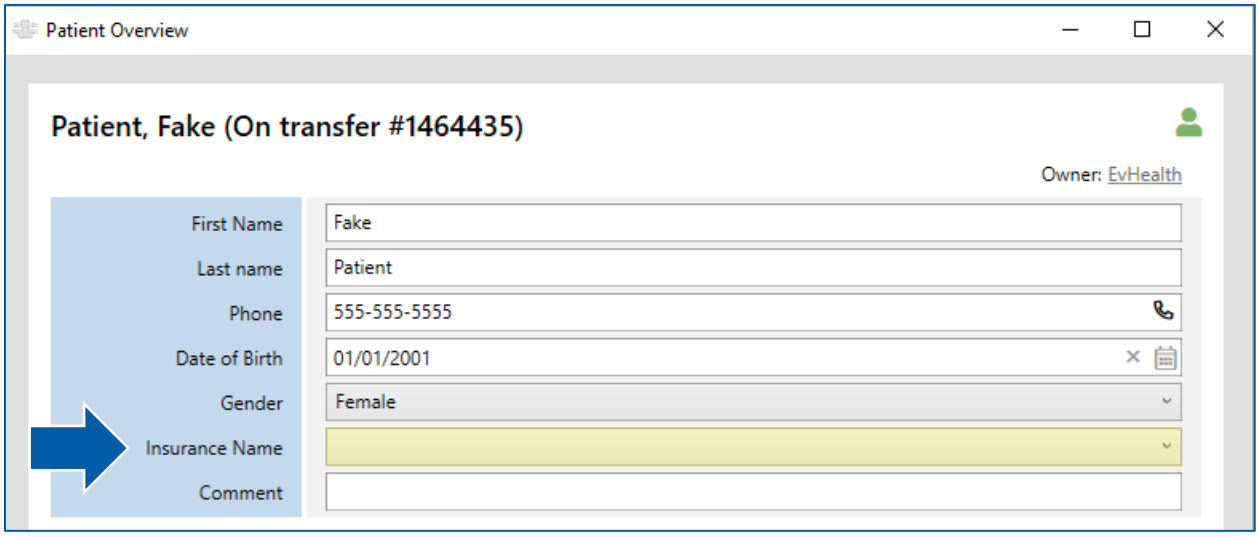
No ***ABD pain*** to clinic.

Documentation

Insurance Documentation

Enter the patient's insurance information listed in ***CRISP DC*** into the Patient Overview window:

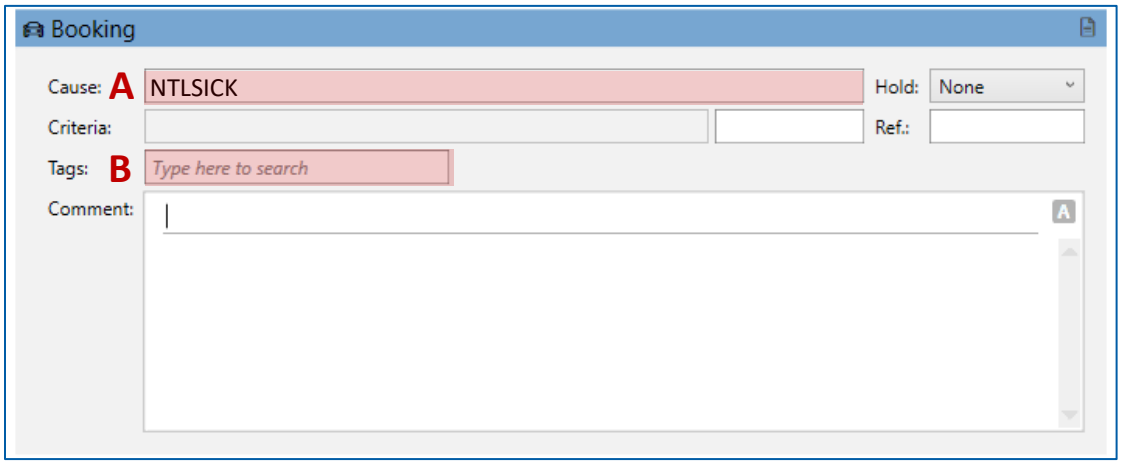
- Use the drop-down to select the specific insurance, if listed.
- Place the Membership ID No. in the ***Comment*** field.
  - If not listed in ***CRISP*** and the patient does not have the information available, you may leave this blank.



While insurance information is not required for a patient to use our services, it is important to attempt to get this information. DC FEMS monitors this data and uses it for reimbursements from the insurance providers.

The "Cause" rule for CAD-to-CADs

- If it's blank: Fill it in! (w/ Corti pathway)
- If it has data: Leave it alone!



A. This is DC specific, internal coding.

- This field gets reported back to DC. Changing it will skew their data.

B. Use DC appropriate tags:

- Fall Taker
- Fire Field

Dispositions

For this (our first) contract, we're a little old-school:

- Logis includes button-types and terminology for DC that we have updated elsewhere.
- Buttons will be updated if / when their CAD team allows for it.

Other Differences

- DC does not currently have RideER or post-VC buttons.
- DC has five buttons for different call types usually encompassed with the "Other" button:
  - ADMIN: Administrative Issues
  - FEMS: FEMS was dispatched prior to receiving call
  - FOLLOWUP: Follow up call from either patient or partners
  - NPRNTL: No Phone Call Received
  - TESTNTL: Test Call

Disposition Key

Left = DC FEMS | Right = NN Standard

ALS = ALS	REFRR = MUC
AMRBLS = BLS	RSC = RSC
TELE - VC	Multiple = Other
REF = REF	

DC FEMS Dispositions

ALS	AMRBLS
TELE	REF
REFRR	RSC
Admin	Follow-Up
FEMS	NPRNTL
TESTNTL	

Standard Dispositions

ALS	BLS
VC	NN-RideER
REF	MUC
Other	RSC
VC-EMS	VC-RideER
VC-REF	VC-MUC

AMRBLS is to be used if the patient needs BLS transport.

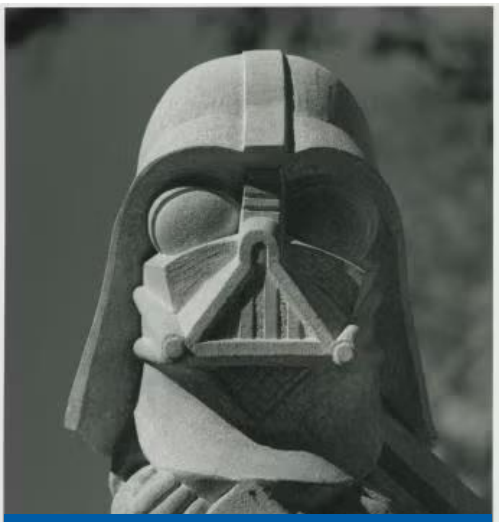
- It does not matter which field service is providing the transport (FEMS or AMR).
- If the recommendation is BLS, use AMRBLS for the final disposition.

More About Washington DC

- *DC FEMS was the 1st Nurse Navigation contract!*
- *The city hosts more than 170 embassies and international cultural centers.*

Washington DC gets more rain than Seattle!

Standing at 555 feet, the Washington Monument obelisk is not only the tallest structure in Washington, D.C. but the tallest stone structure in the entire world.



The Washington National Cathedral has an image of Darth Vader hidden on one side.

(Image provided by the National Cathedral Archives)

911 GA, Barrow County

Founded: September 2026

Contact With

Barrow County

PSAP

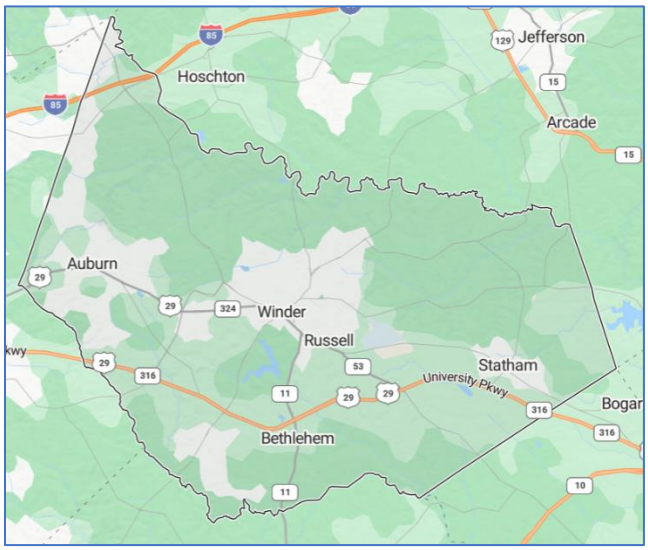
Barrow County 911

State

Georgia (GA) Eastern Time

Unique Scripting

There are no unique scripting requirements for this contract.



Communications

ALS	AMR
BLS	AMR
CAD-to-CAD	*No*
Field Referrals	*No*

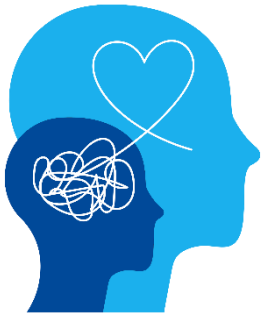
Test Your Knowledge

Cities, Towns, & Neighborhoods in Barrow County

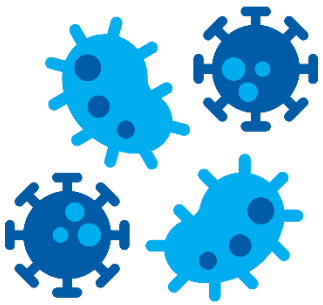
- Winder (County Seat)
- Statham
- Auburn
- Carl
- Bethlehem
- Braselton

Barrow County is named after David Crenshaw Barrow, Jr., who served as Chancellor of University of Georgia from 1906-1925..

Health Priorities



Improving mental health resources



Managing Chronic Diseases

Expanding access to health coverage and specialized care

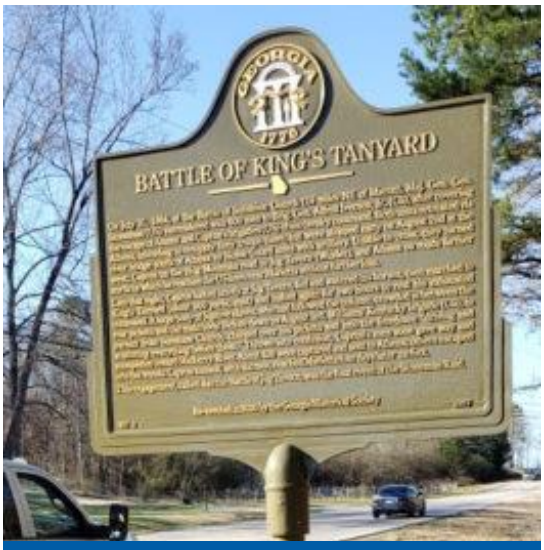
Major Health Concerns

- Heart disease
- Obesity
- Diabetes
- Mental Health
- Uninsured Rates

Explore the Area

Fort Yargo State Park located in the county seat of Winder, features a historic 1792 log fort built to protect early settlers.

The Barrow County Museum is located in the former county jail built in 1914. It still has original jail cells alongside local historical exhibits.



Barrow County was the site of the Battle of Kings Tanyard during Stoneman's Raid in 1865.

911 GA, DeKalb County

Founded: July 2020

Contact With

DeKalb County

PSAP

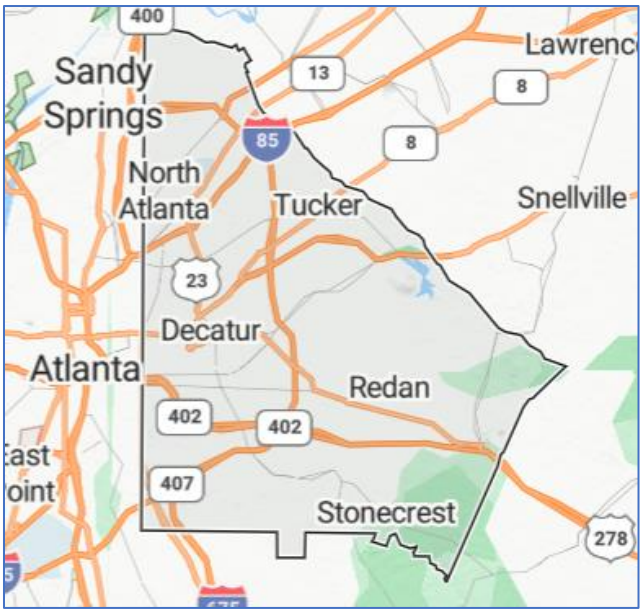
DeKalb County E-911

State

Georgia (GA) **Eastern Time**

Unique Scripting

There are no unique scripting requirements for this contract.



Communications

ALS	PSAP
BLS	PSAP
CAD-to-CAD	*Yes*
Field Referrals	*Yes (AMR + Fire)*

Test Your Knowledge *(pending)*

Cities, Towns, & Neighborhoods in DeKalb County

- Stonecrest
- Stone Mountain
- Tucker
- Decatur
- Dunwoody
- Brookhaven
- Lithonia

Health Priorities

Obesity	Heart disease	Opioid use	Diabetes

AMR Field Refusal

Field staff on scene must call 911 NN for clearance refusal for low acuity calls per their policy.

①

Interview the Patient

Interview the patient and attempt to navigate to an appropriate care resource.

②

Refusal Documentation

If the patient refuses, the nurse will document the refusal of care appropriately.

Exclusion Criteria

- Calls that meet exclusion criteria should not be cleared for refusal and the nurse should inform EMS to follow their process for high acuity calls.
    - The nurse should also compete the NN Feedback Form to report the incident

Explore the Area

Stone Mountain is the world's largest piece of exposed granite, standing 1,686 feet above sea level. Quartz mined from the site was used in the U.S. Capitol, the Panama Canal, Lincoln Memorial, and Fort Knox.

Home to the Center for Disease Control and Prevention (CDC), one of the leading national public health institutes in the United States.

The iconic 24-hour restaurant chain, Waffle House, was founded in DeKalb county in 1955.

911 MA, Baltimore

Founded: October 2025

Contact With

City of Baltimore

PSAP

Baltimore City 911 Center

State

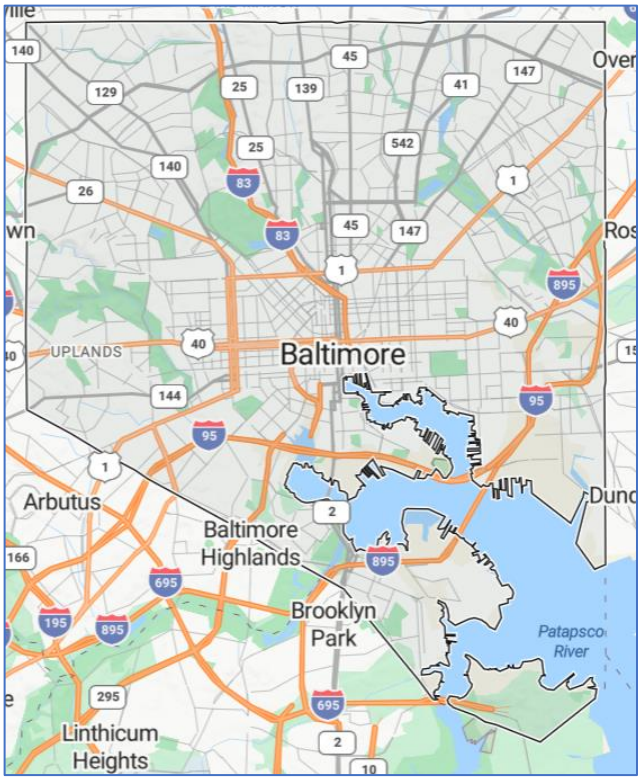
Maryland (MA) **Eastern Time**

Unique Scripting

There are no unique scripting requirements for this contract.

Communications

ALS	PSAP
BLS	PSAP
CAD-to-CAD	*No*
Field Referrals	*No*



Test Your Knowledge *(pending)*

Explore the Area

Baltimore is the most populous city in Maryland with a population of 568,271 according to 2020 census data.

Baltimore is the birthplace of “The Star-Spangled Banner” - inspired by the turning point in the War of 1812.

Baltimore is the birthplace of several notable figures; Babe Ruth, David Hasselhoff, Billie Holiday, and others.

Nicknames: Charm City, B’more, and Mobtown Baltimore’s name means ‘town of the big house’ in Irish.

Baltimore established the first post office system in what became the United States

911 MI, Kent County

Founded: September 2025

Contact With

Kent County

PSAP

Kent County Communications Center

State

Michigan (MI) **Eastern Time**

Unique Scripting

There are no unique scripting requirements for this contract.

Communications

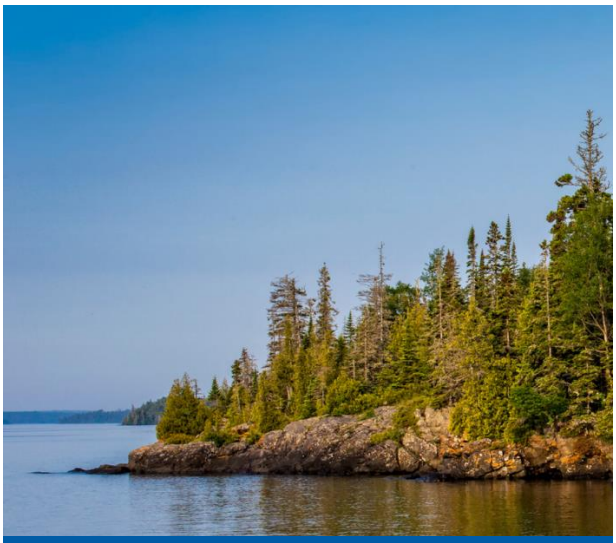
ALS	CAD-to-CAD
BLS	CAD-to-CAD
CAD-to-CAD	*Yes*
AMR Field	*No*
Fire Field	*No*

Test Your Knowledge *(pending)*

Cities in Kent County, MI

- Grand Rapids
- Wyoming
- Kentwood
- Plainfield Township
- Gaines Charter Township
- Forest Hills

Explore the Area



Michigan has more miles of freshwater shoreline than any other state in the nation.

The name, "Grand Rapids" comes from the Grand River, Michigan's largest inland river.

Grand Rapids

Grand Rapid is the birthplace of the 38 U.S. President, Gerald R. Ford.

Grand Rapids is nicknamed Furniture City as it was once a worldwide leader in the production of fine furniture. It remains a world leader in the production of office furniture.

"The Polar Express" author, Chris Van Allsburg, was born in Grand Rapids.

Grand Rapids was the first city in the U.S. to add fluoride to its drinking water - way back in 1945.

911 NC, Wake County

Founded: February 2022

Contact With

Wake County EMS, Cary 911 &

Raleigh-Wake 911

PSAP

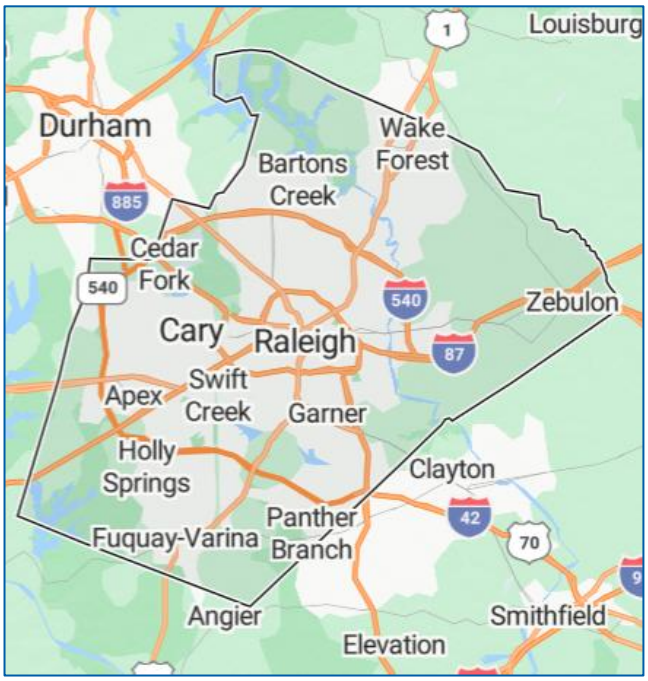
Wake County EMS

State

North Carolina (NC) Eastern Time

Unique Scripting

There are no unique scripting requirements for this contract.



Communications

ALS	PSAP
BLS	PSAP
CAD-to-CAD	*No*
Field Referrals	*Yes (field AMR)*

Test Your Knowledge *(pending)*

Cities / Towns in Wake County

- Raleigh (county seat and state capital since 1792)
- Wake
- Cary
- Garner
- Fuquay-Varina
- Holly Springs

More About Wake County

The county is named after Margaret Wake Tryon, wife of Governor William Tyron.

Wake County is in the rolling hills of the Piedmont called the "fall line" where waterfalls appear in rivers as land drops towards the coast.

Home of the Research Triangle Park (RTP), one of the largest industrial parks in the country and serves as a major hub for high-tech and biotech research.

911 NY, Erie County

Founded: December 2025

Contact With

AMR Erie County / Buffalo

PSAP

911 MERS Communications Center

State

New York (NY) Eastern Time

Unique Scripting

There are no unique scripting requirements for this contract.

Communications

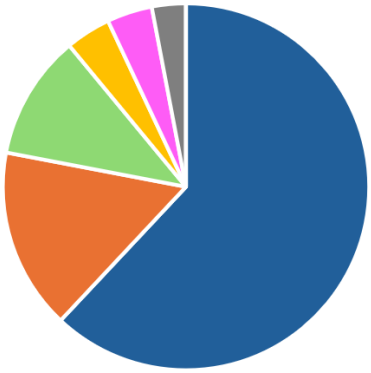
ALS	PSAP
BLS	PSAP
CAD-to-CAD	*No*
Field Referrals	*No*

Test Your Knowledge

(All NY Contracts)

Additional Information

BLS	62%
NN ER	16%
RSC	11%
REF CCR	4%
ALS	4%
VC	3%
VC EMS	0%



Total Calls / Month ≈ 270

More About Erie County

Erie County is home to Buffalo, the second largest city in the state after NYC. It is also the birthplace of buffalo wings!

The Eternal Flame is a natural phenomenon located in Erie County's Chestnut Ridge park. Despite the constant waterfall rushing over it, the flame is kept alive by natural methane gas emissions .

911 NY, Onondaga

Founded: May 2022

Contact With

County of Onondaga

PSAP

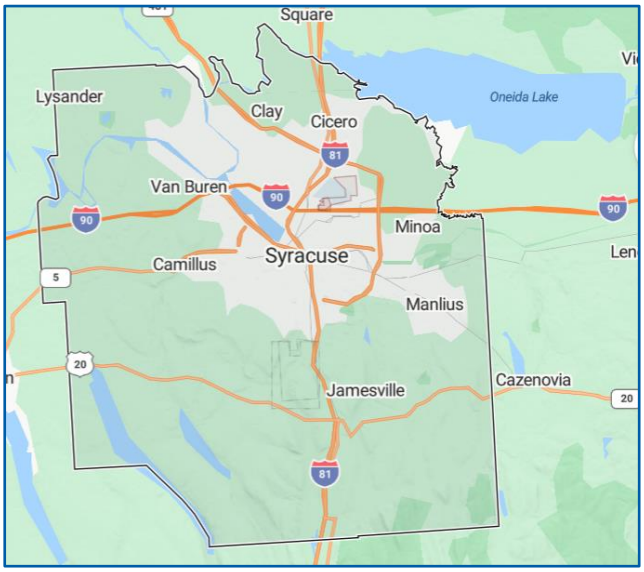
Onondaga County EMS

State

New York (NY) **Eastern Time**

Unique Scripting

There are no unique scripting requirements for this contract.



Communications

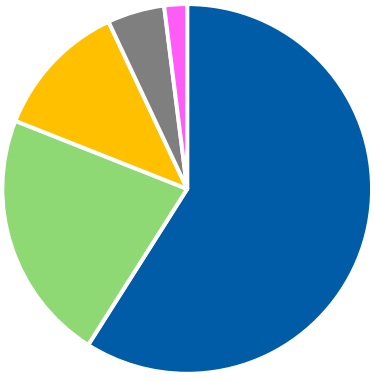
ALS	Local FDs
BLS	Local FDs
CAD-to-CAD	*Yes*
ALS / BLS Verification	*No*
Field Referrals	*Yes (Field AMR)*

Test Your Knowledge

(All NY Contracts)

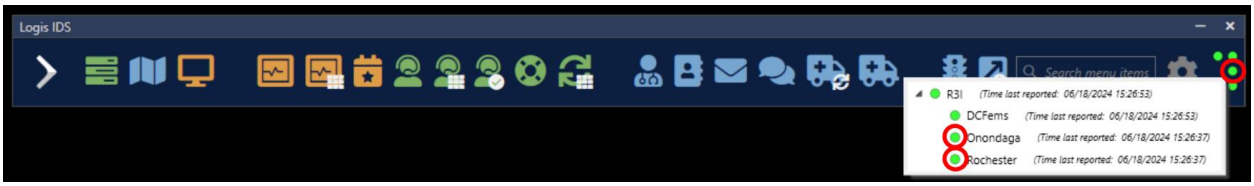
Call Data

BLS	56%
RSC	23%
REF	10%
VC	5%
ALS	3%
NN ER	3%



Total Calls / Month ≈ 150

CAD-to-CAD Information



- Onondaga uses Hexagon (same CAD as DC).
- No feedback / verification loop for incidents saved ALS / BLS
  - Assume it's working fine until we have evidence otherwise.
  - If any of the circled dots are orange or red, *the CADs aren't talking*!
    - Click the middle dot on the far right to bring up the grey box with details.
- If you get calls from Rochester or Onondaga asking for dispositions that show closed on our end, notify your Team Lead. May need to test CAD connection.
- Both ROC and ONO tend to share connectivity issues at the same time.
  - This can sometimes affect DC as well depending on nature of the issue.

Explore the Area

Oneida Lake borders Onondaga County to the northeast. It is the largest lake wholly within the state of New York.

The Onondaga, known as "People of the Hills", are "Keepers of the Central Fire". Their traditional territory serves as the capital and meeting place for the Grand Council of the Six Nations.

911 NY, Rochester

Founded: January 2022

Contact With

City of Rochester & County of Monroe

PSAP

Rochester / Monroe County 911

State

New York (NY) Eastern Time

Unique Scripting

There are no unique scripting requirements for this contract.

Communications

ALS	Local FDs
BLS	Local FDs & 3rd party providers
CAD-to-CAD	*Yes*
ALS / BLS Verification	*No*
Field Referrals	*Yes*
NN Ride ER	*Yes*

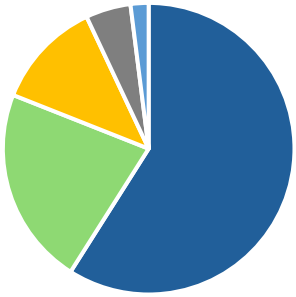
Test Your Knowledge

(All NY Contracts)

Additional Information

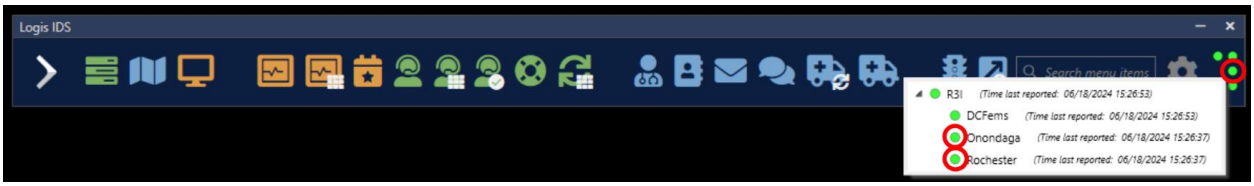
Call Data

BLS	59%
RSC	22%
REF	12%
VC	5%
ALS	2%



Total Calls / Month ≈ 200

CAD-to-CAD Information



- Rochester uses Hexagon (same CAD as DC).
- No feedback / verification loop for incidents saved ALS / BLS
  - Assume it's working fine until we have evidence otherwise.
  - If any of the circled dots are orange or red, *the CADs aren't talking*!
    - Click the middle dot on the far right to bring up the grey box with details.
- If you get calls from Rochester or Onondaga asking for dispositions that show closed on our end, notify your Team Lead. May need to test CAD connection.
- Both ROC and ONO tend to share connectivity issues at the same time.
  - This can sometimes affect DC as well depending on nature of the issue.

Explore the Area

Rochester is home to the Strong National Museum of Play, the only collections-based museum anywhere devoted solely to the study of play, and although it is a history museum, it has the interactive characteristics of a children's museum, making it the second largest museum of that type in the United States.

Jell-O, French's Mustard, Granula (the first breakfast cereal) all originated in the

Workflows

In This Section

This section explains different call types and how we process them. The call flow process is outlined in the Conversation Guide and includes required talking points and reminders for each step of the call. This section also includes resources for documentation, call handling, and the QA rubric.

Reference Topics

- Call Types
- Identifying the Patient's Complaint
- Exclusion List
- Leading with Empathy
- De-Escalation
- Conversation Guide
- Potential Call Barriers
- Frequently Asked Questions (FAQ)
- Dispositions
- Documentation Do's and Don'ts
- Follow-Up
- Quality Assurance
- Transportation Guide

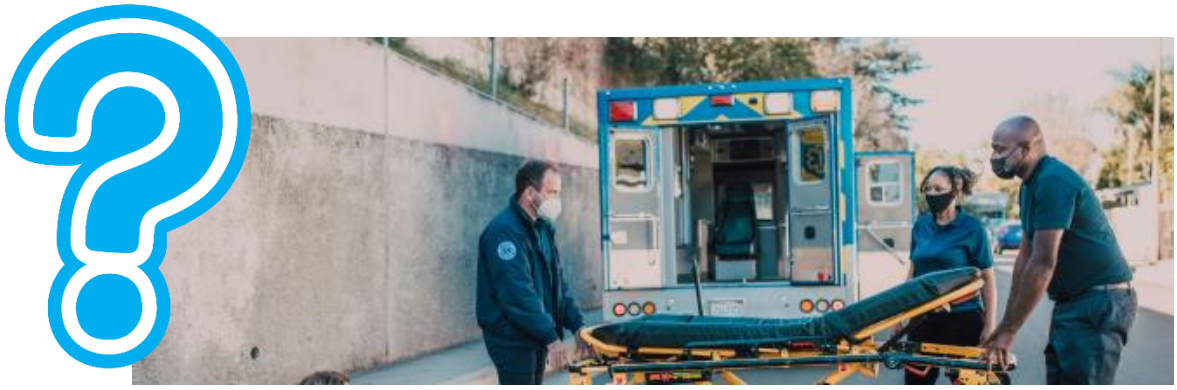
Call Types

911 Call	FLD (Field Fire)	AMR (Field AMR)
911 Call Reported from EMD Center	Referral from on-scene Fire Department or EMS staff	Referral from on-scene AMR Staff

911 Call

The quality of your interview with the patient will determine:

- If the caller needs an ambulance
  - If EMD needs to use lights / sirens if an ambulance is required
- Is the patient's condition life threatening
  - What care resources match the patient's complaint.



FLD (Field Fire)

- ALS / BLS response requested by 911 dispatch
- Paramedic / EMT on scene evaluated pt to be alternate destination likely
  - Vital signs
  - Medically trained observations
- In-person assessment can reassure that condition is not life-threatening
- Higher chance of successful referral recommendation
- May not have ambulance on scene if pt needs transport (fire truck)

AMR (Field AMR)

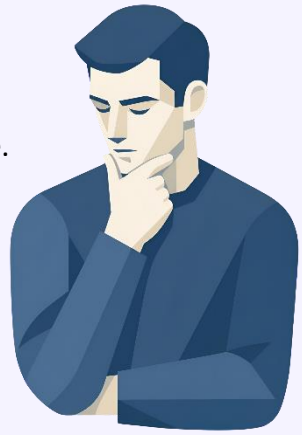
- BLS response initially requested by 911 dispatch
- Paramedic / EMT on scene evaluated patient to be alternate destination
  - Vital signs
  - Medically trained observations
- In-person assessment can reassure that condition is not life-threatening
- Higher chance of successful referral recommendation
- They are an ambulance on scene if pt needs transport

Identifying the Patient's Complaint

Patient Interview

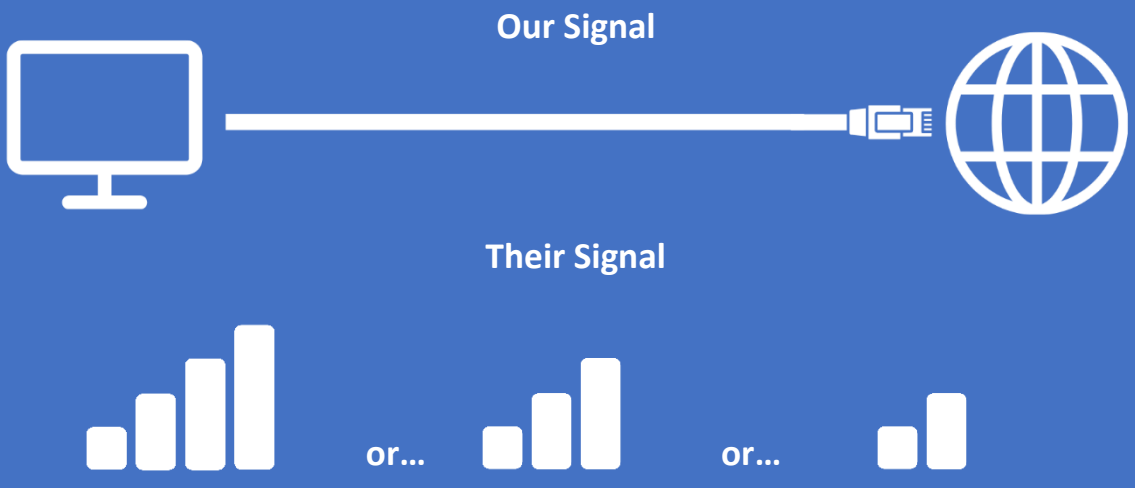
With telephonic interviewing, our diagnostic abilities are limited:

- We rarely have objective data from diagnostic tools.
- No direct exam / eyes on patient
- No physical exam



- We're asking objective questions about a subjective experience.
- Relies on self-reporting / non-medically trained observations
- Influenced heavily by caller/patient's state of mind
  - ➢ Anxiety, Fear, Pain, Anger

The patient's phone signal may effect the quality of the call…



How to Analyze Pain

PAIN is what the patient says it is because:

- It is the individual's experience
- There aren't definitive diagnostic tools to measure it.

This is the #1 rule of 911 Nurse Navigation.

The ***symptoms*** are what the patient says they are.

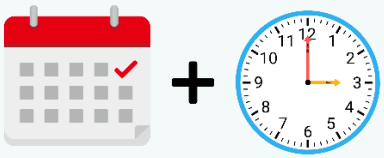
Questions

We use questions to probe where our eyes, fingers, and stethoscopes can't.

- **USE DISCRETION**; we don't have to take everything a patient says at face value...

Ask the patient to quantify their pain – 0-10

When did it start?



How does the patient describe it?

Area, type, characteristics, & persistence

How would the patient evaluate it?

What makes it better / worse?

Example: General Communication

Caller

"I have a complaint of…"

GMR Patient Interview Questions

- Have you had these symptoms before?
- Did you seek treatment?
- What did you/they do about it?
- Have you tried that this time?

Example: Immobilization

Caller

"I can't walk."

GMR Patient Interview Questions

- Are you incapable, or are you being limited by pain?
- Are you too weak, or because you can't move your legs?
- If you need to pee, can you make it to the bathroom?

Example: Immobilization

Caller

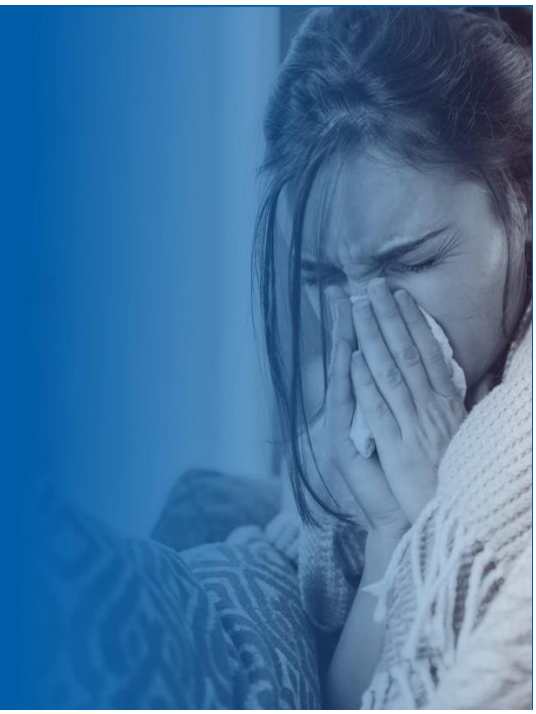
"My head, neck, back, and stomach all hurt!"

GMR Patient Interview Questions

- Which one started first?
- Which hurts the worst?
- Have you had these symptoms before?
- Did you seek treatment?
- What did you/they do about it?
- Have you tried that this time?

What comorbidities change the complaint?

- Headache with HTN?
- Numbness / tingling with diabetes?
- Nausea / vomiting with a skipped period?



Exclusion List

Overview

911 Nurse Navigation is designed to help match low acuity patients with appropriate care. Unfortunately, there are a handful of patient situations, even when low acuity, that 911 NN does not have the resources to help navigate. These situations have been compiled into an **Exclusion List**.

Why do we have an Exclusion List?

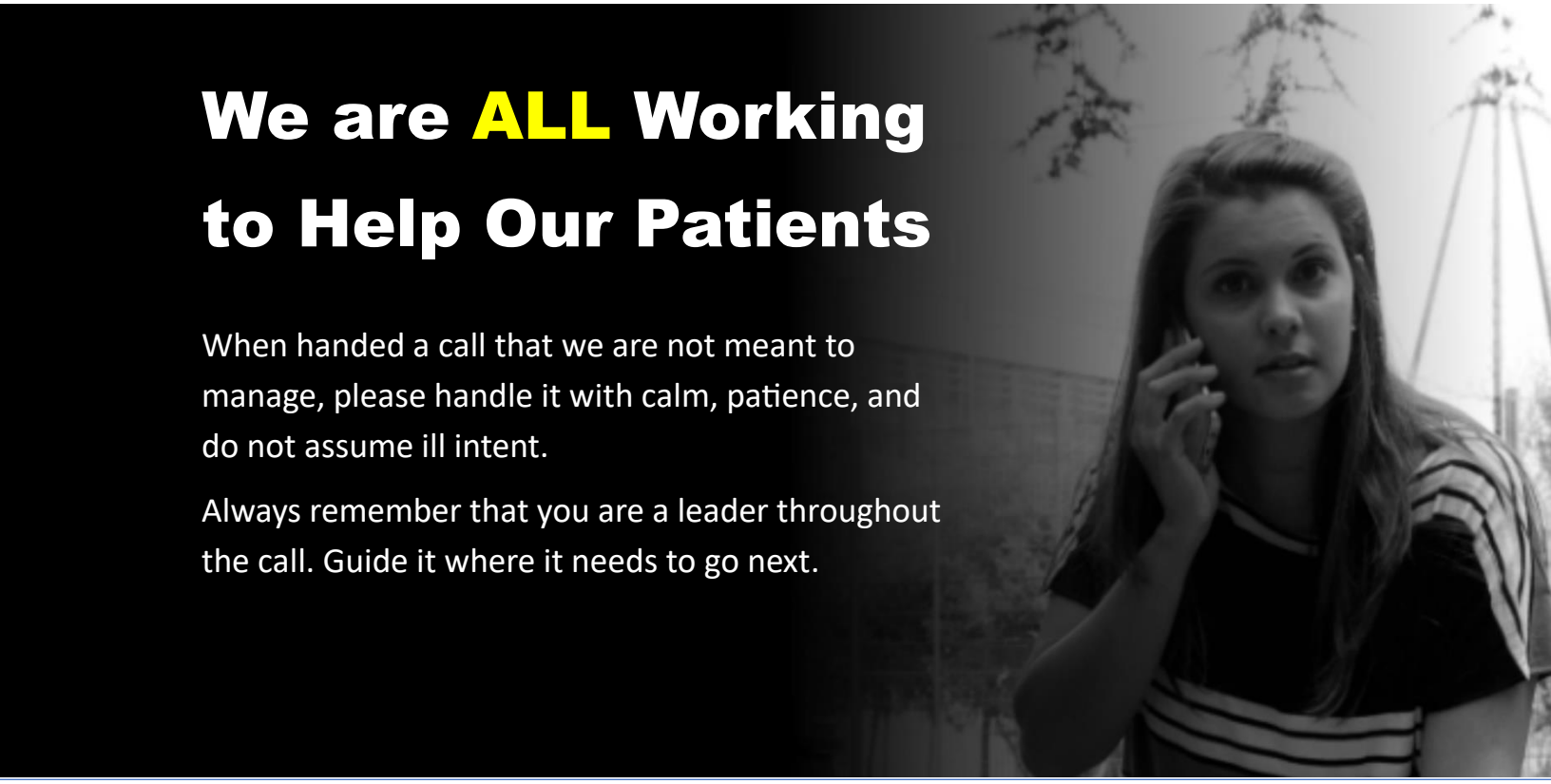
The Exclusion List is part of the Call Flow process that is mapped out during contract implementation. This foundation allows the contract to educate their call takers, and most of these calls are screened out before reaching 911 Nurse Navigation. Some exclusions will slip through because:

- The nurse is allowed to probe and get data that an EMD process may not reveal
- Mistakes happen.

We are ALL Working to Help Our Patients

When handed a call that we are not meant to manage, please handle it with calm, patience, and do not assume ill intent.

Always remember that you are a leader throughout the call. Guide it where it needs to go next.



Exclusion Scenarios

- **Life threatening illness or injury**
- **Incarcerated / Arrested / Detained by Law Enforcement**
- **Impaired or unable to communicate due to drugs or alcohol**
- **Pregnancy for a duration longer than 20 weeks**
- **Homicidal or Violent Intent**
- **Nursing Home / SNF Patients**
- **Third-Party Callers**
- **If the patient is directed to an ED by a clinical provider**
- **If the patient is under the care of a medical professional at a facility**
- **If the patient was directed to an ED by their residing facility's protocol**

Exclusion Workflows

During Handoff

Scripting

Tell the dispatcher:

"Unfortunately, I cannot accept this call as this patient meets [exclusion criteria]."

- Be sure to cite the specific exclusion criteria that has been met.

If the dispatcher asks you what to do with the call and immediate life threat is not suspected, tell them to send the response they would have sent if NN was unavailable.

- *If you suspect life-threatening illness or injury, request ALS.*

Documentation

Warm Transfer	Do not document the call.
CAD-to-CAD	Cancel the call with reason, "Exclusion Criteria"
Tracking	fill out the Feedback Form with the contract, date, and time of the call for tracking purposes. • We report repeated behaviors to the PSAPs for individual or systemic correction.

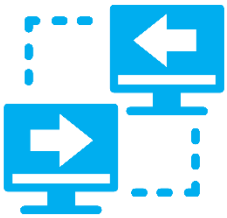
After Handoff

Scripting

Often, nurses will identify exclusion criteria during their assessment/triage. When this happens, please tell the patient:

"It looks like we don't have any resources that match your current situation. I'm going to contact dispatch and ask them to send you an ambulance."

- Also provide your normal ambulance instructions.



Next Steps

1. Warm Transfer to dispatch and tell them you were unable to finish assessing because the patient met exclusion criteria.
   - ➢ Specify the exclusion criteria.
   - ➢ If life-threat is suspected, request ALS for your patient.

Documentation

Warm Transfer	Finish documenting the call with the knowledge you acquired prior to identifying the exclusion criteria and save as the type of ambulance requested.
CAD-to-CAD	• Put the exclusion criteria met in the Comments. • Choose an appropriate ambulance type for the patient (ALS/BLS) and save. • RSC should only be chosen if the patient states they do not want an ambulance.
Tracking	fill out the Feedback Form with the contract, date, and time of the call for tracking purposes. • We report repeated behaviors to the PSAPs for individual or systemic correction.

The Seattle Exception

Guidelines

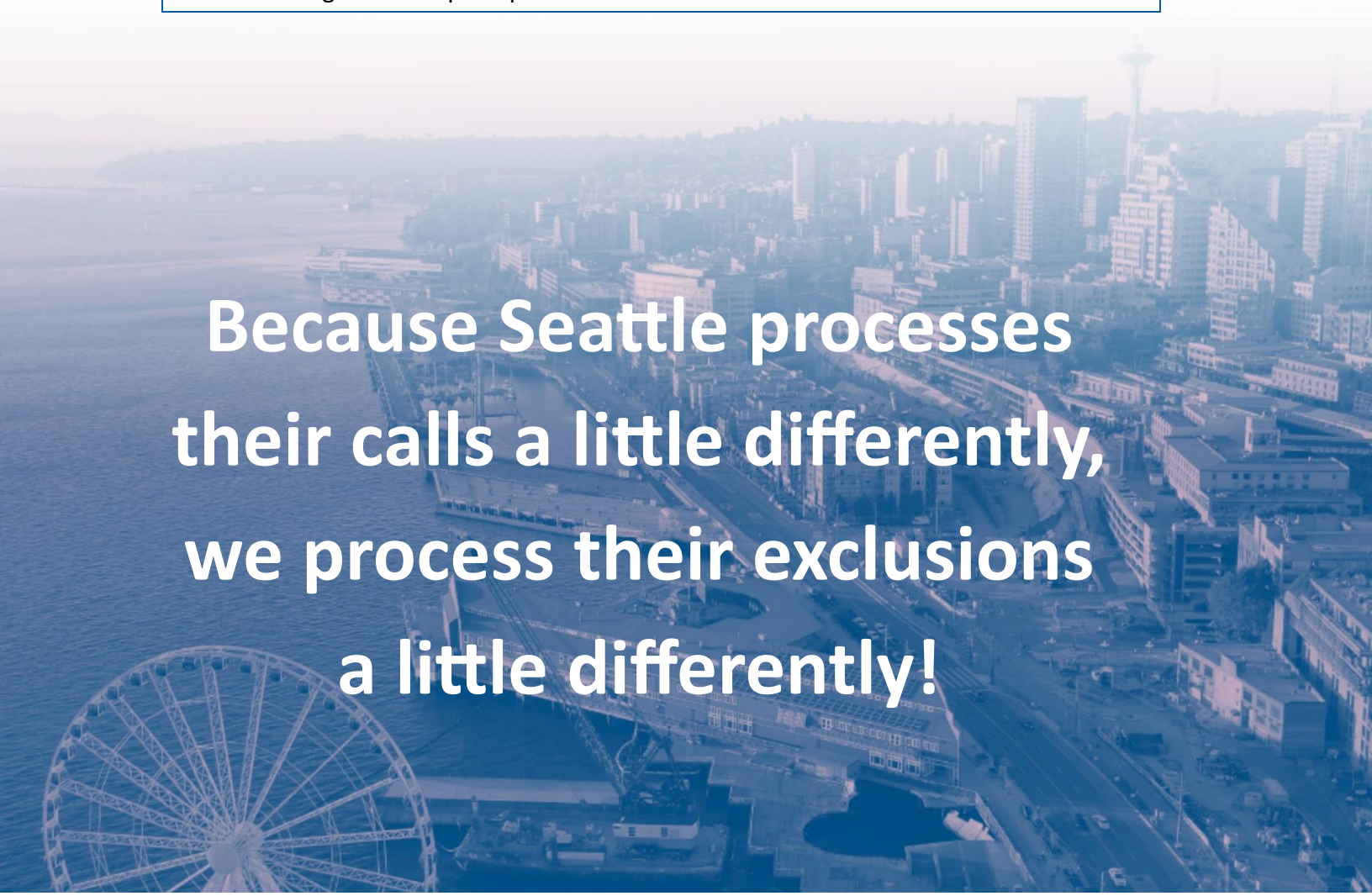
If your Seattle patient's exclusion is life-threatening with the need for an ALS respons

Please request an ambulance through Seattle Fire.

- Do this during or after handoff

For *all other exclusions in Seattle*

1. NN will accept the patient
2. Verify demographics and the basic patient situation
3. Contact Seattle AMR to request an ambulance for their exclusion
4. Triage not Complete patient**.**

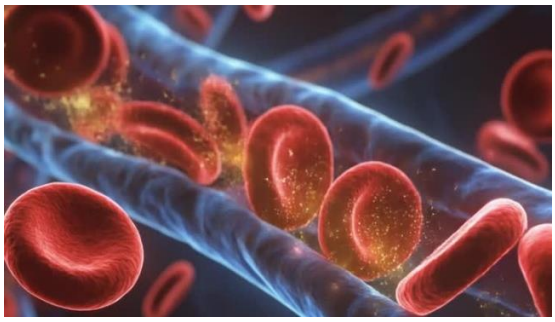


Exclusion Criteria Examples & Workflows

Example 1: Life-Threatening Indicators

Exclusion Identified

The patient's hemodynamic status is suspected to be compromised, and the patient's life could be in danger without quick, on-scene assessment and intervention.



How Do We Know?

Terms like "Shortness of Breath" and "Chest Pain" rightfully cause concern for most clinicians.

They also have a wide range of subjectivity. Follow the ABCs and be vigilant when clinical data points to life-threating symptoms.

Use Your Clinical Judgement!

Gather Additional Data When Needed

- Unconscious or sudden, unexplained change in neuro status
- Unable to complete sentences due to respiratory distress or audible wheezing
- Hemorrhage: uncontrolled and/or uncontrollable bleeding. "Spurting"
- Chest Pain / MI: may radiate to arms, jaw, abdomen, present with feelings of anxiety / dread, feeling of something heavy on chest, diaphoresis, dizziness, or weakness.
- Stroke: B.E.F.A.S.T. – Balance Loss, Eyes (Vision changes), Facial Droop, Arm Weakness, Speech Difficulty, Time to Call 911

Why Exclude?

Time to care is of the essence for these patients and non-EMS resources (clinics, virtual care, rideshare drivers, etc.) aren't equipped to handle medical emergencies.

Example 2: Incarceration

Exclusion Identified

The patient is currently in the custody of law enforcement personnel. This could be someone that's been arrested and is handcuffed on the sidewalk, or it could be a call from inside a jail or detention center.

This One Is Straight Forward

How Do We Know?

Typically, the officer or staff member of the correctional facility will identify themselves and the situation. If there's any doubt, ask the officer / nurse if the patient is currently in custody or under arrest.

Why Exclude?

Law enforcement has strict protocols for handling medical needs for those in their care, and CCRs are ill-equipped to handle the extra oversight and security considerations needed for patients in custody. Rideshare is *not* an option!

Example 3: Drugs or Alcohol Impairment

Exclusion Identified

The key here is ***Impairment***.

- The patient shows signs / symptoms of diminished LOC.

How Do We Know?

Don't focus on the substance consumed or the amount taken:

- A 120 lb patient that never drinks and a 250 lb alcoholic get impaired at *very* different quantities
  - ➢ Focus on clinical signs/symptoms
- **Slurred or rapid speech**
- **Disorderly thought processes**
- **Slow to react to questions**
- **Reports of uncoordinated balance/movement**
- **Ask Standard LOC questions**
  - ➢ Day of week, time of day, location, President, et.

Assess To Determine Impairment!

Why Exclude?

If a patient is suspected to be impaired, their ability to consent to and follow-up on treatment is compromised.

- Additionally, these patients are best served by specialized resources and/or treatments that are not available at standard walk-in clinics.

Example 4: Pregnant > 20 weeks

A Straight-Forward Scenario

Exclusion Identified

As soon as you find out a patient is pregnant, verify how far along they are.

How Do We Know?

As soon as you find out a patient is pregnant, verify how far along they are.

Why Exclude?

Almost any complaint from a patient >20 weeks pregnant will also include a wellness check on the baby. This generally requires specialty resources only available at Obstetric clinics and hospitals.

Example 5: Homicidal or Violent intent

Use Clinical Judgement

Exclusion Identified

The patient has taken actions, or is threatening / considering taking actions, that will harm themselves or others.

How Do We Know?

Triggering this exclusion requires more than ideation.

- Consider if the patient has a plan, the means, and intent to commit harm.
- Are the thoughts of doing harm fleeting or consistent?
- Passive or active?
- Controllable or overwhelming?
- Are voices or other hallucinations involved?

Why Exclude?

Someone who is actively a threat to themselves or others need resources to ensure scene, patient, and first responder safety. Community Care Resources (CCRs) that don't specialize in Behavioral/Mental Health (BMH), and all known rideshare services, do not have these capabilities.

Example 6: Nursing Home/SNF Patients

Identify The Facility

Exclusion Identified

Patients in a full-time care facility with medical direction.

How Do We Know?

If the caller / patient doesn't identify the facility as a Nursing Home or Skilled Nursing Facility (SNF), you can Google the facility and see from there**.**

- **This exclusion specifically targets full-time care facilities with on-site medical staff.**
- **This exclusion does not apply to Assisted Living.**
  - Assisted Living homes are not full-time care facilities, and residents can typically come and go as they please.

Why Exclude?

Residents of these facilities are under the care of medically trained staff with medical direction. When they call for an ambulance, it is typically via physician's direction – either directly or by standing order/protocol.

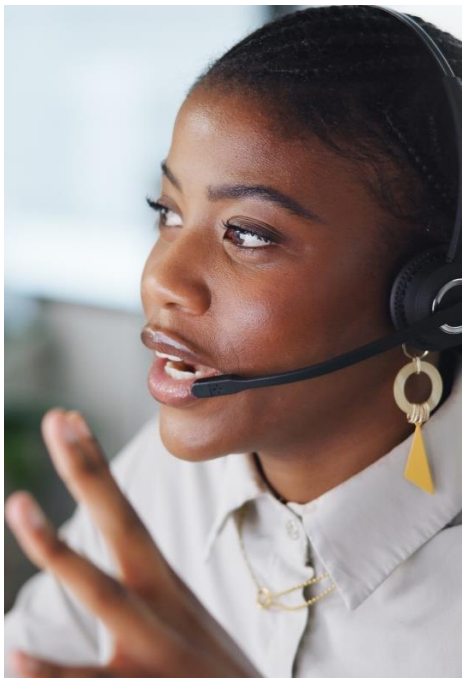
Example 7: Third Party Callers

Exclusion Identified

Technically speaking, a third-party call is when an incoming call comes from someone who is not on-site with the patient.

- Typically, these are via alarm companies (like LifeAlert) or family members who called 911 on a relative's behalf after a phone conversation with the patient.

Internally, we also use this designation for calls where the patient is unable to answer our questions and the caller (typically a bystander, staff member, or friend) is unable to provide meaningful data on the patient or their condition.



How Do We Know?

If the patient and the caller are unable to supply information about the patient and their current condition, we won't be able to adequately triage and obtain an accurate NMTARA level for safe patient care.

Always Ask to Speak to the Patient!

Why Exclude?

Our clinical pathways require a willing and knowledgeable participant to give answers to specific questions. Without someone on-site who can assist us to obtain up-to-date answers, our NMTARA accuracy and patient safety may be compromised.

Example 8: Physician Directive

Doctor's Orders

Exclusion Identified

Some patients or care givers have already spoken to a physician prior to calling 911 – either via phone, by being under their direct care, or by facility protocol.

How Do We Know?

Generally, the patient or caregiver will tell you they've already called the doctor and have been advised to go to the hospital.

If the patient's symptoms aren't life-threatening, and if the patient agrees to it, we *can* use our clinical pathways to find out if we can safely transport the patient via Rideshare to save them from an expensive ambulance bill.

Why Exclude?

When patients have consulted providers who instructed them to report to an ED, or when a caretaker is following standing protocols to escalate care through 911, Nurse Navigation is unlikely to provide an agreeable solution outside of that guidance. As nurses, we help enact physicians' directives when it is possible within our scope of practice.

Leading with Empathy

The Cornerstones of Empathy

Active Listening

Listen to understand, not respond. Think about how they are feeling as they describe their situation. For example “that must be frustrating” or “That sounds painful” to acknowledge the caller’s concerns.

Avoid Assumptions

Assumptions lead to unconscious bias which can inhibit equitable and compassionate care. Approach each patient as an individual with unique experiences, beliefs, and concerns.

Offer Comfort

While we cannot hold our patient’s hand, we can still tell them that we are sorry they aren’t feeling well and they we are committed to getting them care quickly.

Avoid Burnout

Take care of your own well-being. Maintaining a self-care routine can motivate you to provide quality patient care. Nurses need to be emotionally and mentally available to advocate for their patients

How Empathy Benefits our Patients

Reduce Patient Distress	Higher Patient Satisfaction	Prevents Emotional Harm	Increased Efficiency
Patients who are anxious or scared believe they are having an emergency. Recognizing this and responding with empathy can greatly reduce their distress and anxiety.	Empathy leads to more positive patient experiences. Patients report higher satisfaction scores when they feel that someone cares.	Empathy helps validate emotions, prevent misunderstandings, and reduce feelings of isolation.	Nurses are more likely to successfully navigate patients to a clinic or other alternate destination

Empathy Phrases

Acknowledgment	Encouragement	Validation
"I can see that this is really distressing for you right now; I'm here to help."	"You're doing great by reaching out; it's a big step to ask for help."	"Your feelings are valid, and it's important that we address your concerns."
"It sounds like you're going through a tough time, and it's completely understandable to feel this way."	"You're taking the right action by calling us; we'll figure this out together."	"It's completely normal to feel anxious in situations like this; let's see how we can help."
"I can see that this is really distressing for you right now; I'm here to help. How long have you been feeling this way?"	"I understand that this situation is very frustrating, and it's okay to feel overwhelmed. Let's talk about what's been happening and see how we can assist you."	"That sounds really difficult; I understand how concerning this must be for you. Can you tell me more about what you're experiencing?"

Clarifying Intentions	Reassurance	Supportive Promises
"My goal is to ensure you get the best care possible; let's talk about what's happening."	"I'm here to listen and support you. We'll work through this together."	"I will do my best to assist you and get you the help you need."
"I want to make sure we understand your situation fully, so we can provide the right support."	"You're not alone in this; I'm here to help you get the care you need."	"Let's take this step by step; I'm here with you throughout this process."
"Right now, you are my priority. Take the time you need."	"You did the right thing in calling us."	"I'm going to talk you through this, but please let me know if you have any questions along the way.

***REMEMBER**: Your patient may be perceiving a life-threatening emergency, so they are not having a good day. Your job is to assess the situation and provide reassurance and empathy while navigating them to an appropriate care destination.*

Your patient is more likely to agree to navigation if they feel heard and cared for.

Why does it matter to de-escalate?

- Callers who are in crisis cannot think clearly and make decisions even when a nurse is providing a reasonable solution to their problem.
- Prevents unnecessary emergency utilization: Proper education and assessment can avoid overuse of 911 and ER resources.
- The patient's expectations are based on their limited knowledge and understanding of the program.

Core Concepts

Empathy

Active Listening

Validation

Clear Communication

Set Expectations

Prioritize Safety

Remain Calm

- Be calm and composed, this isn't about you.
- Use a softer, reassuring tone, avoid raising your voice.
- Avoid arguing or dismissive concerns, promote a safe space.
- Ask open ended questions with slow, deliberate speech.
- Breathe deeply to maintain your own composure.
- Maintain a professional, compassionate demeanor regardless of caller behavior.

Validate what the Caller is Saying

- Show genuine interest and focus on the patient's words.
- Maintain a respectful attitude.
- Acknowledge feelings without judgment.
- If you actively listen, you can demonstrate they are understood.
- Listen with the intention of understanding instead of arguing.
- Repeat back what the patient says in a different way.

Get the Caller to Participate

Speak to the patient as you would a good friend or family member.

"Let's take a moment to breathe together."

"Let's focus on one step at a time."

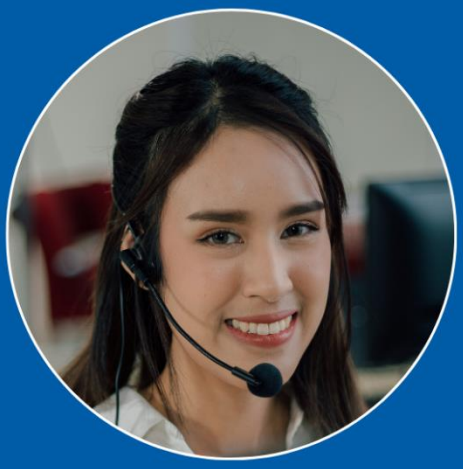
"Can you tell me what you see around you? What's something you can focus on right now?"

"Can you tell me what you see around you? What's something you can focus on right now?"

Set & Manage Expectations

Transparency is key:

- This includes the scope of 911 Nurse Navigation.
  - ➢ Explain the purpose of 911 Nurse Navigation and what the caller can expect.
- Inform them about available resources and support systems.
- Involve the patient in developing a plan, confirm they are on board.
- Provide education surrounding the plan.
- Allow patient to write call back number down.



“Would you like me to go over anything we’ve talked about so far?”

Conversation Guide

This conversation guide training reconceptualizes the call flow of the 911 Nurse Navigator by chunking the call into segments for easy recall. While there are talking point requirements, this conversation guide is not meant to be a script, but a reminder of what to say and when.

Call Overview

Reference Quick Links

		Info & Suggested Scripting	Barriers
1	About Call Handoffs	View	View
2	Greeting the Caller	View	View
3	Verify Patient Details	View	View
4	Clinical Discovery	View	View
5	Clinical Pathway	View	View
6	Care Suggestions	View	View
7	Plan of Care	View	View
8	Arrange Care	View	
9	Partner Contact	View	

For post-call activities, please see the Frequently Asked Questions (FAQ), Dispositions and Documentation Do's & Don'ts sections of this reference guide.

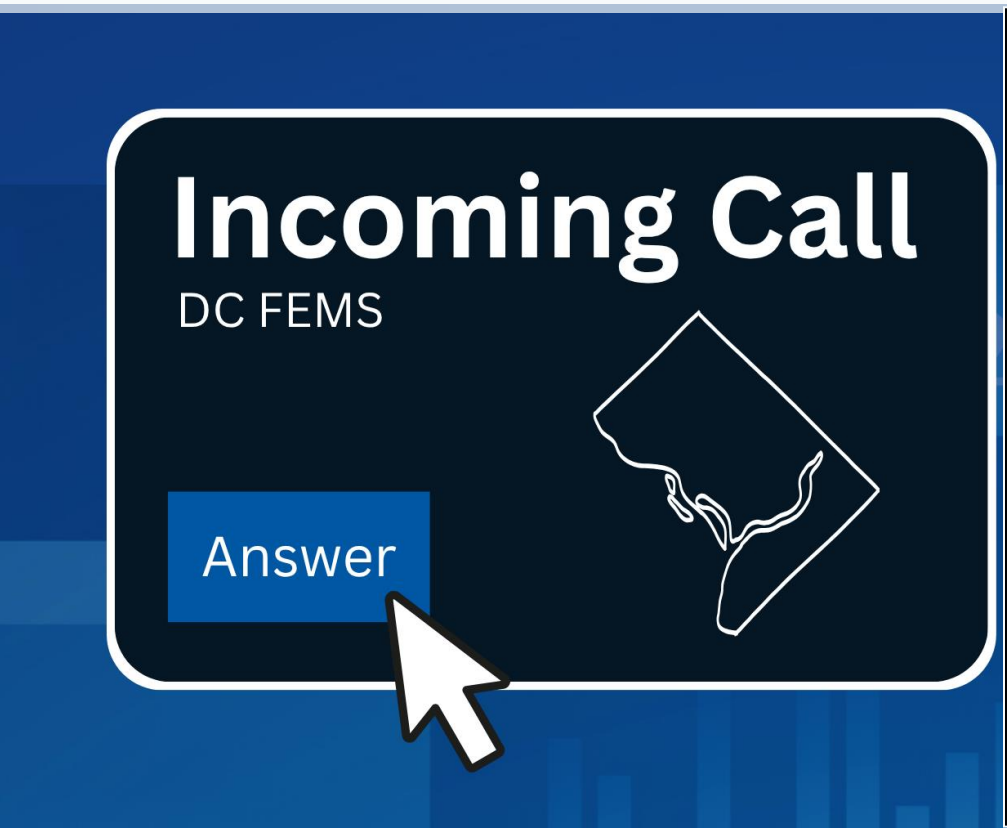
About Call Handoffs

Handoff of the patient from the 911 system (911 operator or Field Staff) to 911 Nurse Navigation. Assume the patient is on the line (or on speaker phone), listening to the interaction. This handoff should be short, professional, and instill confidence in the patient and 911 partner that they are in good hands.

- Listen for the whisper
  - ➢ It tells you where the call is from and if the contract has CAD-to-CAD
  - ➢ Reminds you to use the appropriate greeting for the contract.

Call Handoff Types

911 Dispatcher	CAD – CAD	Field
1. Incident address 2. Phone number 3. Brief situation *Accept calls without excessive delay*	1. Incident address	1. Incident address 2. Brief situation 3. Patient vital signs *FLD=Field Fire* *AMR=Field AMR*



Call Greetings should match our contracts and include the nurse's identification.

Call Handoff Information Exchange

Push

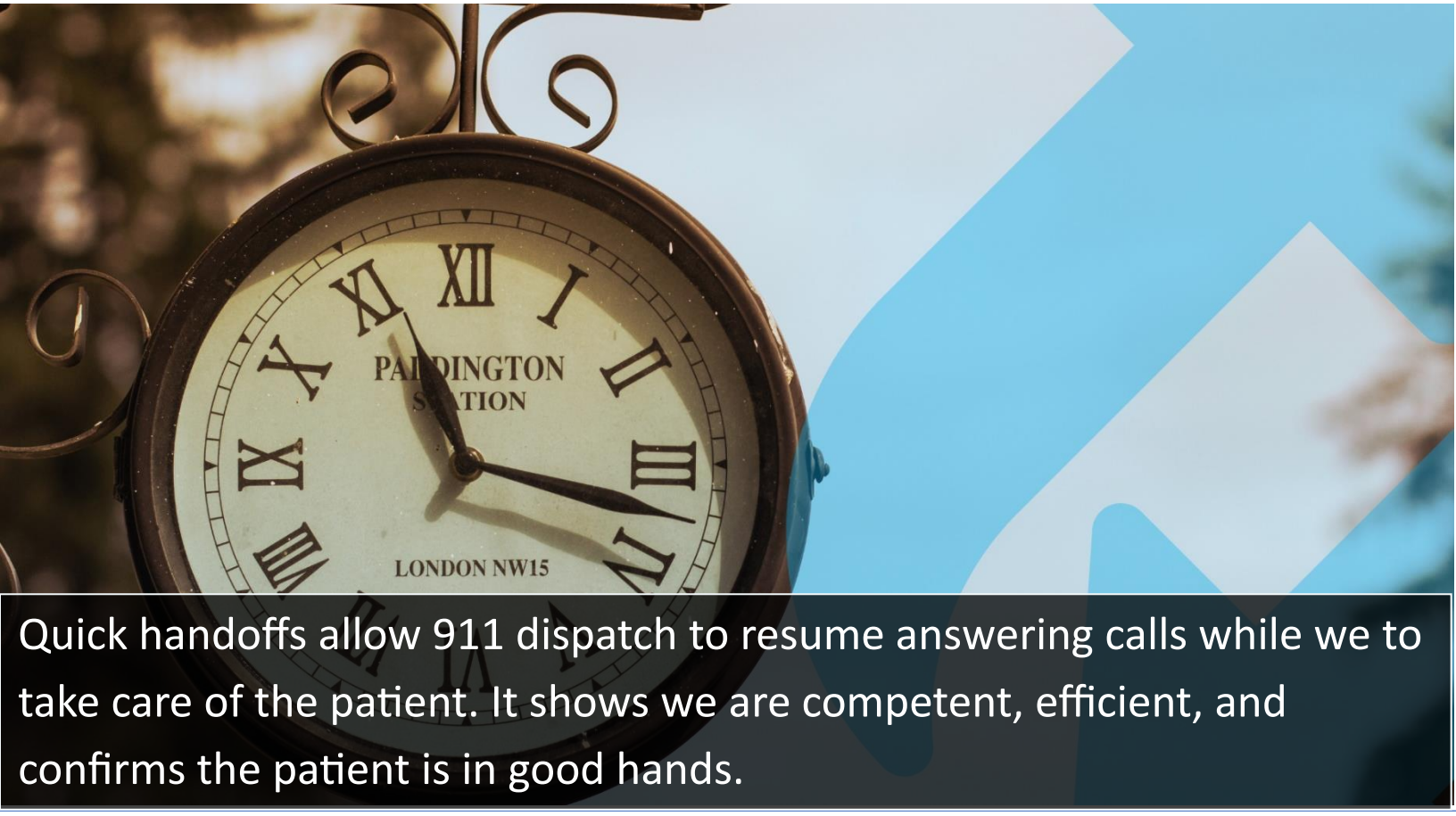
Info given to 911 Nurse Navigation by dispatcher / EMS

- Should include the “expected” info for call type
- May be more than “expected” info – reference number, patient name, etc.
- Document reference numbers when given

Pull

Info requested by 911 Nurse Navigation from dispatcher / EMS

- Only pull “expected data” when it is not being offered by dispatcher / EMS
- Do not pull info outside of the “expected data”
- It is okay to clarify any pushed data

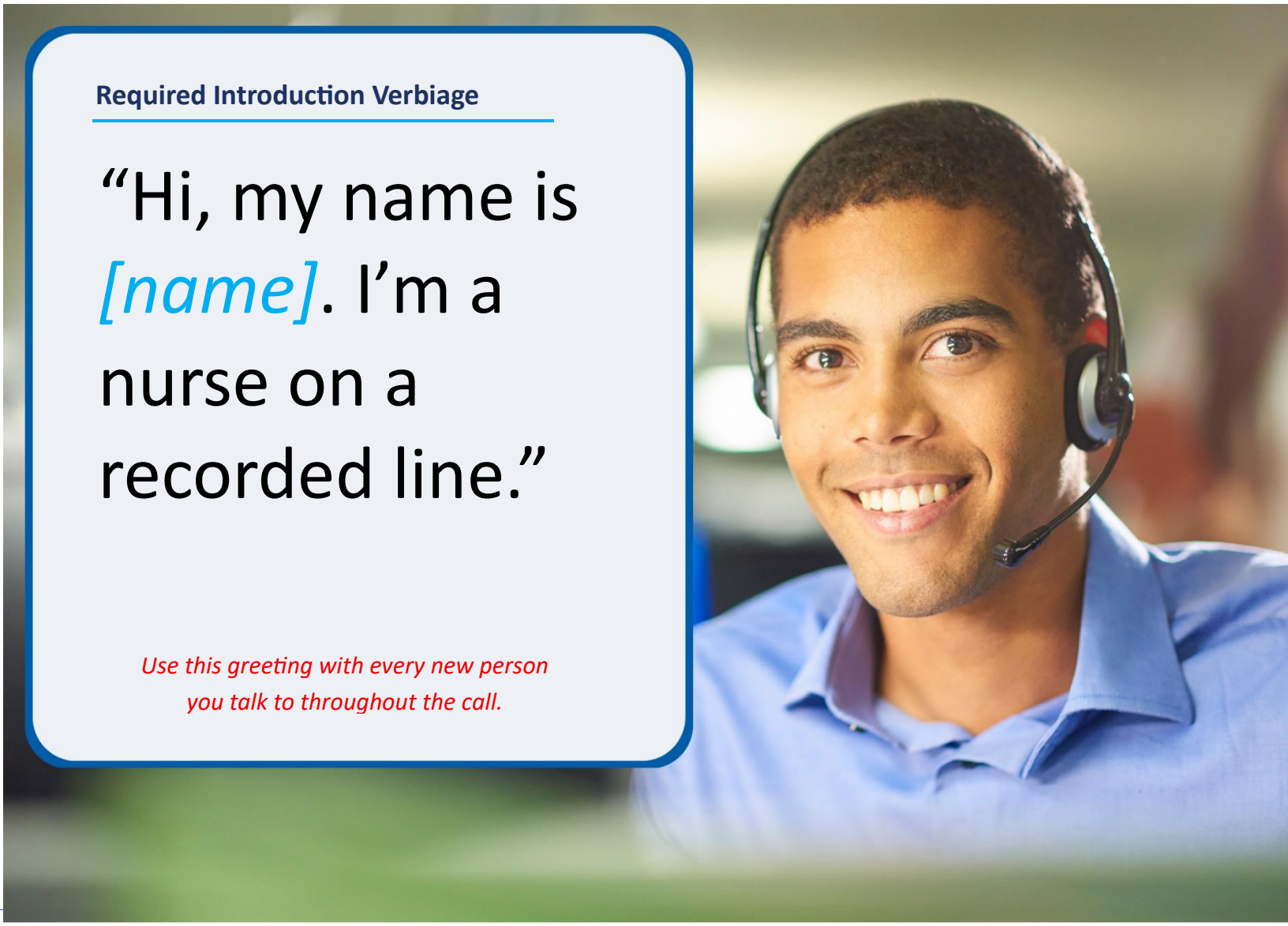


Quick handoffs allow 911 dispatch to resume answering calls while we to take care of the patient. It shows we are competent, efficient, and confirms the patient is in good hands.

Greeting the Caller

This greeting explains why the caller/patient is talking to a nurse. Include required talking points but make it your own. This statement conveys empathy and explains to the patient why they are speaking with a nurse.

This reduces confusion, increases understanding and buy in, and allows the patient an opportunity to opt out.



Required Introduction Verbiage

"Hi, my name is *[name]*. I'm a nurse on a recorded line."

Use this greeting with every new person you talk to throughout the call.

DO

Set expectations with the required talking points:

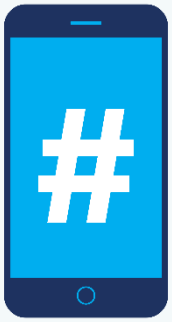
- ✓ Reintroduce yourself
- ✓ Recorded call disclosure
- ✓ Contract Name
- ✓ No ambulance is on the way at this time
- ✓ Explain why they are speaking with a nurse

DON'T

- × Say "You don't qualify for an ambulance."
- × Say "It's going to be a long wait"
  (or specify several hours of wait time)
- × "EMS is backed up or no ambulances available in your area"
- × "This is not an emergency"

Verify Patient Details

Verify the patients identify by asking 3 data points:

First and Last Name	Date of Birth	Phone Number
		

Confirm the spelling of the patient's name!

Example: Patient Verification Verbiage

"For your security, please verify your full name, date of birth, and phone number. Is that the traditional spelling?"

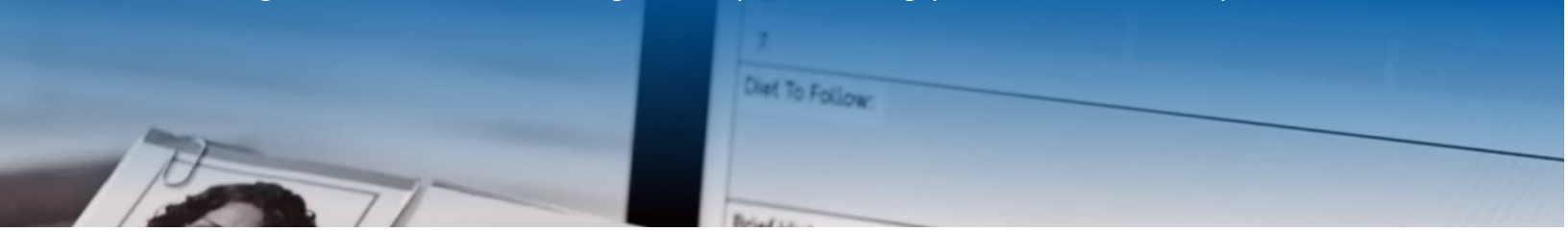
Other Data Points

Age & Gender

Care recommendations, diagnosis, and patient education are more precise

Insurance Type

Should be collected for all patients to match with the best care options, allow 911 Nurse Navigation to trend resource usage, and explore service gaps with our contracts / partners.

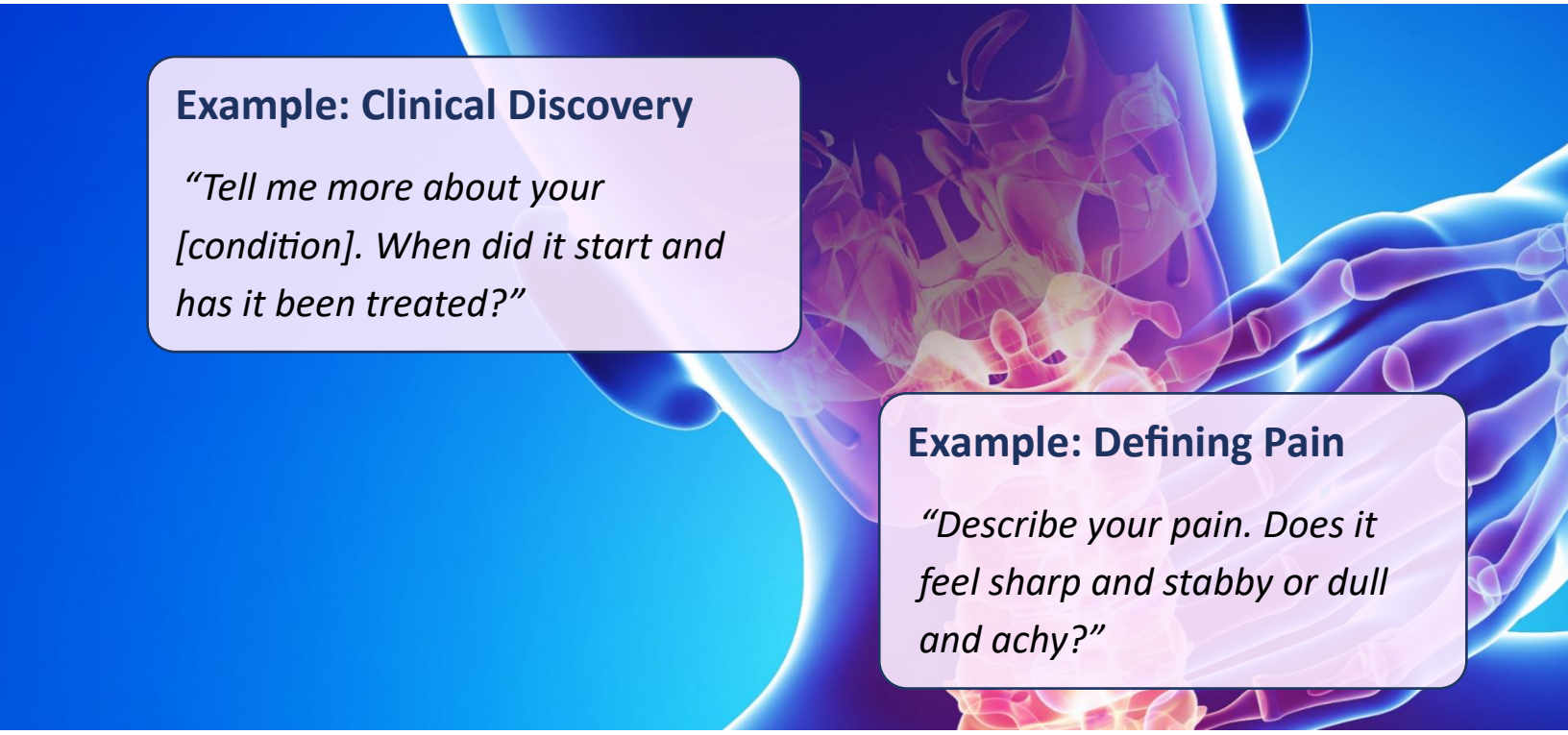


Clinical Discovery

Ask probing questions to understand their chief complaint, allowing you to match the patient with the most appropriate protocol, resulting in a more accurate NMTARA level.

- Your goal is to gather enough relevant information to be confident in the pathway.
- Primary source-data from the patient is priority.
  - Always ask to speak with the patient unless inappropriate (i.e. age, developmental level, aphasic, etc).

Use **OLDCARTS** or another mnemonic to guide your probing:	
O	Onset
L	Location
D	Duration
C	Characteristics
A	Aggravating / Alleviating factors
R	Radiation
T	Timing
S	Severity



Example: Clinical Discovery

"Tell me more about your [condition]. When did it start and has it been treated?"

Example: Defining Pain

"Describe your pain. Does it feel sharp and stabby or dull and achy?"

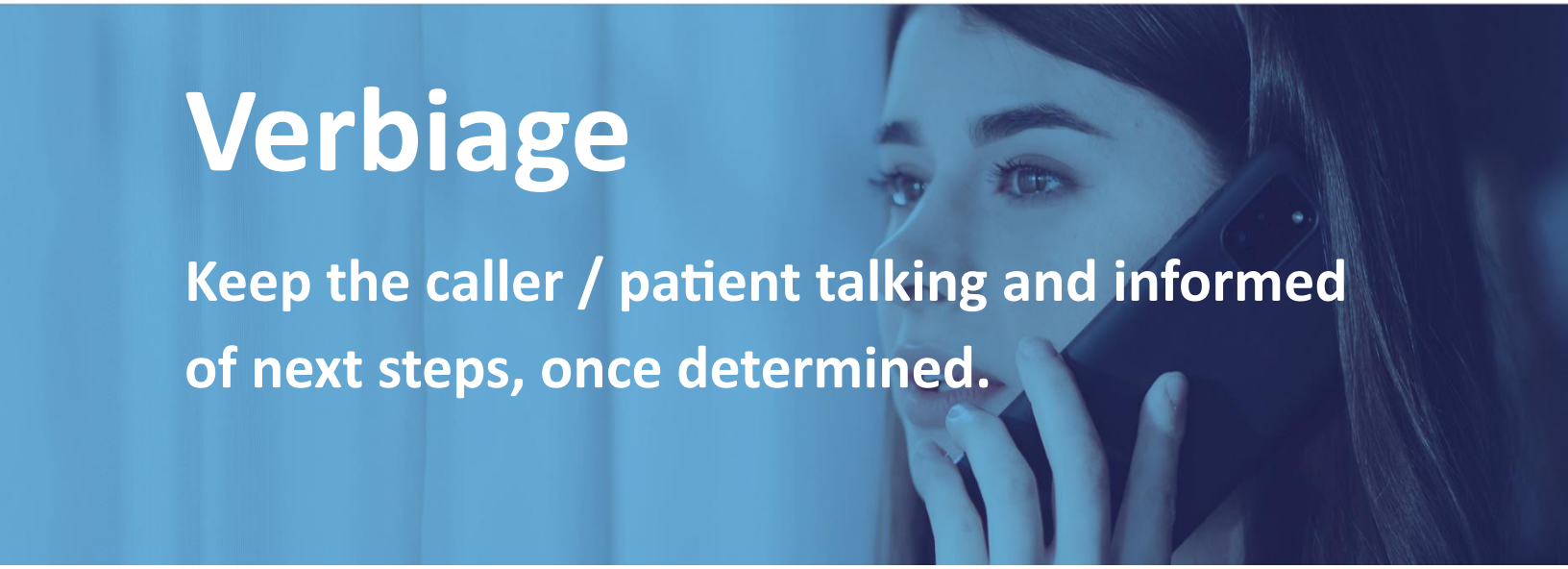
Assess patient responses using your best judgement and immediately escalate to an ALS response if needed.

Clinical Pathway

Follow the clinical pathway to completion. These consist of MD-approved questions.

Clinical protocols are NOT optional. Think of them as your failsafe.

- Ask all questions to ensure you reach an accurate ***NMTARA*** level.
  - Nurses can always escalate care with a reasonable concern, but ***NMTARA*** can't be downgraded.
- If you believe the NMTARA level is falsely high, report it so we can track and work on system solutions.



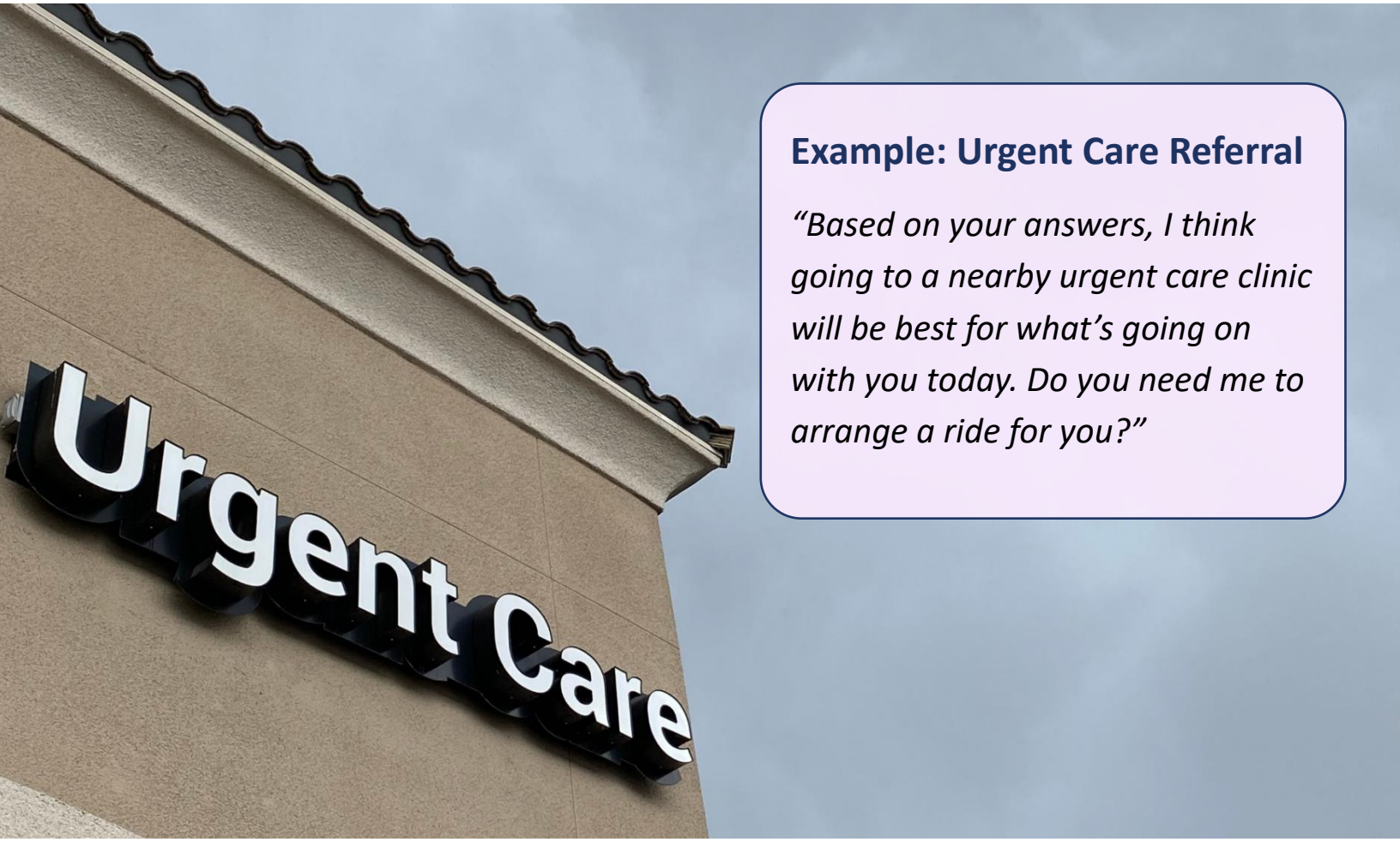
Clinical protocols are followed to the end; All questions asked or confirmed. All steps completed.

Care Suggestions

Consider the patient's circumstances and resource availability. A hierarchy of choices will be presented to you.

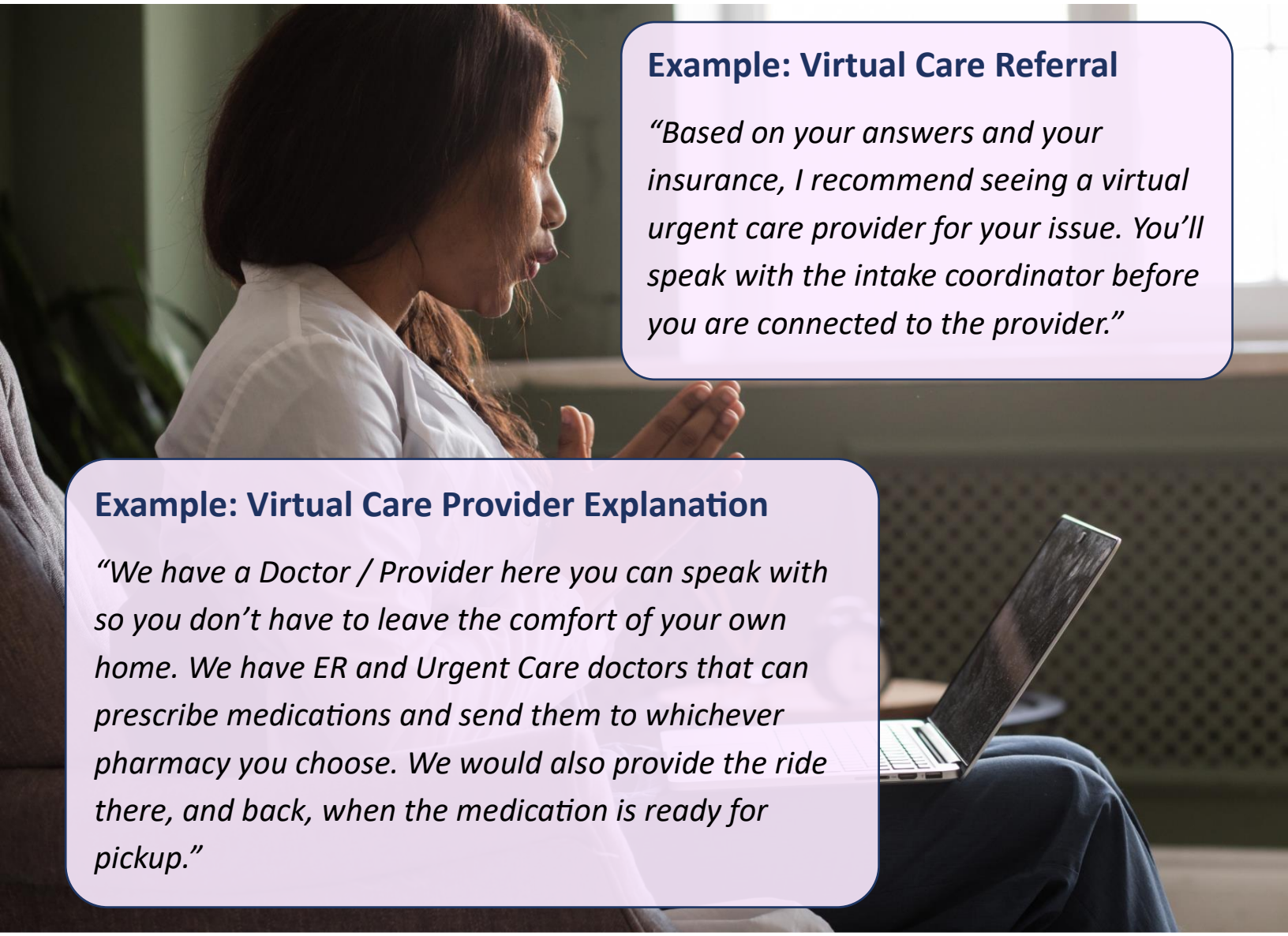
- Select the closest match to the **NMTARA** level (i.e., REF or VC for NMTARA 1-5)
- For your consideration:
  - Does insurance play a factor?
    - Clinics, specialty resources, Kaiser, etc…?
  - Will they potentially need imaging or lab services?
  - What care resources are available for their area?
  - Does the patient have mobility restrictions?
  - Can the patient be safely transported in a rideshare?

If any of these options limit your choices, document it!



Example: Urgent Care Referral

"Based on your answers, I think going to a nearby urgent care clinic will be best for what's going on with you today. Do you need me to arrange a ride for you?"

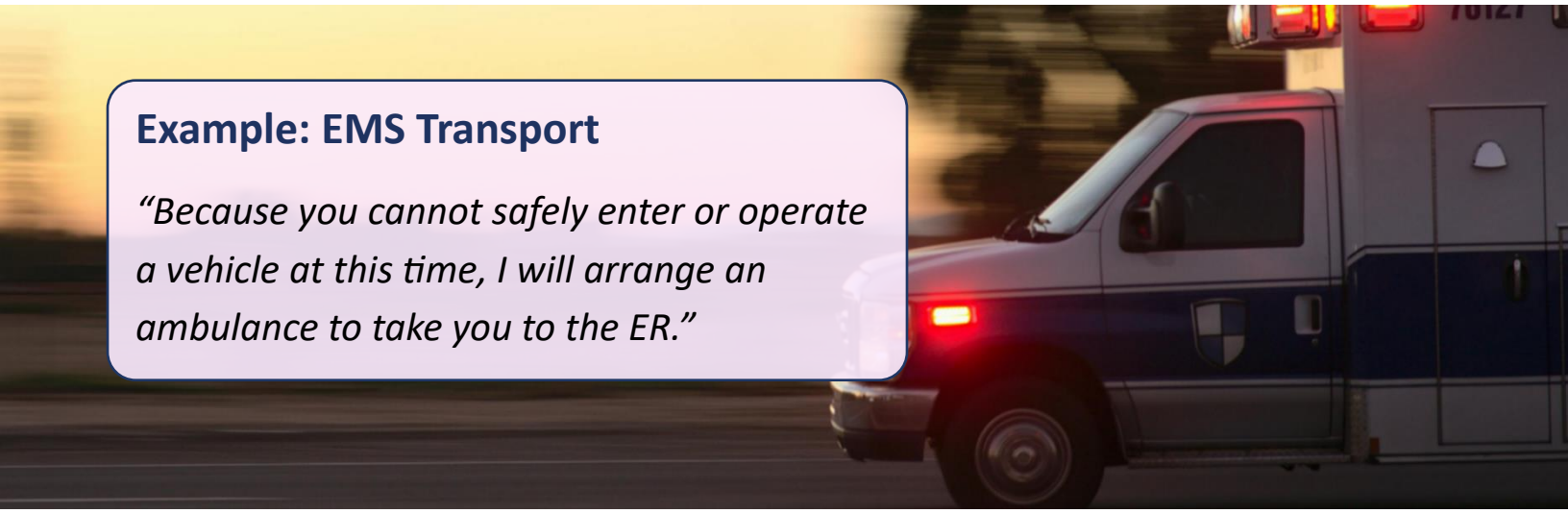


Example: Virtual Care Referral

"Based on your answers and your insurance, I recommend seeing a virtual urgent care provider for your issue. You'll speak with the intake coordinator before you are connected to the provider."

Example: Virtual Care Provider Explanation

"We have a Doctor / Provider here you can speak with so you don't have to leave the comfort of your own home. We have ER and Urgent Care doctors that can prescribe medications and send them to whichever pharmacy you choose. We would also provide the ride there, and back, when the medication is ready for pickup."



Example: EMS Transport

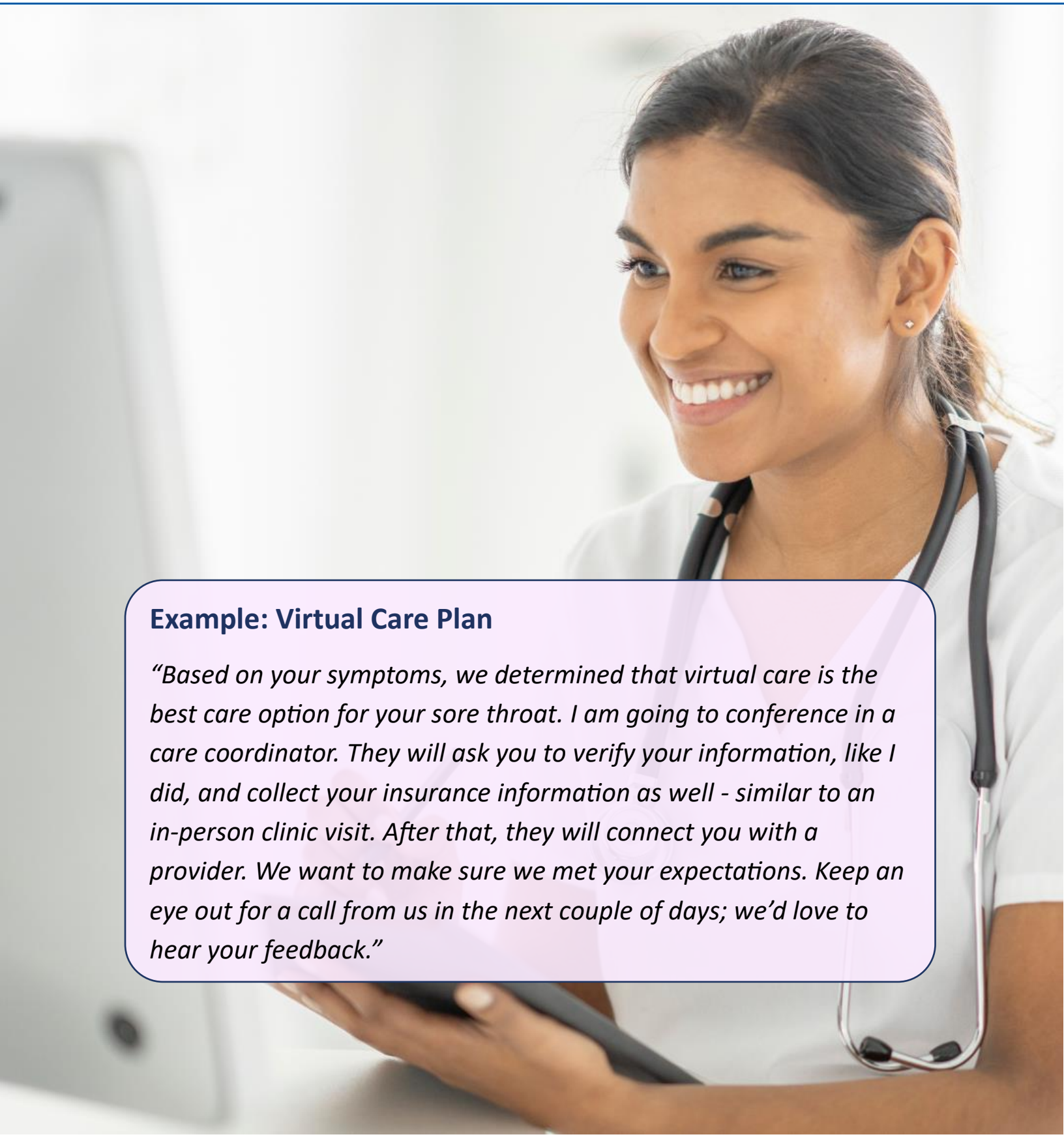
"Because you cannot safely enter or operate a vehicle at this time, I will arrange an ambulance to take you to the ER."

Plan of Care

The patient must be informed of what their plan of care is. ***This includes verbal consent to the plan.*** Summarize the call and actions taken to ensure the patient has a clear understanding of next steps:

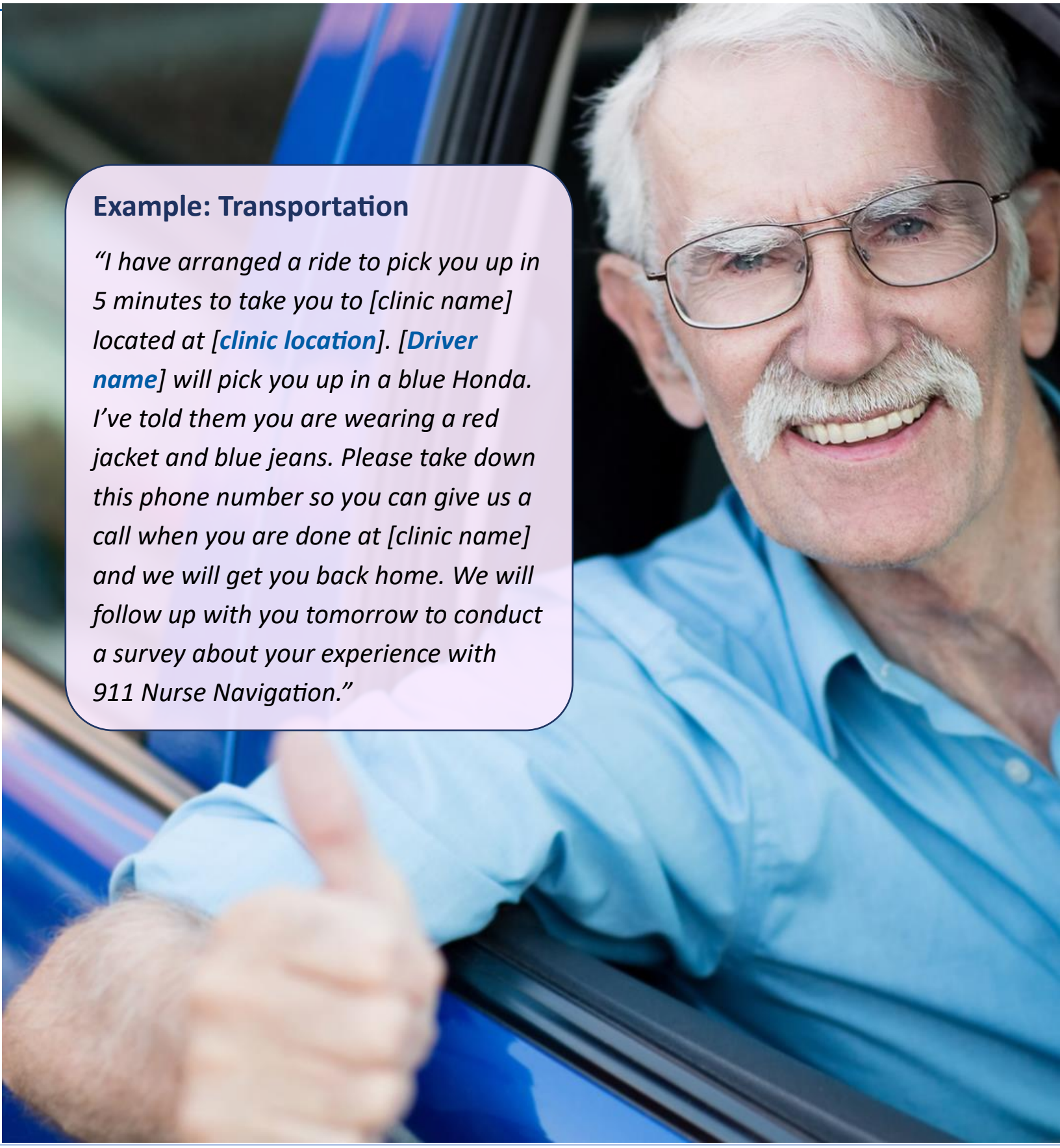
- **Where they are going?**
  - Clinic or hospital name and address.
- **How will they get there?**
  - Rideshare, ambulance, or private vehicle.
- **What should they take with them?**
  - Photo ID and insurance care; the clinic may also collect a co-pay.
- **Who to call when the plan breaks or they need a ride home?**
  - Provide the patient call back number and explain what it's for.
- **Home care instructions?**
  - Documenting only what was verbalized.
- **Will there be a follow-up call or communications?**
  - Follow up to get their thoughts and feedback regarding the care they received.

The patient should be aware of the specifics regarding the plan of care and they know who to call if the plan breaks.



Example: Virtual Care Plan

"Based on your symptoms, we determined that virtual care is the best care option for your sore throat. I am going to conference in a care coordinator. They will ask you to verify your information, like I did, and collect your insurance information as well - similar to an in-person clinic visit. After that, they will connect you with a provider. We want to make sure we met your expectations. Keep an eye out for a call from us in the next couple of days; we'd love to hear your feedback."



Example: Transportation

*"I have arranged a ride to pick you up in 5 minutes to take you to [clinic name] located at [**clinic location**]. [**Driver name**] will pick you up in a blue Honda. I've told them you are wearing a red jacket and blue jeans. Please take down this phone number so you can give us a call when you are done at [clinic name] and we will get you back home. We will follow up with you tomorrow to conduct a survey about your experience with 911 Nurse Navigation."*

Arrange Care

The nurse arranges care agreed upon by the patient. Ensure the patient understands the plan of care and who to contact if the plan breaks. The patient has the chance to ask questions and decline before care is arranged.

Examples of Care Options

- Select an ***open*** clinic and verify they accept the patient's insurance
- Arrange transport to a community care resource
- Conference RelyMD for a virtual care visit
- Correctly activate an ambulance response based on contract directions

Example: Virtual Care Next Steps

*"Let's recap your call to 911 Nurse Navigation, your initial complaint was [**xyz**] and based on the assessment, we agreed that a virtual care visit is our next step. Please take down this phone number in case you can't complete your virtual care visit or if you have any issues. You may hear silence followed by a brief hold as I get that set up for you. You will answer some questions with the care coordinator before speaking with the provider."*

Partner Contact

When calling any partner (dispatch, VC, Crisis Connections, etc.) always:

- Identify yourself
- Who you work for
- Why you are calling

Example: Ambulance Request

*"This is [**name**] with 911 Nurse Navigation. I'm calling for an ambulance for [**address**] due to [**patient condition, refusal of resources, limited mobility, etc**]".*

Example: Virtual Care Request

*"This is [**name**] with 911 Nurse Navigation. I have a patient in need of virtual urgent care."*

Virtual Care-RelyMD Process

1	**Check the VC exclusion list before proceeding**
2	**Explain the RelyMD process to the patient (set expectations)**
3	**Follow Corti to arrange care.**

Potential Call Barriers

Call Handoff

The dispatcher has cross streets, but no specific address.

Enter the cross streets into Google maps (i.e. “intersection of 7th and Main (city/state)”). Ask the patient to describe what they see around them and correlate with information from maps.

The nurse is having trouble geolocating the address in Logis.

Don’t geolocate here. Wait until you are on with the patient to clarify address and geolocate.

The patient is yelling / belligerent during the handoff.

Wait until the patient stops talking and offer empathy, provide context, and preview next steps as needed.

The nurse continues to ask dispatch for excessive information

Only ask clarifying questions. All other information can be confirmed with the patient / caller, including geolocation for addresses

While on the phone, our tone can cause both intentional and unintentional emotional ripples throughout a conversation. Even our physical facial expression has a subtle, but meaningful, impact on our tone, and therefore, the other party's willingness to participate.

We should always aim to acknowledge, respect, and respond to the emotional environment of the room.

Greeting the Caller

The caller / patient interrupts and demands an ambulance

Wait until the patient stops talking and offer empathy and a 'why':

- "I'm sorry you're not feeling well today. I'm a nurse and I am here to get you the help you need today."

The caller / patient does not speak English

Conference in the language line services; see Corti for instructions

The patient refuses to speak on a recorded line

Explain that we record for quality assurance. if they still refuse, end the call and send it back for BLS.

- "I'm sorry. I cannot proceed with this call. I will have an ambulance sent to your location.

Patient Verification

The patient refuses to verify demographic information

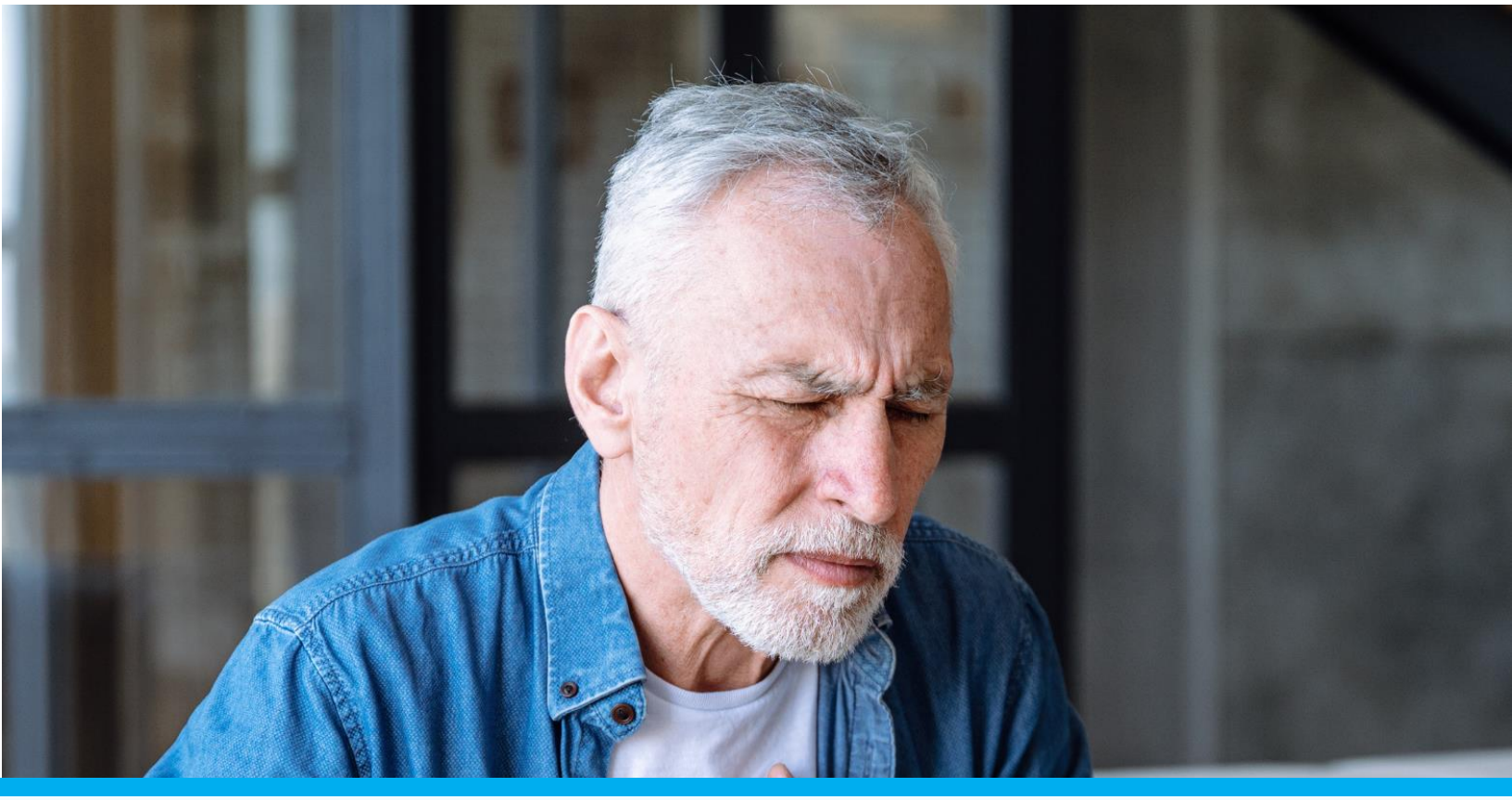
Take what information you can and explain the purpose of your request.

The patient identifies as non-binary

Respectfully explain that some questions are included or excluded based on biological sex so that we can best match them to the proper care.

The patient has a 911 only phone

Ask the patient / caller to confirm if it's 911 only. If so, put '911 only' in the number field. If the patient / caller has no phone, put 'no phone' or 'unknown' in the number field



Clinical Discovery

The patient reports red-flag symptoms

Based on your nursing judgement, **immediately** escalate care to an ALS response, if needed, in the event of:

- **New** onset of weakness
- Persistent or worsening pain that interferes with activity
- New onset of confusion or cognitive issues
- Continuous vomiting / diarrhea
  - Especially in the very young and very old
- Difficulty breathing
  - With or without blue lips, mouth, or tongue
- Chest pain/pressure that radiates up to jaw or down arm

The caller is providing answers for patient without consult

Ask to speak directly with the patient if possible

Clinical Pathway

The patient's reply indicates they misunderstood of the question

- If they don't understand the question as written, rephrase to ask the spirit of the question
- 'And' and 'Or' are very important aspects of the question and should not be omitted
  - Example: (Age 50+) Is there a tearing or ripping pain, or is the pain rapidly getting worse?
  - Example: Are (you/they) having persistent nausea and vomiting?

The patient gives vague answers

- Ask the patient to clarify their answer if needed (i.e., "When you say sometimes, does that mean right now?")
- Ask for a concrete 'Yes' or 'No' answer
- Ensure that your click matches the patient's answer

Data Collection Issues

- System glitches
- Refresh system [Log out and log back in]
- Use Briggs book if Corti is not functional
  - Need backup for contract routing
- Report pathway issues to Nathan-if questions don't make sense or unable to move forward.

Care Suggestions

The clinic is closed or doesn't take the patient's insurance

- Place the caller on hold and call the clinic to verify their hours and accepted insurance

There are no providers available

Pivot and offer the patient a ride to a clinic

The patient demands an ambulance and refuses other resources

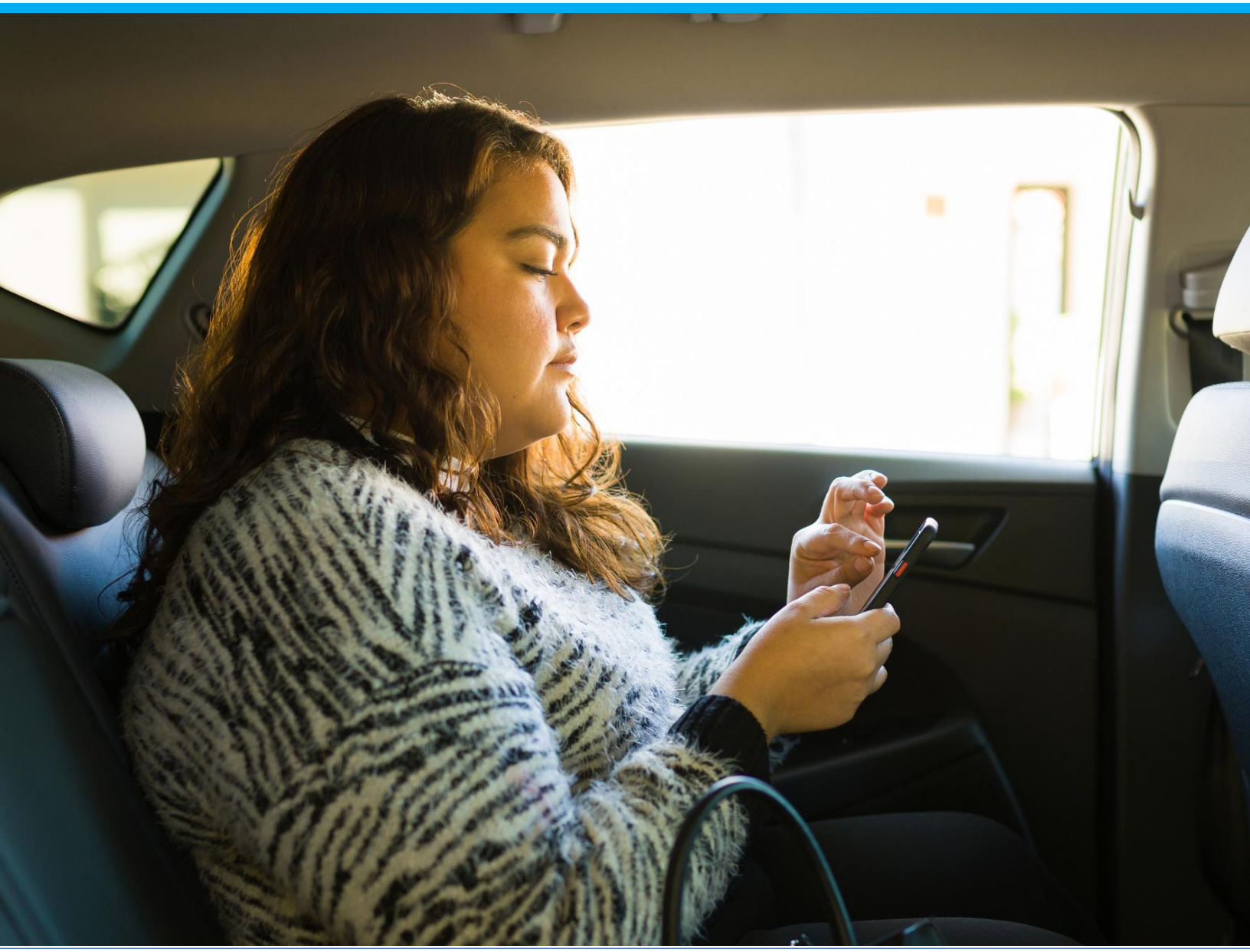
De-escalate and explain that a clinic or virtual care visit is often faster and more cost effective than an ED visit.

- If they still decline, document refusal and which resources offered.

Plan of Care

The patient has no time to note or save the patient callback number before getting into the rideshare

- Provide the information before requesting the ride so the patient has time to take down the number and fully grasp the plan.
  - ➢ Nurse can also schedule the ride to give patient time to get ready (as NMTARA level allows)
- If sending a patient to virtual care, go over the plan with them before conferencing in the care coordinator. Provide the callback number and advise them of a follow-up survey via next business day phone call.



Frequently Asked Questions (FAQ)

Question

How do we process calls from minors?

Answer

If a minor (***younger than 18 years old***) calls with no adult present, we will send back to EMS to dispatch an ambulance.

Question

Is there a list of call scenarios / types we should receive?

Answer

Each Contract has a list of determinants that tells them which calls are eligible for 911 Nurse Navigation. This list is not for us to screen which calls we accept and assess.

Question

How do I decide between an ALS or BLS response?

Answer

Based on the nurse's identification and evaluation of red flag symptoms and the clinical protocol's care suggestions

- **BLS Response:** Non-Emergent Response
  - At the normal speed of traffic
  - Units could be diverted to an emergency call nearby
- **BLS Response Consideration**:
  - The patient requires services only a hospital can provide
  - The patient is immobile (bed bound, unable to walk, etc…)
  - Recommended mode of transportation by a healthcare provider
- **ALS Response:** Emergent Response
  - Lights and sirens
  - Full speed ahead (goes through red lights)
  - Won't be diverted
- **ALS Response Consideration:**
  - The patient requires advanced medical care such as:
    - cardiac monitoring
    - intubation
    - medication administration
    - IV access
    - Life-threatening emergencies like:
    - respiratory distress
    - MI related chest pain
    - stroke-like symptoms

Dispositions

Virtual Care (VC) / Telehealth

Partner: RelyMD

NMTARA 0-5

Clinical Staff

Emergency Department Boarded Physicians as well as other providers like PAs and NPs.

Availability

Available per provider's self-schedule: “Uber”-style model

Check availability before offering!

Platform

Prefer smart-device, but not necessary to connect on scene.

Insurance

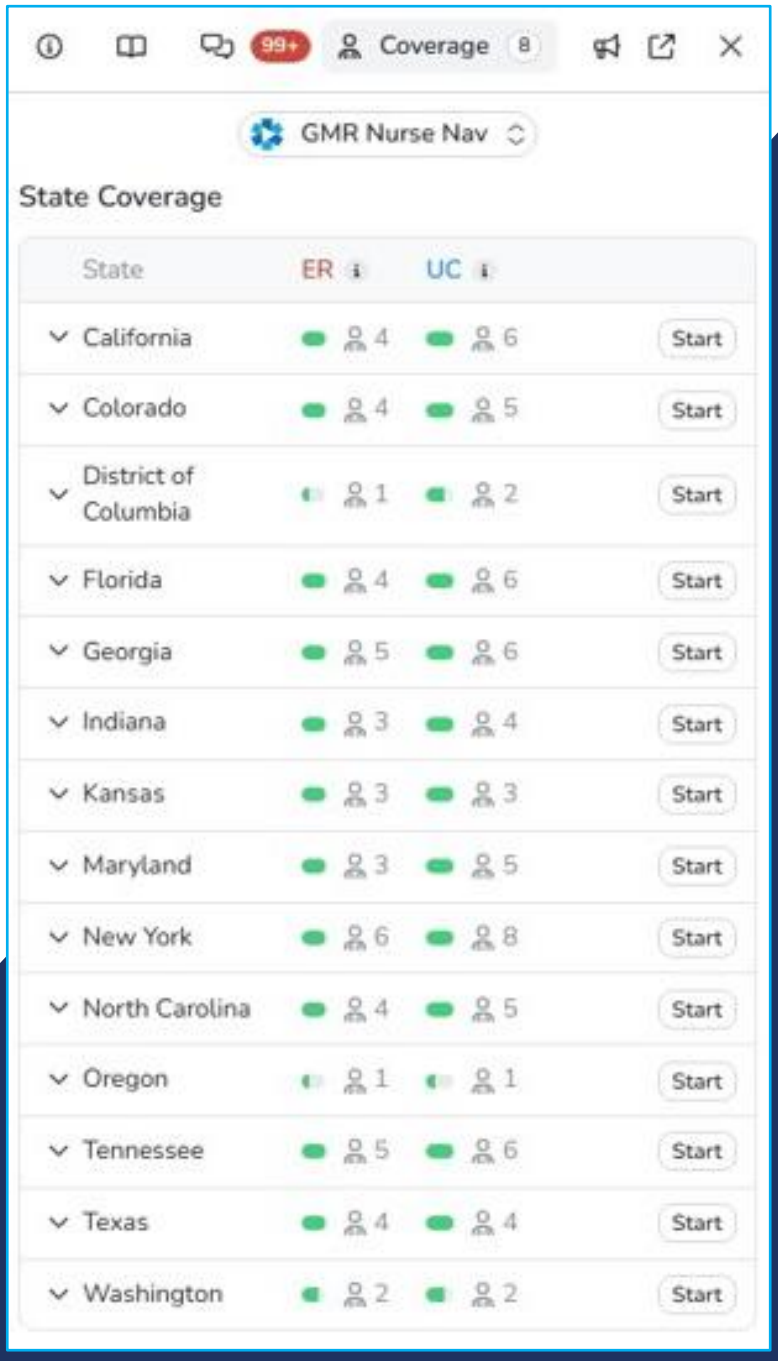
NOT insurance dependent

Age

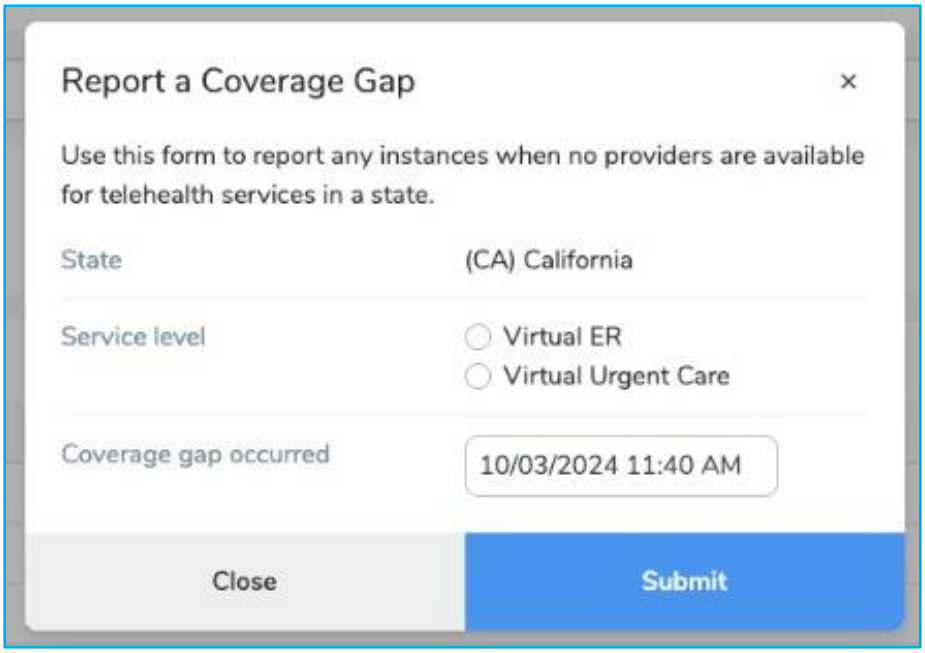
>2 years old

Coverage Check

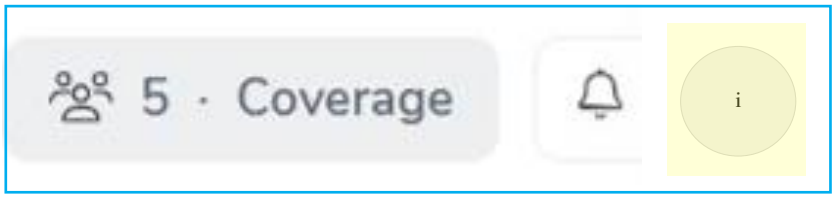
- Make sure you have a provider available in the state your patient is currently in.
- ER means there's an ER doc available for all your NMTARA 0 needs.
- UC means there's an urgent care provider on for all your NMTARA 1-5 patients.



Coverage Requests



- If you're missing a provider for your patient, you can click the Report button, to help track the specific gap in coverage.
- You can also ask a care coordinator in the chat if a provider available in the state your patient is in.
- Check this Info icon if you are on the fence about VC. You can get to this list by clicking the info icon to the right of coverage:



Virtual Care Exclusion Criteria

General Exclusions

Age Criteria Exceptions

- All patients under two years old
- Patients 17 or younger without parental / guardian consent

Mobility Exceptions

- Patient is non-ambulatory (cannot walk) without available or reasonable assistance from transportation

Medication Management

- Patients likely to required scheduled narcotics or controlled substance refills

Advanced Live Support (ALS) Management

- Any condition clearly requiring immediate ALS intervention

Neurological Symptom Exceptions

Sudden Onset or Severe Headache

- Example: “Worst headache of my life” within the last four hours

Sudden Onset Sustained Weakness, Numbness, or Paralysis

- Especially unilateral within the last 4 hours

Virtual Care Preparation & Contact

Overview

1. **Complete Prescreen**
2. **Confirm Virtual Care**
3. **Contact Virtual Care**
4. **Track Consultation**
5. **Followthrough**

1. Prescreen Questions

Before you enroll this patient for a virtual care visit, please review the following information:

5. Is the patient a minor (< 18 y.o.):
    - RelyMD does not see patient < 2 years of age.
    - Patients age 2-17 need a parent / guardian present to consent for VC treatment.
6. RelyMD does not prescribe scheduled medications.
7. RelyMD is not an appropriate solution for patients with:
    - ALS symptoms
    - Active suicidal ideation or behaviors
    - Active homicidal ideation or behaviors
    - Active psychosis or severe agitation

2. Virtual Care Teams Confirmation

Inform the patient to start their virtual visit, there will be an intake process very similar to checking in at the doctor or ER. Advise the pt that if there is a disconnect during transfer, they should turn off spam blockers and expect a call from an 855 number to complete this intake.

At this time, inform the patient that you will follow up with them if they require further care and if not, they will receive a follow up call to

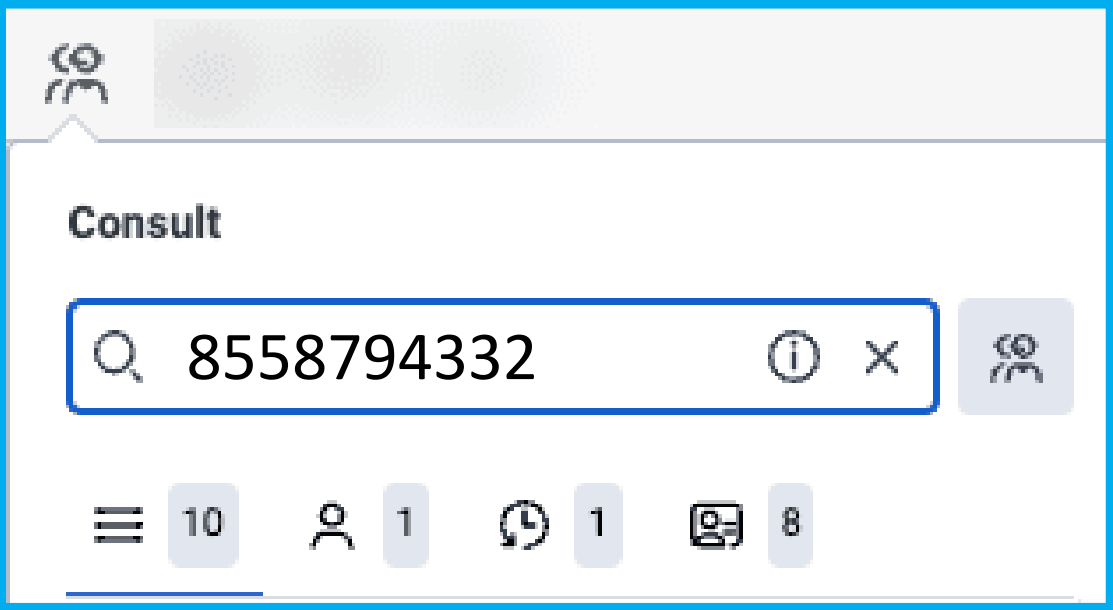
Then, put the following in the **NN / Care Coordinator** chat:

> *Pt Name, DOB, Address, Active callback number, U.S. State pt is calling from.*

3. Initiate Virtual Care Contact

1. Keep the patient on hold to start the transfer.
2. Dial (855) 879-4332
   - Press 1 for English
   - Press 1 for patient Care
3. Once the Care Coordinator answers confirm the information sent in the chat and verbally give pt age, gender and situation.
4. Bring patient on the line and confirm everyone is on the call before disconnecting.

Example: Genesys Consult Transfer



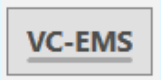
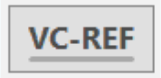
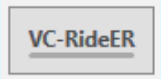
**When a consult transfer is initiated, the patient will be placed on Hold automatically.*

4. Track the Consultation

Monitor the RelyMD chat for updates from the provider. In the event the provider does not send a message, be sure to check the chart and follow through to make sure you do not miss further necessary care.

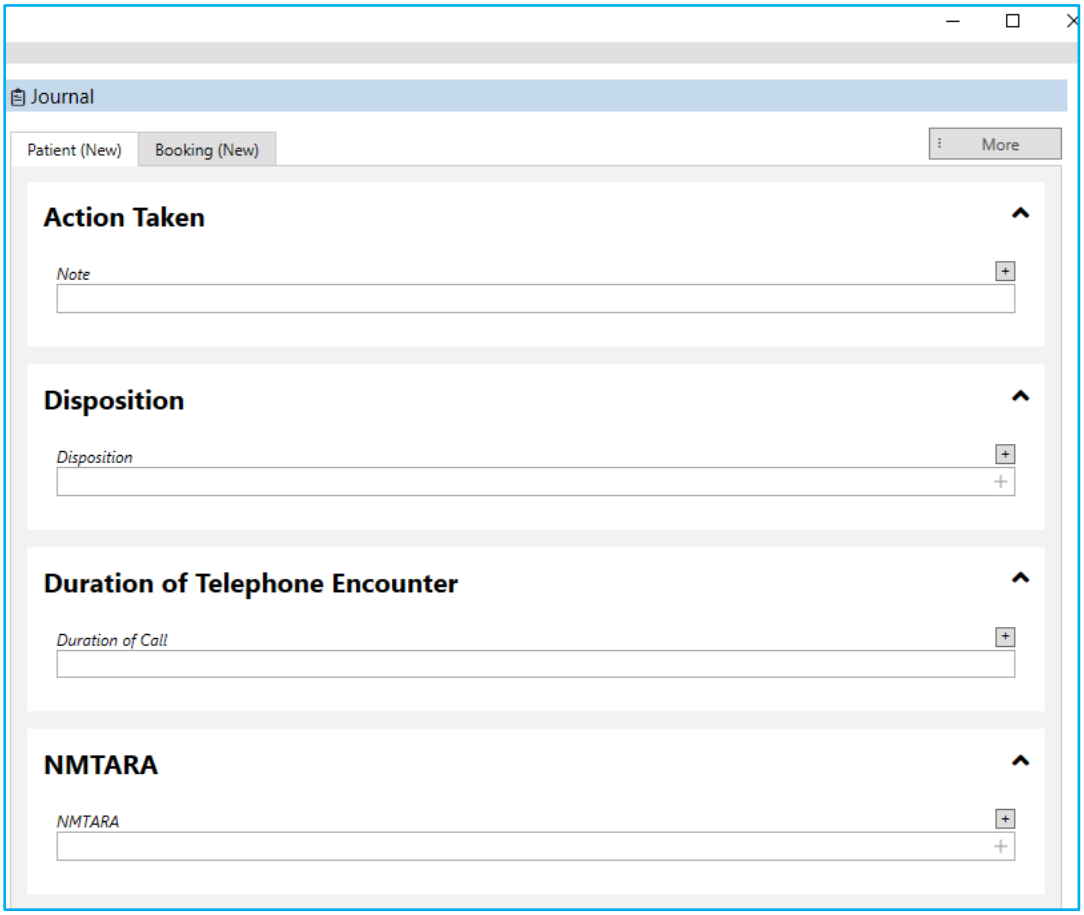
5. Follow-Through

- If your patient has been seen by the RelyMD provider and they recommend in-person evaluation, follow the steps below:
  - Copy the incident (with comments)
  - Update the Response Plan / Status on the new incident

Selection	Purpose
	The provider recommends they go to the **ER via ambulance** (ALS or BLS)
	The provider recommends they go to an **urgent care clinic**
	The provider recommends they go to the **ER via rideshare**

Journal

- Click the plus sign to add a note that includes the provider's recommendation and the action taken by the nurse.
    - ➢ Example: "Nurse Navigation assist to nearest ED via EMS". Patient notified of the provider's recommendation that he/she go to the ER via ambulance. Dispatch called for BLS.
- Click the plus sign to update the disposition to the recommended care option.

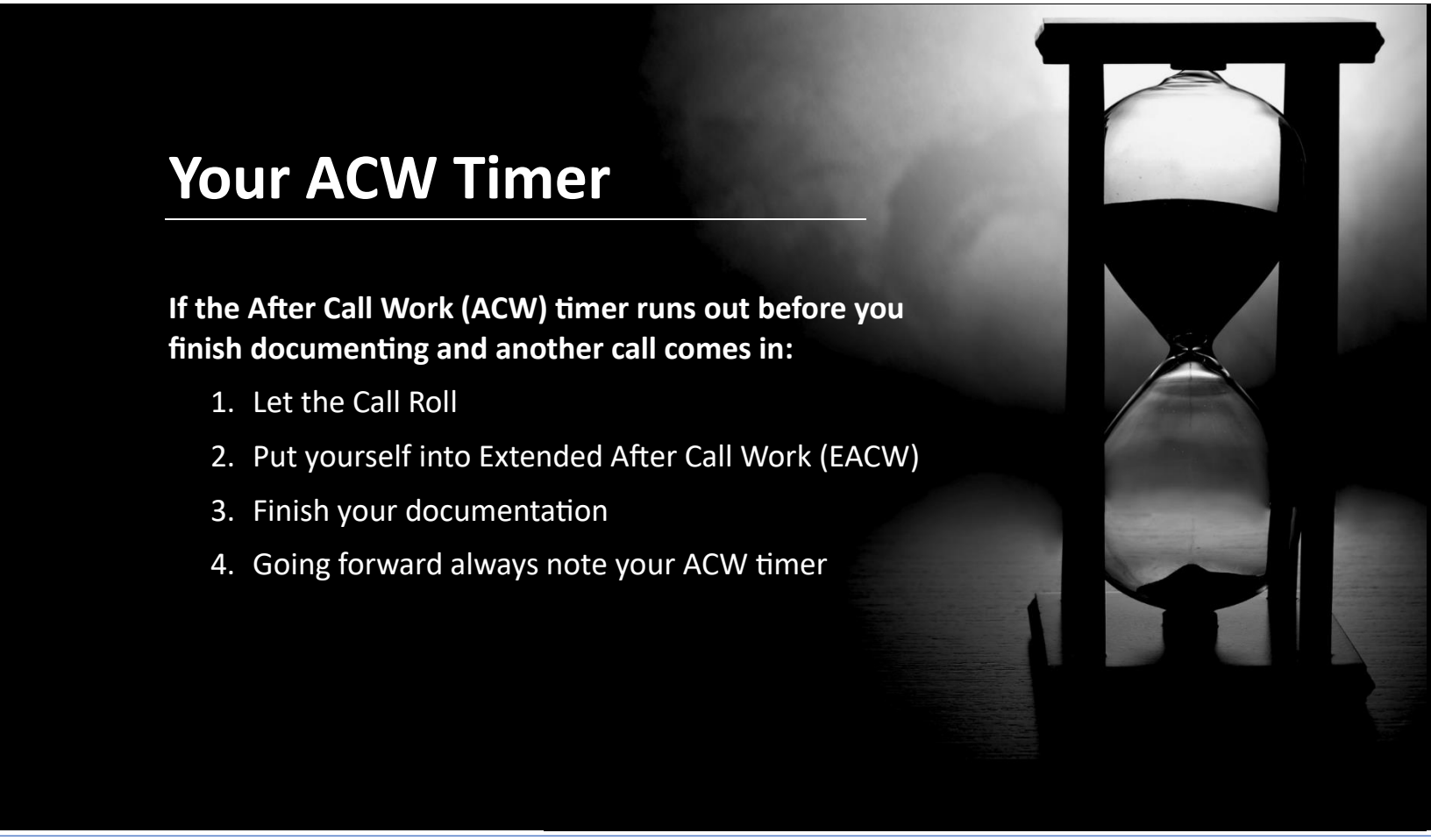


Documentation Do's & Don'ts

Documentation

- Tells the story of the patient encounter
- Provides follow up information
- Demonstrates that we did good work
- Stands as a legal record of patient care.

Someone reading your after-call note should understand clearly what happened during the call without having to listen to it for additional context.



Your ACW Timer

If the After Call Work (ACW) timer runs out before you finish documenting and another call comes in:

1. Let the Call Roll
2. Put yourself into Extended After Call Work (EACW)
3. Finish your documentation
4. Going forward always note your ACW timer

SBARF

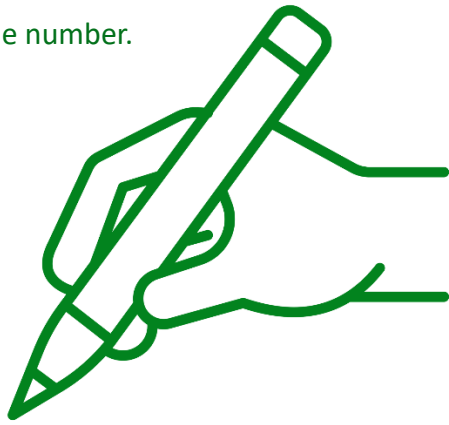
Situation	What is the current chief complaint?
Background	Relevant history
Assessment	Include vitals and Corti summarization
Recommendation	Care options offered / declined / accepted
Follow Through	Actions taken by nurse to facilitate care

All Deviations from Protocol **MUST** be Documented

Guidelines

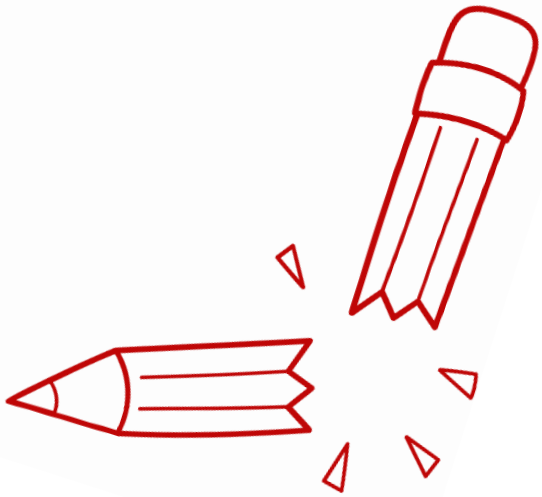
✓ DOs

- Accurate spelling of patient's full name and physical address
- Appropriate capitalization and inclusion of apartment / building / unit numbers
- Be concise and clear in your communication
- Enter patient information in the appropriate fields
- Verify who the caller (name) is and their relationship to the patient
- The patient's phone number may be different from the caller's phone number.
  - Do not enter the EMT / Medic's number here.
- Call tags are ***NOT*** optional! You must select one of three:
  - 911 call taker
  - Field Fire
  - Field AMR

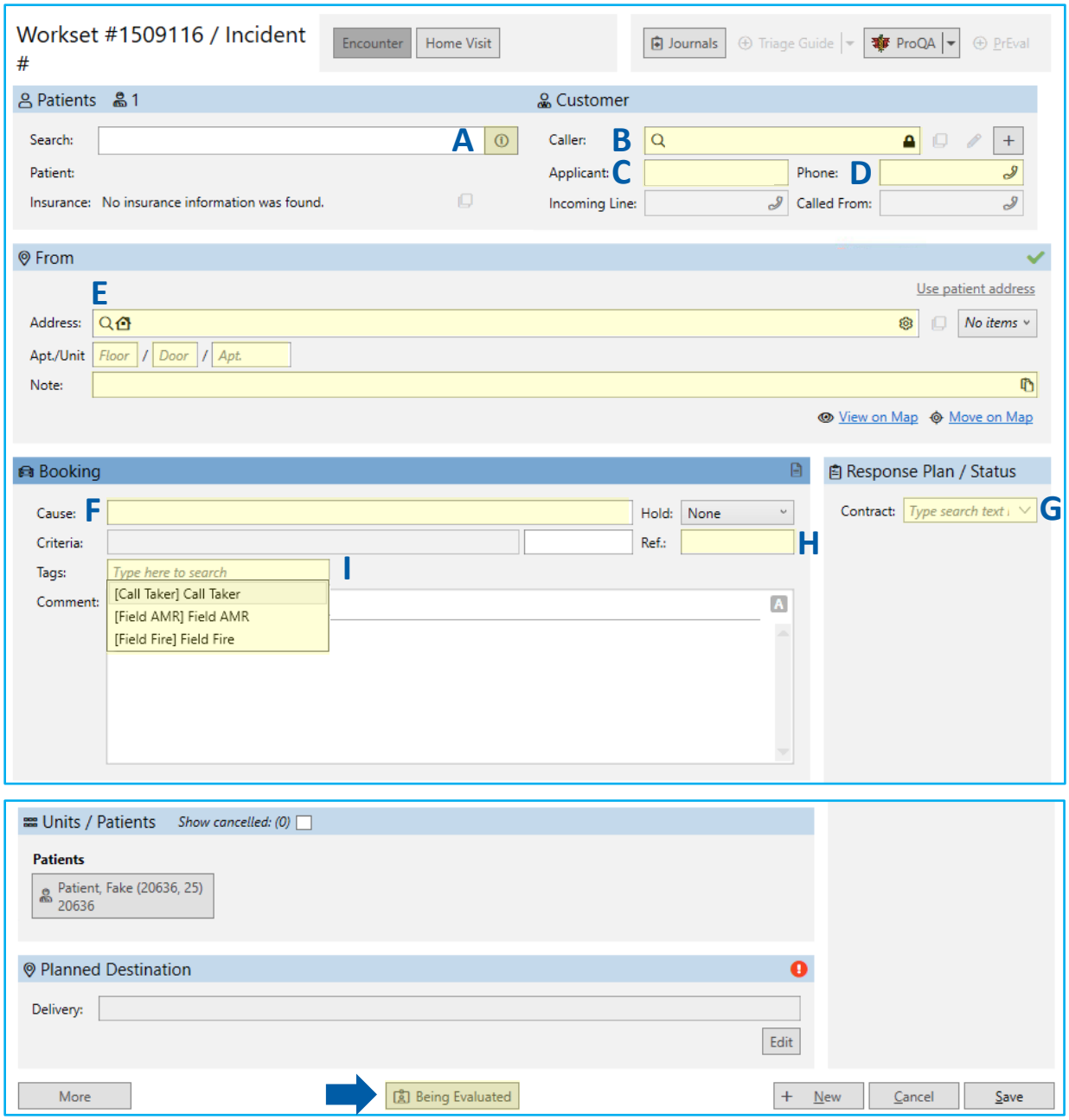


☒ DON'Ts

- Chart opinion-based notes. Only document facts.
- Copy and paste the unedited Corti AI summary.
- Submit journal without checking spelling /grammar
- Document what isn't reflected in the Corti session



Documentation Layout



After you open your CAD-to-CAD call on the right side, click the **Being Evaluated** button so that your team will know you are working on the call.

Overview

A. Click for Patient Info Screen.

- Enter the patient's information and ensure all required fields are completed.

B. Enter Contract Name.

C. Enter the caller's name and relationship to the patient.

D. Enter the caller's phone number.

- The caller's phone number may be different from the patient's phone number.
- Do not enter the EMT / Medic's number in this section.

E. Enter the patient's address details.

F. Enter required Cause documentation.

- If it's blank: Fill it in! (w/ Corti pathway)
- If it has data auto-filled: Leave it alone!

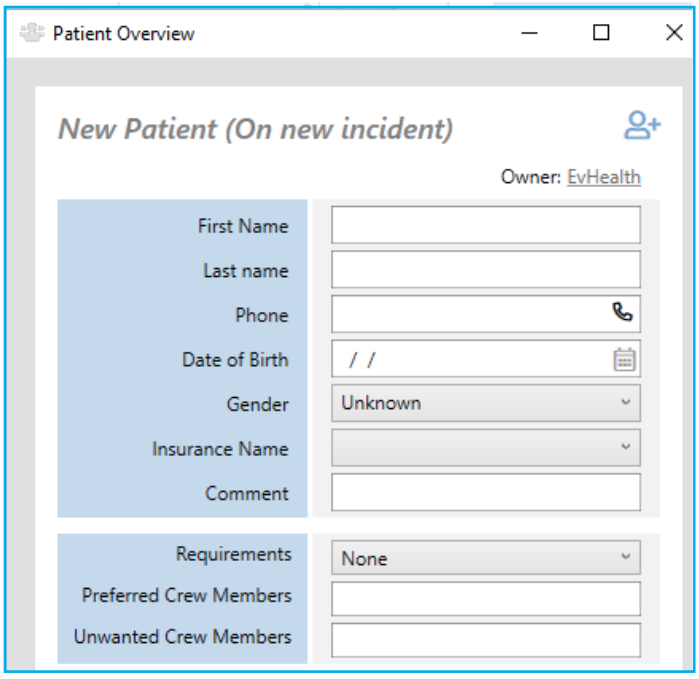
G. Select insurance type.

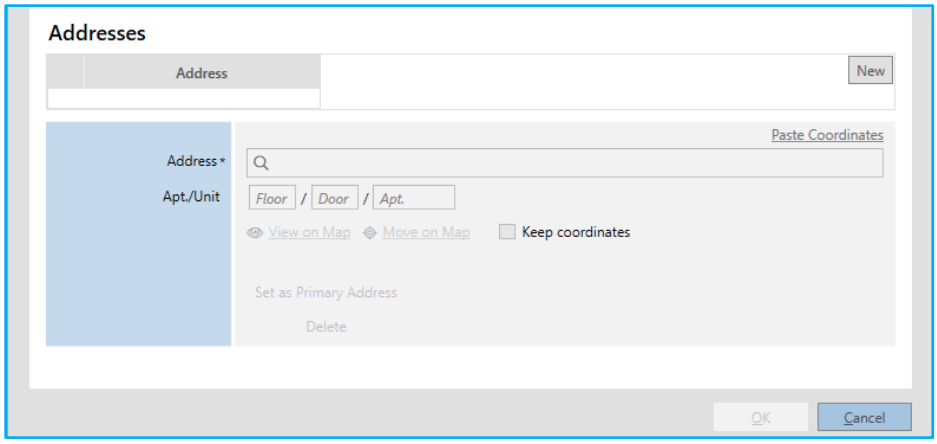
H. Input the Reference / Event Number.

I. Call tags are NOT optional! You must select one of three:

- 911 call taker
- Field Fire
- Field AMR

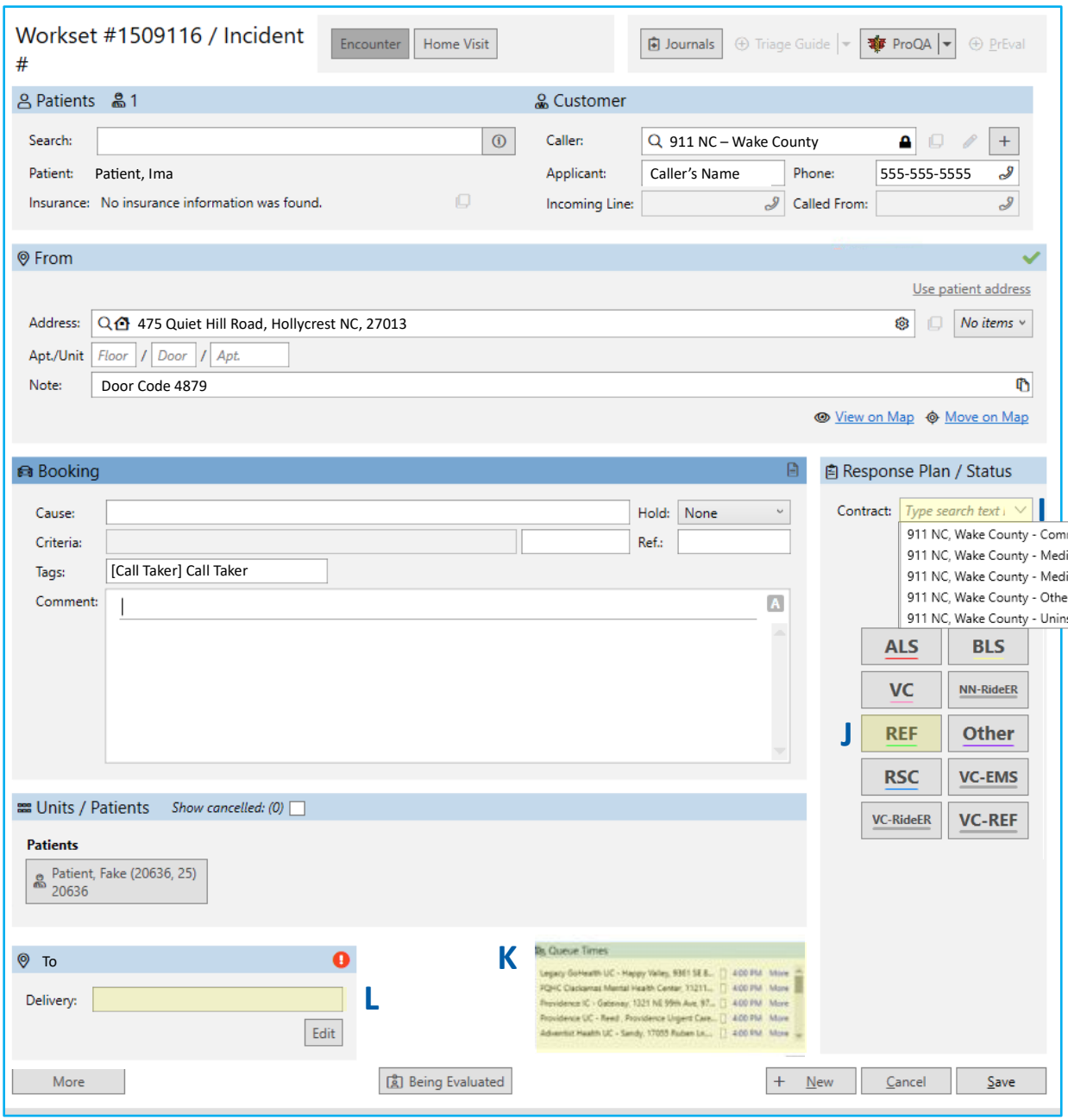
Patient Overview Window





Required entry fields are:

- First Name
- Last Name
- Phone Number (use dashes)
- Date of Birth
- Gender
- Address
    - ➢ Include apt. / building / unit number



J. Select the contract type from the Response Plan / Status menu on the right.

K. For a clinic referral, select REF from the Response Plan / Status menu.

L. From the list of clinics, select the one with the closest time.

M. If the name and address of the desired clinic are not listed in the Queue Times window, enter them manually into the Delivery field.

Editing the Corti Summary

Patient Age	**Patient Sex**	**Chief Complaint**
Reported Symptoms	**Relevant Past Medical History**	**Current Medications**
NMTARA Level	**Recommended Disposition**	**Patient's Acceptance of Plan**
Education Provided to Patient	**Corti Session ID**	**Date / Time Information**

Follow-Up

Circle back to document patient outcomes, if known.

Add a note, ***DO NOT*** edit an existing note.

✓ **Ride Completed**

- Ride home requested / completed
- Virtual Care follow up
- TIP, VC > REF, VC > EMS

✓ **Clinical Disposition**

- What is the consultation outcome?

✓ **Do *NOT* include the provider's name in the note**

✓ **Did patient need a ride to a different clinic?**

Ride completion should be documented in a timely manner to confirm completion of transportation and ensure the patient requires no further care from us.

Quality Assurance

Guiding Scope & Principles

- Observers of 911 NN's efforts to ensure safe and appropriate patient care
- Use objective metrics based on stated standards and processes
- Every call reviewed is an opportunity to reinforce what we're doing well, move us closer to a unified patient experience, and identify areas for NN improvement

What We Do

Routine Evaluations	Each nurse is scored twice per month
Call Investigations	Both Internal & External
Special Assignments	Analysis of call types for better understanding, workflow improvement, and systems checks.
Reporting	Research and report trends and findings to the proper stakeholder(s)

Reporting

- Calls within normal expectations will be discussed at scheduled 1:1s
  - Goal: same-day visibility on NN "report card"
- Flagged calls allow for timely guidance supporting continual improvement
  - If you report to a Supervisor, reports go to Supervisor and Manager
  - If you report to a Manager, reports go to Manager and Director

QA Rubric Goals & Development

	Developed from the Conversation Guide		Designed to minimize subjectivity and eliminate bias
	Evaluates adherence to workflows		Call Handling & Documentation are both graded

Call Handling

Raise your Hand – 3 pts

- ☐ The nurse used the appropriate greeting for the call. (3 pts)

Keep Care at the Center – 35 pts

- ☐ The nurse informed the patient/caller they were on a recorded line. (5 pts)
- ☐ The nurse appropriately verbalized that there was no ambulance on the way / why the call was routed to a nurse. (5 pts)
- ☐ The nurse verified HIPP data of the patient (name, date, and phone). (10 pts)
- ☐ The nurse verified insurance with the patient/caller. (5 pts)
- ☐ The nurse verified the incident address and disconnect phone number with the patient/caller. (10 pts)

Seek to Understand – 28 pts

- ☐ The nurse spoke with the patient / asked to speak with the patient. (8 pts)
- ☐ The nurse probed chief complaint and red flag symptoms. (10 pts)
- ☐ The nurse chose and accurately followed an appropriate triage pathway. (10 pts)

Find a Way Together – 24 pts

- ☐ Care resources were presented one at a time. (3 pts)
- ☐ Care recommendations were appropriate given NMTARA and known barriers. (8 pts)
- ☐ Plan of Care was outlined with the patient. (8 pts)
  - Where they're getting seen
  - How they're getting there
  - What to take with
  - Who to call for next steps or breaks in the plan
- ☐ The patient agreed/verbalized understanding of the PoC. (5 pts)

Be Accountable – 10 pts

- ☐ Care arrangements initiated per disposition specifications (10 pts)
  - Ambulance – responsible party was informed per contract instructions
  - Private transport – destination selected in software and transport finalized
  - Telephonic – followed instructions to successful warm handoff

Kudos: Did the nurse do something exceptionally well, or go above and beyond on this call?

Documentation

Customer Field – 16 pts

- ☐ Caller (911 Contract) (6 pts)
- ☐ Phone (Disconnect callback number) (6 pts)
- ☐ Applicant (Disconnect phone owner, with relationship if not patient) (4 pts)

Patient Info Field – 24 pts

- ☐ First and Last Name (spelling verified) (8 pts)
- ☐ Phone (verified as pt's) (6 pts)
- ☐ DOB (8 pts)
- ☐ Gender (2 pts)

From Field – 9 pts

- ☐ Address (8 pts)
- ☐ Additional Data (if needed) (1 pts)

Booking Field – 9 pts

- ☐ Cause (triage pathway) (6 pts)
- ☐ Tags (source of call) (2 pts)
- ☐ Referral (if given) (1 pts)

Response Plan Field – 16 pts

- ☐ Contract (insurance) (4 pts)
- ☐ Disposition (8 pts)
- ☐ Resource selected (if applicable) (4 pts)

Journal Screen – 28 pts

- ☐ Note: SBARF used – 4 pts
- ☐ Note: Deviations (NMTARA or process) documented – 6 pts
- ☐ Note: Narrative details match call – 8 pts
- ☐ Note: Corti ID documented – 6 pts
- ☐ NMTARA: correct for call – 4 pts

Flags

Customer Service Issue

- Unprofessional conduct (lack of empathy, frequent interruptions, rude, dismissive, etc.)
- Negative statements re: pt eligibility/qualifications for EMS or pt situation
- Nurse statements disparaged EMS system or capabilities
- Nurse's actions were not in alignment with contract-specific guidance
- ALS escalation reasoning unclear
- NMTARA 6 / Triage Not Complete reasoning unclear

Patient Safety Issue

- Red flag symptoms
- Did not utilize triage pathway or used inappropriate triage pathway
- Care recommendation lower than NMTARA classification
- Exclusion criteria met, continued triage

Keys to Success

Follow the Conversation Guide

It was designed to keep all of 911 NN on track!

Use the Tools that are There to Help You

Corti, Genesys tags, whisper, etc.

Verify HIPAA Data

The importance of this only grows as we evolve our work with clients and payers.

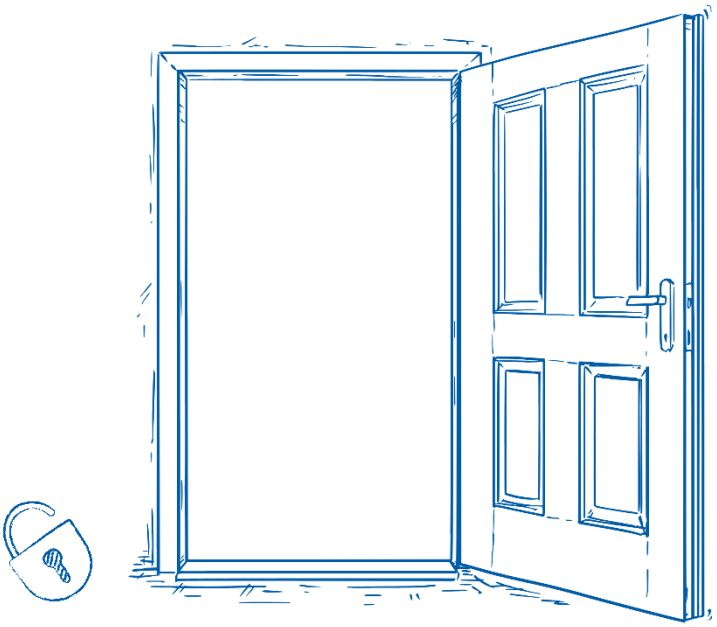
Document Clinical Reasoning for Deviations

Triage Not Completed - NMTARA 6 / Exclusion Criteria

Care escalation above NMTARA expectations

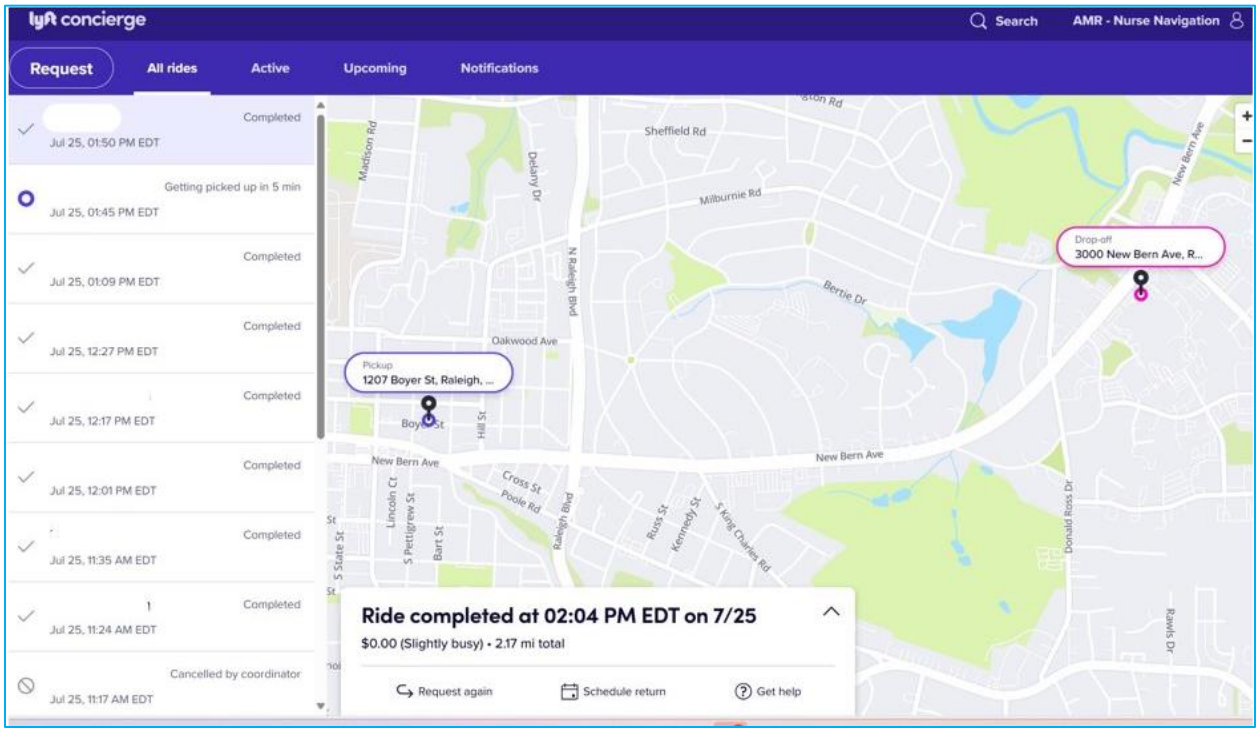
Ask for Help When You Need It

We are here to help you!

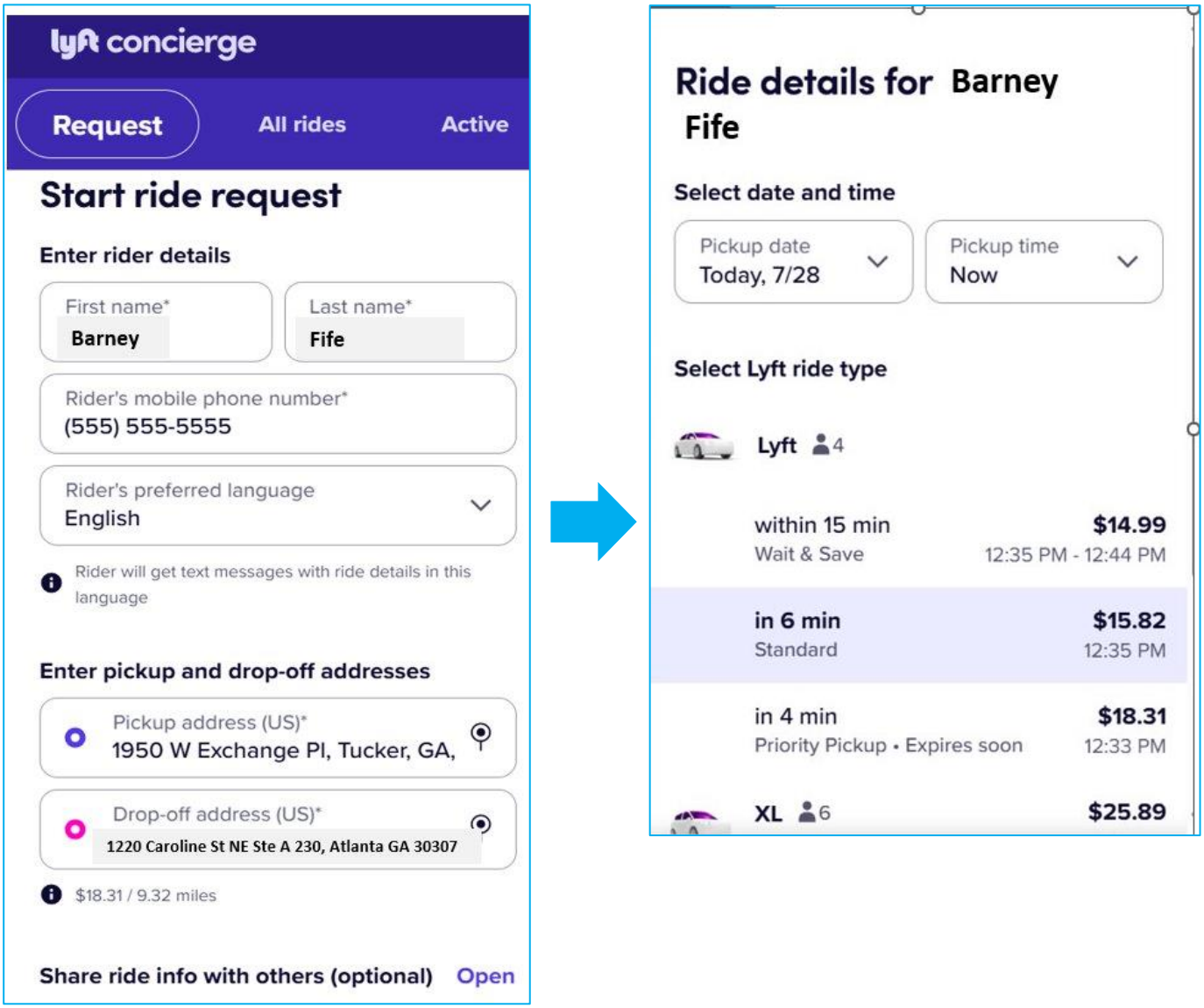


Lyft: Preferred Transportation in most contracts

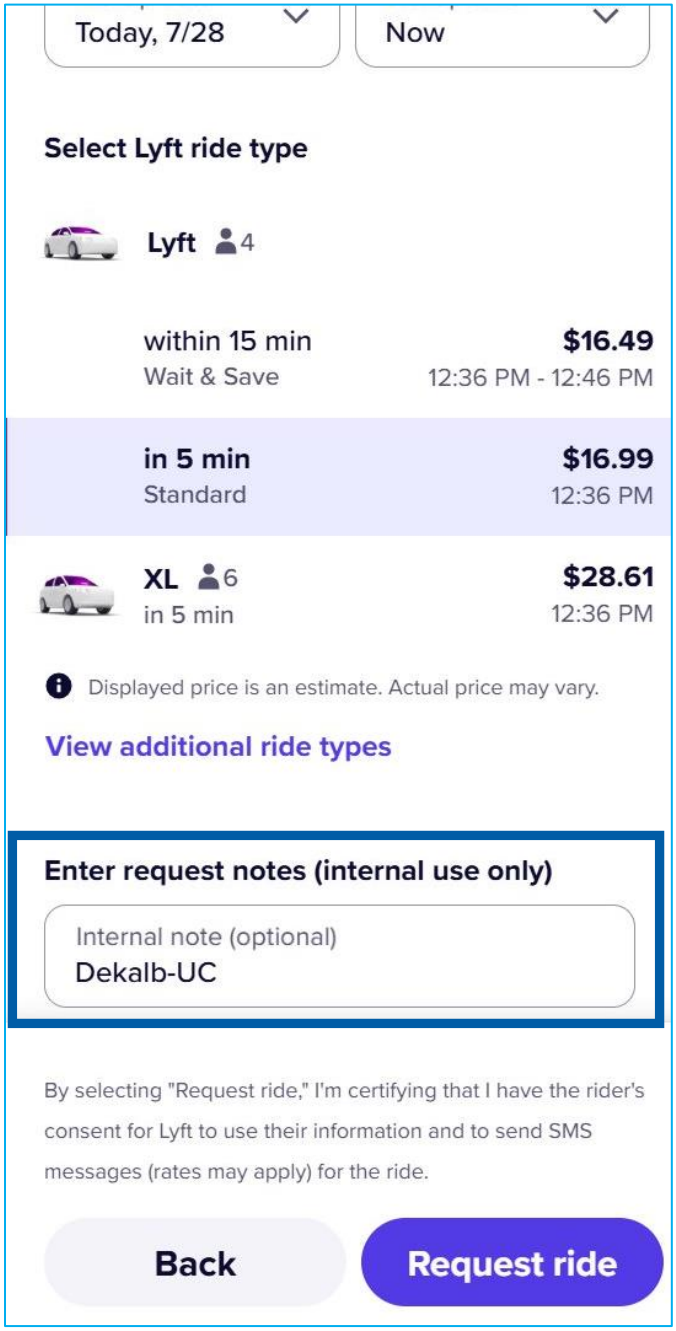
Log in at the beginning of your shift and leave open for the shift.



1. You must enter the patient's first and last name, telephone number (does not need to be cell but it is preferred). Pick up address and drop off address should be entered manually instead of copy and paste.
2. Note for the driver: information that is important or helpful to the driver. "Rider is elderly and may take a few minutes." Or "Rider wearing a yellow hat."
3. Select standard ride or wait and save if the patient needs more time to get ready.

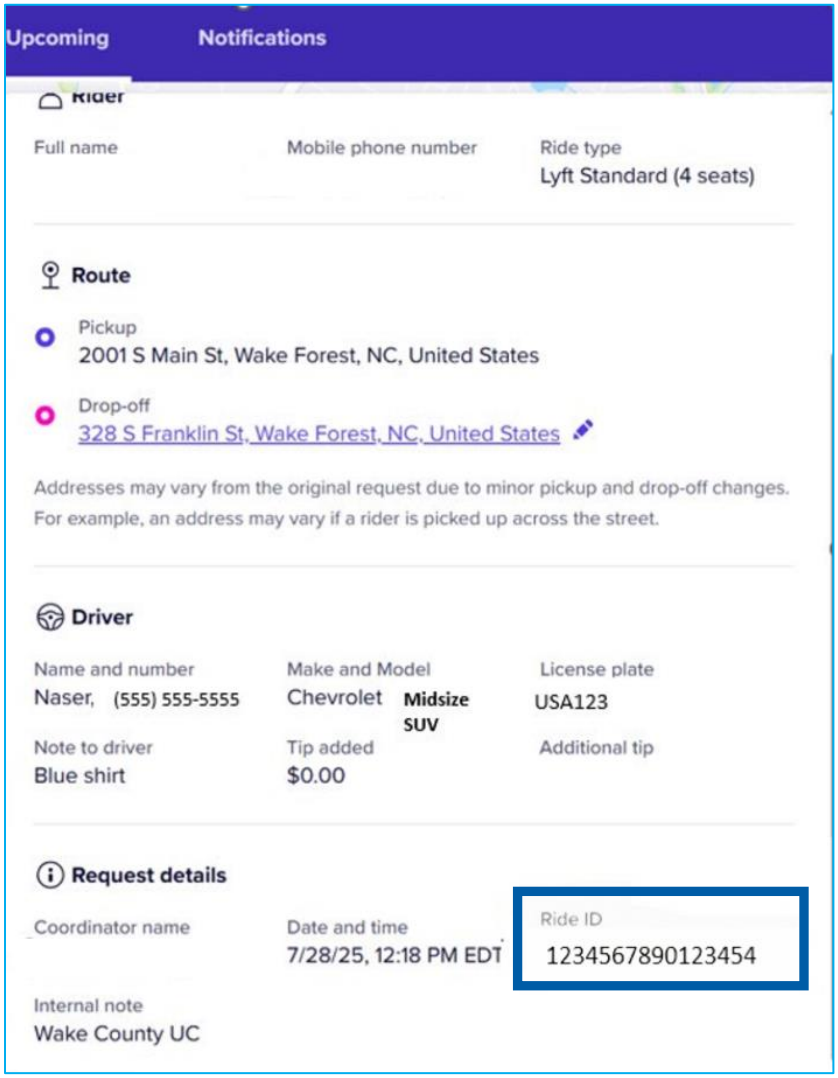


4. Enter request notes (Internal note):
   - You must enter the contract and destination.
     - Ex: Seattle-ER, Knox-Pharmacy, Wake-Urgent Care, etc
5. Select Request Ride for driver information



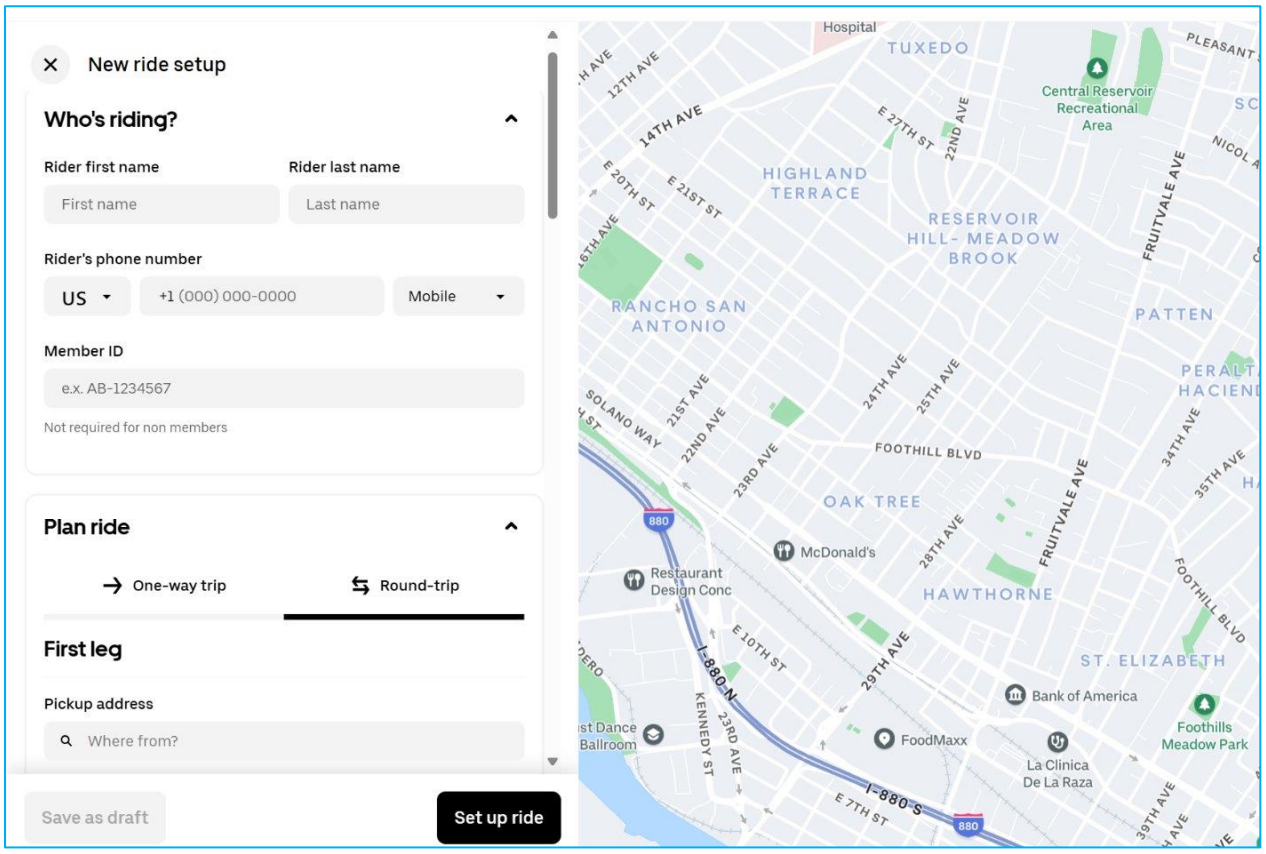
Driver Details

- Patients usually receive a text with driver info but if they have not, provide name and car info with ETA.
- You *MUST* document the Ride ID in your journal as well as confirmation that your patient has reached their destination.

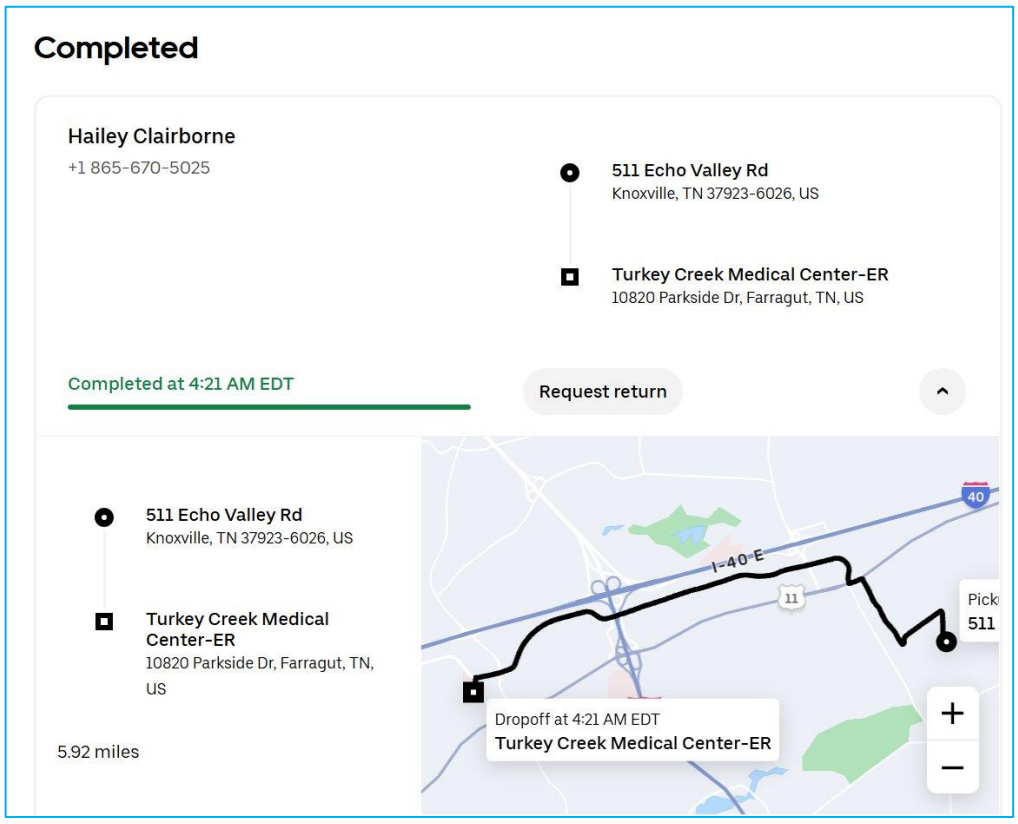


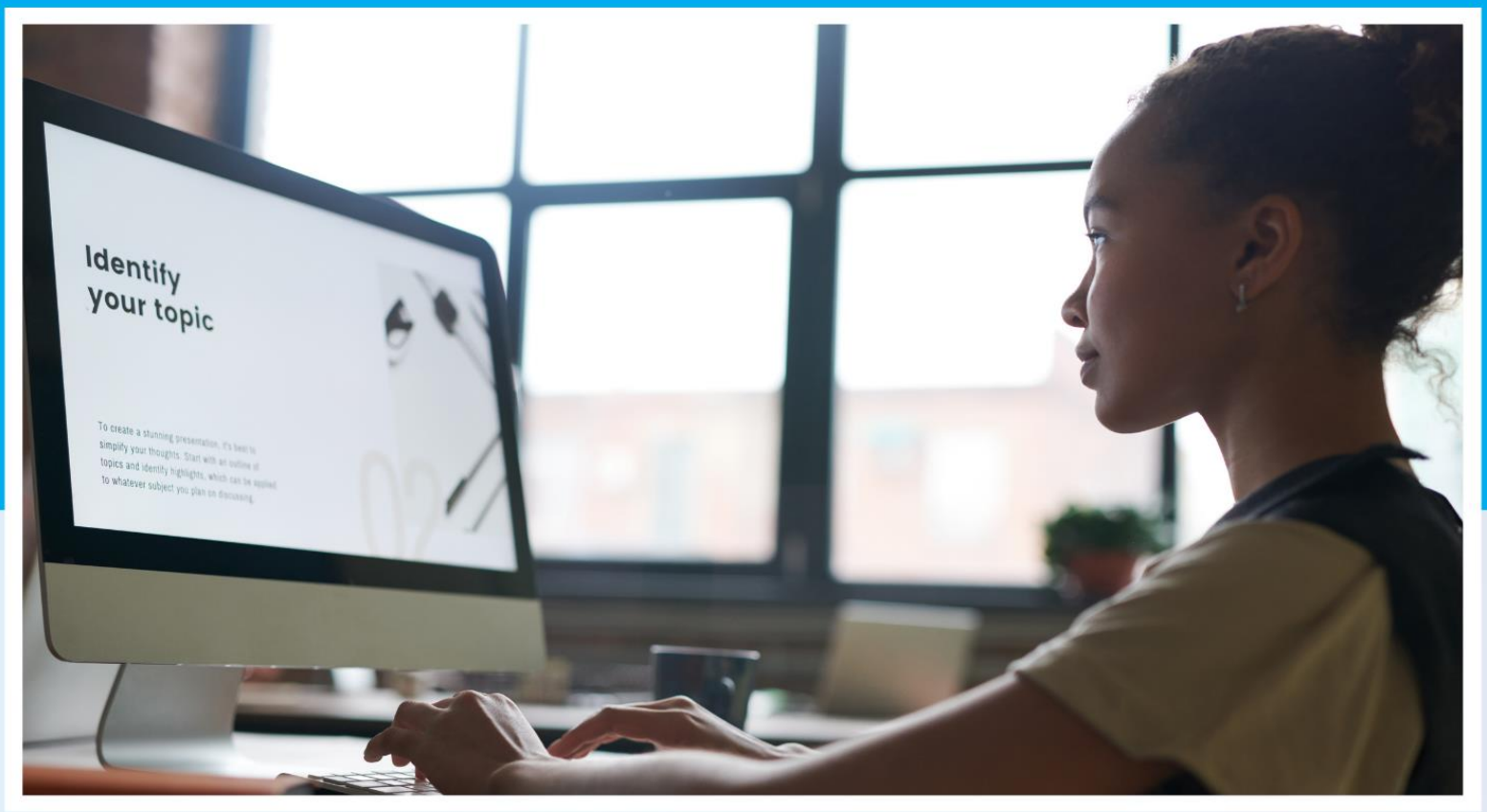
The Uber Alternative

Same process, different platform.



1. Confirm the rider arrived at their destination
2. Document
   - ➢ Document Trip ID to the journal.
     - Example: Abc12345-abc1234-abc123456-abc123





System Applications

In This Section

Can’t remember how to do a warm handoff in Genesys? Need a refresher on how to edit your signature? Systems and Applications is your stop for all programs and platforms that make your job easier.

Reference Topics

- Workstation Setup
- Ultimate Kronos Group (UKG)
- Genesys
- CAD-to-CAD
- Logis
- Crisp DC
- DC Taxi

Workstation Setup

Introduction

This section will explain how to access and set up vital applications such as Teams, Okta, and Outlook. Learn how to customize your Outlook signature and Teams notifications.

Logging In

- Log onto the workstation with your username (email address) and temporary password
  - Your email address is usually: firstname.lastname@gmr.net
- Your temporary password is your:
  - [Birthmonth][Last 5 of SSN]!
  - (no spaces; capitalize firstletter of month)
- You will be prompted to create a new password and set up forgotten password questions

Applications Access

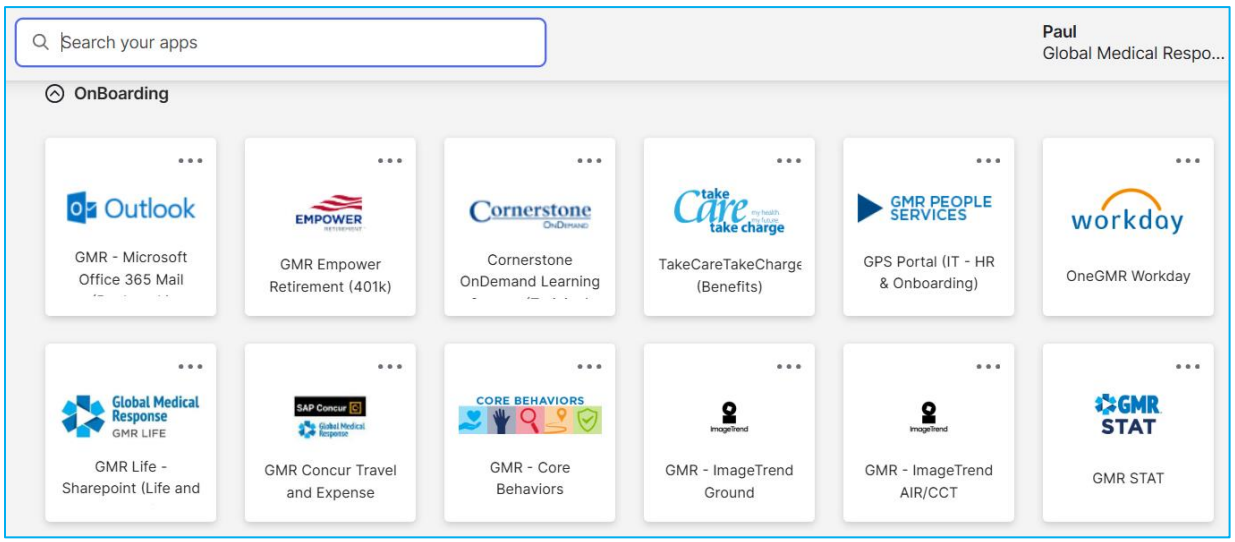
Okta	Teams	Outlook
Digital applications hub used to access Teams, Outlook, Cornerstone, Genesys, and more!	Used to communicate with your trainer, supervisor, and team. Also use to access NN-specific resources.	Review emails regarding organization updates and announcements.

Okta

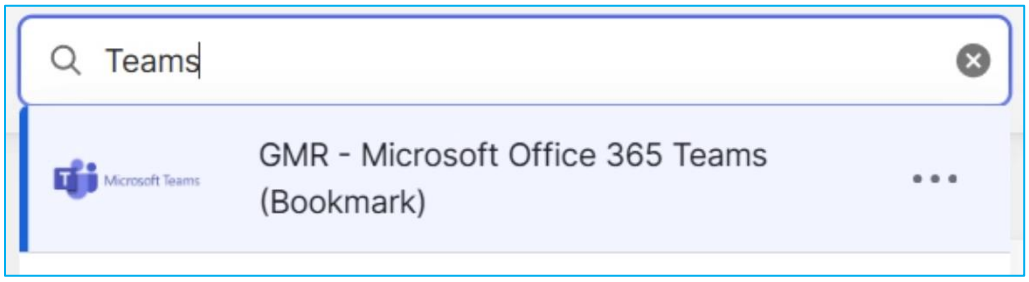
To access Okta, open Microsoft Edge and navigate to: https://gmr.okta.com/

- Enter your GMR username, password, and use Google Authenticator to login

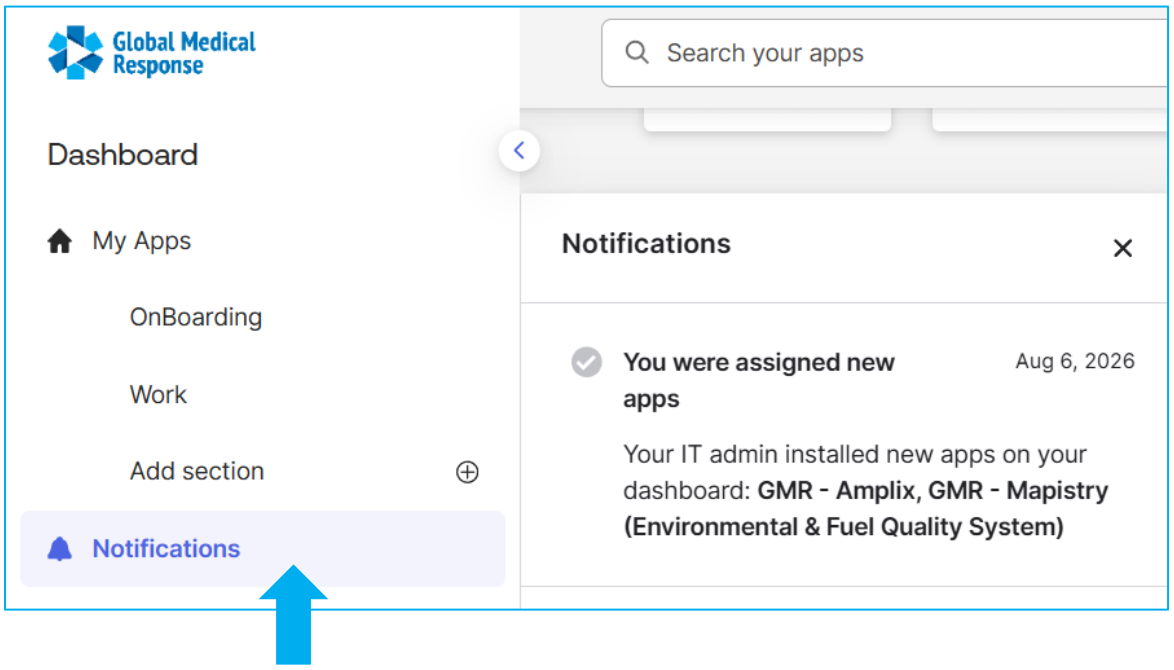
Use Okta as to quickly find and open job-essential web applications:



Can't find the app you're looking for? Use the search bar at the top of the page:



When new apps are added to your Okta dashboard, you will receive a notification on the left-side navigation menu:



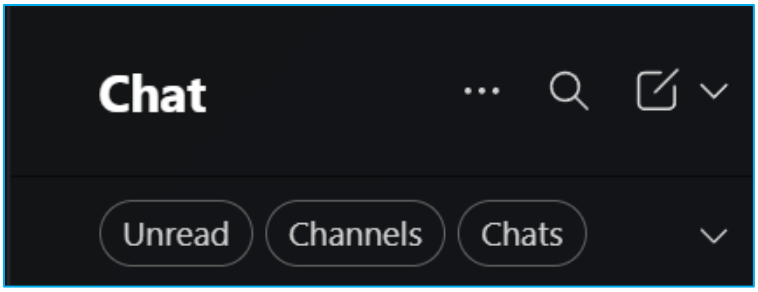
Why Use Okta?

Okta ensures a safe, secure, and seamless navigation experience when you switch between applications.

Teams

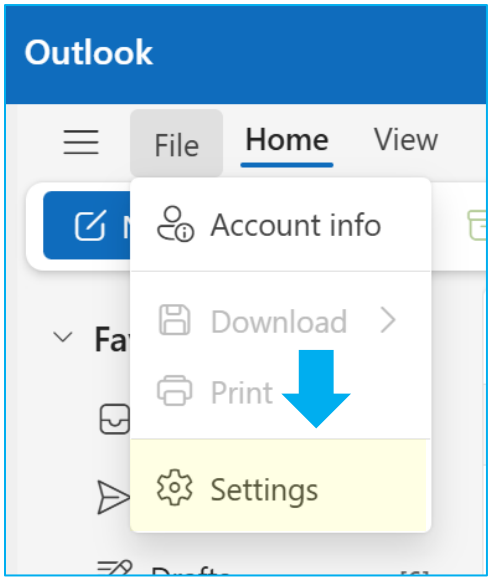
Access Teams from Okta (and bookmark for future use)

- Teams can filter your communications into Unread, Channels, and Chats

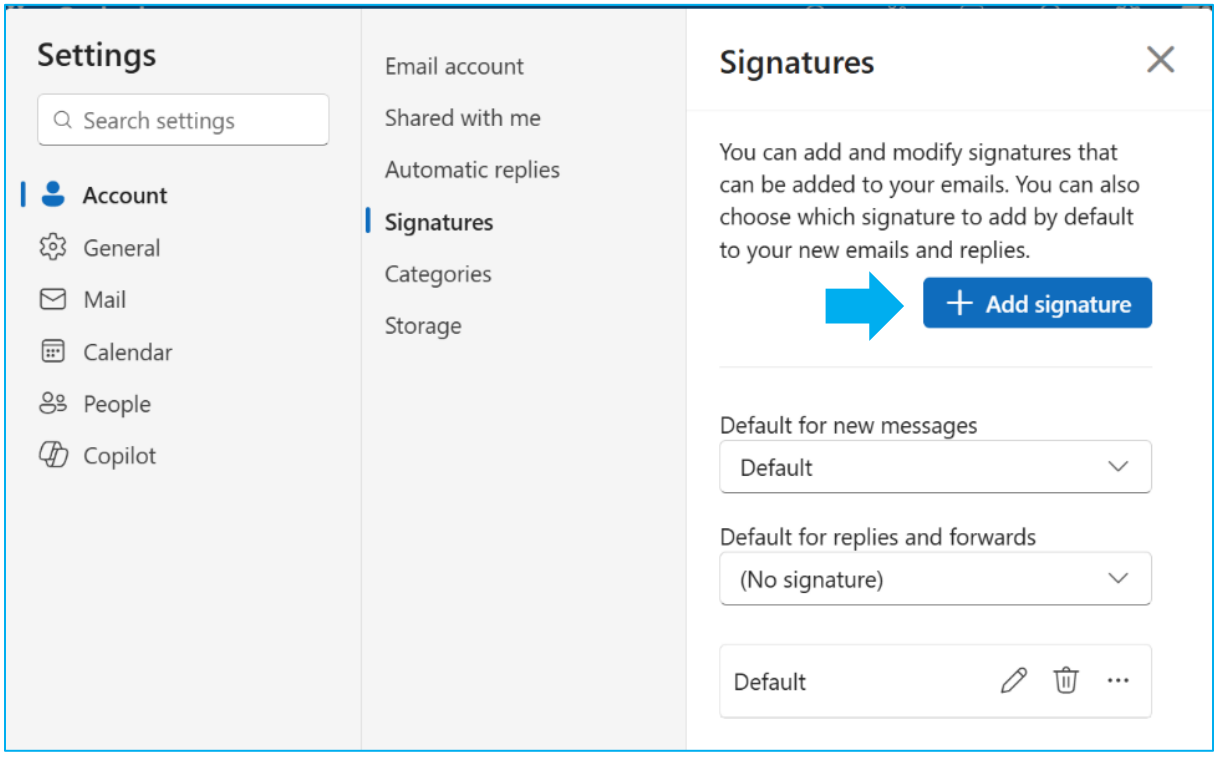


  - Select the Unread filter to display any unread message you have received in a chat or channel
  - You will be added to organization-specific channels, such as NN Academy, to help you access resources necessary to do your job
  - Select Chats to view individual or team chats that are not affiliated with an organization-specific channel

Setting your Signature in Outlook



1. Open Outlook from your web browser
2. Under File, select Settings
3. Under Account > Signature, select “Add Signature”

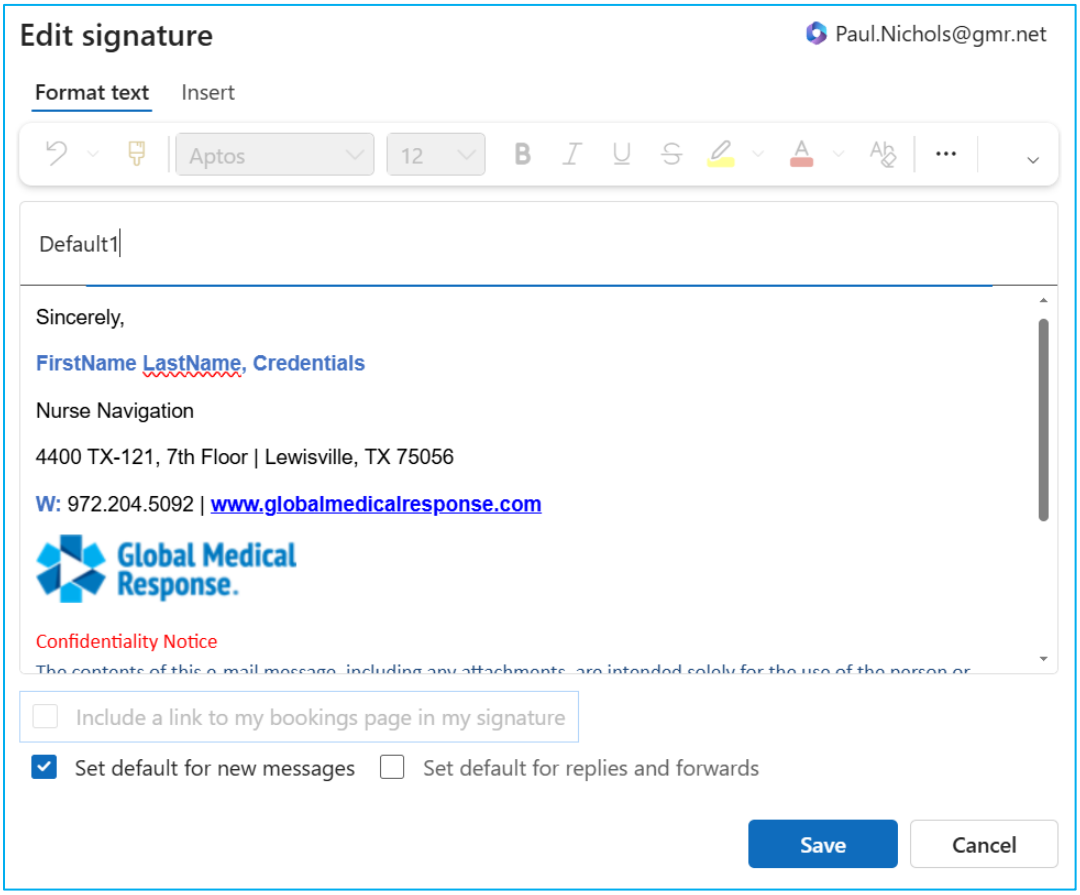


4. Name your signature
    - Example: “Default”
5. Copy / Paste the email signature located in the NN Academy Teams folder:

Folder Navigation

Day 1 > Policies > Workstation Setup

6. Once pasted into Outlook, edit the signature to match your information
    - Your name and credentials
    - Nurse Navigator, 911 Nurse Navigation
    - Phone number
        - Lewisville: 972-204-5092
        - Phoenix: 623-201-6069
7. Check “Set default for new messages”
8. Save



Ultimate Kronos Group (UKG)

Introduction

Ultimate Kronos Group: This application is our nurses' clock in and out, check the schedule, and enter PTO requests. You can also clock in and out using the time clock on the wall near the printer.

Agents can use UKG to:

1. **Clock in and Out**
2. **Request Time Off**
3. **Review Attendance Records**

UKG helps managers balance staffing levels with expected call demand by:

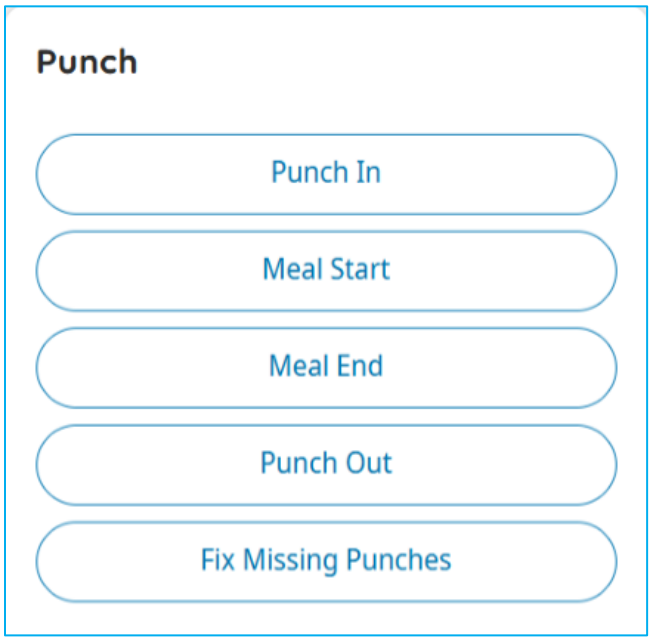
- **Forecasting Workforce Needs**
- **Identifying Staffing Gaps**
- **Managing Overtime**
- **Supporting shift swaps and schedule adjustments**

For additional reference, there are videos available in Cornerstone. While these videos provide a general overview of UKG and how request PTO / Shift swaps, please note that what we discussed was specifically related to 911 Nurse Navigation.

Troubleshooting	Schedule Alterations
For UKG-related issues: marika.adebayo@gmr.net	Scheduling issue, PTO / Shift swap / Overtime request: Helen.jackson@gmr.net

Clocking In and Out

Click ***Punch In*** to clock in for your shift. Click ***Punch Out*** at the end of your shift. We do not clock in or out for lunch.



You cannot clock in any earlier than 10 minutes before your shift start time without management approval.

Missing Punches

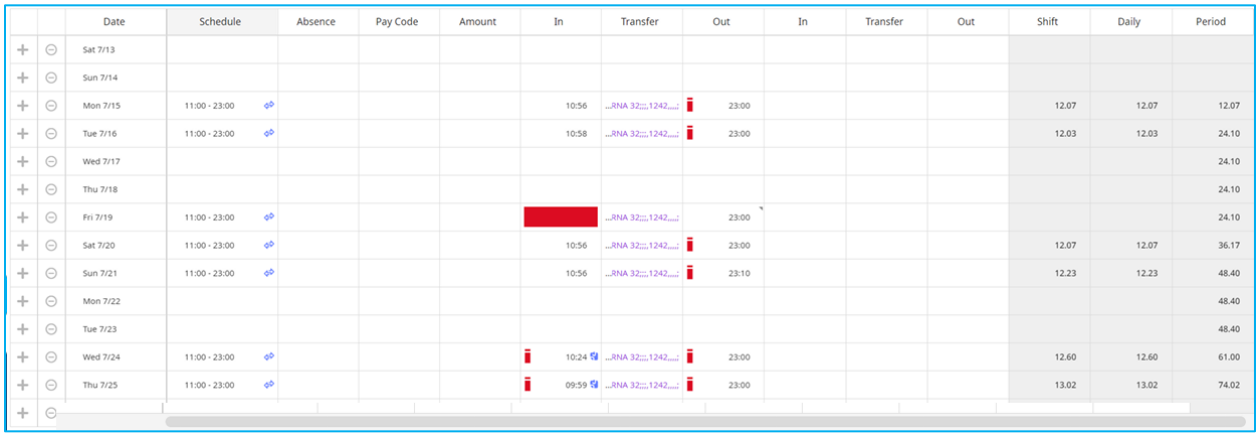
The ***Fix Missing Punches*** button is used if you forget to clock in or out. It allows you to enter the missing time to ensure your hours are correct.

- If you arrive on time but experience delays clocking in, do not punch in late. Instead, use the 'Fix Missing Punches' option at the end of your shift.
- When using ***Fix Missing Punches***, enter the date and time for the missed punch, then save your entry.
  - After submission, Sydney will review and approve the missing punch request accordingly.
- Please allow Sydney time to review and approve your missing punch request.

Avoid Duplications

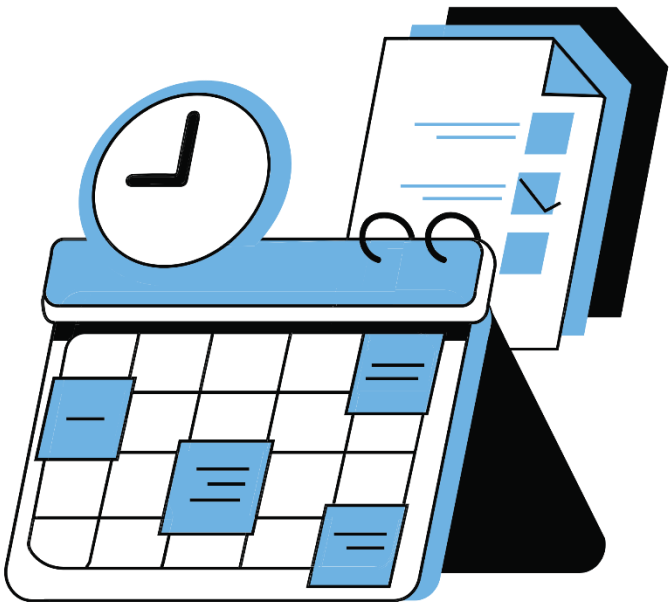
There's no need to submit it more than once. Submitting multiple requests can result in duplicates, which may lead to one being approved and the other denied after her reviews. To avoid confusion, it's best to submit your request only once and allow time for processing.

My Time Card



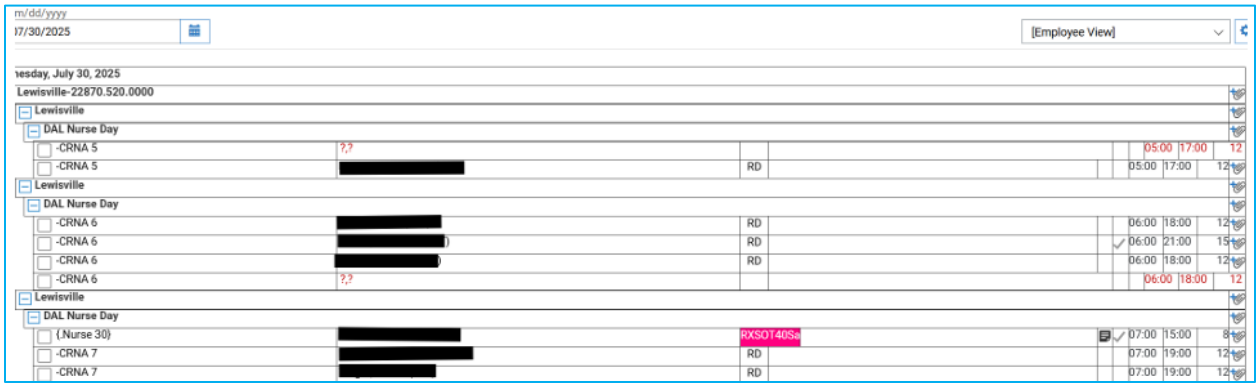
	Date	Schedule	Absence	Pay Code	Amount	In	Transfer	Out	In	Transfer	Out	Shift	Daily	Period
	Sat 7/13													
	Sun 7/14													
	Mon 7/15	11:00 - 23:00				10:56	...RNA 32;;;;1242;;;;;	23:00				12.07	12.07	12.07
	Tue 7/16	11:00 - 23:00				10:58	...RNA 32;;;;1242;;;;;	23:00				12.03	12.03	24.10
	Wed 7/17													24.10
	Thu 7/18													24.10
	Fri 7/19	11:00 - 23:00					...RNA 32;;;;1242;;;;;	23:00						24.10
	Sat 7/20	11:00 - 23:00				10:56	...RNA 32;;;;1242;;;;;	23:00				12.07	12.07	36.17
	Sun 7/21	11:00 - 23:00				10:56	...RNA 32;;;;1242;;;;;	23:10				12.23	12.23	48.40
	Mon 7/22													48.40
	Tue 7/23													48.40
	Wed 7/24	11:00 - 23:00				10:24	...RNA 32;;;;1242;;;;;	23:00				12.60	12.60	61.00
	Thu 7/25	11:00 - 23:00				09:59	...RNA 32;;;;1242;;;;;	23:00				13.02	13.02	74.02

- You are expected to check your timecard at the beginning and end of your shift.
- If you encounter issues submitting the correction, take a screenshot (using the Snipping Tool) and email the issue to Sydney the image along with a completed Time Exception Form so Payroll can address the issue.
- You can check your PTO balance by selecting the Accruals option at the bottom of the page in this tile.



Telestaff Roster Tile

This tile displays who is scheduled and their start and end times.

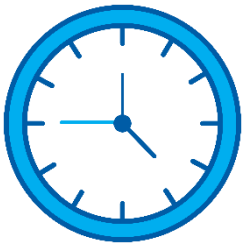


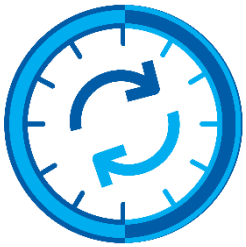
Telestaff Roster codes you will see:

"RD" Regular Duty
A Standard Shift

RXSOT40Sa
An Overtime Shift

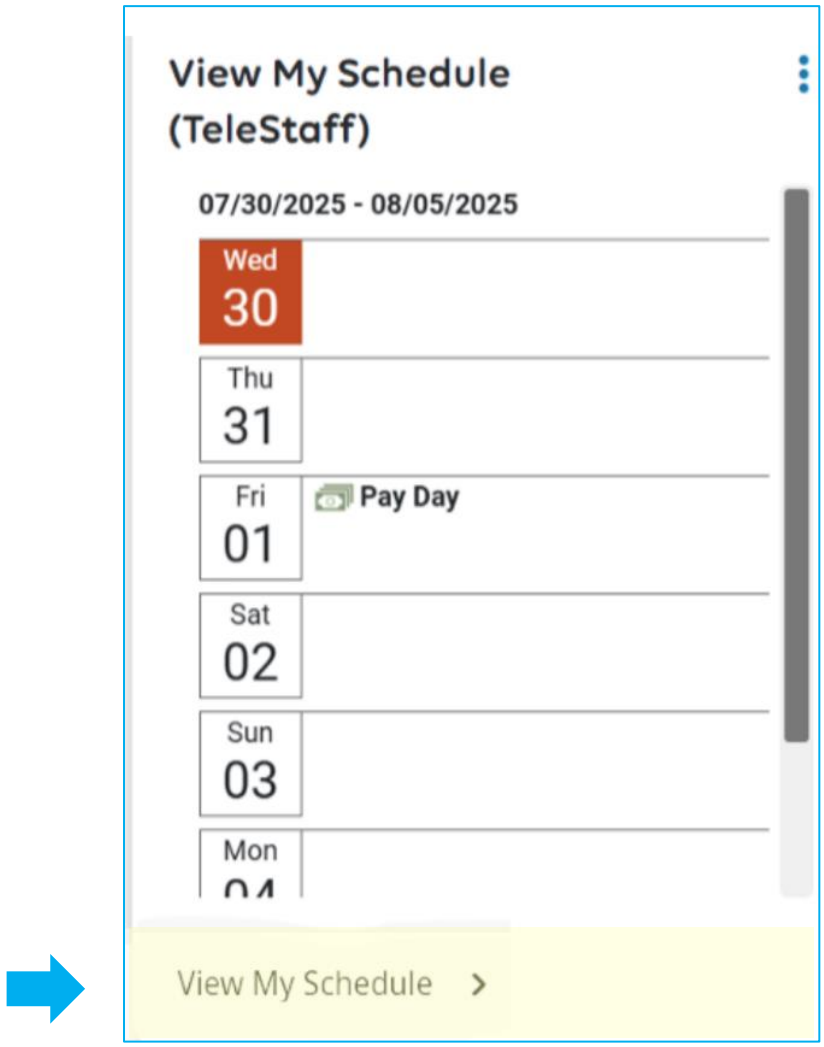
STW
Shift Trade Working
(Shift swaps)





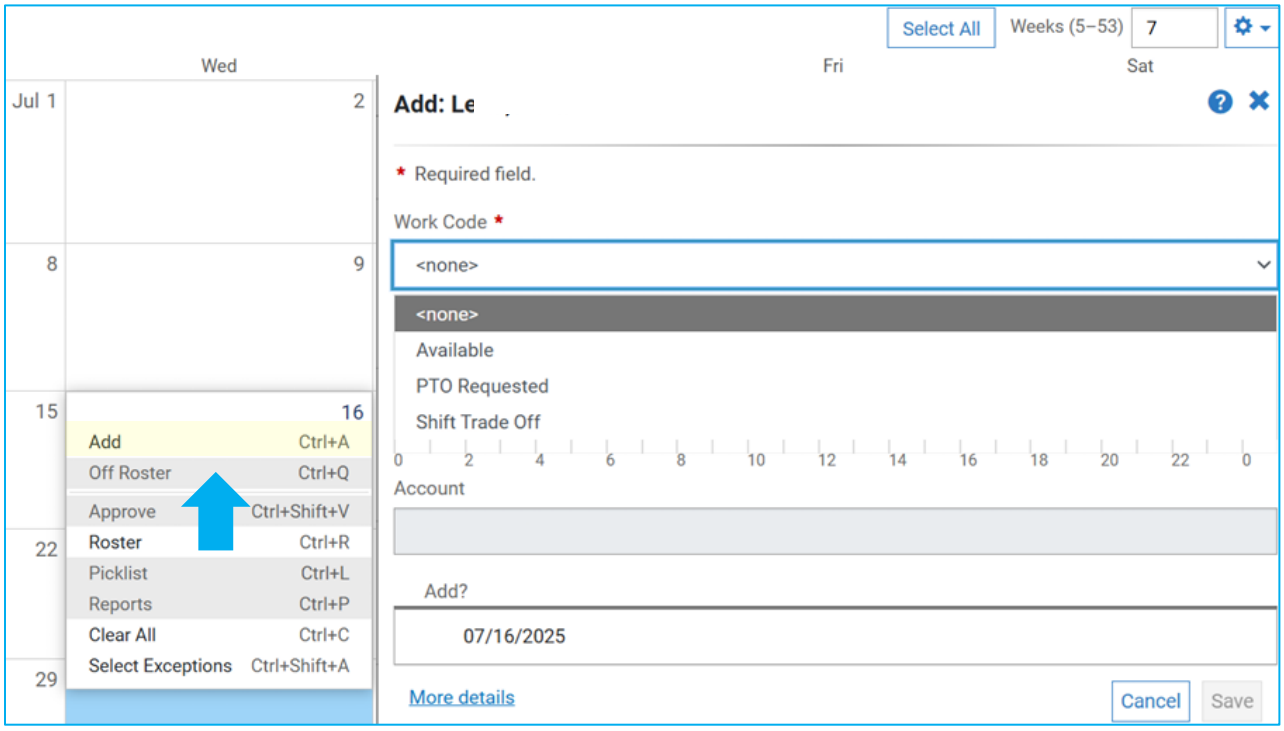
Requesting Time Off or Shift Swap

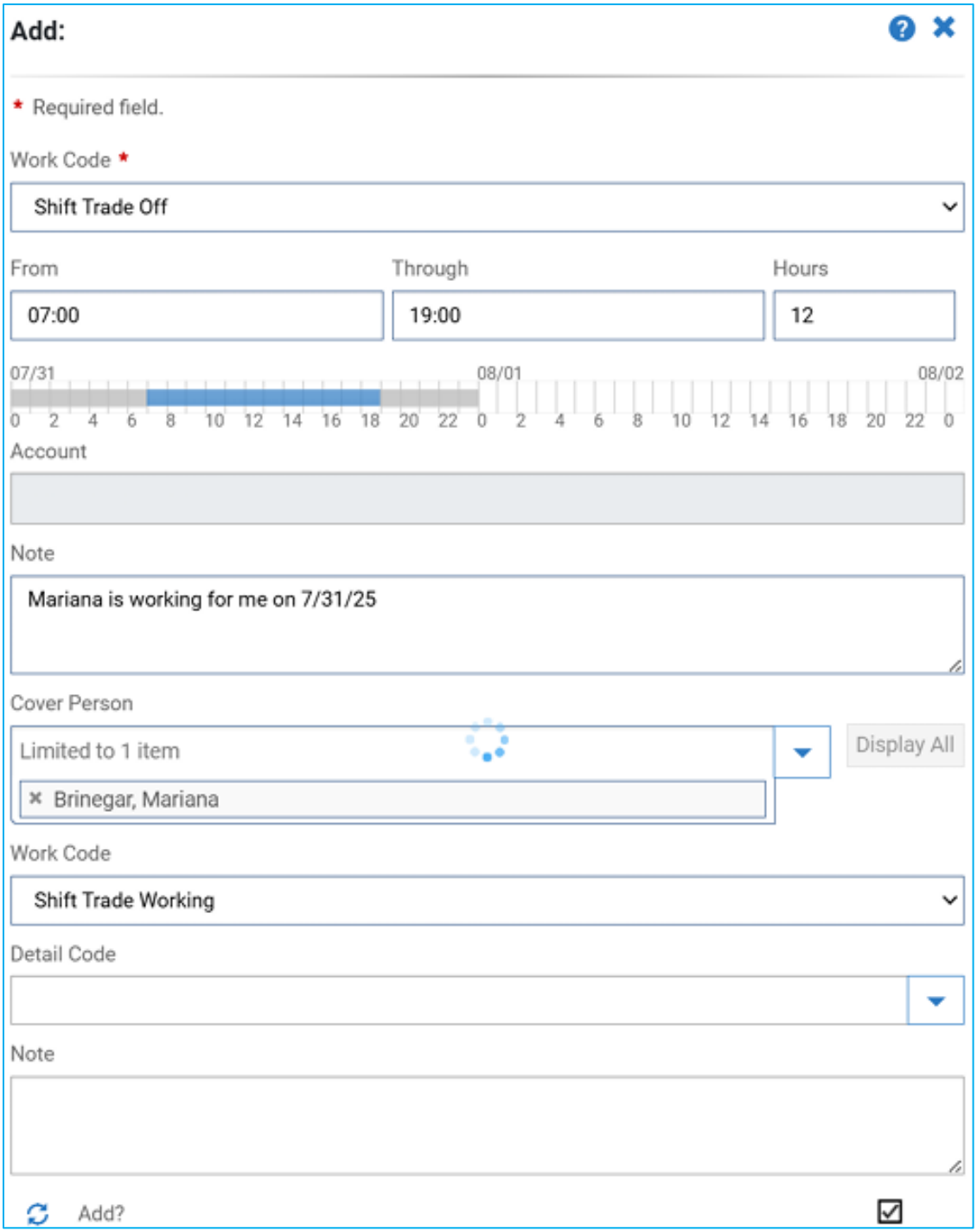
- On the View My Schedule tile, select view my schedule.
- This tile shows your current and upcoming shifts.
- You can also submit PTO and shift swap requests here.
  - Please follow the PTO Policy for all PTO and shift swap requests.



PTO / Shift Swap

- Click on the day you want to request. Select Add.
- Choose if it is PTO or Shift Swap request.
- The box will expand to include more areas to complete.





You are not allowed to request OT or shift swaps during the training, and you must wait until week 10.

Genesys

Introduction

Genesys is our phone and communication system used to manage inbound and outbound calls, customer interactions, and team collaboration.

The platform:

1. **Helps ensure calls are routed to the right team members**
2. **Provides access to call history and reporting**
3. **Supports a seamless customer service experience**

Genesys call routing ensures customers are connected to the right person as efficiently as possible. The system uses information gathered from the caller, business rules, agent skills, and real-time availability to determine the best destination for each call, helping improve both customer experience and operational efficiency.

To learn more about how we are able to help our patients once their call is received, please see the Workflows section of this document.

Quick Links

Select a topic to view that section of the Genesys tutorial:

Introduction

- Definitions
- Sign Into Genesys
- Cloud Components
- Set Notification Preferences
- Sound Preferences
- Public Profile Overview
- Log Out of Genesys

Interactions

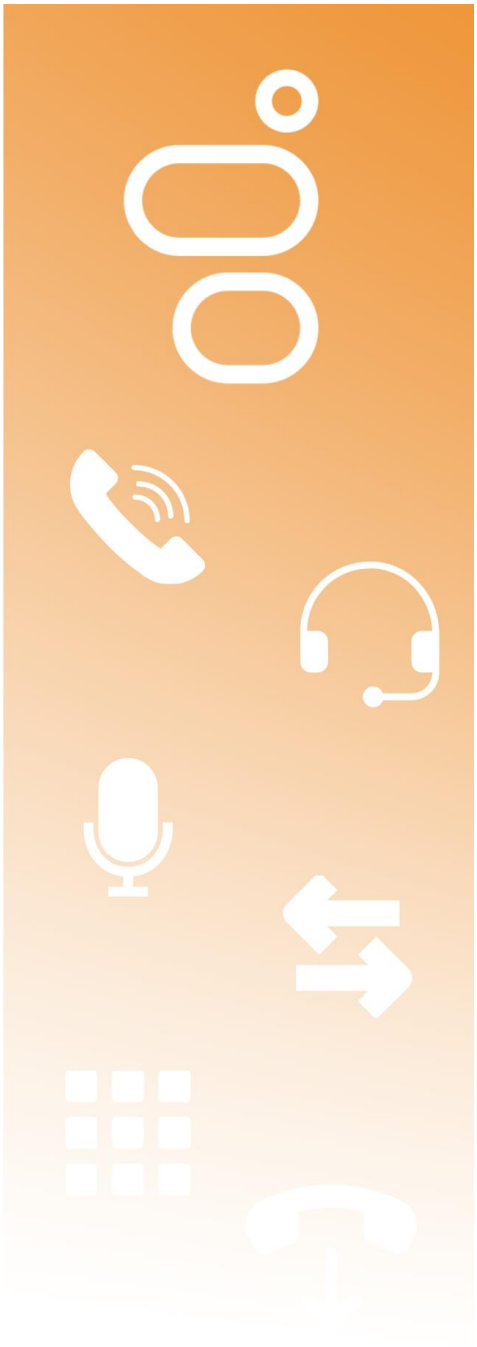
- Non-ACD (Internal) Calls
- ACD Interactions on Behalf of Queue
- Call Dashboard
- Call Consults & Conferences

Your Status & Activities

- Check Your Status
- After Call Work (ACW) & Wrap-Up Codes
- Failing to Answer an Interaction
- View Performance & Queue Activity

Troubleshooting

- Common Errors



Definitions

Genesys Cloud

An interaction management application that supports managing interactions including voice calls, voicemail, conference calls, chat, video calls, and email; media types may vary by organization, department, and / or queue.

WebRTC Phone

WebRTC Phone runs directly from the internet browser with no special hardware requirements or additional software; a user's computer acts as their phone.

Queue

A "waiting line" of interactions ready to be answered.

Non-ACD Call Menu

The menu and associated calls that are on behalf of a queue.

Interaction

Communications (calls) routed through the Queue to eligible nurses.

Inbound Call

Calls arriving to 911 Nurse Navigation.

Outbound Call

Calls made from NN to an internal or external contact.

Consult

During a call, this is an option in Genesys that allows a contact to be placed on hold while a secondary contact is reached. It is possible to warm-transfer a caller with this feature.

Conference

During a call, this is an option in Genesys that allows all parties to speak to, and hear, one another in real time. It is possible to transfer a contact using this feature.

Wrap-up Code / After Call Work (ACW)

Required ACW that appears after a call has ended on behalf of a queue.

Agent Workspace

The main working component of Genesys Cloud where external inbound and outbound interactions are managed.

Cold Transfer

Cold transfers will forward the caller to another contact without introduction or confirming their ability to take the call.

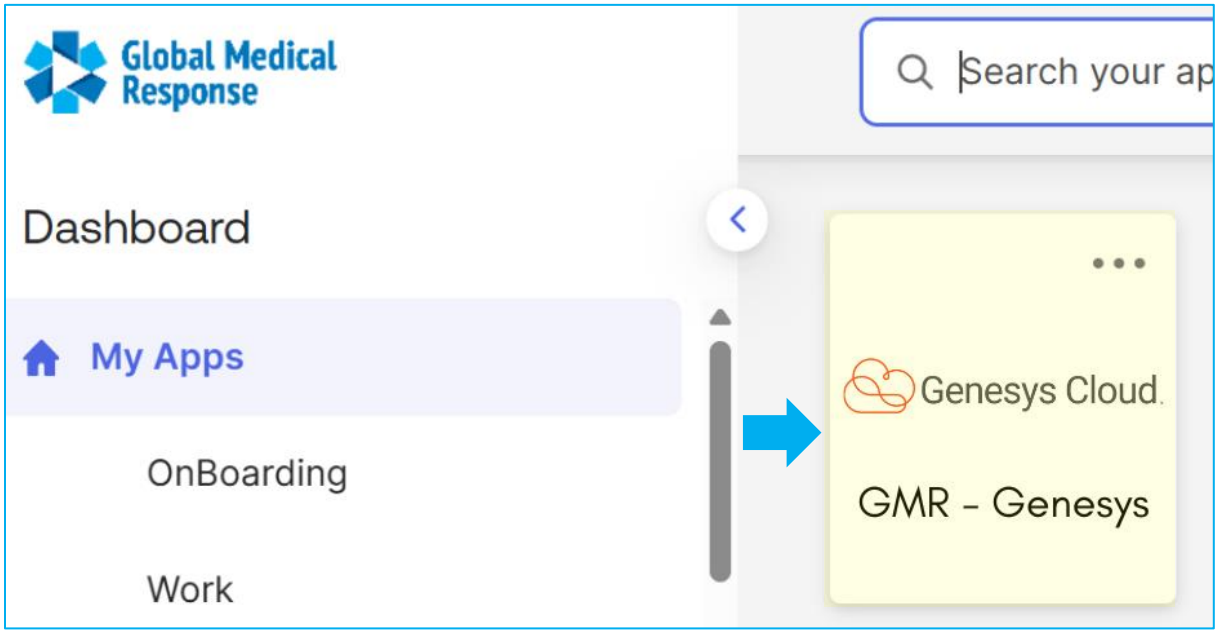
Warm Transfer

Outreach is made to another contact before the caller is transferred. This allows us to provide caller details, confirm the contacts availability to take the call, and summarize the caller's concerns.

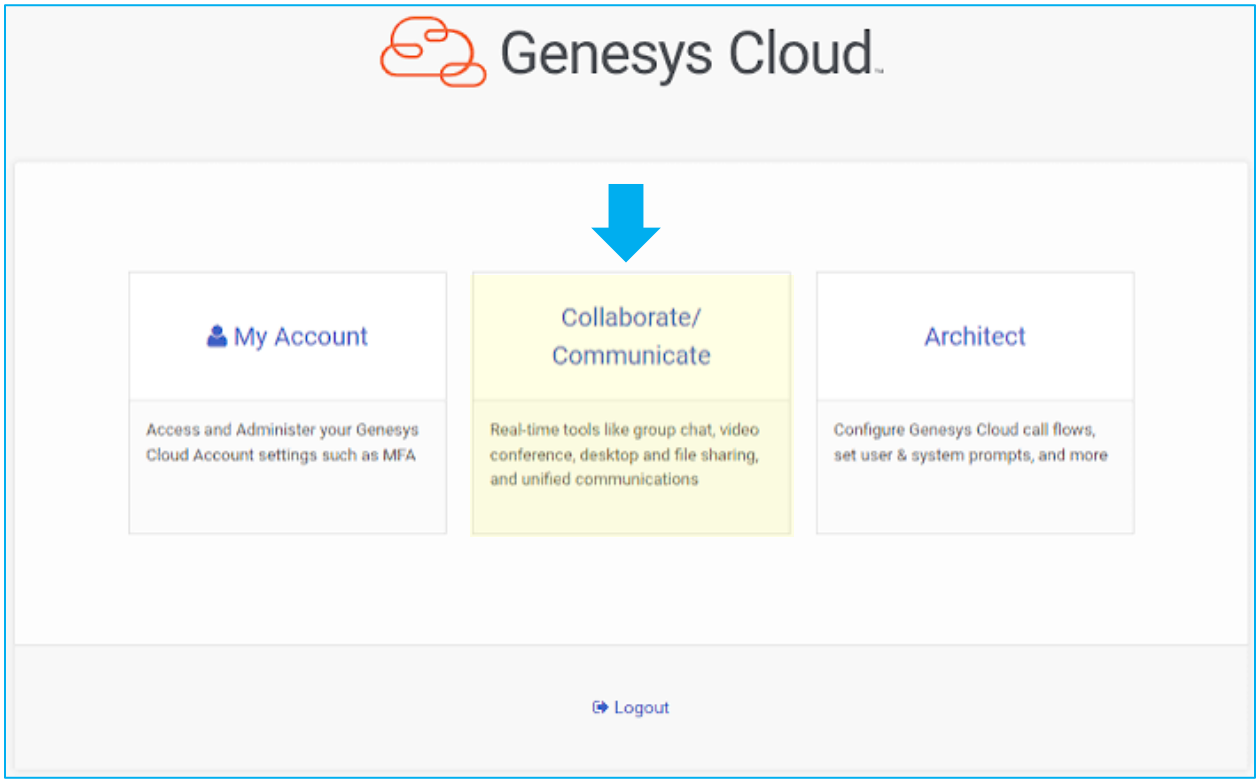
Sign Into Genesys

1. Login to Okta:
   https://gmr.okta.com/
2. Click on the Genesys chicklet.

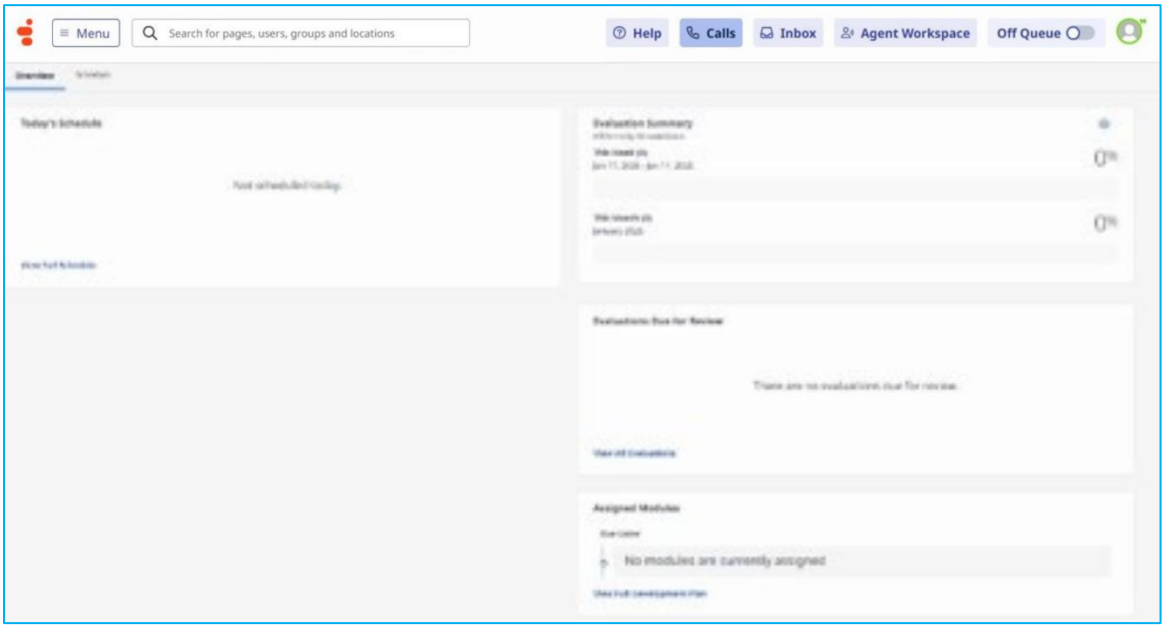
Please use Google Chrome when accessing Genesys Cloud.



3. When prompted, click Collaborate / Communicate.

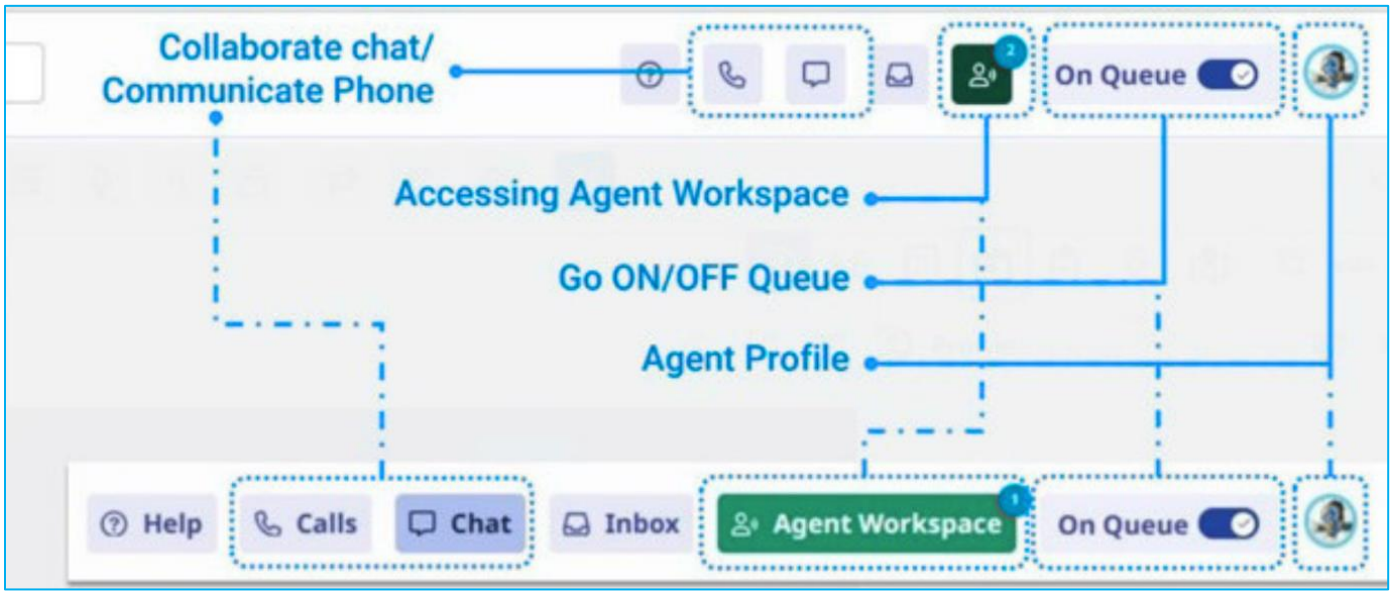


4. A successful login will direct you to this screen:



Cloud Components

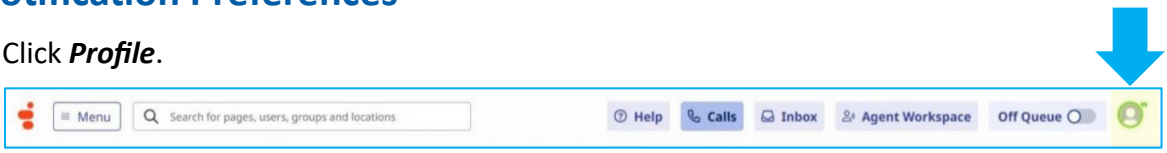
Components / Icons may vary depending on screen resolution, browser window size, or browser zoom level:



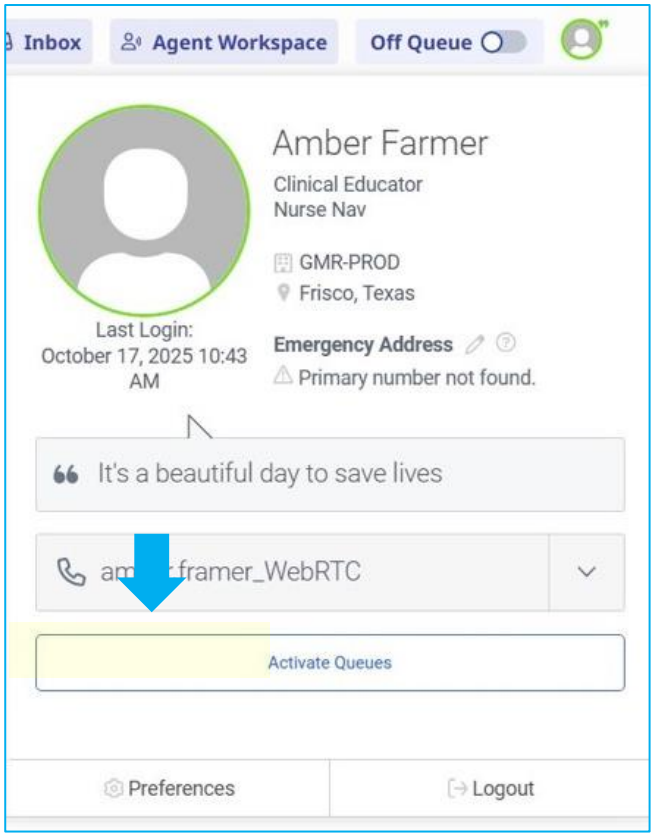
Your ***PROFILE ICON*** (located in the upper-right corner of the Genesys window) is ***ESSENTIAL*** for setting your preferences, status management, and logging out of Genesys.

Set Notification Preferences

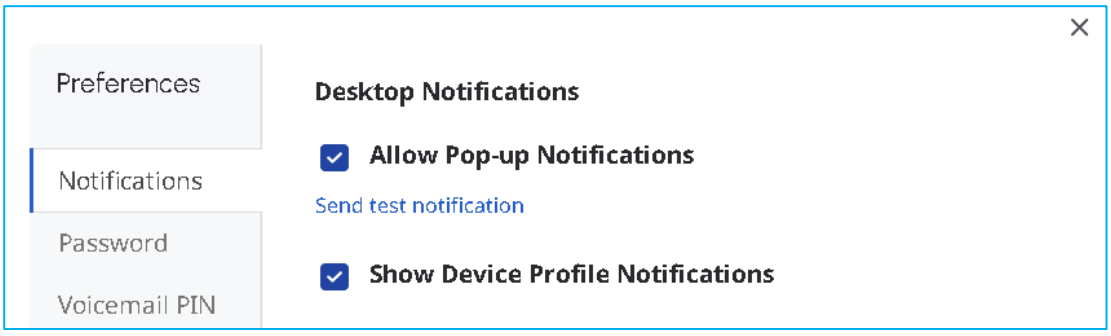
5. Click ***Profile***.



6. Click ***Preferences***.



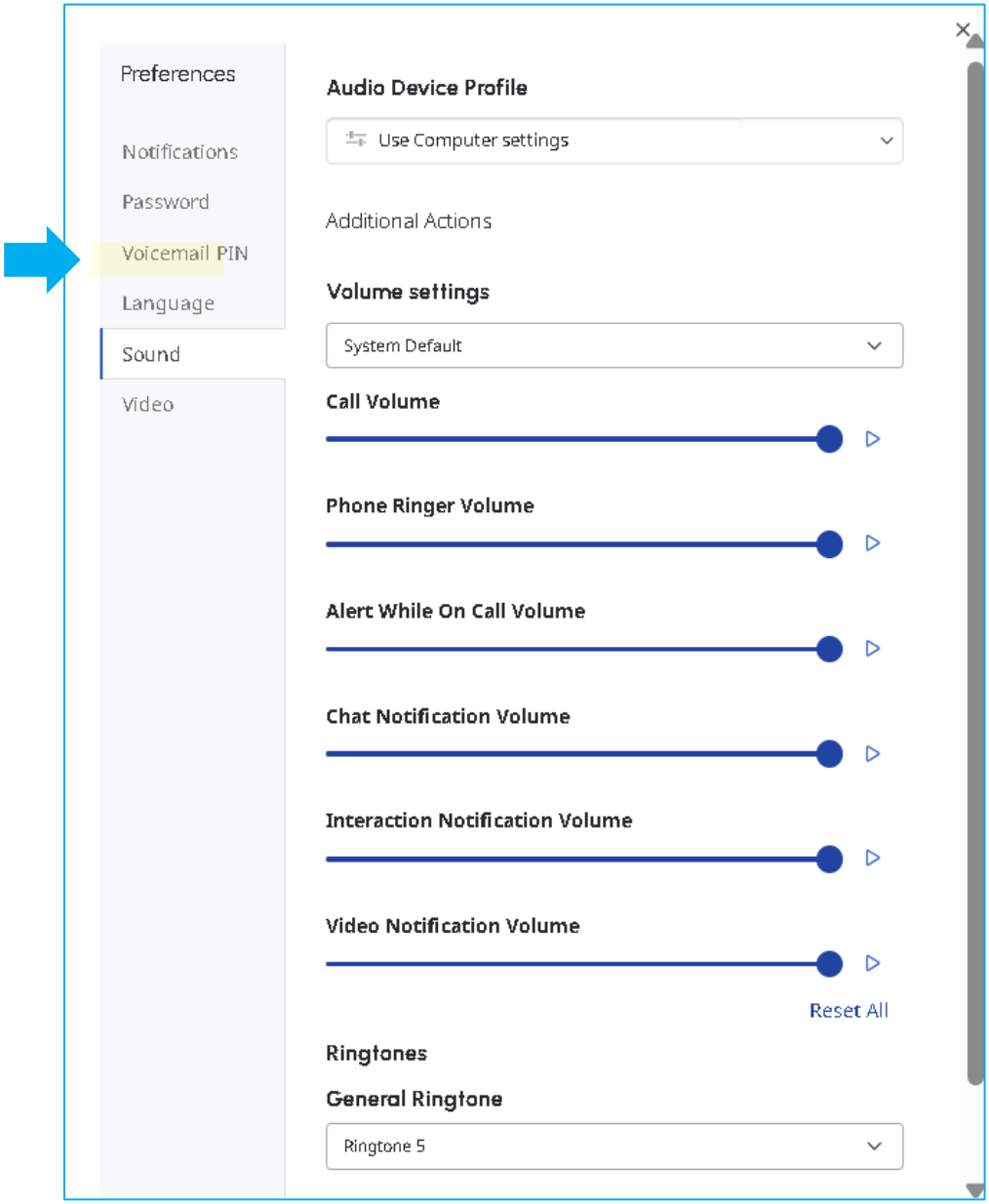
7. Ensure Notifications is selected from the left.
8. Check the box for Allow Pop-up Notifications.
9. Users may also need to enable pop-ups in their browser.



Sound Preferences

By default, Genesys will match the Sound Settings on your system.

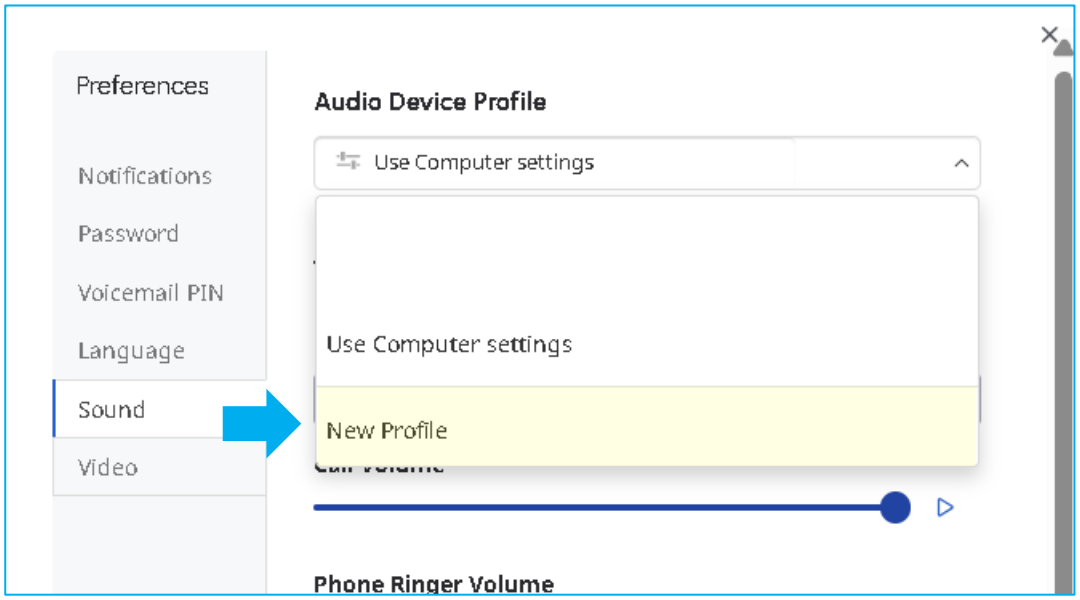
1. From the ***Preferences*** window, click ***Sound***.



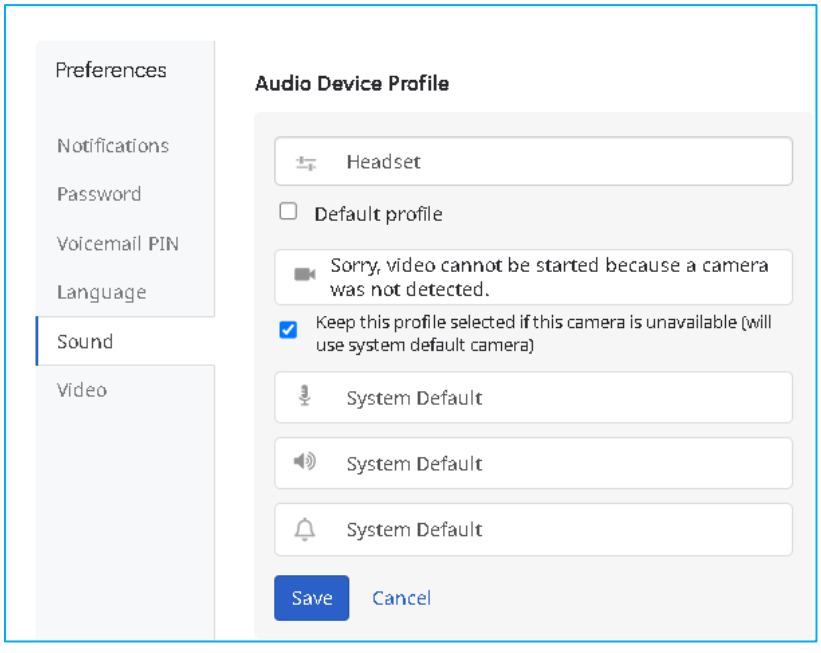
2. Review the Audio Device Profile at the top.
3. The profile listed in the dropdown is being used for calls, speakers, and microphones.

To create a new Sound Profile:

1. Select ***New Profile*** from the Audio Device Profile dropdown.



2. Name the ***Profile*** something recognizable:
   - Follow the steps in the Review Sound Preferences document
3. Set ***Microphone***, ***Speaker***, and ***Alert*** selections.
4. Click ***Save*.**

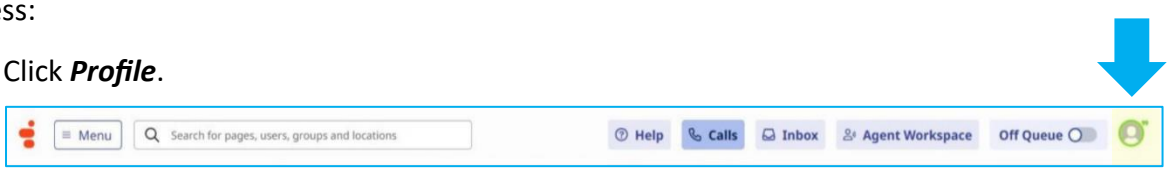


Public Profile Overview

Information about a user that is visible to all the organization's Genesys Cloud Users

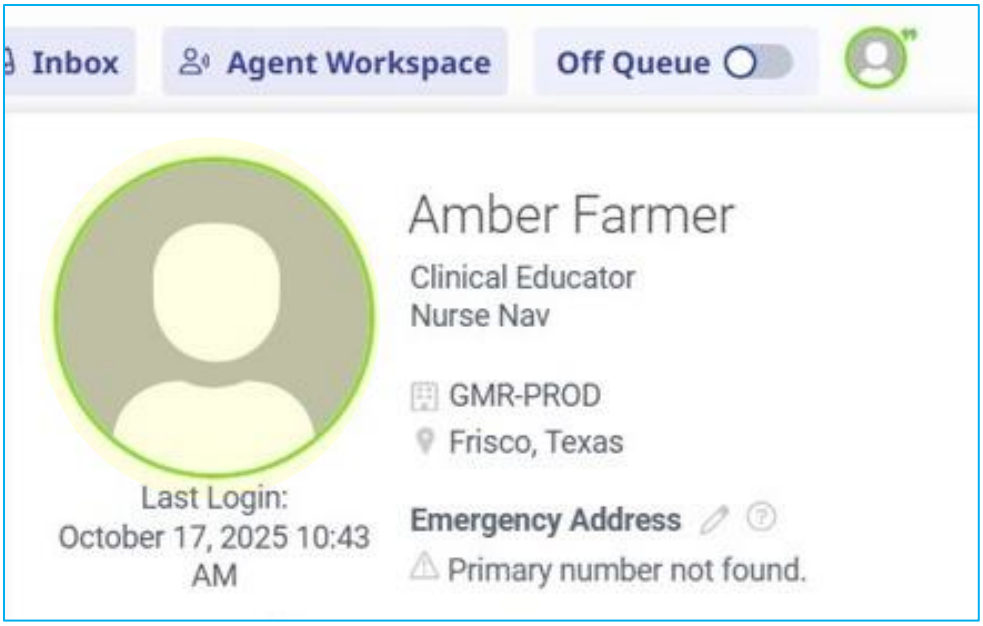
To access:

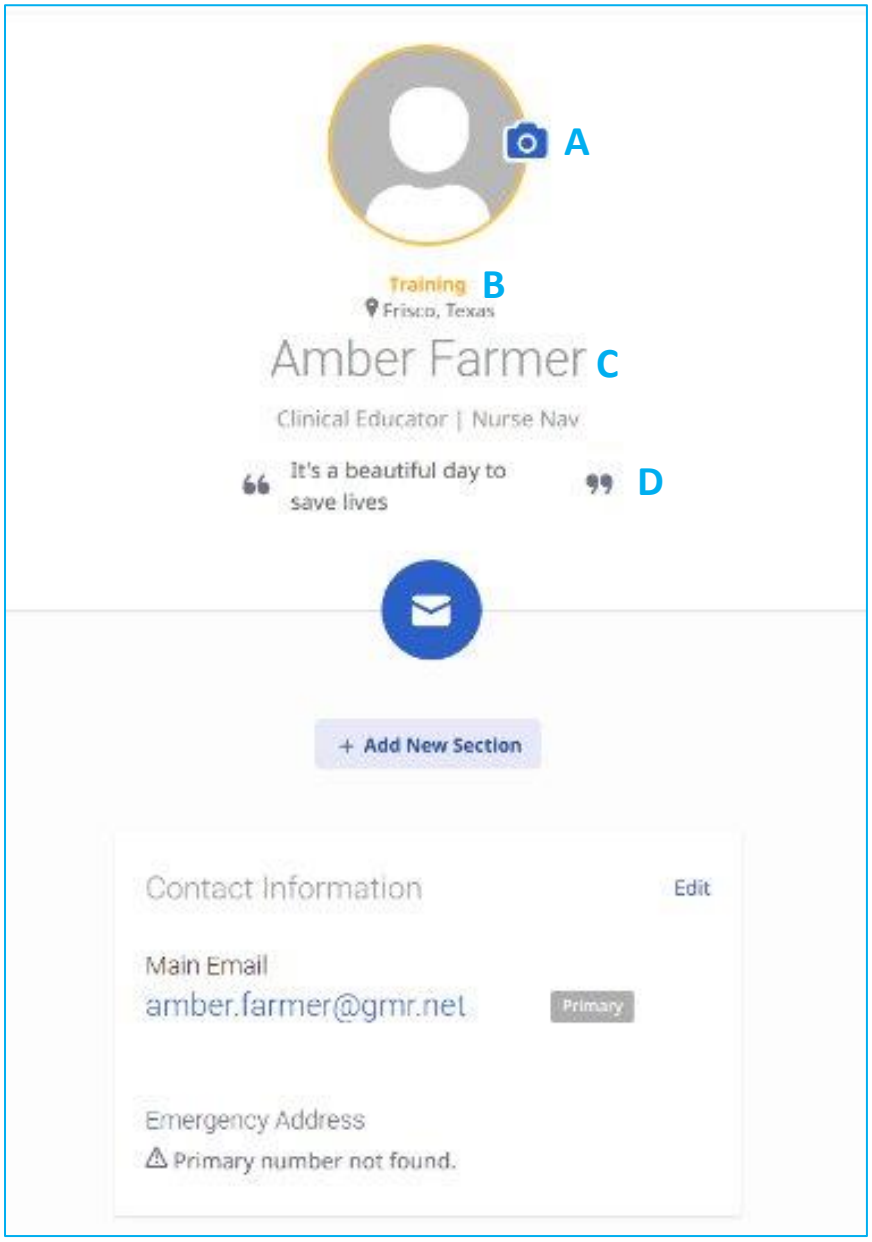
1. Click ***Profile***.



2. Click ***large profile icon*** or circle.

   Here, you can review your public profile as seen by others in the organization:





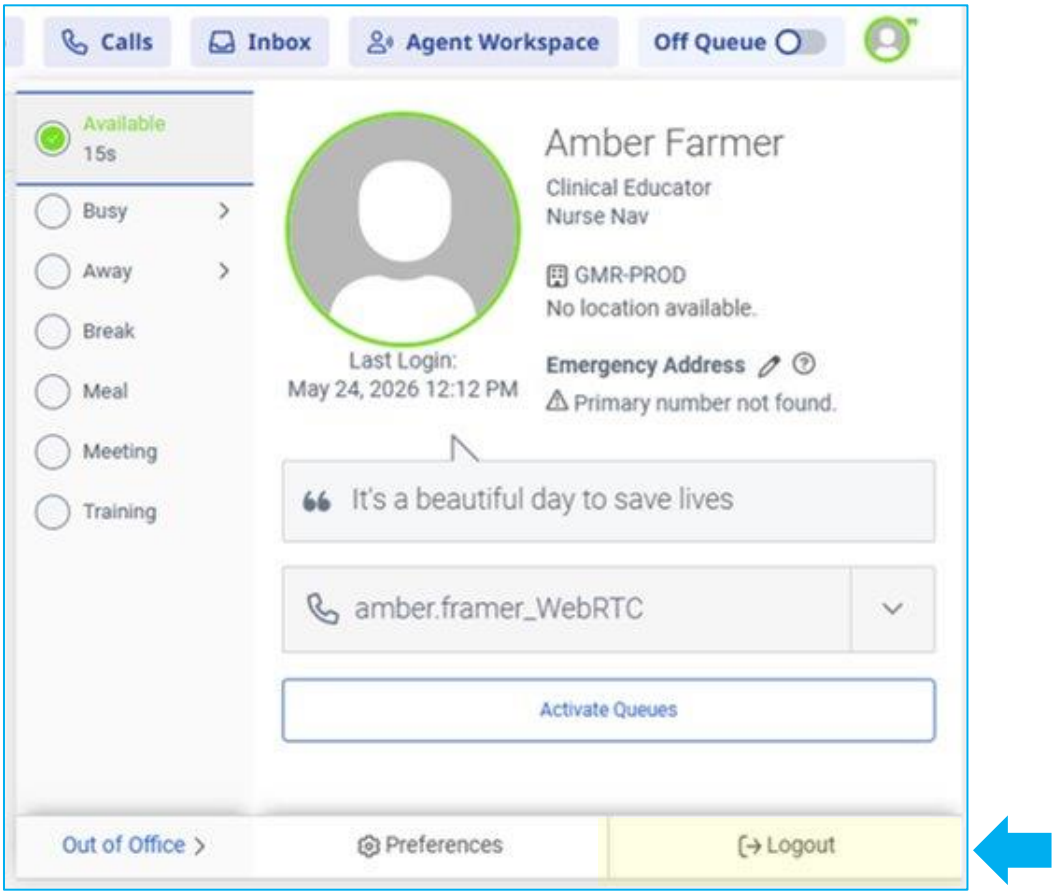
A. Change Profile Picture

B. Status

C. Your Name

D. Status Note

Log Out of Genesys



1	2	3	4
Go ***Off Queue***	Open your Profile Menu	Click Logout	Close your tab / browser

Wait for the browser to fully sign you out of Genesys before closing it.

Non-ACD (Internal) Calls

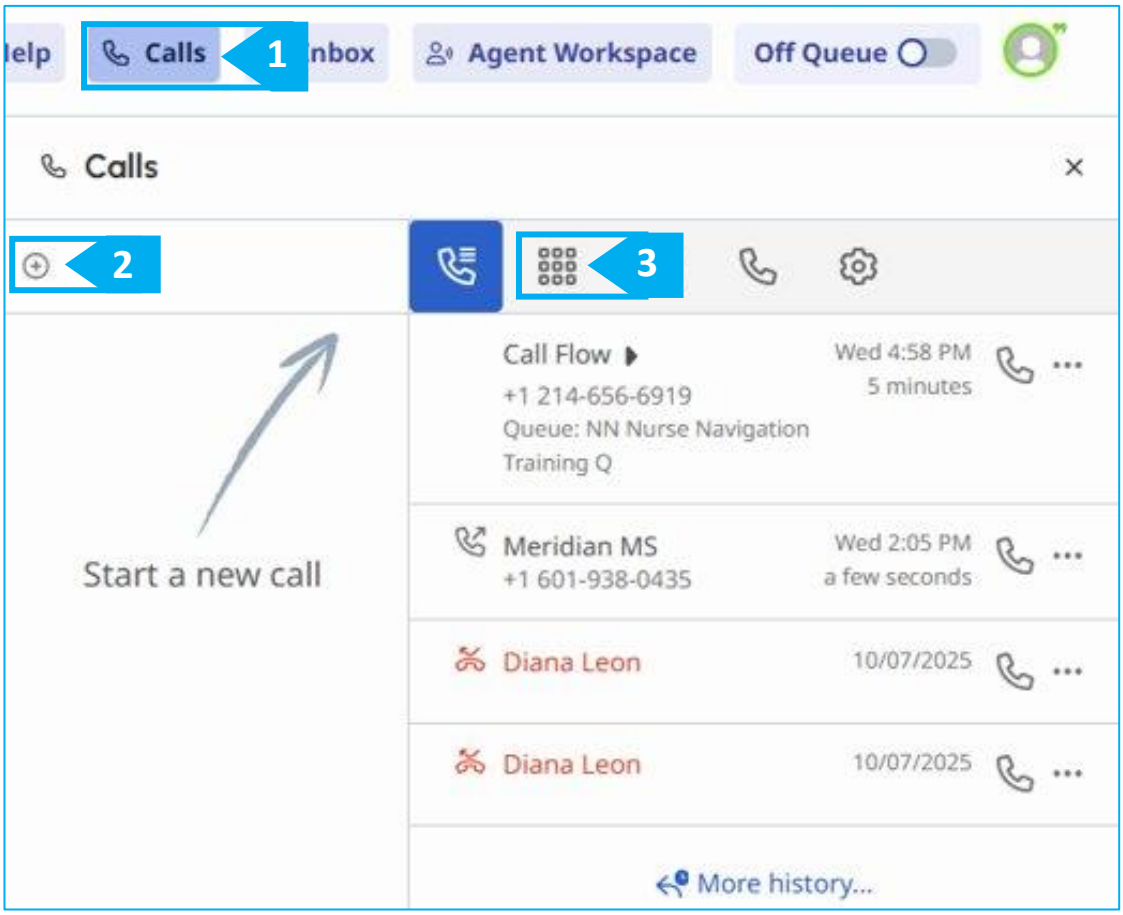
Outbound Non-ACD Calls

Use the ***Calls*** component to make non-ACD internal and outbound calls:

- These are calls NOT on behalf of a queue.

You will still be eligible to receive calls if you make non-ACD calls or calls **NOT** On Behalf of a Queue.

Outbound Non-ACD Calls



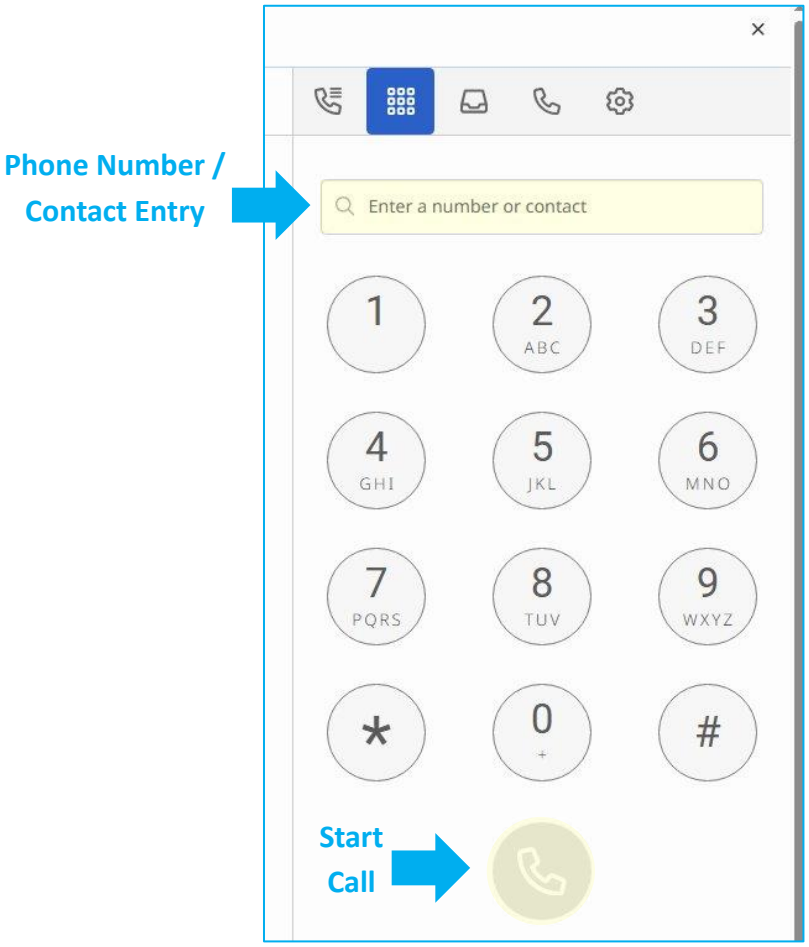
1. Click the Calls icon.
2. Click the + button from the top of the panel.
3. Alternatively, you can click to enter a number or contact.

To dial a Genesys Cloud User…

1. Type their name into the Number or Contact field at the top.
2. Select it from the dropdown.

To dial externally…

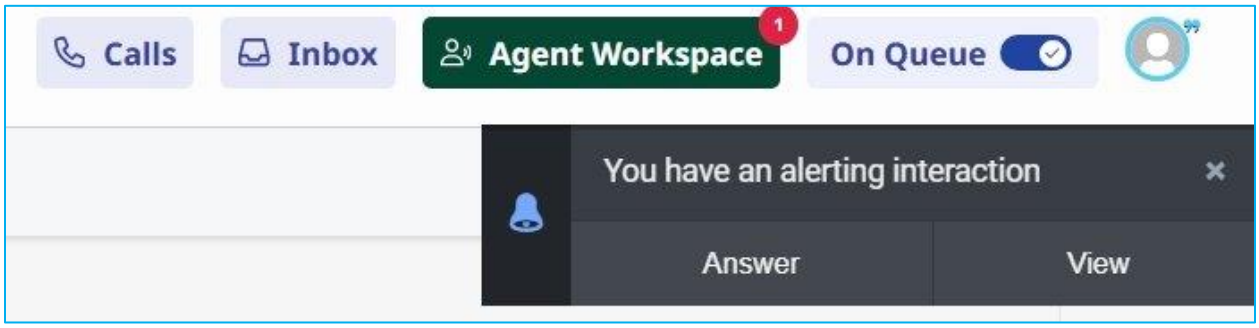
1. Use the dial pad or your keyboard to type in a full, 10-digit number
2. Press Enter on the keyboard.
3 Click Enter on the keyboard OR the Green Phone Button *(turns green when you enter numbers or letters)*



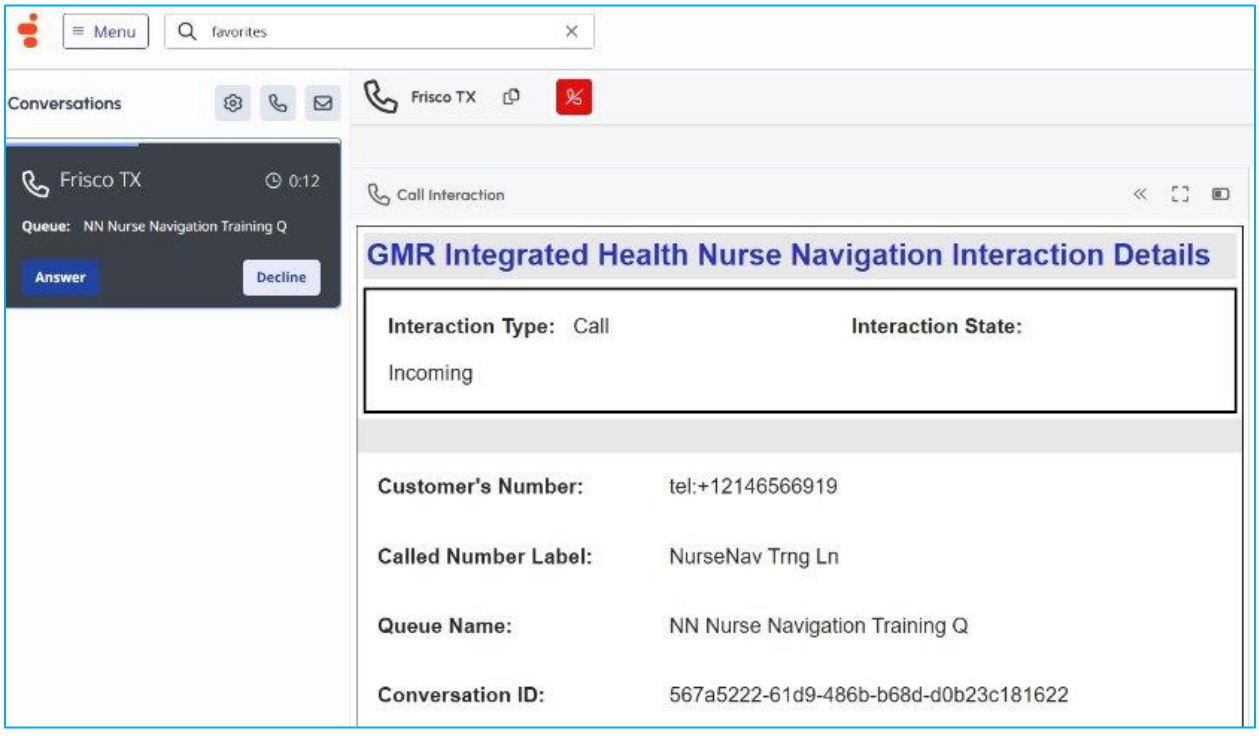
Inbound Non-ACD Calls

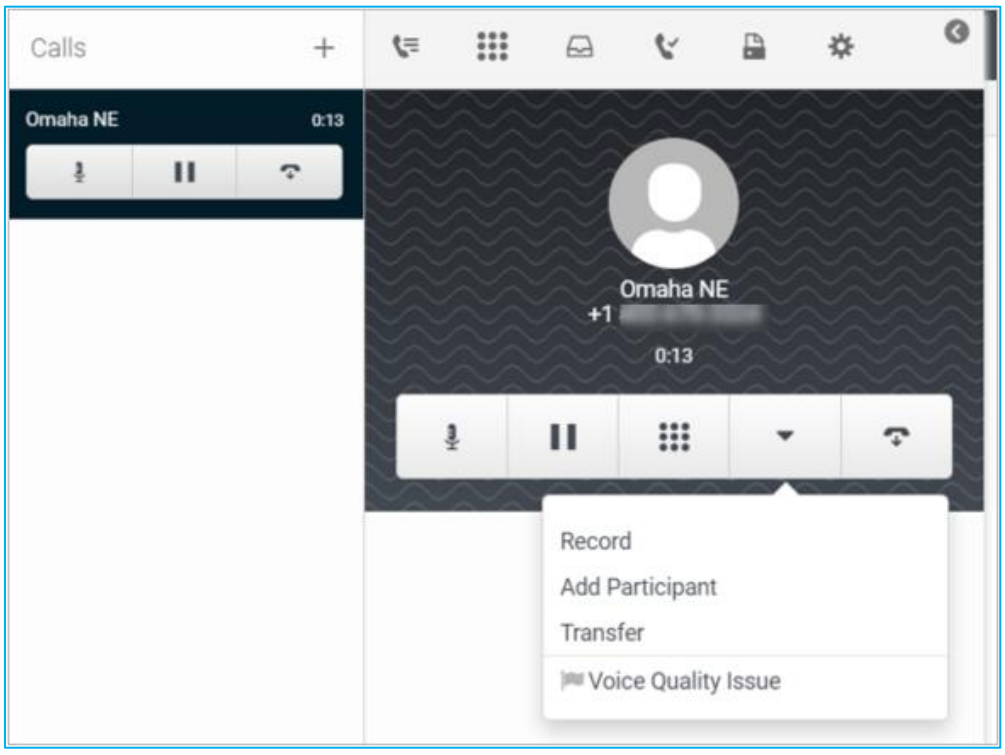
When someone within the business attempts to contact you through Genesys, you will receive a notification in the upper right corner of your window:

- Click Answer to take the call
- Click the green Agent Workspace component to view interaction details (caller ID)



If already in the agent workspace, click Answer to take the call:





Mute / Unmute

Hold

Touch-Tone Dialpad

More Options

- Record
- Add Participant
- Transfer
- Voice Quality Issue

End Call

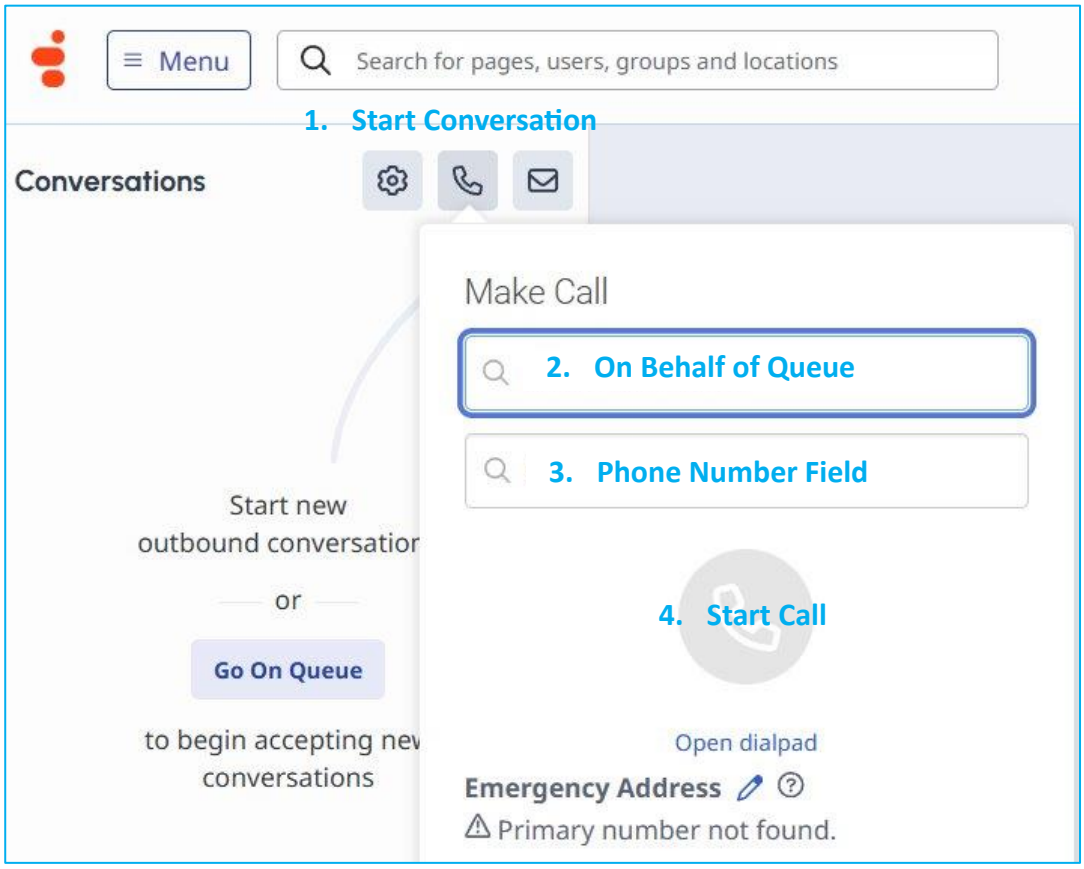
ACD Interactions On Behalf of Queue

Outbound Calls

ALL external calls ***MUST*** be made On ***Behalf of Queue***

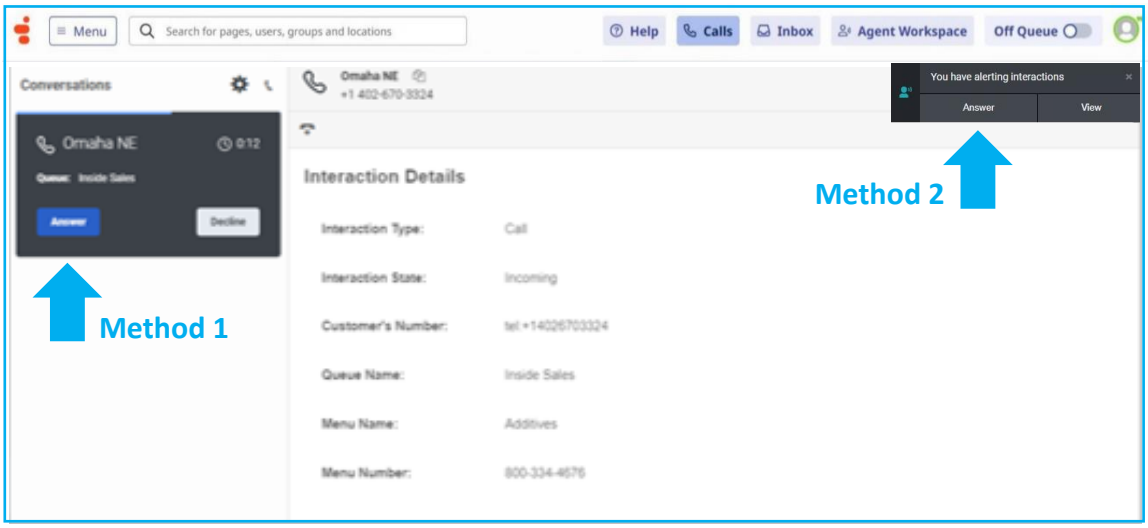
- Interaction recording policy is applied.
- If dialing while On-Queue, agent is not eligible for inbound interactions.
- The caller ID number (determined by the ***On Behalf of Queue Field***) routes the customer / patient back to the queue rather than the individual agent.

To make an outbound call on behalf of a queue:



1. From the Agent Workspace component, click the Start Conversation (phone) button.
2. Search and select an outbound queue (the phone number we are calling from).
3. Enter the phone number you wish to call
   - Genesys will attempt to search its address book as you type the phone number. To dismiss the address book, press enter on your keyboard after the phone number after completing your entry.
4. The ***Start Call*** button will turn green when both ***On Behalf of Queue*** and ***Phone Number*** fields are completed. Press ***Enter*** on your keyboard or select this button to make a call.

Inbound Calls



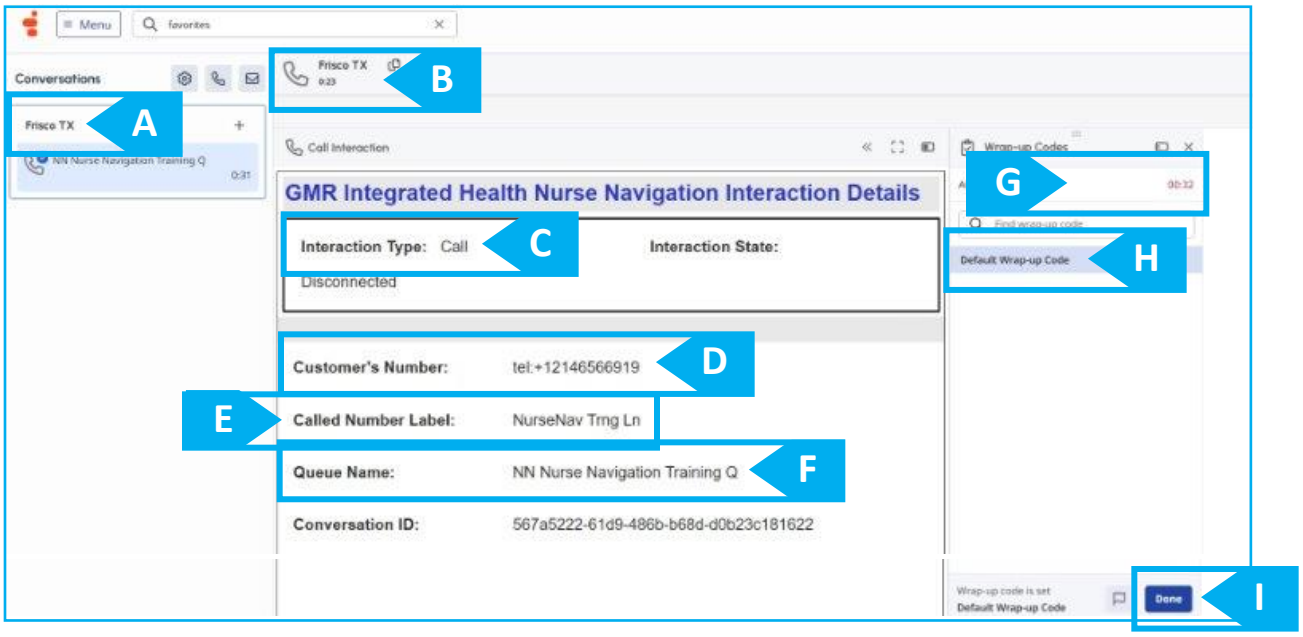
Incoming interactions are accompanied by a ring as well as two types of response methods:

Method 1: While in the Agent Workspace, select the blue Answer button when the interaction notification appears on the left-side of the screen.

- This notification will indicate the queue through which the interaction was routed.

Method 2: Anywhere in Genesys, you will see a smaller notification appear in the upper-right corner of the window (below your profile icon).

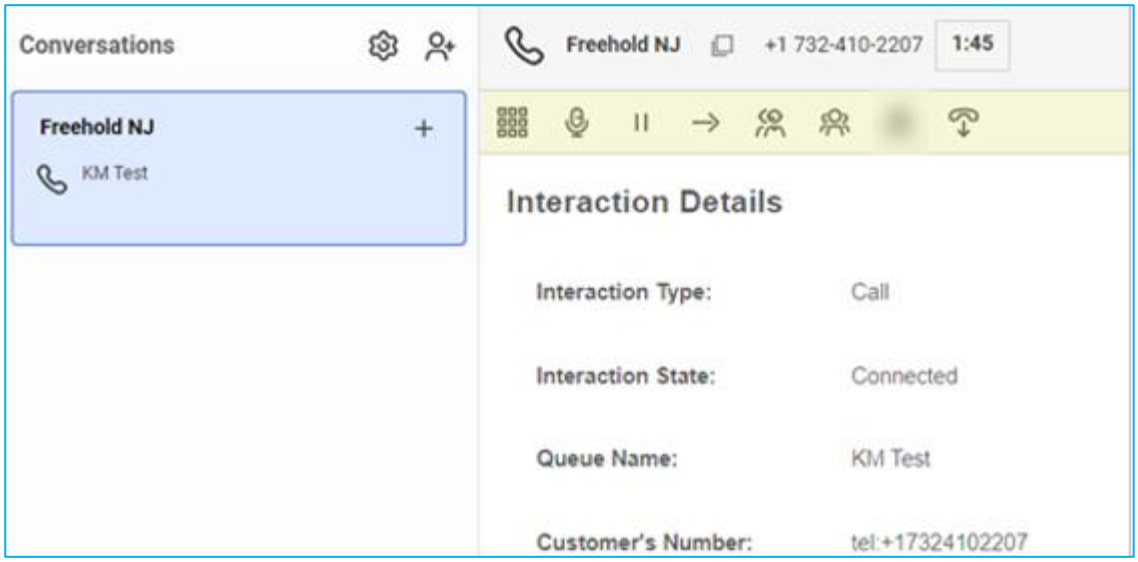
- Click ***Answer*** to accept the interaction.
- Click ***View*** to open the Agent Workspace component.



A	Displays the general location the call is coming from: • In this example, the call is coming from Frisco, TX
B	This is the call's timer
C	Interaction Type
D	The caller's phone number
E	Contact name (if it is logged as a Genesys contact)
F	Defines the queue through which the Interaction was routed / called
G	ACW Timer
H	Wrap-up code (select to define the type of call received)
I	After selecting a wrap-up code, you *MUST ALWAYS* select the ***Done*** button

Call Dashboard

When answered, you will be able to manage the call using the dashboard located above the interaction's details:



	Touch-Tone Dial Pad (click to toggle visibility)
	Mute / Unmute
	Hold
	Transfer
	Consult (Warm Transfer)
	Conference (three, or more, parties in the call can speak to, and hear, one another)
	End Call

Call Consults & Conferences

There are three ways to add parties to, or transfer, a call:

Transfer (Cold Transfer)	***DO NOT USE!***
Consult (Warm Transfer)	✓
Conference (three, or more, parties in the call can speak to, and hear, one another)	✓

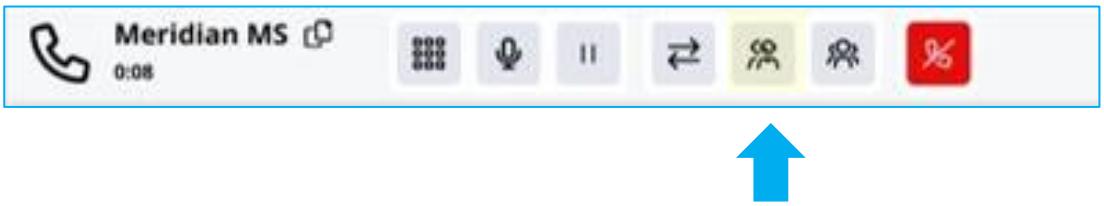
If every call were a hike, we would be the Guide. A ***cold transfer*** is like leaving an inexperienced hiker alone on a trail while the sun is setting.

We are responsible for leading our callers along the best path of care, even when the terrain gets rocky.

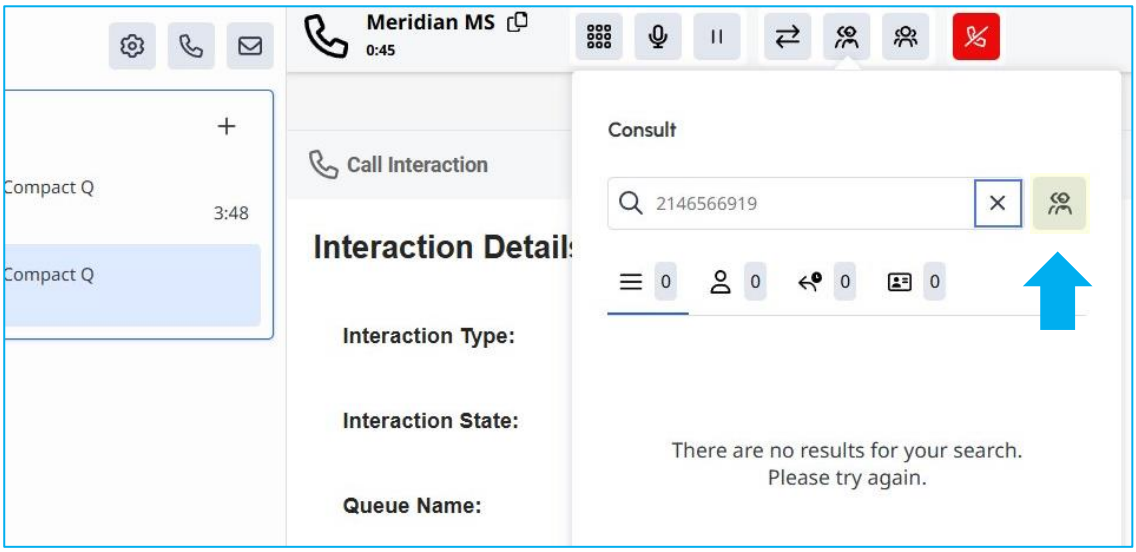
Start a Consult

Inform the caller that they will be placed on ***Hold*** once you begin the consult.

1. Select the Consult Icon:

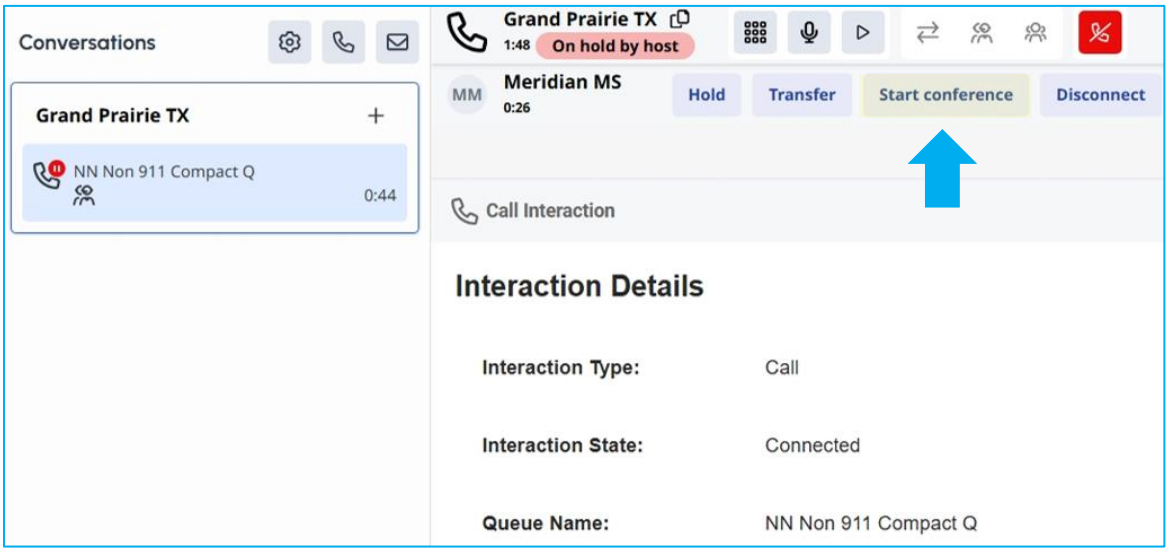


2. Enter the phone number you want to call.
   - If calling a phone number, select the secondary Consult button to begin calling.
   - If calling another Genesys user, or a contact in the address book, select the listing to start calling.
   - The caller will be placed on hold automatically once the consult is initiated.

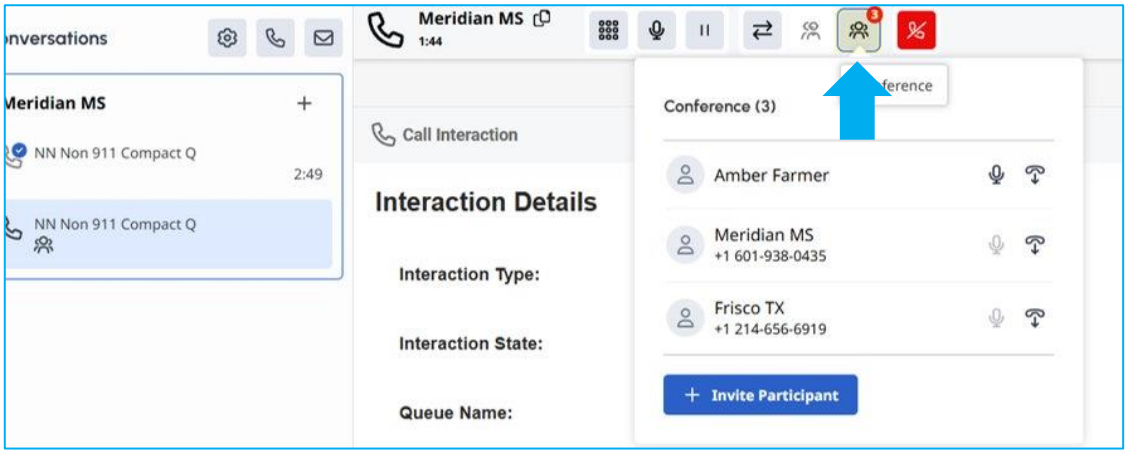


3. When your consulted contact answers the call, a new set of options will become available on your dashboard:
   - ***Hold***: This option will put your consulted contact on hold. You can manage your initial caller on the original call dashboard.
   - ***Transfer***: This will transfer your caller to the consulted contact and disconnect you from the call.
   - ***Start Conference***: All contacts in the call will be taken off of hold and be able to speak to, and hear, one another.
     - Use this if more than three contacts need to participate in the call.
   - ***Disconnect***: This will disconnect the consulted contact from the call, but your interaction with the original caller can continue.
     - This is *DIFFERENT* from the End Call button.
     - Selecting the red End Call button will disconnect **all parties from the interaction.**

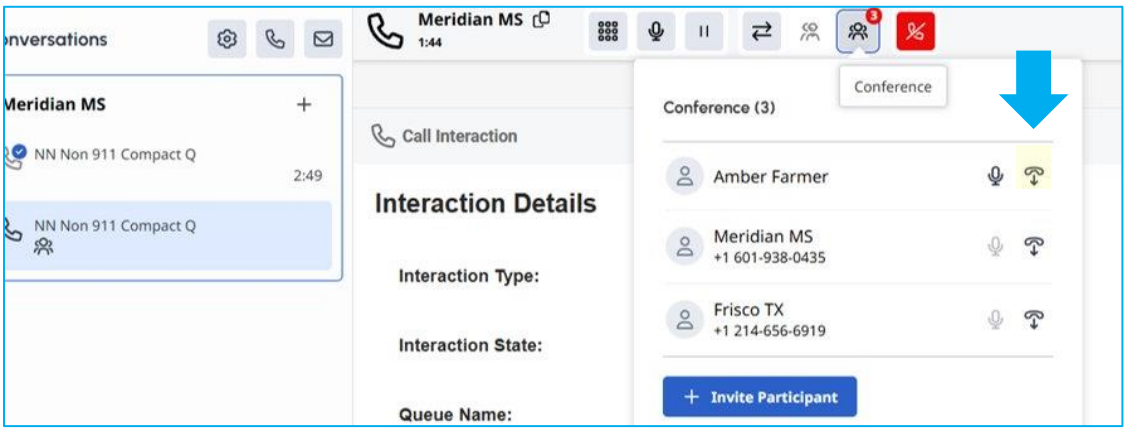
4. Summarize the patient's concern, then use the ***Start Conference*** option.
   - Selecting ***Start Conference*** will automatically take all parties off of hold.



5. Select the Conference icon from the main Dashboard to view all parties in the call:



6. When ready for transfer, disconnect yourself from the call.
   - The call will continue with the remaining parties after you disconnect.



7. Complete your after-call work (ACW) by selecting a wrap-up code and then ***Done***.

Check Your Status

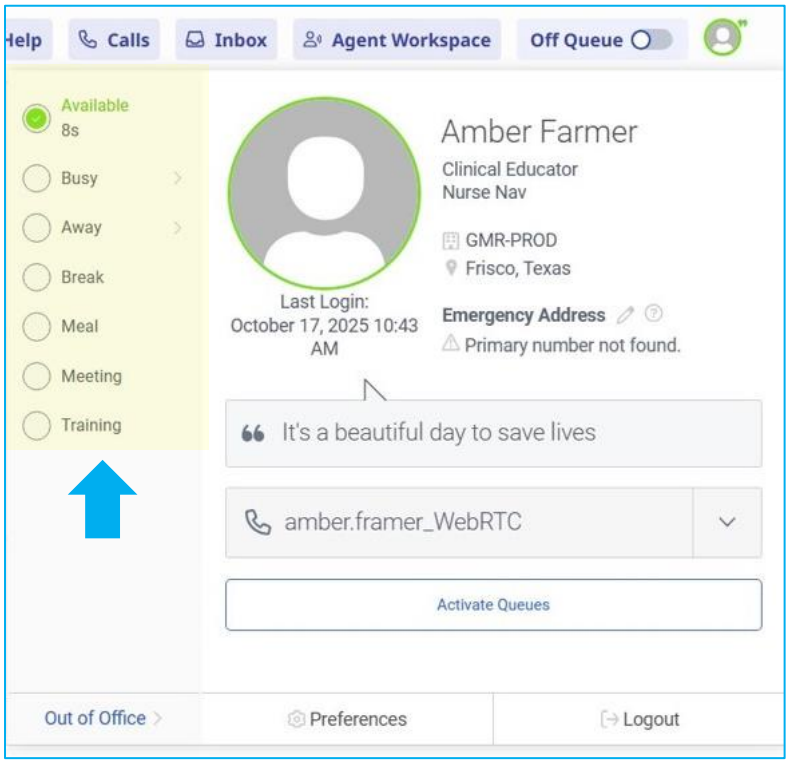
About Status Selection

- Determines an agent's eligibility to receive Calls and Interactions.
- Manually adjusted to match current activity; does not sync with any other systems.
- Agents do not need to adjust their status while connected to calls / interactions; the system will automatically put them in a special ***Interacting*** status for the duration.
- Supervisors use statuses to track users' availability and adherence to schedules.
- Colored ring around profile picture corresponds with status:
  - The ring around your profile will be Blue if you are On Queue or Interacting
  - The ring around your profile will be Red if a Busy status is set.
- Agents will see other users' statuses when attempting to call or transfer.
- Some status options include secondary situational status options to better inform the team of current activities.

Status Selection

You will choose your current status from your ***Profile*** icon:

- Upon selection, a status will begin to display a timer.



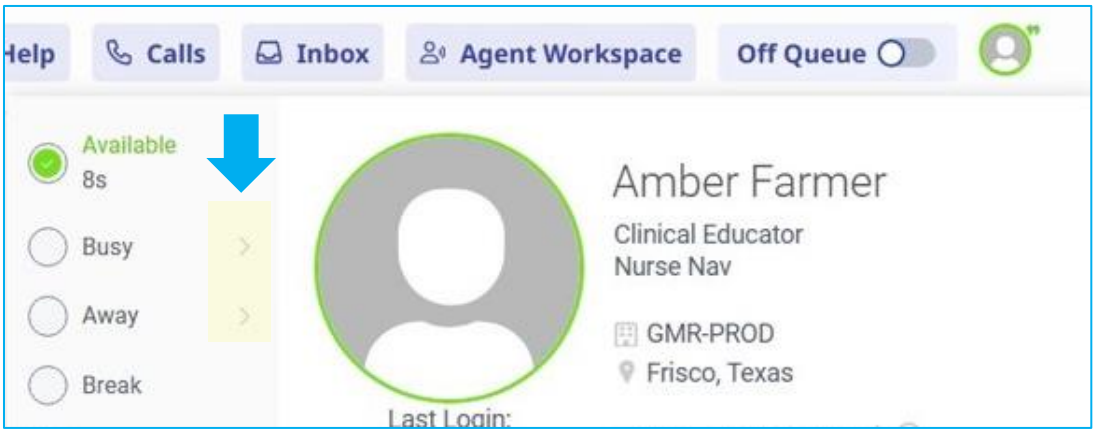
Status Key

Available	This will *NOT* allow the user to receive calls, but it does allow the user to receive internal calls within the organization. • This IS NOT the same as being On-Queue. ***Available*** auto-populates when you first log in but should be changed right away. You should *NEVER* be in this status during your shift.
Away	Indicates nurse is not at their desk or is unavailable; all calls and transfers will be routed to voicemail. Use for restroom, water / coffee, DoorDash, etc. • No more than 20 minutes per shift *IN TOTAL*.
Break	Used for breaks (2x15 min) per shift.
Meal	Used only for your lunch (30min).
Busy	Indicates nurse is not at their desk or is unavailable; all calls and transfers will be routed to voicemail. • Nurses should use ***extended ACW*** as needed to complete documentation
Meeting	Coachings, Projects, etc…
Training	This should only be used as directed by a Team Lead or Supervisor.
On-Queue	Indicates the user is available to take external calls for NN. • Selecting any other status will take the user Off-Queue • If switching Off-Queue using the toggle, the user will be placed in their last-used status.

Secondary Statuses

Primary statuses with a carrot icon (>) to the right have a ***Secondary Status***.

- For any Primary Status that include Secondary Status options, a Secondary Status must be selected to confirm your activity.
- To return to the primary status list, click the primary status at the top.
- Secondary statuses are division-aware; therefore, they may vary from team to team.



Statuses & Locking Your System

What happens when your system is locked while you are in the Available status?

- Your status stays **Available**.
- Internal calls continue to alert.

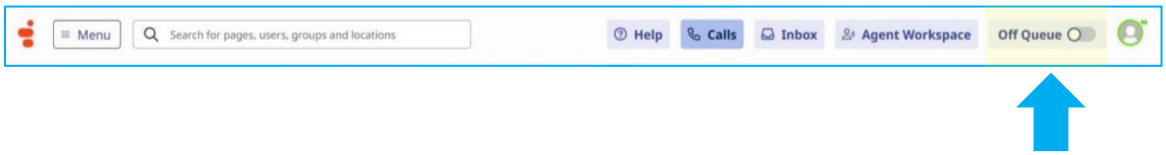
What happens when your system is locked while you are On Queue?

- You stay **On Queue**.
- Calls and Interactions continue to alert.

Going On-Queue to Receive Calls

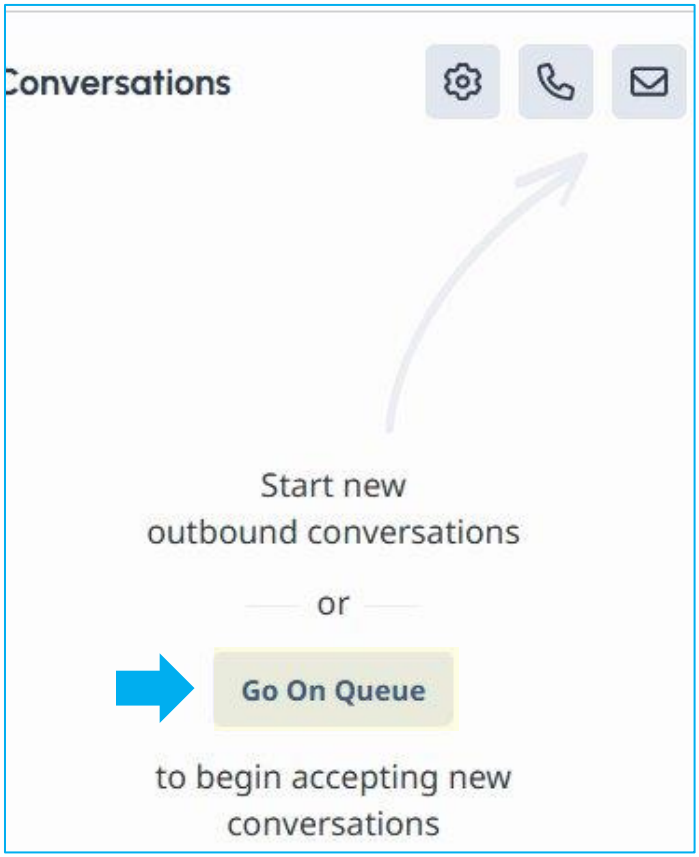
You can set yourself as On-Queue by two methods:

Method 1: Use the On / Off Queue Toggle beside your profile icon in the upper-right corner of the window.

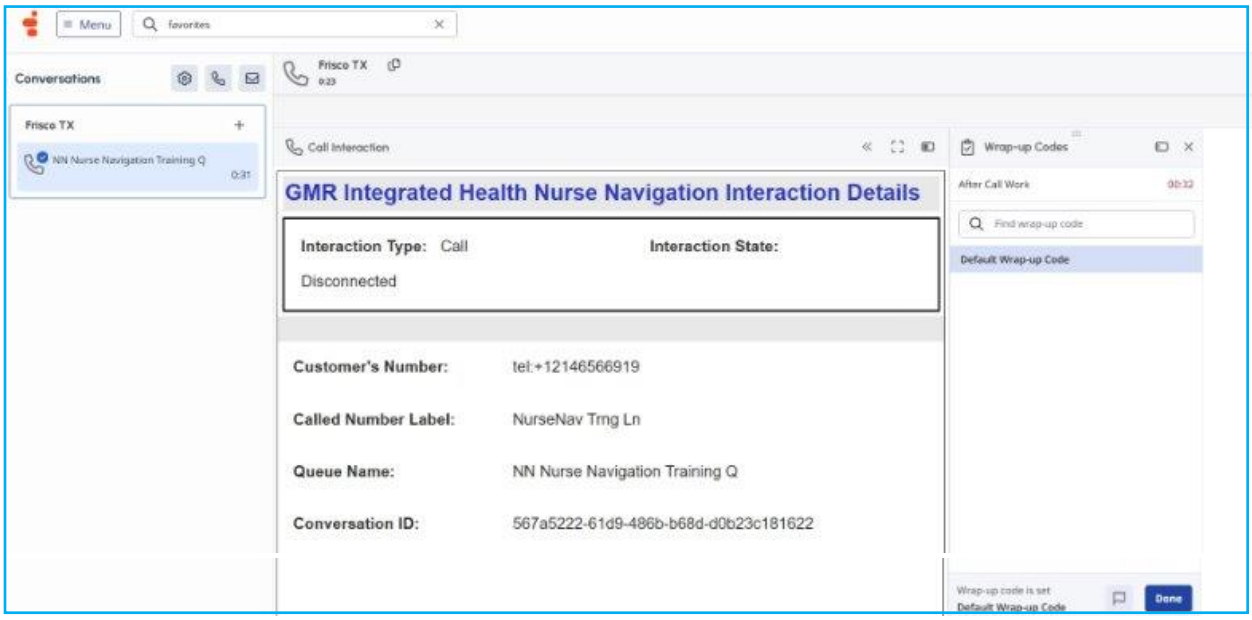


Method 2: In the Agent Workspace screen within the Conversations pane.

- It is recommended that you are already in the ***Agent Workspace*** when going On-Queue as this is where you will be able to most effectively manage incoming calls, or dial out, as needed.



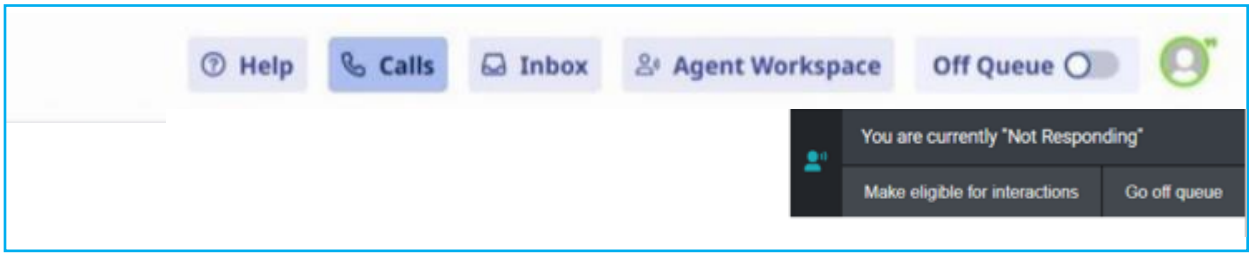
After Call Work (ACW) & Wrap-Up Codes



- Agents are automatically placed into After Call Work (ACW) after a call ends.
- Agents will not receive voice interactions from the queue while connected to ACW.
- To exit ACW early and become eligible for waiting interactions, select a wrap-up code and click Done.
- The timer indicates the amount of time an agent may stay connected to ACW; this varies from queue-to-queue.

911 Calls	Non-911 Calls
5 Minutes	30 Seconds

Failing to Answer an Interaction



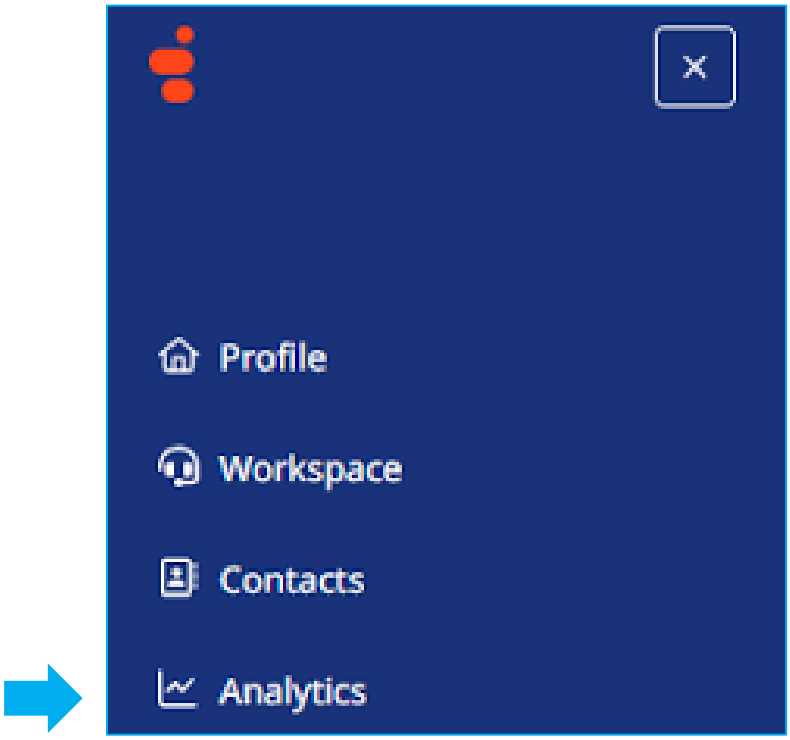
If an incoming interaction / call is missed, you will be placed automatically into a unique ***Not Responding*** status and be ineligible to receive additional interactions until you manually go On-Queue. The notification proves two options; ***Make Eligible for Interactions*** (go on queue), and ***Go Off Queue***.

A Not Responding event may occur as the result of miss-clicking a Break or Meal status. Ensure the ring around your profile icon has changed to the correct status color before locking and departing from your system.

View Performance & Queue Activity

To view your individual performance data, or the activity of a particular queue:

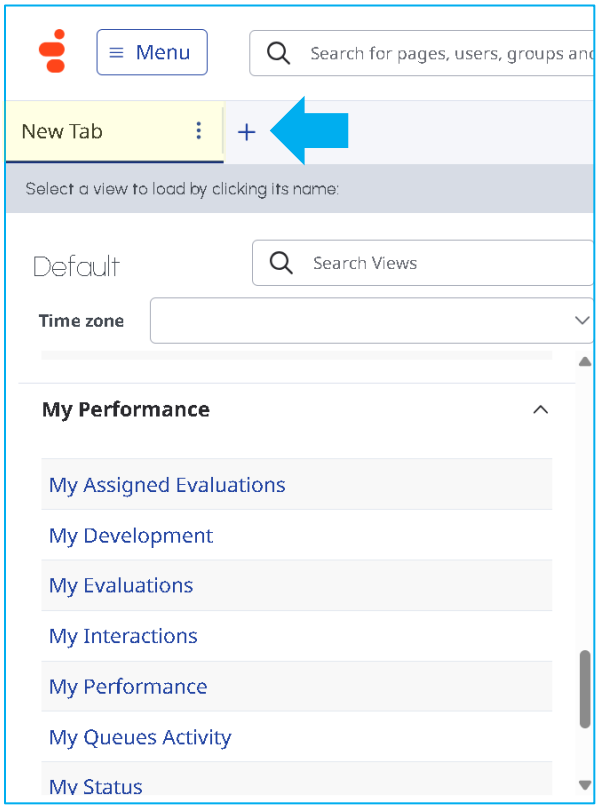
1. Open the Menu located in the upper-left corner of the Genesys window.
2. Select Analytics > Analytics Workspace



The options available to you in this menu will depend on your individual user permissions.

The Analytics Workspace opens as a tab within the Genesys window:

- Selecting additional options in the Analytics Workspace will open additional tabs.
- Select the three dots (⋮) beside the tab's name to reload, close, or refresh it.

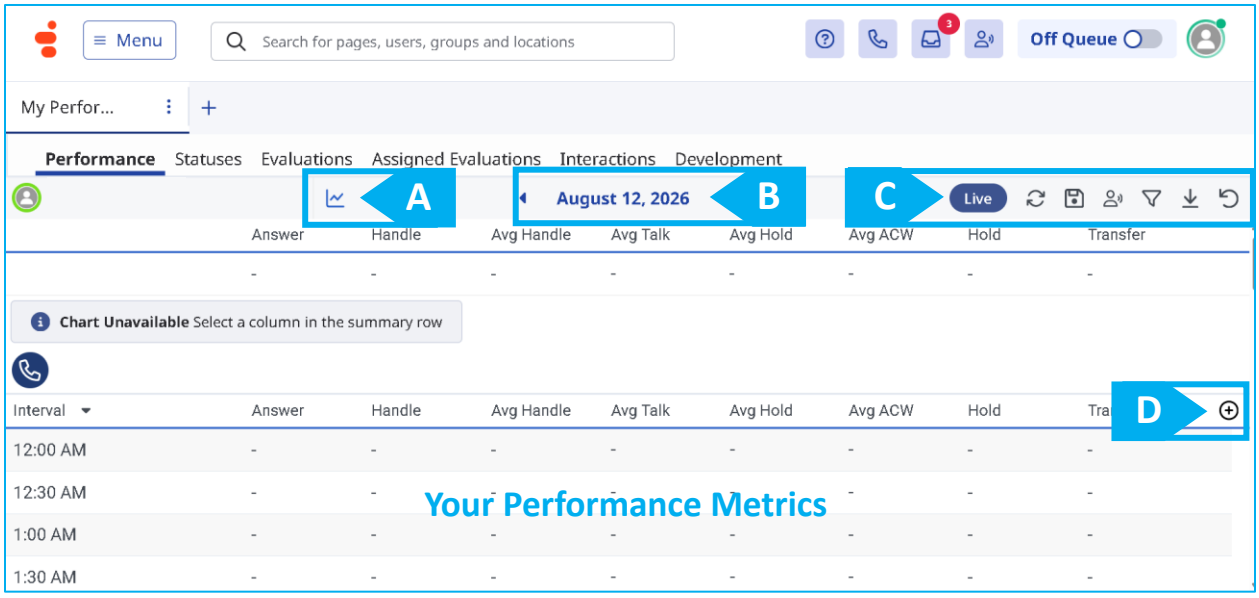


Tab management is key to successful navigation of the Analytics Workspace. Genesys will remember your open tabs between login sessions.

My Performance

The My Performance section of the Analytics Workspace includes the following options:

My Performance	• View total interactions managed • Average interaction length • Average After Call Work (ACW)
My Interactions	• View previous interactions (calls) • Listen to previous interactions • View ACW assigned to each interaction
My Queues Activity	• Review current and historic interaction traffic of assigned queues.
My Status	• See historic use of, and total time spent in, a status.



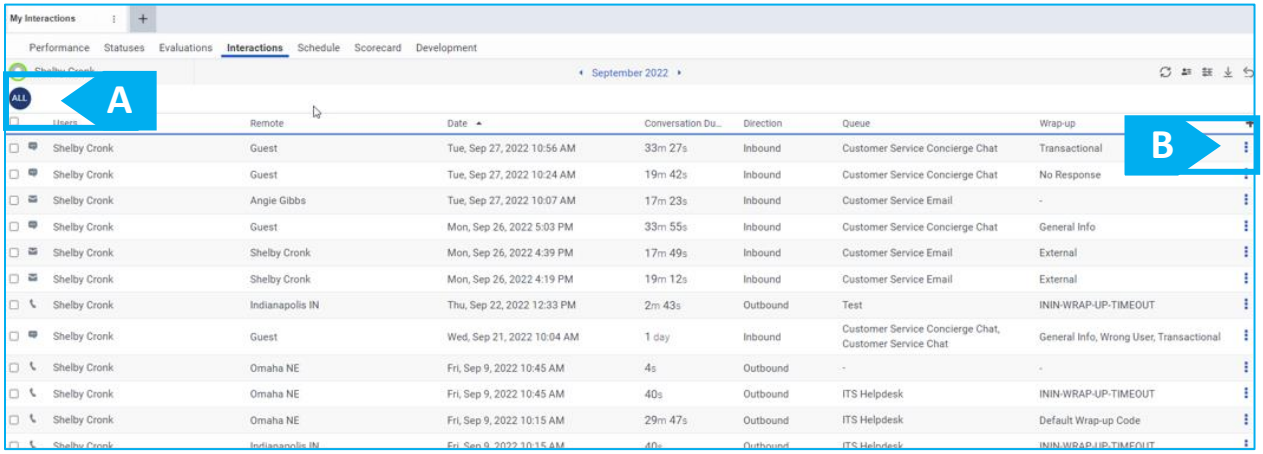
A	Toggle the chart on / off
B	Click to select a date range
C	View Options • Refresh • Save View (to the main Analytics Space window) • Search • Filter • Export • Reset
D	Organize Columns

My Queues Activity

1. Queues that the agent is a member of are listed
2. Real-time Metrics:
   - Interactions
   - Service level
   - Agents on Queue
   - Agent Status
3. Metrics can be added / removed using the (+) option to the right of the column listings.

My Interactions

Each line represents an individual interaction. Click the interaction to view the call's recording.



A	Set the ***Filter by Media*** option to ***All***
B	Click the three dots (⋮) on the right side of each interaction to copy the Conversation ID

Common Errors

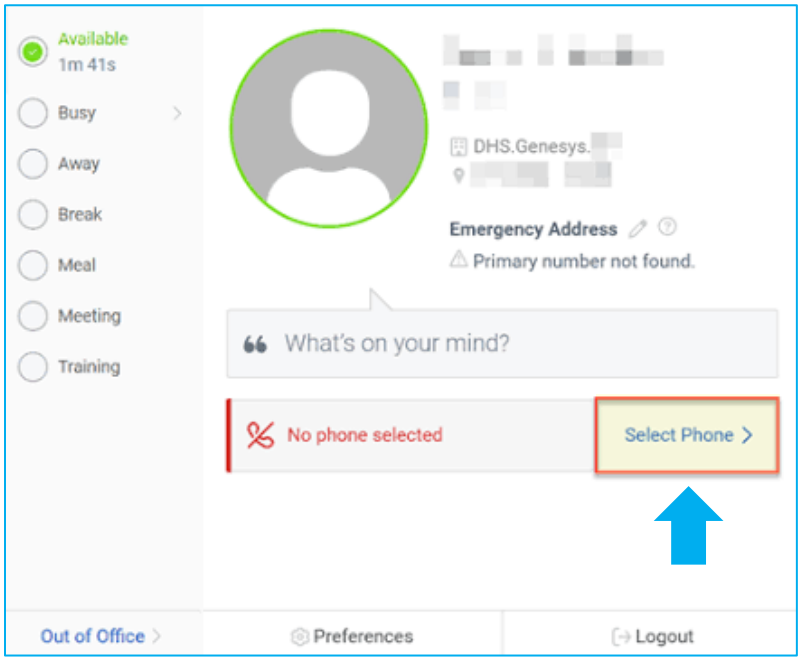
Logging In

- If the agent receives an error:
  Sorry, there is a Problem with the Organization You Provided
  —there are two possible explanations:
  - Wrong Region (US East/US West)
  - Typo or space in Organization's name
- If the agent says their email address and password are not working, there are two possible explanations:
  - Have not activated their account and set a password.
  - Trying to log in using email / password when configured for SSO
- Trying to log in on the wrong screen (must be on the purple panel for SSO)

No Phone Selected

Agents with a red phone will not be able to make or receive calls or interactions; three common Red Phone issues:

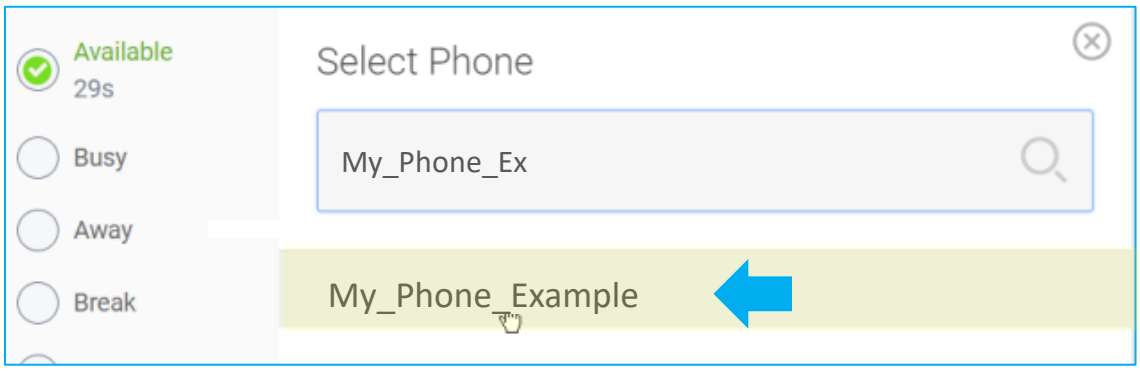
- Select a phone to make / receive phone calls
- Your browser does not have permission to access your microphone.
- You have a WebRTC phone selected, but Genesys Cloud is unable to connect to a compatible microphone.



A selected phone is necessary to make outbound calls and receive incoming interactions in Genesys.

Select a Phone

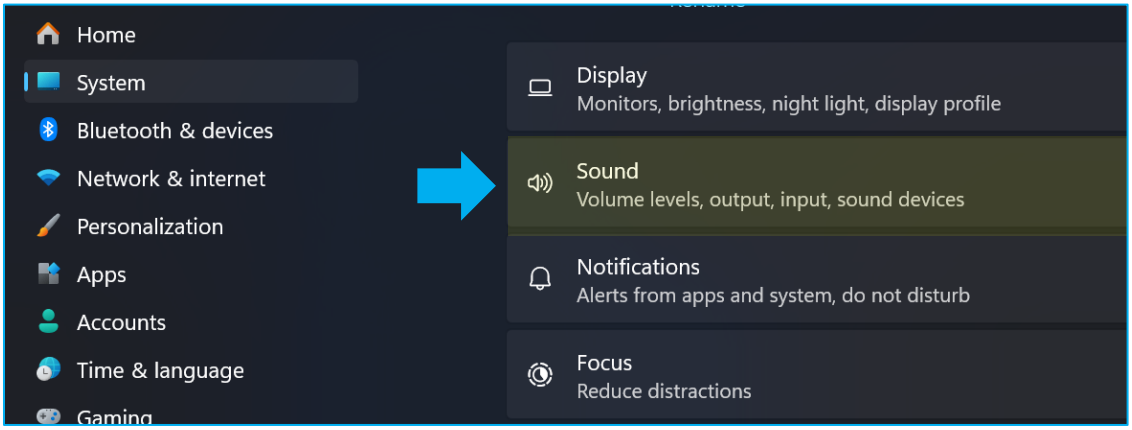
1. Click your profile icon in the upper-right corner.
2. Search for, and select, your assigned phone:



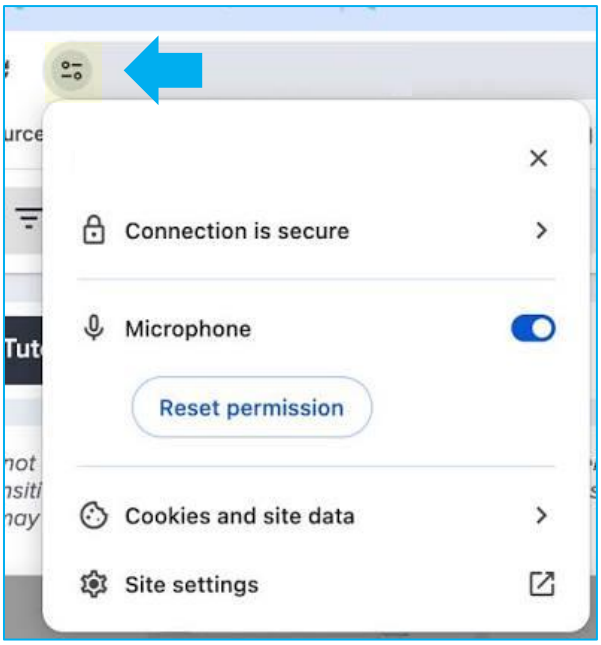
Check Windows Sound Settings

If your system or browser does not recognize a microphone, or if the microphone assigned through Preferences is disconnected, Genesys may be unable to make calls:

- Double-check your system's ***Sound Settings***.
  - Access your system's settings menu through the Windows / Start icon ( )
  - Check input settings.



- Double-check your Preferences settings in Genesys.
- In your web browser, select the ( ) icon of your address bar and allow permissions for your preferred microphone.



CAD-to-CAD

Overview

1. 911 / Field gets an NTL eligible call
2. 911 / Field provides details to their CAD
3. Their CAD contacts our CAD to check for an available nurse in Logis
    - If no nurse is available in Logis, the incident is not sent to 911 Nurse Navigation, and contract sends resources to patient.
4. The available nurse is provided the incident
5. The available nurse receives the associated call

Status Management

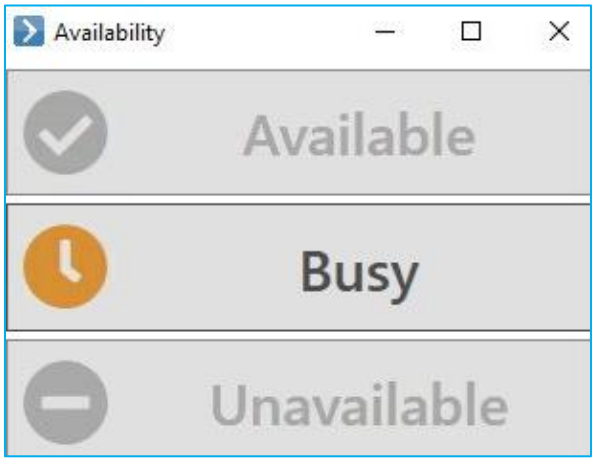
Because CAD-to-CAD checks your status in Logis to see if you are available for a call, accurate status management is key to the successful operation of NN.

If you're not ready to take a call, you should be "Busy" or "Unavailable" in Logis.

When the system says we're available, but we don't pick up the phone:

- Contracts blows up our phones and our abandoned calls increase.
- Call Takers and Field Staff lose faith in our program.

It's easy to fix staffing, BUT It's hard to repair lost faith



Matching Statuses

- Contracts see the system is working as designed.
- Reports of “No nurse available” from our partners are monitored by leadership and we look into the cause (number of staff on shift, staff in Aux, etc…), and we work towards a solution internally.

Keys to Success

Adjust your Logis Window Proportions

The Logis Availability window can be stretched and resized to match your economy.

- Hover your mouse curser on the window’s edge to stretch it to the desired size.
- Keep it near your Genesys window

Position your Logis Window

Logis Availability stays on top of all other programs. You can even put it over your Genesys window which we have found to encourage the fastest system navigation.

Update Logis for Live Calls

When you answer a call in Genesys, immediately click “Busy” in Logis.

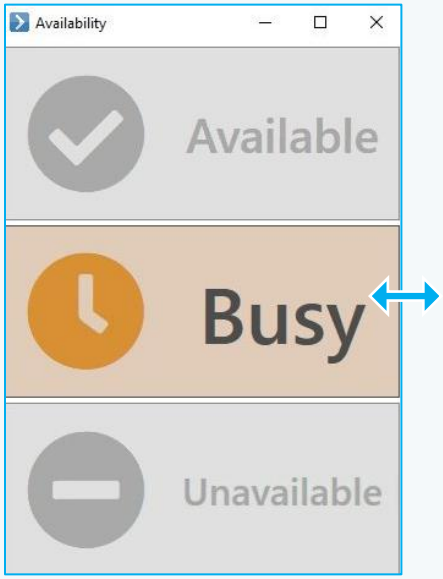
Set Your Availability in Logis

When you switch back to “On Queue” in Genesys, immediately mark yourself “Available” in Logis.

Window Adjustment

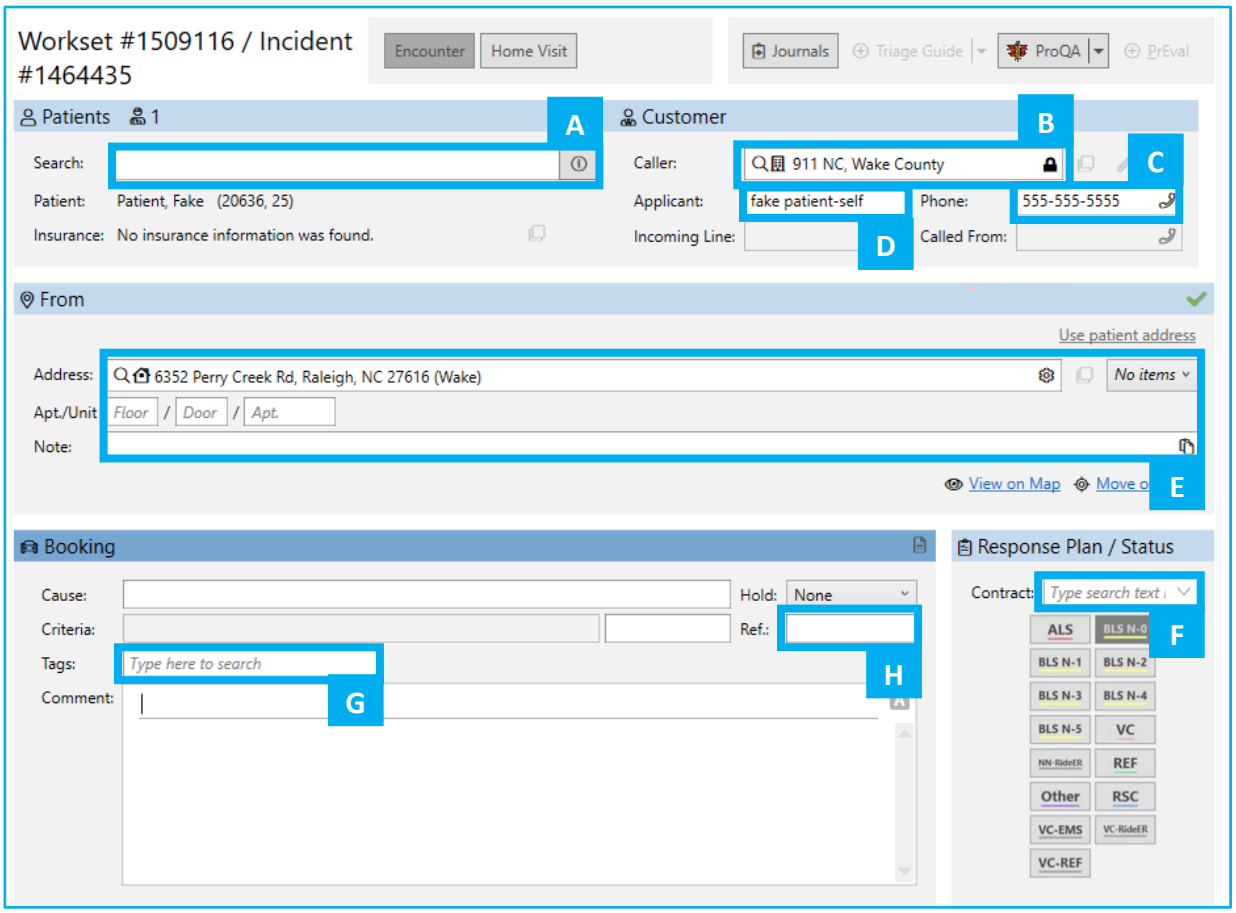
Wide-Width Window Adjustment

Narrow Window Adjustment



Window Placed Over Genesys

Overview

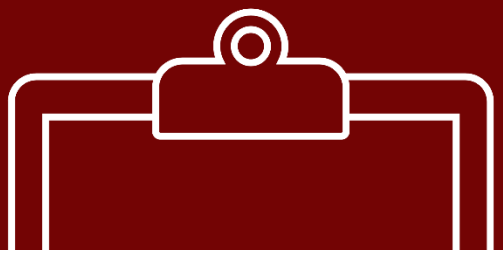


A. Search the patient's name here. Verify their address
B. Provide the contract name here
C. Enter the patient's phone number
D. Caller Name and relationship to patient
E. Input the patient's address and include their apartment / building / unit number and gate code (if applicable)
F. Enter the patient's insurance type to see the dispositions
G. Tag the call as 911 Call Center, AMR Field, Fire Field
H. Reference Incident / Call for Service Reference Number

Call Tags Are NOT Optional

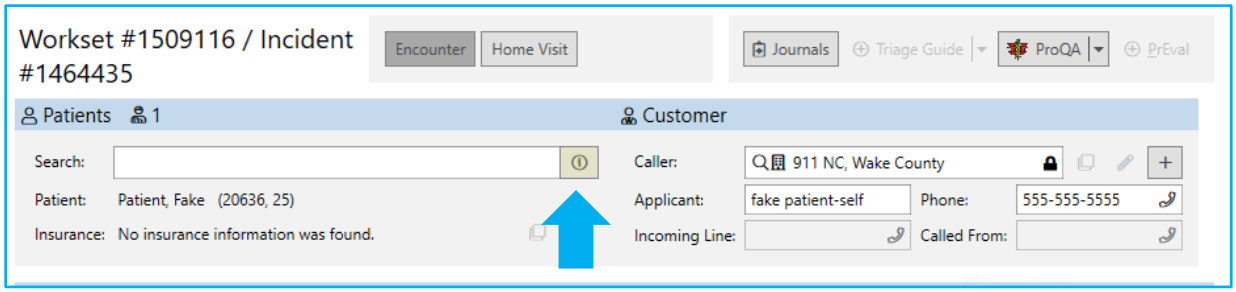
Check the Caller ID in Genesys

Follow Documentation Do's and Don'ts

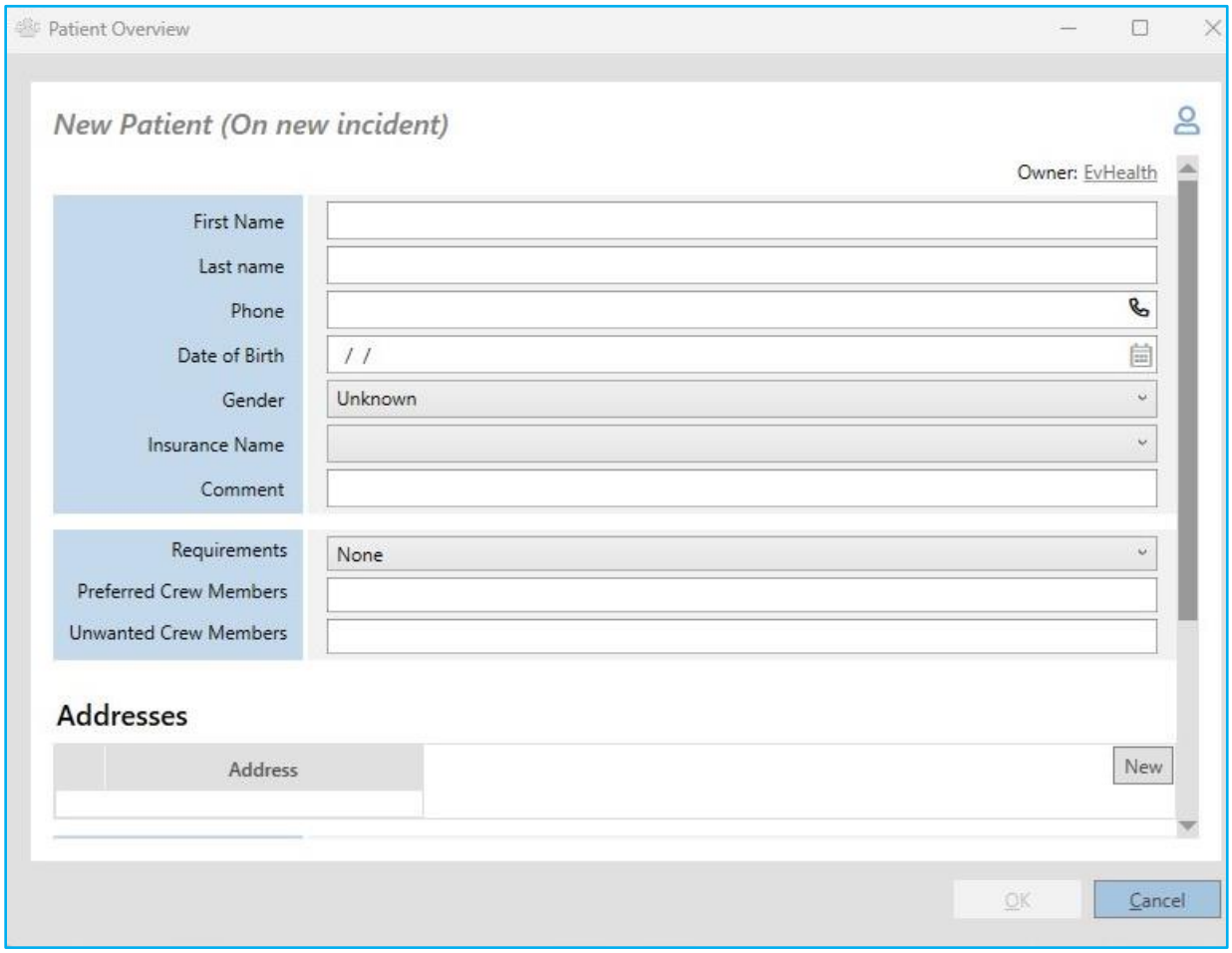


Proofread!

Can't Find the Patient?



Enter the patient's demographics here to create a new patient, if not found.



Six Required fields

1 **First Name**

2 **Last Name**

3 **Date of Birth (DOB)**

4 **Gender**

5 **Phone Number**

Use dashes

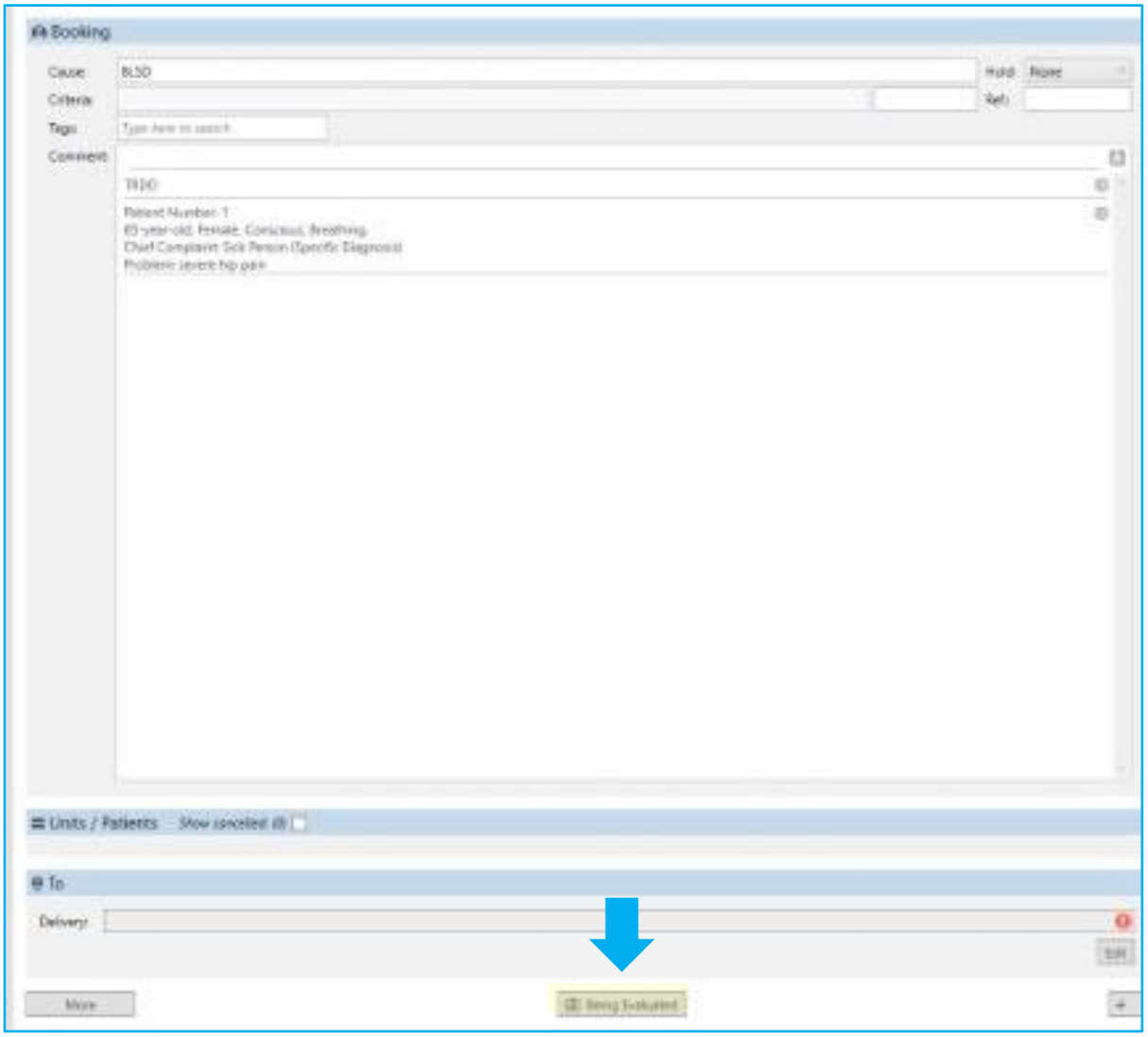
6 **Address**

Include apt / building / unit number

Insurance type is required only for DCFEMS and Riverside contracts

Being Evaluated

- After you open your CAD-to-CAD call on the right side, click the Being Evaluated button (bottom left) so that your team will know you are working the call.
    - ➢ If not CAD-to-CAD, enter patient information and click Save before clicking Being Evaluated to claim the call on the incident list.



★★★★★ Visit the Quality Assurance segment of this guide to review specific QA evaluation guidelines.

Scheduling in Gantt

Schedule follow-up calls for the next business day

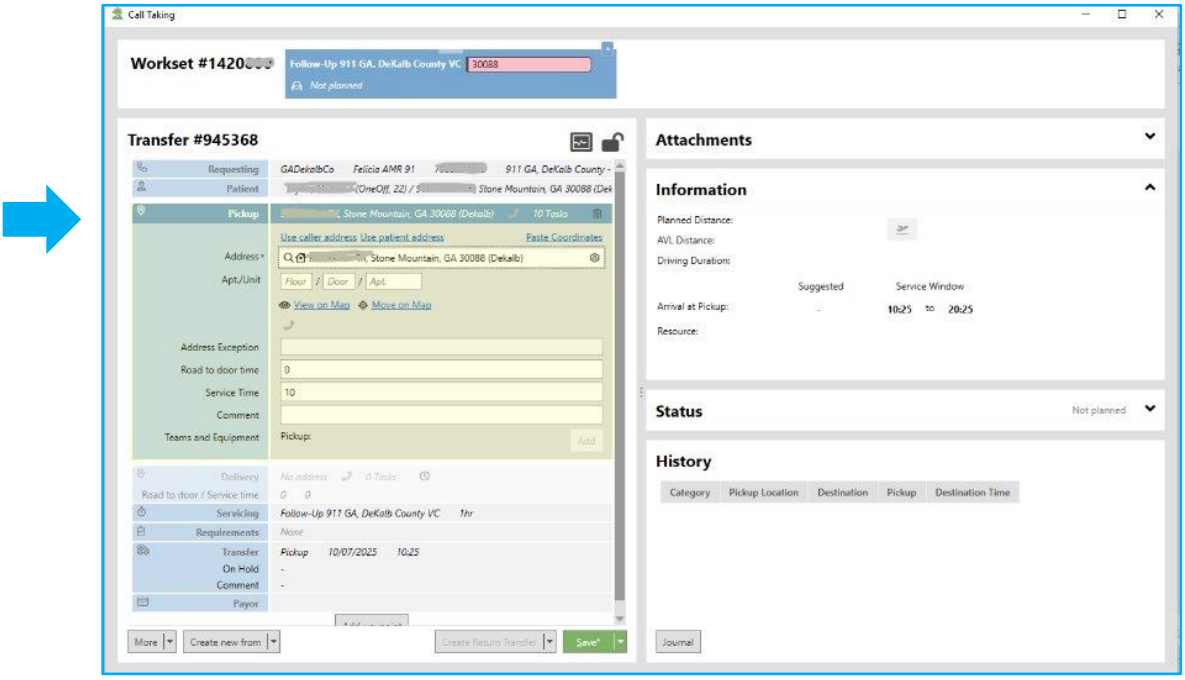
Edit the pickup location to “home”

Change the time to reflect 10:00 AM in the patient’s time zone

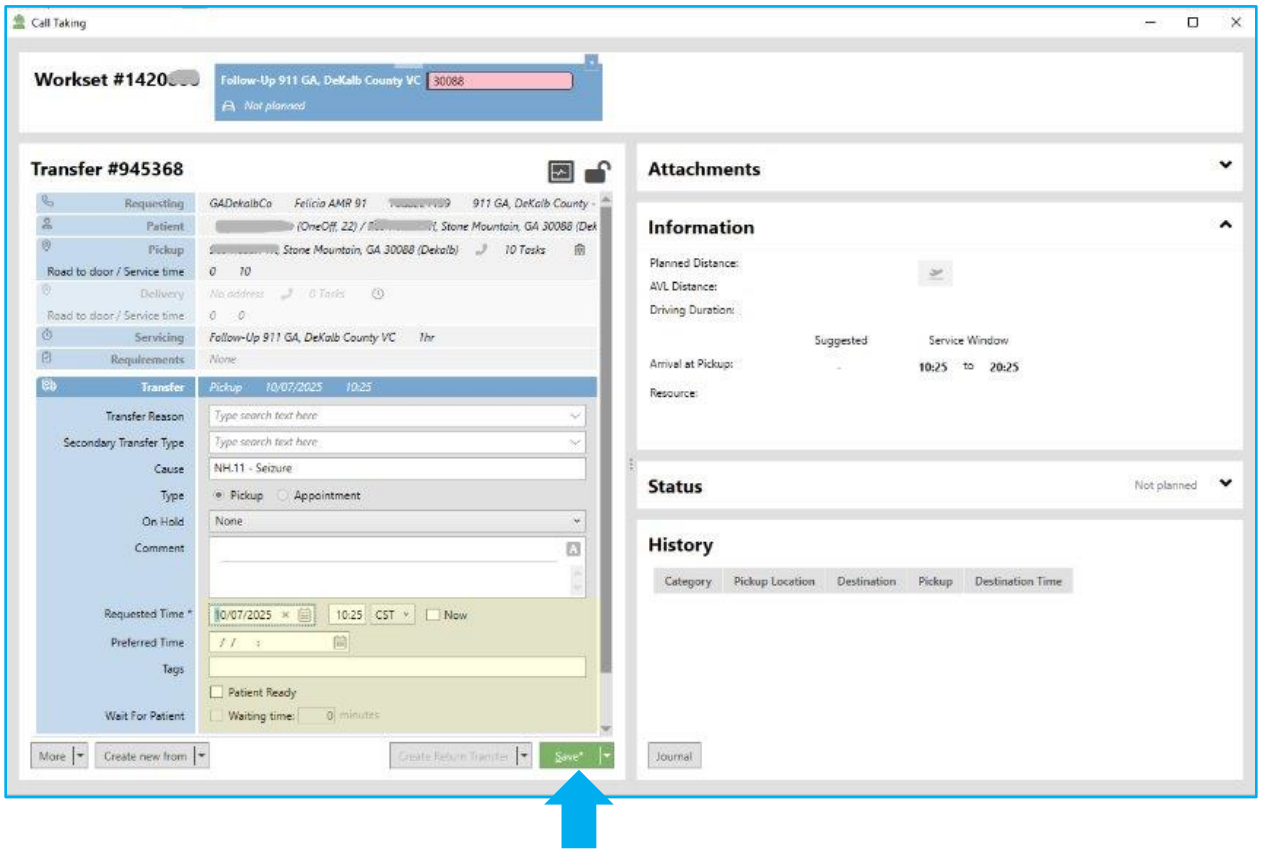
Schedule Follow-Up Calls

1. Select the Lock icon to open and edit fields:

2. Under the Pickup section, click the address field and delete the address listed.
3. Type ‘Home’ and select 4400 TX-121 Lewisville, TX
   - It is the first in the dropdown list



4. Edit the date and time to reflect 10:00 AM in the ***patient's time zone*** on the following business day.



5. Click ***Save*** and exit.

Crisp DC

Overview

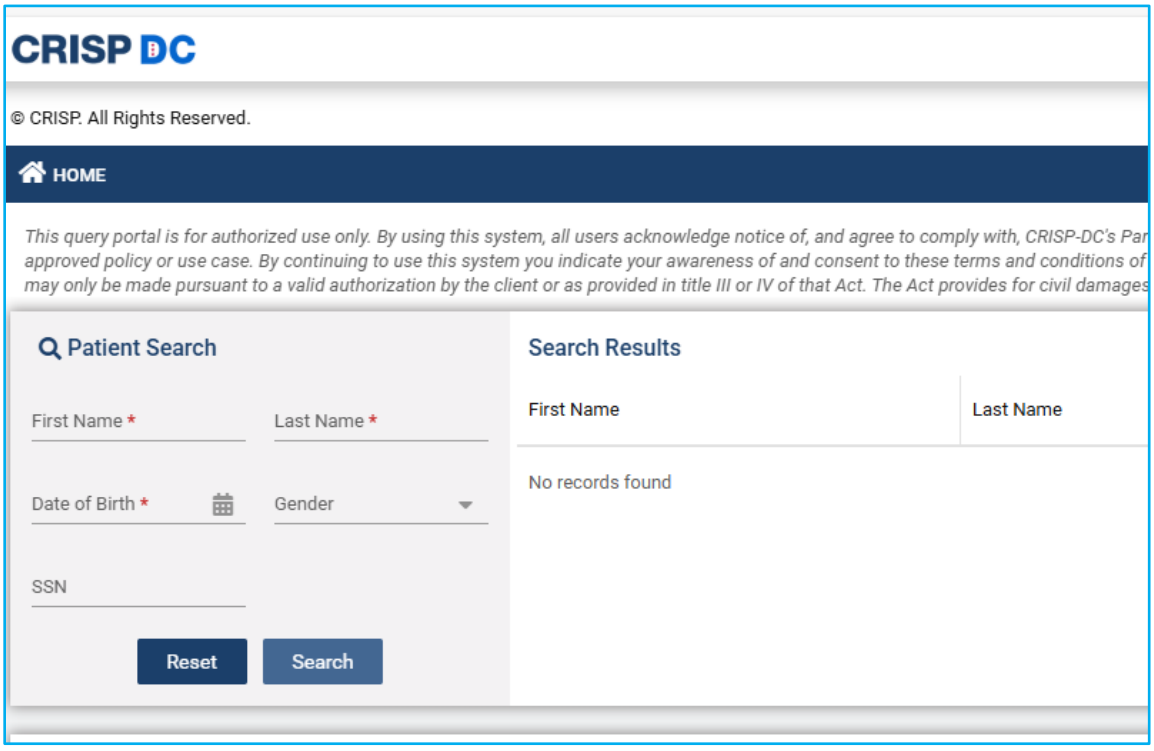
- Nurses will use Crisp DC to verify pt insurance and identify pt's 'home' clinic
- You will receive an email to set up your CRISP account.
  - The set-up email is only good for 72 hours.
  - You must log into your CRISP account at least monthly to maintain your access
- For documentation guidelines in Logis, carefully review the *DC FEMS Contract*.
- While it is not *required* for a patient to have insurance to receive our services, it is important *attempt* to get this information
  - DCFEMS tracks this data to use for reimbursements from insurance providers

Login

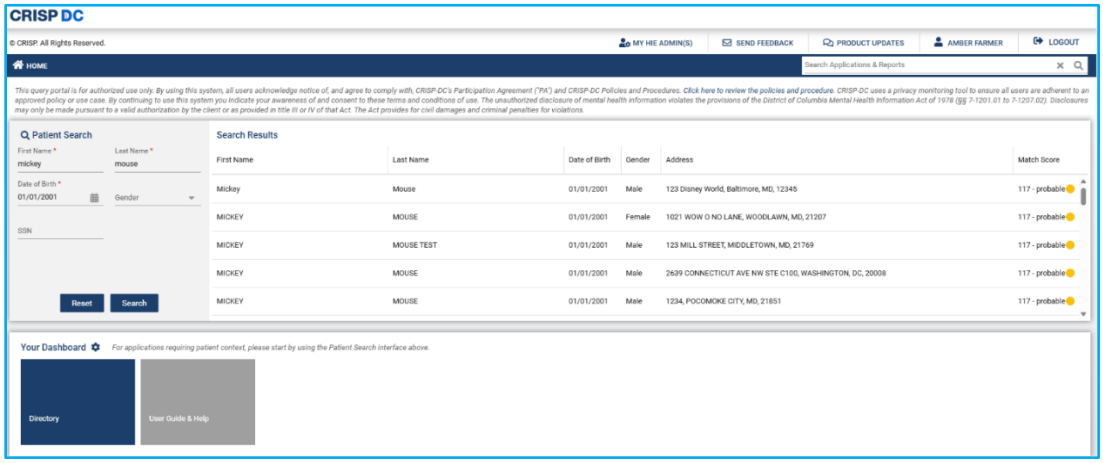
Link: Open DC Crisp

Patient Search

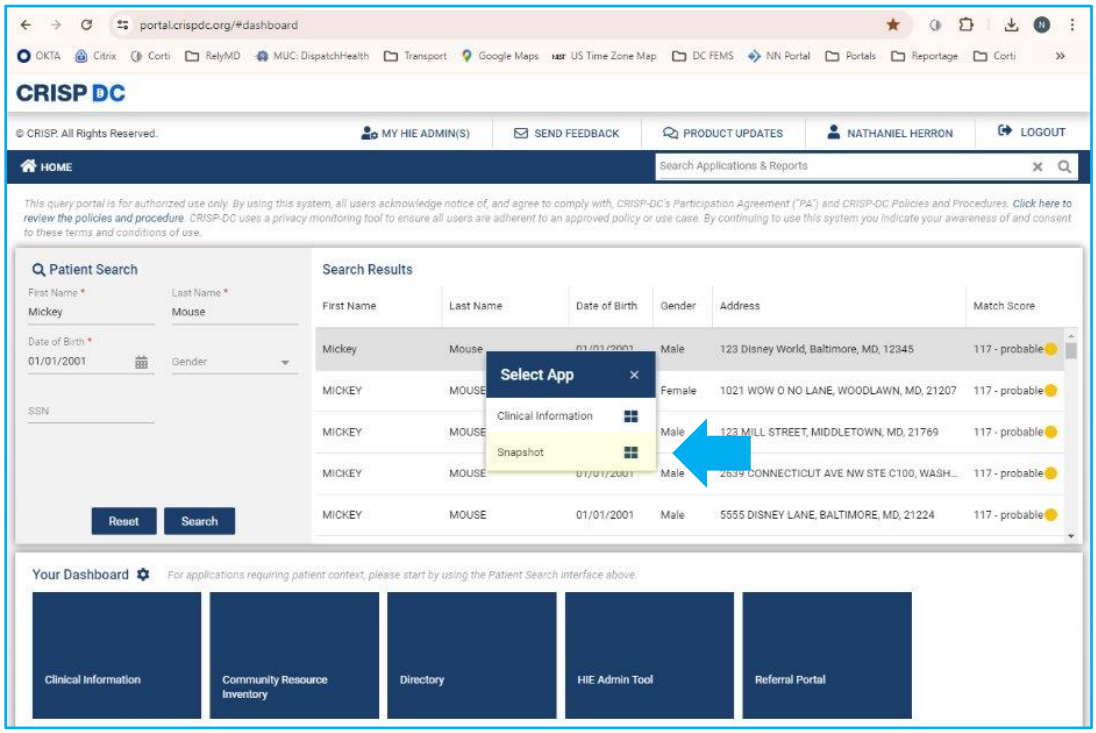
- Enter the patient's first name, last name, and DOB to start your search. Results are displayed to the right of the search menu



- Search results include first name, last name, DOB, gender, address, and match score:

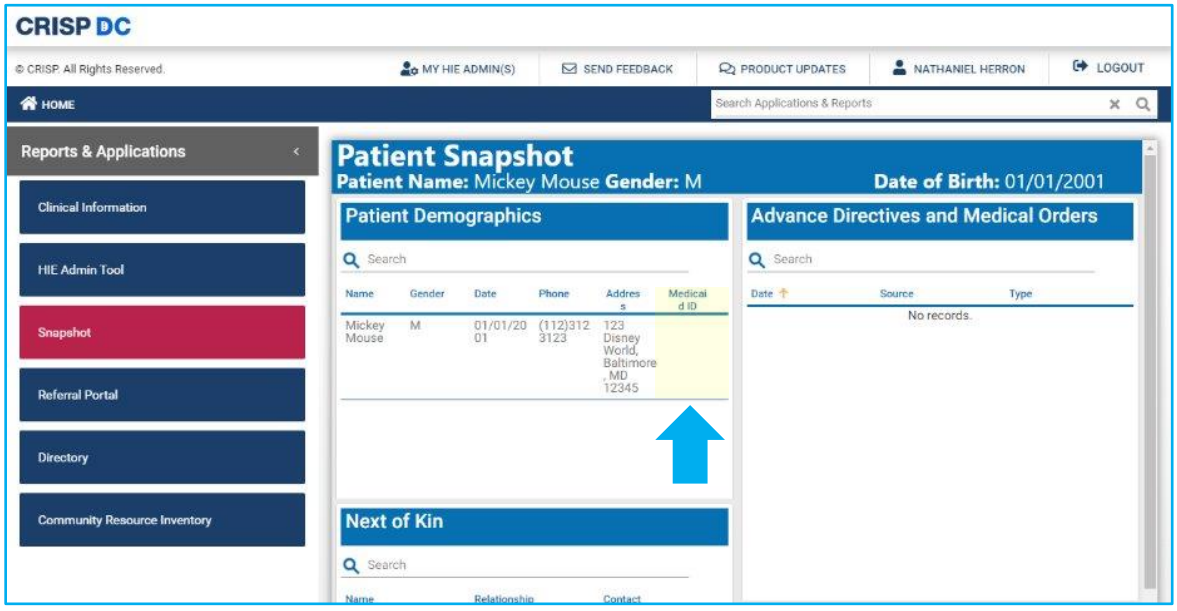


- Select the patient you wish to view
- From the app selection menu, choose ***Snapshot***
- You will be asked to verify why you are viewing the patient's data.
  - Select ***Care Coordination***

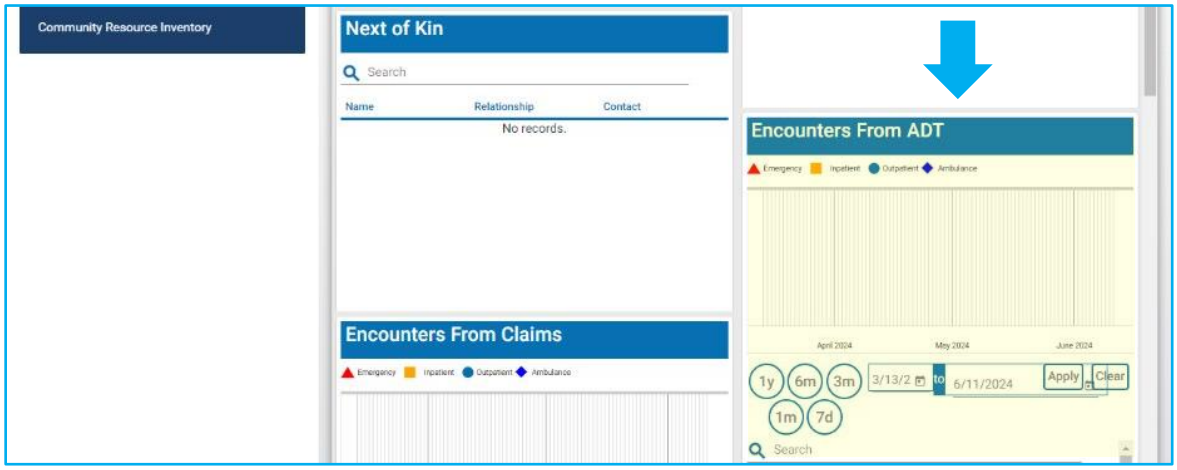


Snapshot

- Find the Medicaid ID in Patient Demographics:

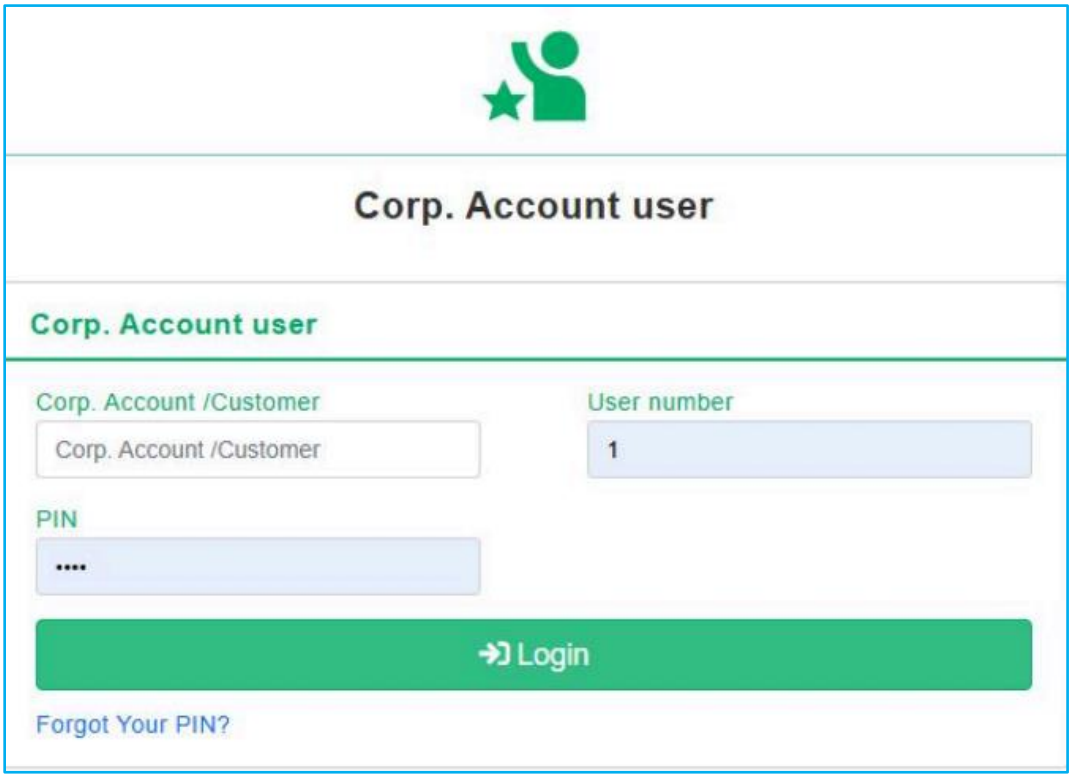


- Encounters from A1T keeps a list of the patient's most recent clinic / hospital visits
- Prioritize sending the patient to the most recently visited clinic
- If uninsured or no insurance, Unity Clinics have on-site enrollment



Login

Link: Open DC Taxi

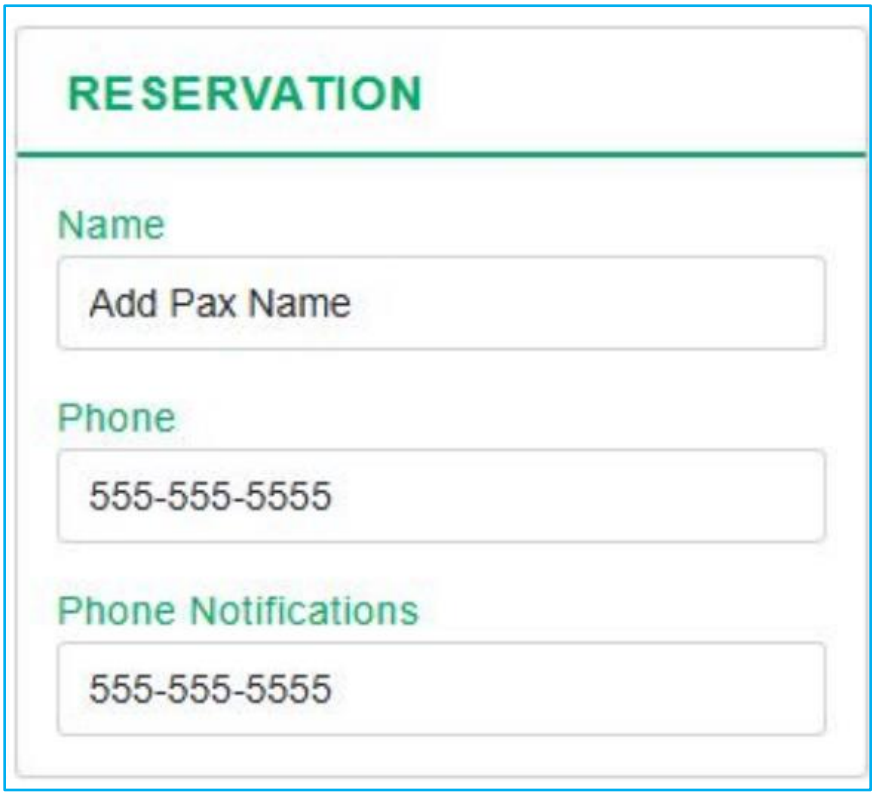


Corp Account / Customer	User Number
911	1

PIN
1234

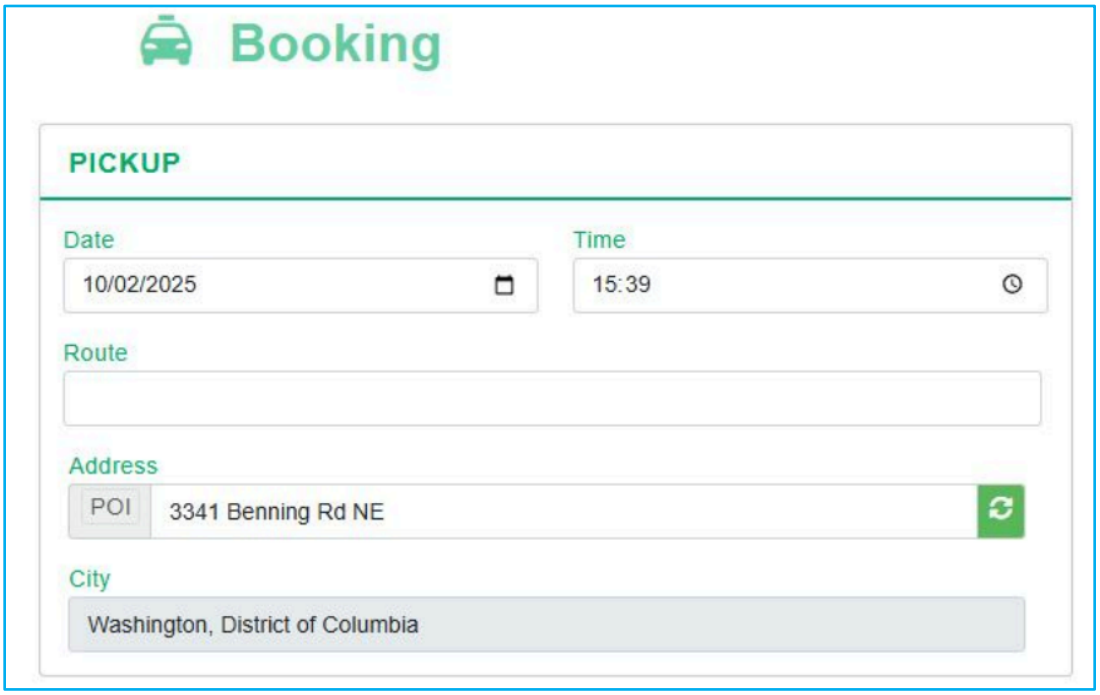
Reserve a Ride

1. Delete the pre-existing information auto-prepopulated in each field.
2. Enter the patient's name under ***Name***.
3. Enter the patient's phone number twice
   - In the **Phone** field
   - In the ***Phone Notifications*** box



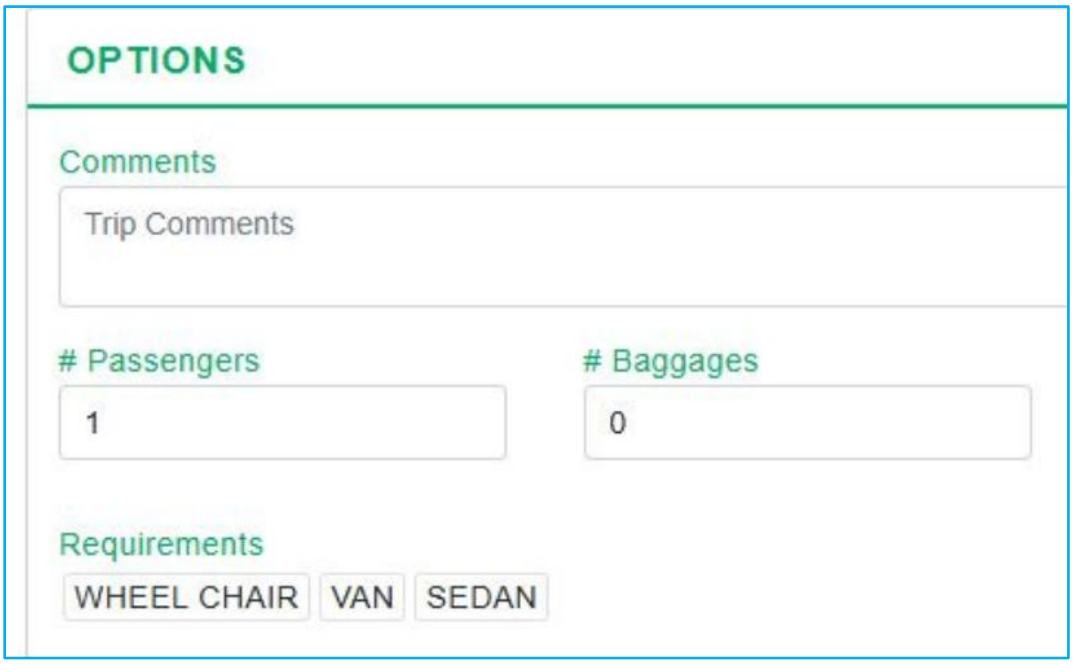
Pick Up

4. Date / time: Enter the date and time for right now or you can pre-schedule the ride for next day.
5. Enter the address where patient needs to be picked up at.



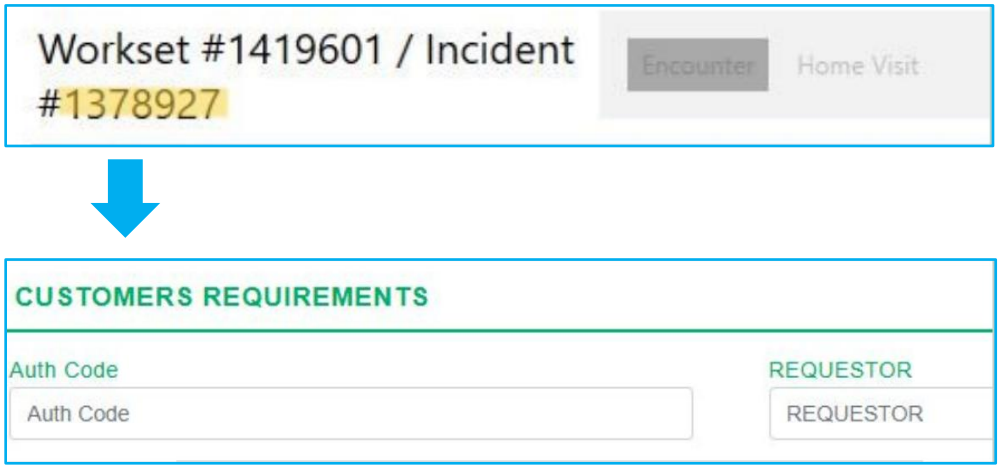
Options

6. Comments can be:
   - "Please call patient upon arrival"
   - "Patient is elderly and will need time to get to the vehicle"
   - …anything that is important.
7. If the patient needs a van, select ***Wheelchair***.



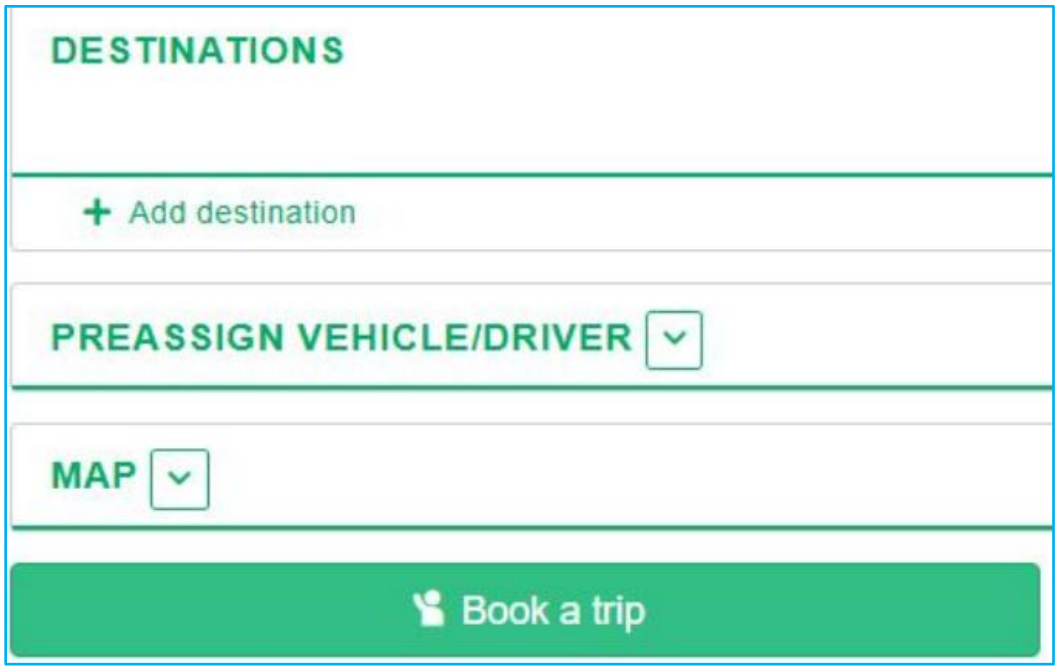
Customer Requirements

8. ***Auth Code***: Enter Logis Incident Number
   Requestor: Your name



Destinations

9. Enter the address the patient is going to.
10. Click “Book a Trip”.



Pending Future Update

We are working hard to bring you the all-new Recognition section!
Thank you for your patience.